

Dr. Lal Path Labs Ltd. (LALPATHLAB)

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National Stock Exchange of India Limited

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Corporate Relationship Department

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Mumbai- 400001

Subject: Financial Results Conference Call Transcript for Q1 FY23

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements)

Regulations, 2015, please find attached herewith Financial Results Conference Call Transcript of the Company for Q1 FY23.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Rajat Kalra

Company Secretary and Legal Head

Encl.: As above

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Dr. Lal PathLabs Limited

Q1 FY23 Earnings Conference Call

July 28 , 2022

Call Duration • 1 hour 23 minutes

Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman

- Dr. Om Prakash Manchanda - Managing Director
- Mr. Bharath U - Chief Executive Officer
- Mr. Ved Prakash Goel – Group Chief Financial Officer
- Mr. Shankha Banerjee – CEO Suburban and other group companies
- Mr. Rajat Kalra - Company Secretary and Head of Investor Relations

Participants who asked

questions • Pooja Bhatia – Morgan Stanley

- Sriram Rathi – BNP Paribas
- Shyam Srinivasan – Goldman Sachs
- Prakash Kapadia – Anived Portfolio Managers
- Bharat Shah – ASK Investment Managers
- Shalini Gupta – East India Securities
- Shaleen Kumar – UBS
- Saion Mukherjee – Nomura
- Sayantan Maji - Credit Suisse
- Prakash Agarwal – Axis Capital
- Dheeresh Pathak – Whiteoak Capital Management
- Nikhil Chaudhary – Kriis PMS

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Moderator : Ladies and gentlemen, good day and welcome to the Dr. Lal PathLabs Q1 FY23 Earnings Conference Call. As a reminder, all participant lines will be in the listen - only mode, and there will be an opportunity for you to ask questions after the

presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' and '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference, over to Mr. Siddharth Rangnekar of CDR India. Thank you, and over to you, Sir. Siddharth Rangnekar : Good evening, everyone and welcome to Dr. Lal PathLabs' Q1 FY23 Earnings Conference Call. Today we are joined by senior members of the Management team, including:

- o (Hony) Brig. Dr. Arvind Lal – Executive Chairman
- o Dr. Om Prakash Manchanda – Managing Director
- o Mr. Bharath U – CEO
- o Mr. Ved Prakash Goel – Group CFO
- o Mr. Shankha Banerjee – CEO Suburban and other group companies
- o Mr. Rajat Kalra – Company Secretary and Head of Investor Relations

Standard Disclaimer: Some of the statements made on today's call could be forward-looking in nature, and the actual results could vary from these forward-looking statements. A detailed update in this regard is available in the results presentation, which has been circulated to you and is also available on the stock exchange websites.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspectives.

Thank you .

(Hony) Brig. Dr. Arvind Lal: Thank you. A very good evening and a warm welcome to everyone present on the call. We are here to discuss Dr. Lal PathLabs' Q1 FY23 Earnings.

The industry has more than adequate headroom to grow in FY23, where within effective Vaccination Drive and improved patient care infrastructure, India has successfully met the challenges posed during the pandemic. Meanwhile, the Indian diagnostic industry is undergoing a transformation and is estimated to grow at a CAGR of 14% over the next five years. With very few barriers of entry, it makes a great ground for new entrants that include investors, entrepreneurs, and innovators.

To remind you all, when COVID-19 pandemic hit in 2020, we were amongst the first private labs to be approved by ICMR for COVID-19 RT-PCR testing. And last year, we performed over 32 lakhs COVID-19 RT-PCR tests. We have continued to serve our patients in the best possible manner even with the challenges posed by COVID.

I am happy to share with you all that we were adjudged the 'Best Star SME of the Year', by the prestigious Business Standard Financial Newspaper. For this,

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I would like to thank the senior Management team, and all the 5,000 odd employees in our organization, which is the oldest diagnostic lab chain in India, having been started in the year 1949 by my late father, Dr. Major S.K. Lal. We are all equally driven to achieve our large vision to be the most trusted healthcare partner, enabling healthier lives. Just like COVID-19 pandemic which saw rapid spread across the nation over the past two years, we too faced both challenges and opportunities to improve our infrastructure, systems, and procedures.

The public now has a greater understanding of the importance of keeping a healthy lifestyle, which is more vital. Since then, people have been monitoring their health and wellness better, which from a nation building point of view is a very welcome step.

Dr. Lal PathLabs is committed to control the epidemic of non-communicable diseases or lifestyle

diseases that are responsible for nearly 65% deaths in India. For this to take place, we shall further increase our organic network expansion, and to create structural levers for driving volume growth. Our methodical network capacity creation nationally, investments in digital technology, SwasthFit and focus on better service parameters will prepare us comprehensively to lead this space for time to come.

Today, LPL has become one of the most reputed laboratories in the world, with a network of 277 labs, 4,731 collection centers, and 10,599 pickup points, serving across 1,500 plus cities with 5,000 plus tests and panels. We have record number of 33 labs accredited by NABL, and two labs by College of American Pathologists.

As a forward-thinking Company, Dr. Lal PathLabs has been at the forefront of incorporating technology into its business strategy, by doing so, we can cut costs while giving our patients a smoother, more coherent experience. Companies like ours must adapt to stay ahead of the competition given the speed at which the world is changing. India continues to be a grossly untapped market. For a brand like ours, there is a lot of room for expansion. So, we want to take advantage of our position and create a mark for ourselves in the history.

Thank you very much, and I would now like to hand the floor to Dr. Om Manchanda.

Dr. Om P. Manchanda : Thank you. Welcome everyone to Dr. Lal PathLabs Q1 FY23 Earnings Call. I hope you and your loved ones are safe and healthy.

I will talk to you about the current trends as well as strategic focus of Dr. Lal PathLabs. The Indian healthcare sector, particularly the diagnostic phase, holds significant growth potential as was evident by the response of the industry through the pandemic. And organized national brands like ours have met these challenges without raising prices. The industry has seen entry of many new

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players, I foresee growth of organized sector, both due to overall market growth as well as accelerated shift from unorganized to organized segment. Our customers appreciate the certainty of quality and effectiveness that Dr. Lal PathLabs provides, which the unorganized players will not be able to successfully deliver.

Going forward, we will continue to build and drive growth through the following:

- 1) Organic expansion of labs and collection centers infrastructure
- 2) Through inorganic expansion route
- 3) Use of technology to enhance customer experience and also provide value-added services at one level and drive internal process efficiencies at another level to achieve productivity

On organic front, the initiative of creation of Hub Labs has started yielding good results especially in northern part of India. This will also give us capability to go deeper in Tier-II and Tier-III towns in large states like UP, Bihar, etc.

As the markets get competitive, I foresee segmentation and differentiation will play a greater role in either maintaining or growing market share. For example, first thing is seeing a good momentum and patients find greater utility in the offering. And we believe that it will further build up credibility of our brand.

Similarly, I do see in super-specialty business, we plan to focus on segments like Oncology, Autoimmune Disorders, etc. On channel front, network of collection centers is going to play a very important role. We expect CC network to service all types of customers like home collections, walk-ins, hospital pickups, etc.

Last year, COVID-19 contributed nearly Rs. 400 crore to the Company top-line. That was roughly about 19% of the Company turnover of which nearly 55% that is Rs. 221 crore came from 1st Quarter of FY22. From the current quarter that is Q2 of

2022 onwards, this overhang of large COVID-19 base, to a large extent is going to be behind us. And we hope to stay focused on more stable part of the business that is non-COVID business going forward from here.

With that now I would like to invite our CEO, Bharath, to continue this conversation.

Bharath Uppiliappan : Thank you. I warmly welcome you all to this call today. I will now take you through all the business highlights.

In Q1 FY23, we served 6.9 million patients generating revenue of Rs. 503 crore. As you are all aware that the COVID-19 wave has abated significantly and as a consequence, revenue from COVID-19 and allied test is Rs. 21 crore for Q1

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with a contribution of 4% to overall revenue. This is in contrast to 36% contribution in last year, same quarter and also the lowest in last two years.

Our non-COVID revenue of Rs. 482 crore registered a robust growth of 25% over Q1 last year. This growth in non-COVID revenue is led by patient volumes which registered a growth of 15%. The tests of patient for the quarter was 2.6,

the highest ever for the Company.

This quarter was marked by strong market activation program across our B2B and B2C channels. Our product and activation innovation has led to a robust growth on our SwasthFit bundled test program contributing to a strong patient and RPP growth.

As leaders in the diagnostic industry, we have taken upon the task of establishing India's first Center of Excellence for Autoimmune Diseases. And this has shown very encouraging results in Q1 itself. This initiative combines the focus on super specialized test including genomics has contributed to our growth in this quarter.

Our digital marketing initiatives and efforts on growing this hub business organically, is also progressing well. We remain focused on our strategic agenda and execution focus.

With that, I would like to invite, Ved to take you all through the financial performance. Over to you, Ved.

Ved Prakash Goel : Hello everyone and thank you for joining this call. I trust each of you and your families are safe and healthy.

Please note that Q1 FY23 includes Suburban results, hence not strictly comparable with previous year, same quarter. We clocked highest quarterly non-COVID revenue of Rs. 482 crore, a growth of 25%.

Our dependence on COVID -19 revenue has significantly reduced due to COVID -19 revenue declining by 91% from Rs. 221 crore to Rs. 21 crore in this current quarter. Though our non -COVID revenue increased by 25%, reduction in COVID business by Rs. 200 crore as compared to last year resulted in overall decline of 17%. Total revenue came in at Rs. 503 crore versus Rs. 607 crore last year same quarter.

As Dr. Om mentioned that out of total COVID -19 business of Rs. 396 crore in FY22, almost 55% business was in Q1 only.

Revenue realization per patient for Q1 FY23 is Rs. 727 as against Rs. 860 last year same quarter. The lower realization is due to sharp reduction in COVID -19 and allied testing. Non -COVID revenue realization per patient for Q1 FY23 is Rs. 707 as against Rs. 653 for Q1 FY22.

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Normalized EBITDA after eliminating the impact of RSU and CSR for Q1 FY23 is Rs. 126 crore as compared to Rs. 199 crore reported in Q1 FY22. Normalized EBITDA margins for Q1 FY23 is at 25%.

Q1 FY23 margins are inclusive of Suburban, which is relatively a low margin business.

Normalized PBT after eliminating the impact of notional depreciation of Rs. 12.3 crore on consolidation of Suburban for Q1 FY23 is Rs. 94 crore. Normali

zed

PBT margin is 19% for this quarter.

Normalized PAT for Q1 FY23 is Rs. 71 crore. Normalized PAT margin is at 14%.

Net cash and cash equivalents after adjustment of borrowing at the end of Q1 is Rs. 436 crore.

We are pleased to share that the Board of Directors of the Company have approved an interim dividend of 60% that is Rs. 6 per equity share.

At last a quick update on Suburban performance, Suburban revenue for Q1 FY23 is Rs. 39 crore of which non -COVID revenue is Rs. 36 crore. Please note this revenue is recorded on net basis, due to transition to Ind AS from IGAAP. This is equivalent to Rs. 52 crore as far as erstwhile accounting practices as concerned. EBITDA margin for Q1 FY23 came at 12%.

With that, I request the moderator to open the forum for questions. Thank you.

Moderator: We take the first question from the line of Pooja Bhatia from Morgan Stanley.

Please go ahead.

Pooja Bhatia : Just wanted to know, if you hold on to this year's guidance of mid -teen revenue growth, which was given last quarter. And how would this be, if you had to split the growth, how much would be volume and value, if you could give a little color on this?

Dr. O m P. Manchanda : We probably have not, in so many terms given the guidance, because one doesn't know the trajectory of COVID business. Now it's becoming a little clearer so we are not focused on COVID at all, because we still don't know how it's going to pan out for the year.

The focus for our teams has been on non -COVID part of the business. And as the current quarter indicates, we have done 15% ex-Suburban and including Suburban, we have done 25%. But one must keep in mind that last year, there are many months which are softer, many months which are very high, because of uneven trajectory of non -COVID. So, if I go by month -by-month, April, May of last year were a softer but then July, August was a little higher. After that, again, it goes down from I think November onward till at least February.

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So, our best estimate is we will still try to maintain that. It's very difficult currently to put a number. But we are still very confident that we should fall in non-COVID growth rate that we used to have before COVID times, which was I think around 13%, 14%, 15%. But I would say that it's very important to wait it out for another quarter to see as to how it pans out.

Pooja Bhatia : On the acquisition front M&A, what are the assets that would potentially interest you? Has there been any moderation in valuations that you are seeing? And is there anything in consideration at this moment?

Dr. Om P. Manchanda : So, there is nothing right now that I can share. But definitely I think teams are focused on making the Suburban asset integrate and work well. But right now, I think industry is in a bit of a state of flux. So, I think we will probably have wait and watch our strategy.

Pooja Bhatia : And what is the roadmap to improve margins in Suburban from currently 12% to say Company level 30%, what priorities are there to leverage this asset?

Dr. Om P. Manchanda : So, if you study last two -year data, it clearly showed that if you have a high top -line, there is an operating leverage in this business, which suddenly fell because COVID disappeared overnight. And we believe that if we focus on top -line aggressively in this asset, we should really be able to improve our margins, because the business is quite concentrated in city of Mumbai. And the potential is quite high. So, we just want to aggressively drive top -line that's our first priority.

Pooja Bhatia : So, we have spoken about the competitive outlook in every call for the past 24 months now. Like this month, there has been a new pharma player wh

o has

announced their plan to join the business as we speak, there are now three pharma players along with many others. Are we really undermining the new players that have entered into the market in terms of how we are looking at the industry growing? What is the outlook on this?

Dr. Om P. Manchanda : So, we are definitely not undermining any player who enters in the market. So, I think there is definitely a higher competitive intensity that we have seen especially in the last 24 months or 12 months. I think the important thing is to know what are the long -term implications of what's really happening, it's my personal judgment and I think India is highly underpenetrated from the diagnostic standpoint though there is no public data but my sense is beyond metros and mini metros diagnostics is very little right now.

And more players entering into the market they are going to drive two factors:

I) One is of course acceleration of unorganized to organized shift which is going to be very rapid. That's going to be I think number one effect, I see a very high level of competition

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II) Second is penetration levels will go up, so we will see a lot of growth coming from Tier -II, Tier -III towns

So, I personally believe that this competition is good for two things, one is it will accelerate probably the market growth, even more for organized player. The second thing is the penetration levels in Tier -II, Tier -III towns will actually go up, which means that accessibility and affordability of Quality Diagnostics will improve in this country. Now, obviously, for a Company to do well, Management teams will have really figure out a way on how do we maintain or increase our market share .

Moderator: Thank you. Next question is from the line of Sriram Rathi from BNP Paribas.

Please go ahead .

Sriram Rathi : Firstly, if you can provide some details in terms of how much of our revenue is

coming from franchisees as such, because it seems that the fees to collection centers as a percentage of sales has increased this quarter. So, just to get an idea.

Bharath Uppiliappan: So, maybe 40% plus is franchisee business for us. And I don't think there has been a sharp increase on the fees to collection center, it has been as for the normal trends.

Dr. Om P. Manchanda: I thought it has slightly come down this quarter.

Ved Prakash Goel: We have not increased the revenue share for any franchisee, fees to collection centers have come down, but the business contribution is increasing in that share. But in this quarter fees to franchisee is reduced as compared to last year.

Sriram Rathi: Yes, Y-o-Y it has reduced because of COVID I think, yes.

Dr. Om P. Manchanda: The contribution of a franchisee business to our total business is on the rise. It went up, now, I think as the COVID is receding and we are seeing a slight dip in that which essentially means the patients are coming back to a bit of walk-in format. Now, obviously, we still will not go back to where we were before COVID, but I think it's going to settle down at a slightly higher level.

Sriram Rathi: So, my second question was on that only, I mean, are we seeing the trend again reversing towards walk-in patients now versus what we would have seen in the last few quarters?

Dr. Om P. Manchanda: I think there is a comeback to that.

Sriram Rathi: And secondly, on the other expenses, I think that amount is higher this quarter on sequential basis when Suburban was there in Q4. So, should we assume this Rs. 105 crore kind of run rate to continue or it could be fluctuating quarter-on-quarter?

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Ved Prakash Goel: If you are comparing last quarter to this quarter, I think there are a few investments we have made, especially in this quarter. One is on digital side in IT. And second is, spend on marketing, which is A&P, which is also high in this quarter as compared to last year. I think these are the two broad heads where we have spent a little more in this quarter. And that's where you are looking that there is a difference between other expenses.

Sriram Rathi: So, I mean that kind of high amount may not continue in the coming quarter. Is that fair understanding?

Ved Prakash Goel: So, there are expenses, which we are continuously spending, especially on this digital side and obviously on A&P, it's quarter-on-quarter. But this is not something exception, which we have spent in this quarter. These are normal expenses, maybe IT on a special case we are seeing in this quarter, but not otherwise.

Dr. Om P. Manchanda: I think certainly you should keep in mind that margins that we had seen during COVID times were not sustainable. So, we are definitely going to settle down at what we used to do earlier, which was in the range of I think 25% to 26%, that's the way it is.

Sriram Rathi: So, for the full year, it should be in that range more or less.

Dr. Om P. Manchanda: We still don't know as to how it is going to pan out, but I think, right now the way P&L's shape is looking like, it's definitely sharply down from what it used to be during COVID times.

Sriram Rathi: And lastly, just going back to the competition part, I mean, during this quarter in the last three, four months, what part of our business would have seen higher competitive intensity and might have some impact on the business or not that much, so far?

Dr. Om P. Manchanda: I think the intensity is the brand awareness in general, so many brands have gone up. So, you don't want to lose out on top-of-mind awareness. I think that is where probably the heat is being felt right now and that's where probably what Ved mentioned that we had to spend this extra marketing money to stay top of mind. I think one should know that for any brand to remain on top of mind and especially in the healthcare because this is a need-based business. It is, if you just drop on top-of-mind awareness, then you may tend to lose out. I think that's one area, which I find this marketing spend will continue to stay up till the time this intensity dies down. That's the way I look at it. But from a business standpoint, I am not sure because many of these companies are coming in a very routine business, because we have a test menu of 4,000 tests, and not all these players have that kind of portfolio.

Moderator : Thank you. We take the next question from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead, sir.

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Shyam Srinivasan : I am looking at Dr. Lal's own non-COVID sales, excluding Suburban non-COVID. I think you said it grew 15% Y-o-Y. So, just want to disaggregate that into price and volume, because I don't know whether we got the volume numbers for Dr. Lal, non-COVID.

Bharath Uppiliappan : So, our volume growth is about 12.5%, 13% give or take. And rest is mixed impact on pricing. So, you see a price impact of 2% odd. It is not a price increase, it's a purely mix impact because of a higher contribution of SwasthFit. Our SwasthFit program actually did very well this quarter, and also our super specialized test portfolio did extremely well this quarter. And both these two contribute to a mix impact because of it, the RPP went up.

Shyam Srinivasan : This reminds us of like older days, where it used to be largely volume growth driven. And like some contribution from mix maybe some prices, you are denying the price, which is fine so largely volume growth 14%, 15%. So, if you could then disaggregate that into geography, because the worry was that. And again, competition links point, there is some of our core markets like NCR, how would the trend be in volume growth in our core market?

Bharath Uppiliappan : So, we are seeing robust growth as per the past trends across geographies in this quarter. And like I mentioned in my opening comments our South business has also done extremely well this quarter. So, it has been all around performance, North, South, East, West, no specific trend vis-à-vis the past trends, all of them have contributed to a good volume growth this quarter.

Dr. Om P. Manchanda : I think just thinking of the question that you were asking about Delhi NCR versus rest of India right. So, I think we have always said that Delhi NCR will definitely be below our overall company growth. In our scheme of things, we always looked that Delhi NCR should grow at least in high single digits, and that is what we have registered this quarter. And our rest of India is compensating for that. So, the blended average is where the 15% is, and overall contribution of Delhi NCR is steadily declining, which is basically our dependence on Delhi NCR over a period of time has sharply come down. I don't know what is this number this quarter but I think it had come down to 35%. So, the contribution from Delhi NCR is now 35%, so two-thirds of our business actually is coming from the rest of India.

Shyam Srinivasan : My second question, I think I missed it, realization per patient. I think Ved called it out as Rs. 707, is it, is that the number. And can you also explain why it declined, sorry, I couldn't get those two bits?

Ved Prakash Goel : Non-COVID, is Rs. 707 against Rs. 653.

Shyam Srinivasan : Yes, so you are saying non-COVID realization at Rs. 707 versus Rs. 653 last year, right?

Ved Prakash Goel : Yes.

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Dr. Om P. Manchanda : So, I think that's on year-on-year basis. But I think on a full year basis, this number might look very different. I think the overall sense that I get, I don't have full year data with me, is that as the contribution of bundled test is going up, which is probably the likely trend because customers are seeing greater value for money in that format. Maybe a little bit of increase on revenue per patient may happen.

Ved Prakash Goel : Just to give an immediate update, last quarter Q4, it was Rs. 693.

Dr. Om P. Manchanda : So, Rs. 653 is not a representative over here, right. So, don't go by Rs. 653 versus Rs. 707, go by sequential which is last quarter to a Rs. 693 and now it's Rs. 707. I think as Swasthfit contribution goes up; this number may inch a little bit upwards as we go along.

Shyam Srinivasan : And my last one you also called out that 2.6 tests per patient.

Dr. Om P. Manchanda : That is also because of SwasthFit, as a package has much more test per patient.

Moderator : Thank you. We will take the next question from the line of Prakash Kapadia, Anived Portfolio Managers. Please go ahead.

Prakash Kapadia : Post COVID, we are witnessing increased collection from home contribution. So, if you could highlight, what are the kinds of initiatives we have taken to ensure

a more seamless journey using technology or manpower related training or other things?

Bharath Uppiliappan : Prakash , you know our home collection contribution, pre -COVID used to be about 5% to 6%, that has settled around 10%, 11%, 12%, give or take in this range depending on the quarter, on the non -COVID side of the business. So, three, four things have happened now on the home collection side, one

is that

geographic expansion, number of cities we cover.

The second one is that the technology engine which we used has gone up significantly in sophistication. So, slot allotment or ability to look at delayed visits, reallocate visits, and all has gone up tremendously, because we have now perfected this art of how do you manage technology and people together.

The third angle is we introduced new features like Digital ID cards. So, before today, when a home collection memo comes to your house, we send in advance , his ID card to you. So, that it's an added measure of security and safety that he is the guy who is coming to you. So, lot of such initiatives on building confidence amongst people who are users of service.

And secondly, which people are now using technology is upselling or cross -selling while at the point of patient's care, which is the home. So, they are able to now add on tests for their spouses , so that we get more from the same visit. So, lot many more initiatives in the pipeline as well. But we are happy with the progress we are making on the home collection front, leveraging technology .

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Prakash Kapadia : And as a consumer, I see a lot of proactiveness at least in the Mumbai market where there are now calls for new tests or a last test done, due date. So, coming to the question, which you were hinting at of cross -selling and upselling, what is this approach we have taken, is it across newer geographies, or is just Mumbai, if you can highlight some clusters where we are trying to follow this approach, because I see a lot of activity at least in Mumbai. I am talking about Lal PathLabs.

Bharath Uppiliappan : We have an approach on Mar kTech, how do you leverage data for marketing programs, but we just have to be cautious on that side as well, because there are privacy related issues and so on. So, it's a fine balance, but yes, we are getting like I had mentioned in opening comments we had a very aggressive, what I call activation program around both B2B and B2C in this quarter, which we will continue. And some of these are part of that effort.

Prakash Kapadia : And that is specific clusters of cities which you have identified?

Bharath Uppiliappan : Yes, we do have strong points where we want to push the levers , yes.

Prakash Kapadia : And lastly, how are we managing attrition if any due to increased competition and any inflationary pressure or input cost increases, we are witnessing which could be a worry, given the general inflationary environment?

Bharath Uppiliappan : So, inflation is yes , visible in our system because fuel costs have gone up and so on. We have been looking at internal efficiencies and getting those efficiencies into picture. And there is one of the reasons why we were able to keep that P&L healthy without taking a price increase.

Attrition in particular nothing substantial, I would say at this stage. Frontline attrition continues at the previous rate , no significant change. And because we are now more a systems driven company and technology driven company, dependence on people per se specific individuals is far lesser than what may have been many years in the past.

Dr. Om P. Manchanda : We are also seeing another trend because the COVID business is down 90%, it's not only for us, it's for the entire industry. So, a lot of reverse is also true.

Moderator : Thank you. We take the next question from the line of Bharat Shah from ASK Investment Managers. Please go ahead.

Bharat Shah : I think there was a mention of growth drivers. One of the major ones that was highlighted was inorganic. But when we look at kind of recent acquisition, Suburban , the price paid is a multiple of revenue margins are rather very low, not just co

mpared to Dr. Lal business per se, but rather inferior margins.

Therefore, is this a kind of insecurity race that we are running, because growth for the sake of growth is a philosophy of cancer, it doesn't add to the value.

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Dr. Om P. Manchanda : So, I think you are making a very good point. And I value that. I think we have never looked at revenue, any organic sort of option to increase our revenue. We have always looked at any inorganic assets from the point of view of entering into that market. I think in our judgment, we felt that Mumbai is a highly competitive market. On our own, we have tried in the past. And we believe that on our own, we couldn't have really gained that kind of scale in that city without any inorganic sort of an option. That was the reason why we looked at Suburban . Now, coming to your question of, yes, we realized that Suburban has lower margins. Unfortunately, in our judgment, maybe COVID fell much sharply than what we had anticipated, because last two years COVID was very high in Mumbai, especially for Suburban. And I think probably we didn't anticipate that within three months, this decline of 90% would happen. So, that means with that , fall in margins have gone back to what historical margins for this company was. But having said that, what it has told us is that if we are able to at least double the sale of this business, from here on, margins should actually go up to, if not to our parent company level, but at least definitely close to 20%. So, all we need to do is just grow the top -line, and we feel we should be able to do that in next two to three years.

But having said that, if we had not taken this option, on our own, we could have done that, I think in my personal view, it would have been very tough for Dr. Lal PathLabs. And we can't ignore not only Mumbai, but I think State of Maharashtra not being present. That' s been the reason but otherwise, there are a lot of assets which come for to be acquired, we just don't take them at all because unless it makes sense for us, we don't go to those markets. For example, there were a couple of assets available in the eastern part of India, this would have been asset for us, maybe 6, 7 years back, but now our east business is much more stable. So, we are not looking at anything in the East.

There are lots of businesses that come to us from northern part of India, we don't look at any asset at all. I think today if I look at any wide space which is , left is southern part of India, we are seriously evaluating if you want to drive that now organically if not then inorganic, but yes, you are right. But one priority that we have is that in order to tap the overall growth of the diagnostic market in India, we need to have a wider footprint compared to what we have today. And I think the whole objective of Suburban was to how do we establish ourselves in West especially from a presence perspective. But let's see how it goes, but we are still very hopeful that in three to five years' time, we should be able to do something with this asset.

Bharat Shah : So, have I to interpret that Suburban was a bit of a one -off acquisition, and was a strategic imperative, which will eventually after you have worked through would also make financial sense compared to the price that you paid for it?

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Dr. Om P. Manchanda : Yes, it's a very strategic one. Whatever we have paid here does not stand good for any other acquisition that come, so I don't think we will look at any other assets in the way we have looked at Mumbai.

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Bharat Shah : And eventually, it will make financial sense as well?

Dr. Om P. Manchanda : I think so because last year, I think this company did close to Rs. 300 crore of course not in net terms or maybe even FY 21, it was nearly touching Rs. 300 crore and the EBITDA was in the range of 20% odd. So, of course, half of that was COVID. And now COVID has disappeared .

I think it's fairly well placed infra; we also know where are all the levers for cost cutting in this asset, I think we should be able to do that. It may not happen within three, four months or one -year time. But I think over a longer period of time, this asset I am very sure it will turn out to be good for us.

Bharat Shah : My second question was, in the opening remarks Dr. Lal alluded to 14%, 15% growth rate, you also referred to it in your address. And we talked about the huge untapped opportunity apart from conversion highway available from unorganized to organized, but in all of these, I also kind of sense that there is a pressure on the margin. And of course, we are talking of reverting back to the pre-COVID margin, which is understandable that COVID might have just

improved the journey for a temporary period. But eventually growth is unlikely to mean only growth of revenue, but also better profits, because growth of top-line is an affirmation of relevance of the business to society, and growth of profits and the cash flows is the affirmation for the value creation.

So, my fear is the growth of top-line, will it come at inferior performance on the profits and cash flows, compared to reasonably modest 14%, 15% top-line growth or have I understood it wrong.

Dr. Om P. Manchanda : See you all study businesses much more than I do; I look at this one. At the end of the day, pricing and profitability, a lot of it is external market forces driven. I think currently, as I speak to a lot of these consultants, they actually tell me that diagnostics margins are pretty visible. And there are many companies who look at offsetting of what is happening on the other side of business. This could be a noise, which will remain for next 12 to 14 months. But I think we do not want to get ourselves in a situation when our turnover; it starts coming under a bit of pressure.

So, I think two, three things we need to do:

I) One is make sure that our presence is across the country, keep widening our footprint. So, that's number one priority

II) The second priority is that run the business so efficiently that if at all we are competing with somebody like to like, our inefficiency should not be loaded on the price. So, that's productivity which is very important

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II) And I think third is right now this mix of competition that we have. Some of them are very similar cost structure, we don't have much problem with that. I think some of them are resorting to cash burn model. I am hoping that in the next two to three quarters that noise should settle down

But as far as the margin sustainability is concerned that probably market forces will decide but we are very hopeful that it won't crash to any much lower level, but I think at least the mid-20s it should remain for overall industry is the sense that I get for at least next 3 to 5 years. Because market is also opening up in Tier-II, Tier-III towns. I think a lot of people are jostling for space in metros, especially Delhi, Mumbai etc. But as you go deeper in down the pop strata there is a lot of growth coming up there as well.

Bharat Shah : Let me just reword my question, if I did not put it appropriately. Earlier I talked about inorganic acquisition, whether that makes, apart from strategic necessity whether it makes value creating financial change. This one

I am asking whether

in our organic business is the desire or pressure to grow, will it come at the expense of profitable growth? In other words, we may end up chasing shadows without achieving concrete value creation, in a sense that even if we grow the top-line at 15% or 18%. But if profits don't grow meaningfully in sync with it, then it may end up kind of wrong definition of the growth. And I would say both are important, the top-line as well as the cash flow growth.

Dr. Om P. Manchanda : So, I agree with you, I think you are making a very good point. Revenue for the sake of revenue is not good in sense of bottom line, right, when we are running a commodity business. So, it's a fine balance that we will have to maintain. I think this question also got asked in the last quarterly call. Somebody asked me, will you actually chase these lowest price points that are happening in the market? I think clearly answer is no. We will have to really, very carefully maneuver this current market situation, and find a bit of fine balance between revenue and bottom line. And I think the point that you are making is absolutely correct.

Moderator : Thank you. We take the next question from the line of Shalini Gupta from East India Securities. Please go ahead.

Shalini Gupta : You are facing so much price competition and so many strong players are entering the industry, I was just wondering if you would consider setting up a diagnostic center in collaboration with hospitals, say Lilavati, I mean Lilavati is what comes to mind, but I mean, so many others are there, so many hospitals in Mumbai. I mean, you would get a captive set of patients. And what is the downside in setting up such centers? And how easy is it to setup such centers?

Dr. Om P. Manchanda : It's a great idea. Thank you for giving us the input. We call it a Hospital Lab Management segment. And we continue to look for these opportunities. We will probably refine this further and see how we gain a revenue line from this side of

segment, this side of business.

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Shalini Gupta : I just wanted to check how difficult is it to setup such centers ?

Dr. Om P. Manchanda : Yes, so now let's put things in the other side's shoes. Now if the margins on this business is very high, they are reluctant to give it to you because they want to capture everything with them. Second is, pathology is not a very asset heavy business, so barrier to entry is low, they actually want you to run their CT/MR business not pathology business. Other thing is even if they decide to give it to you, you also run a risk of receivables because three, four months down the line he won't pay you the money. So, it's not a very easy business but having said that, I think this is definitely a large size, close to 37% of the industry is lying inside the hospital. So, we can't ignore this segment so we will definitely keep looking at it, but we will have to do probably a lot of hygiene check before we sign up any contract with the hospital.

Shalini Gupta : Okay, but you are open to this.

Dr. Om P. Manchanda : No, we do that, it is part of our research, because we have close to 40. We have 40 such centers inside the hospitals.

Shalini Gupta : Okay.

Dr. Om P. Manchanda : This is integral part of our business strategy.

(Hony) Brig. Dr. Arvind Lal : But Shalini, if you know somebody in Lilavati Hospital, you can introduce them to us.

Shalini Gupta : No, I don't know anybody. I was just wondering because I happen to visit a competitive lab in a hospital. This is a public hospital so obviously it was not very clean and all, not the kind of thing you would expect when you go to say Suburban, but the number of patients there is amazing. And also, I believe you

don't have to pay any referral fees to the doctor, and you don't have to pay for space. So, I was just wondering why is not everybody doing this more aggressively?

Dr. Om P. Manchanda : Because some of these guys don't pay for almost six months

Moderator : Thank you. We take the next question from the line of Mr. Shaleen Kumar from UBS. Please go ahead.

Shaleen Kumar : So, we have noticed one thing in our analysis that the competition from the digital platforms is largely on discount, basically, deep discounting in five tests, and then other five it drops and then substantially it drops after that. So, largely in Top 10 or maximum 15 tests. So, want to understand, what kind of revenue proportion if any, not for you, even if your industry or organized pay rather, you can give us right. What kind of revenue proportion does this test account for you, Top 10 -15? Is it possible to get?

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Dr. Om P. Manchanda : Yes, I think your observation is right, I have gone through your report it's very nicely written. And it's basically just making the news like we go to a shop, 50% off when you walk inside, you suddenly find those 50% of few items lying in the corner. So, it's the same thing happening here as well there all kinds of prices, but overall portfolio of diagnostic if we look at that's much wider. Offhand, I probably may not have that number with me, but I think your report suggested 30%, right, of these 10, 15 tests.

Shaleen Kumar : So, basically after checking with various participants?

Dr. Om P. Manchanda : I probably may not give exact answer on that, but my input here is there are lot of tests which are hematology driven. And there is a manual component in those tests. A lot of tests which are machine driven, especially Biochemistry, that's where the prices are lower. But wherever the human component is high, in any test, you can't drop prices, just like that. And they are not sustainable at all. And for any of these companies to provide a complete portfolio, to increase the scope of testing, they will have to start incurring those costs, which actually is a full-blown company like us or any other similar competition to do that. And at some point of time, at the end of the day, diagnostics is not about 10 tests diagnosis is about diagnosing a disease, so you need to have entire repertoire of testing, rather than just about 10 to 15 tests. While this will definitely grow screening side of testing, this will also grow the overall market, but we believe

that if you screen 100 normal people, and 20 of them or 10 of them actually may fall into the funnel of medical testing, which essentially means the market is likely to go faster.

Shaleen Kumar : Also, intuitively, is it right way to think that prescription driven tests are unlikely to go to the platform. And because of multiple reasons you may not get the booking now or customer is not sure about or patient is not sure about buying the test online, etc., that's one bit, so your views on that.

And second, again, do you track like how much of your testing is prescription driven and how much is non-prescription driven?

Dr. Om P. Manchanda : I think it will be unfair to say that medical things will not go there and if some of them actually widen their test menu, of course they have, they can participate in the segment as well. But I think one thing I want to highlight is that this is not a business where like a daily use item, it's not even monthly use item, last time I mentioned that average visit for patient in our portfolio, combined together is not even one per year. So, so you might just see a lot of trial generation, you do lot of these camps and promotion driven sale, you would say, but suddenly you

will

find that next month nobody is there. So, it's not a very high frequent purchase item. And for any company to show results, it is definitely two to three years of journey before they actually say yes, as we have arrived. So, I think it will take a lot of time to build healthcare brand, they don't get built overnight in my views. And plus, somebody has to sustain the flow of funds as well to sustain those kind of funds as well.

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Shaleen Kumar : Just, on the other expenses you made a comment that there has been an investment in IT, technology, and marketing. Can we get some sense of what kind of investment you are making on tech side?

Bharath Uppiliappan : So, marketing we did in this quarter, like I mentioned, we promoted SwasthFit very aggressively. So, we were around press, radio etc., I think 6 to 8 weeks of the quarter. Second is, there was sort of what you call market activation plan. On IT and digital, lot of stuff around digital marketing, building in data lakes, investing behind data science, putting up the control tower and running that for better service delivery on the front end. So, some of those initiatives have obviously taken some bit of money. But I think they are also helping us get the top-line in place.

Shaleen Kumar : Just last question, in presentation Rs. 12 crore of notional depreciation towards Suburban, so if I can understand what does it mean by notional depreciation and it's not one-off right, it will continue.

Ved Prakash Goel : So, this is on intangibles where we have capitalized intangibles for Suburban. And that intangible apart from goodwill, we are writing over a period of 8 to 10 years. So, roughly, this is Rs. 12 crore in a quarter, will continue for some time.

Moderator : Thank you. We take the next question from the line of Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee : On the organic growth path, can you throw some light, how should we think about, say if you have been increasing your lab or patient service centers at a particular rate, will we see a step up there. And if yes, any geography you have in mind, you want to sort of go deeper in north and east and sort of look for acquisition in south. If you can just throw some light on your organic growth strategy that you are thinking for the next three years?

Dr. Om P. Manchanda : I think our organic strategy has to be two-fold that we improve our service level, at the same time also don't increase our overhead structure. In the past, we have been opening a lot of the satellite labs, but each lab brings in close to crore, crore and a half operating costs as well. So, we will look at judiciously the model of Hub Labs, I think we talked about this in earlier calls, where we increased the test menu in the region now. And that does two things to us. One is it gives us the ability to go Tier-II, Tier-III towns, because otherwise to service Tier-II, Tier-III towns you have to bring those sample load to Delhi. And that will affect the service. So, we want to go closer to the markets by opening more Hub Labs and maybe reducing the number of satellite lab but opening more wider test menu labs, which we call it Hub Labs. And then improve our logistics network so that we are able to service the market. So, that's our strategy.

I think your second question was priority in terms of markets. Clearly, I think the rest of north which is UP, Bihar, Jharkhand, Orissa, even eastern part are

definitely growth markets for us. So, we want to really make sure that we aggressively increase our presence. Southern markets are giving good

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response to our organic strategy, we will have to probably wait and watch whether it's sustainable. If that works, then we will go south as well. But right now, that is still open to start with north, rest of north will be number one priority for us.

Saion Mukherjee : My second question is around competition, basically you know, we have seen the new entrants in the sector. So, at one end, you have the online players who may not have that sort of equity on the healthcare side, but then you have the pharma companies, which are well-known, they have the doctor connect, and they are trying to drive the business through that route and then they have the brand and they also sort of have the funding available from their domestic business. So, in a sense what is the bigger threat to the industry for a player like Dr. Lal in your view?

Dr. Om P. Manchanda : So, I look at both, opportunity as well as threat. I think, opportunity, I talked about, I think some of these organized competition coming into the market, is good for the industry. It's good for the industry because it creates a level playing field. So, far, we were up against a lot of these small-time players, where they have virtually no overhead. And many of these companies that you mentioned will have to have overhead structure which is similar to ours. So, it's to my mind, now we are going to play in a field which is level playing, so that I think is a big upside to this.

Second big upside is that a higher level of competition is technically there, I look at all these money which is being spent in the market is all marketing dollars. I think on a standalone basis, I wouldn't have been able to spend that kind of money. So, the more activity that happens it will grow the market. So, that's another big positive.

I think the only challenge is now as you said, do we stand to lose to these guys? There may be a bit of market share shifts, that will happen depending on who has got strength in which market. I think that's where our team really will have to be smart enough. a) To run the business efficiently so we are able to compete with them, in terms of our efficiency.

Second is carefully pick and choose our battles in terms of geography. And we believe we are very well placed in northern, eastern part of market on the organic front. And that to my mind is roughly half of the country.

And also remember, we have a network of 5,000 + collection centers and 270 + labs and 10,000 + pickup points. That was not built in just overnight one day. It has taken so much of time. And from my personal experience, I can tell you that this is not a frequently purchased item where people keep coming month after month or week after week. So, to sustain one year business, I think all the new customers have to be acquired. So, it's operationally going to be very challenging. So, to me, that actually is a bit of an insurance to us, because operational excellence requirements in this business is huge.

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And also, remember while we may be at heart, we maybe a medical company, but inside we are a huge logistics company. And while you may have doctor connected one end, but to run this operation, those skills are very different than at least running a manufacturing setup. But having said that, overall market I believe will definitely grow.

Moderator : Thank you. We take the next question from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji : So, my question is that how much of the total revenues do we get from routine tests, especially in the metro market cities? And I am asking if the impact of competition would be more on routine tests and maybe

the wellness packages,

so how much do these comprise as percentage of our revenue.

Dr. Om P. Manchanda : I don't think we have readily this data available, and it's also very loosely defined.

I would actually say 21% of our SwasthFit contribution maybe you add another 8%, to 9%, close to 30% odd would be the figure.

Sayantana Maji : And second question is that as per the last disclosure, north India, total I think constitutes around say 65% of the total revenues whereas Delhi NCR is 35%, the remaining 30%. So, do we expect a double-digit growth over there? So, Delhi NCR I understand we already have a higher market share. So, it will be high single digit, but for the rest of north India, do we expect maybe teens growth?

Dr. Om P. Manchanda : Yes absolutely. So, if we are 13% to 15% growth for the company, and Delhi NCR is single digit, so obviously, the balance has to be close to 18% to 19% from rest of India.

Sayantana Maji : And these includes the rest of north India, as well, right?

Dr. Om P. Manchanda : Yes you are right, I probably would tend to agree with you Yes, rest of north India, right.

Sayantana Maji : And a couple of data related question. So, first is, what is the Ind -AS benefit that is there in reported EBITDA? And second is what is the ESOP run rate that will be there going forward? So, I understand we give ESOP and CSR but if we exclude CSR, what is the ESOP run rate that we will have?

Ved Prakash Goel : So, in the past say, it used to be Rs. 24 crore to Rs. 25 crore per annum. I think immediately last year, it was a little high because the large grant was given at a higher price. And that's where the charge was higher. But if you see the last couple of years, it was in the range of Rs. 24 crore to Rs. 25 crore.

Yes, Ind -AS impact it was started 3 - 4 years back. So, pre -Ind-AS we used to have 25% to 26% kind of margin, after Ind -AS it revised to 26% to 27%, so, about a percent or so has impact of Ind -AS so that is all due to Ind -AS.

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Sayantana Maji : And for the ESOP, so we expect the run rate to be like Rs. 25 crore per year going ahead as well? Or would it be at a higher rate as we have right now?

Dr. Om P. Manchanda : See this changes year-on-year because depending on price on the day of grant, I think historically, what we have seen is that it has gone as high as Rs. 32 crore, last year, it was also one of the years which I remember was even Rs. 6 crore. I think the number that Ved is quoting is, I should look at it as sort of where is the settled down number average for the year, is our estimate is close to about Rs. 23 crore to Rs. 25 crore. But year -on-year basis this might just change up and down. It is also a bit of a reversal that happens when somebody resigns and comes back to the pool. So, it's a very dynamic number. But I think it's fair to assume that currently at Rs. 2,000 crore give and take here and there top line, you have close to 1% to 1.2% of ESOP charge.

Sayantana Maji : And this is going to be sustainable for like foreseeable, say next 2, 3 years, right, this ESOP scheme will continue to, it's part of our regular business now.

Dr. Om P. Manchanda : Yes, this is the kind of competition, it is to stay there.

Moderator : Thank you. We take the next question from the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal : A couple of questions and clarifications, so what I understood was 13% to 15%, base business growth, ex acquisition is what you are guiding for, would that be correct?

Dr. Om P. Manchanda : Yes, I think that's the way it looks like as of now.

Prakash Agarwal : And this quarter 25% margin is with 12% of Suburban. So, this is despite higher costs on IT and some expansion and

advertisement that you talked about. So, how do you think your margins will move, I mean, this is clearly the base level margins, would you agree to that? And how do you think, over the year, Financial '23, this margin should settle?

Ved Prakash Goel : We were growing, our margins at pre-COVID level was in this similar range of 25% to 26%, or around 26%. As far as I think if we are able to grow mid-teens, I think we are able to maintain these margins.

Dr. Om P. Manchanda : I think the sweet spot for this business is, I think if you are able to grow between 10% and 12%, my senses remain there, assuming there is no pricing pressure, we should be able to maintain, but since the dynamics of the industry keep changing every now and then. But I think if any business is able to deliver 10% to 12% growth on organic basis, we should be able to maintain these margins as my PAT currently suggests.

Prakash Agarwal : So, what I understand is IT related as well as this advertisement related expenses are here to stay on a quarterly basis.

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Dr. Om P. Manchanda : Yes, absolutely, we can't let the pressure d own on brand awareness in this kind of intensity, so I think they are here to stay for some quarters.

Prakash Agarwal : And with some improvement in Suburban which you are guiding for next couple of years, you are saying t he sales to double. I think next y ear you are expecting sales to double.

Dr. Om P. Manchanda : See right now the challenges are of different nature because Suburban is a fairly routine company by design , we want to invest behind back end, and increase our test menu , hopefully we will have some news to talk about launch of our lab in Mumbai next quarter. I think Suburban needs more company owned, company operated infra, it has to move towards the franchisee driven infra. So, that also will take time. I think next, at least three to four quarters, I want to make sure that we get the fundamentals right. And hopefully if we get the process right, the outcome has to be right. So, that's the way we are looking at it.

Prakash Agarwal : And is there any progress on the online alliances, I think last quarter, you said that you might evaluate something for online alliances, so anything to talk about at the moment.

Dr. Om P. Manchanda : I think online alliances probably we will do that. But I think they are also figuring it out, whether they want to do completely on their own or they want to partner with a lot of players. I think there's a bit of a, whether they are following aggregator model or they are doing their own sort of business. I thi nk I really won't say that the entire model has fallen in place. My personal view is to scale this business, you need partnerships. I think to scale on your own multi -city model will actually be challenging. Let's see what happens this quarter, maybe next quarter because the COVID dependency is virtually come down by 90% I think we would throw up new dynamics in the industry which we will have to wait for another three to four months.

Prakash Agarwal : Lastly, on the pricing side, there are some ads coming with 80% to 90% discounts. And these are, I would say routine tests. But on a general basis, though, you mentioned you are not reacting or your prices have not raised or reduced prices. Your action is on the advertisement and expanding the hub and spoke model. I mean, is there anything else that we are doing to keep the volume intact or grow the volumes?

Dr. Om P. Manchanda: Yes, I think one more thing that is probably likely to happen is that within the portfolio of routine tests versus specialized tests, one may look at price rationalization. Some of our high -end tests maybe underpriced and some of our routine tests

will be overpriced. We will do it in a manner that i t doesn't jeopardize our overall margins for the business. But we will look at case by case basis those kinds of things as well. I think directionally in some ways it has already started happening because the contribution of SwasthFit is going up. In a way , realization per test on the routine tests is coming down. So, some bit of it will have to be adjusted for specialized tests . I think those things will fall in place .

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Prakash Agarwal : One clarification SwasthFit would be wellness, right. So, those would be mostly routine?

Dr. Om P. Manchanda : I actually won't call it wellness; I think it's probably a wrong term to use because wellness sounds as if the patient is not completely healthy. I think it's upgrading of those two or three tests to a bundled test portfolio. So, I think it's a mix of illness and wellness together rather t han only wellness.

Prakash Agarwal : So, you are saying prices there, you could rationalize a bit and increase one with some of the specialized ones?

Dr. Om P. Manchanda : No, I don't mean rationalize, I am just saying realizations per test, because SwasthFit has a lot more number of tests in the package. See what you and I look at normally the revenue per patient, right. But what is also important to look at revenue per tes t. That, to me when you look at SwasthFit revenue per test directionally it is lower than if you order individual tests.

Prakash Agarwal : And how about looking at EBITDA per test, or EBITDA per patient?

Dr. Om P. Manchanda : EBITDA per pati ent, maybe higher but EBITDA per test maybe it's lower

because your overall realisation per test is going down.

Moderator: Thank you very much. We take the next question from the line of the Dheeresh Pathak from Whiteoak Capital Management. Please go ahead.

Dheeresh Pathak : Just trying to understand the non -COVID, ex -Suburban patient growth. So, you said revenue growth was 15% like to like non -COVID, ex -Suburban and then you gave some numbers on realization Rs. 652 and Rs. 707. So , that is like an 8% growth utilization per patient. So, that 15% revenue growth and 8% realization that leaves like a 6% to 7% like -to-like patient growth, but you also talked about 12% growth like -to-like so can you just help me reconcile?

Ved Prakash Goel : So, it has an impact, because in Suburban we have a little higher realization as compared to our own company. So, these patients include Suburban as well, which was not there in last year same quarter. Apart from that, Bharat h was mentioning about the mix and all.

Dheeresh Pathak : This Rs. 652 is obviously, last year base is a keen realization number, right, 15% of the keen revenue growth like to like. So, Rs. 707, which is the realization this quarter, you are saying this is blended realization with Suburban. So, can you help us with like a clean realization ex -Suburban per patient ?

Dr. Om P. Manchanda : Understood, so I think that's about 2% growth, right.

Bharath Uppiliappan : RPP growth is 2.5%, revenue per patient, I don't have a number exactly, right now.

Ved Prakash Goel : So, RPP ex -Suburban; we can come back separately .

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Dr. Om P. Manchanda : Okay, I will talk to you. But I think, basically the Rs. 707 figure is including Suburban business also.

Dheeresh Pathak : But this would not be a whole lot different, right, because you said last year we were closer to Rs. 690 or something . So, even if this is let's say instead of Rs. 707 this was Rs. 690, then also it's like a 6% realization

on growth. So, patient

growth on the like -for-like this thing is only 9%?

Dr. Om P. Manchanda : So, I think we will come back. Sorry, I couldn't get your name ?

Dheeresh Pathak : Dheeresh from Whiteoak. I again touch base.

Dr. Om P. Manchanda : Yes, we will touch base, thank you.

Dheeresh Pathak : The other question I had was on this Suburban , you are saying, so realization per patient is good in Suburban. Gross profit is increasing with Suburban. So, I am assuming the gross margin is not an issue. So, the issue is you are saying the network is company -operated instead of franchisee operated and that is not properly utilized. And that you are going to transition to a franchisee owned, is my understanding correct?

Dr. Om P. Manchanda : We will do both because in any case Labs are even , Dr. Lal Path Labs is also company owned, company operated. So, we will probably make the assets sweat more as the existing ones. And the network business which comes through franchisee that's where our focus would be because I think Suburban traditionally has been focused on walk -in into the lab. And they have been investing more in the lab infra , compared to Dr. Lal Path Labs, there is a scope to improve productivity there.

Dheeresh Pathak : Okay, but this will take time there's no option to right size it like properly because you can always add franchisee centers later right ? And they were quite concentrated in a few cities, right?

Dr. Om P. Manchanda : I don't want to have the pressure of quickly making this margin look very similar to industry margins and Dr. Lal Path Labs margin. I want to make sure that our top line grows in these places. And then only automatically our output -input ratio will improve if we increase our top line, which we are currently hopeful that we should be able to do that.

Dheeresh Pathak : Last question, what percentage of revenue is from aggregators, it shall be very small, but just to get a sense.

Dr. Om P. Manchanda: Single digit .

Moderator : Thank you. We take the next question from the line of Nikhil Chaudhary from Kriis PMS . Please go ahead.

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Nikhil Chaudhary : Wanted to understand on the lab sharing arrangement that we may have with

Suburban, so is it something that we are actually doing it or is it in plans that we will be using our network, meaning our lab network, and taking the samples from the Suburban because I saw an ad of Suburban, expanding in Indore also. So, wanting to understand instead of setting up Suburban lab will we be utilizing our own labs for the Suburban patients also.

Dr. Om P. Manchanda : Yes, it makes sense. It's one company right. We will definitely look at backend infra synergies. I don't think it makes sense to put another lab for Suburban, we are not competing with each other. So, we will figure a way out where one lab is able to service both the brand.

Nikhil Chaudhary : Any thoughts on probably like a question to the earlier participant where probably due to the rising competition, ESOPs probably can help in retaining even mid-level guys. So, because the competitive intensity is increasing, and they may want to take some employees from us, because we have been there in industry for a long time. Any thoughts around that like have you been evaluating or any other strategies that you are employing to probably retain the employees?

Dr. Om P. Manchanda : I think employees are smart enough to evaluate such opportunities. I have seen many of them don't fall for short term raise in their salary. They also know the sustainability of the whole idea. So, as many of them who have actually gone from us have been wanting to come back as well. So, I think some of them are really smart to figure out, they know the career

rs are 30 or 40 years long , they are just not 2 to 3 quarters.

Nikhil Chaudhary : So, last question probably, I asked it in earlier concall also, ads in news papers, have you been evaluating because something probably has created noise has been the ads . So, why can't we spend because we have cash on the balance sheet, spend some money on ad on the front page and get some traction for Dr. Lal also , just asking you guys probably can have a thought around it. On the participation even in the T20 or IPL matches, which actually are good probably areas of getting customer attention, so just probably wanting your thoughts around that.

Dr. Om P. Manchanda : Maybe we can sit separately and discuss this. I mean, I think as all of us have worked for FMCG company as well. So, this business is by the way, still a medical business. It very hyper local city based. IPL is going all over the place your presence is mainly in two or three or five cities, you will sending it to the whole country . I think we can sit and discuss that. And this business is not about you see a hoarding and you go and get your pricked next day; it doesn't work like that. So, I think, ultimately you are trying to solve a problem, which is a medical problem. It's a very need based. And it's a three-way process. You still have to engage with the medical fraternity.

And by the way, you also should know that in medicine, soliciting business this way is actually not a very appreciated format

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Moderator : Thank you. Ladies and gentlemen that was the last question for the day. I would now like to hand the conference over to the management for closing comments.

Ved Prakash Goel : Thank you everyone for being with us on this call today. I wish you and your families remain safe and healthy.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

Call: Nov 2022

November 14, 2022

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurl a Complex, Bandra (E)
Mumbai – 400 051 BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai - 400 001

Subject : Financial Results Conference Call Transcript for Q 2 & H1 FY23

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Financial Results Conference Call Transcript of the Company for Q2 & H1 FY23.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Rajat Kalra
Company Secretary and Legal Head

Encl.: As above

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Dr. Lal PathLabs Limited
Q2 FY23 Earnings Conference Call
November 8, 2022

Call Duration ■ 1 hour 15 minutes
Management Speakers ■ (Hony) Brig. Dr. Arvind Lal – Executive Chairman
■ Dr. Om Prakash Manchanda - Managing Director
■ Mr. Bharath U - Chief Executive Officer
■ Mr. Ved Prakash Goel – Group Chief Financial Officer
■ Mr. Shankha Banerjee – CEO Suburban and other group companies
■ Mr. Rajat Kalra - Company Secretary and Head of Investor Relations
Participants who asked questions ■ Rahul Agarwal – InCred Capital
■ Sriram Rathi – BNP Paribas
■ Shyam Srinivasan – Goldman Sachs
■ Prakash Kapadia – Anived Portfolio Managers
■ Sayantan Maji - Credit Suisse
■ Sameer Gilani – Individual Investor
■ Nitin Agarwal – DAM Capital
■ Sameer Baisiwala - Morgan Stanley
■ Rishabh Tiwari - Allegro Capital Advisors
■ Rishi Modi - Marcellus Investment Managers

■ Arshita Jain - Nuvama Wealth Management
■ Dheeresh Pathak - WhiteOak Capital Management
■ Prashant Nair - Ambit Capital

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Moderator: Ladies and gentlemen, good day, and welcome to Dr. Lal PathLabs Q2 & H1 FY23 earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Nishid Solanki of CDR India.

Nishid Solanki: Thank you. Good afternoon, everyone, and welcome to Dr. Lal PathLabs' Q2 & H1 FY23 earnings conference call. Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal, Executive Chairman; Dr. Om Prakash Manchanda, Managing Director; Mr. Bharath U, CEO; Mr. Ved Prakash Goel, Group CFO; along with Mr. Shankha Banerjee, CEO of Suburban and other group companies; and Mr. Rajat Kalra, Company Secretary and Head of Investor Relations.

I would like to share our standard disclaimer here. Some of the statements made on today's call could be forward-looking in nature, and the actual results could vary from these forward-looking statements. A detailed statement in this regard is available in the results presentation, which was circulated earlier and is also available on the stock exchange website. I would now like to invite Dr. Lal to share his perspectives. Thank you, and over to you, Sir.

Arvind Lal: A very good evening and a warm welcome to everyone present on the call today. We are here to discuss Dr. Lal PathLabs Q2 & H1 FY23 earnings. I would like to take you all through some of the key initiatives and perspectives that have unfolded during this quarter.

Dr. Lal PathLabs has been very tactfully increasing its presence across the country, which is otherwise fragmented and dominated by unorganized players. According to news reports, there are approximately 3 lakh labs operational in India. During COVID-19, we also witnessed a new phenomenon of carrying out RT-PCR test by certain operators, who had never been in the diagnostic field earlier, but were able to pick up COVID samples, and we have learned that many such companies have now closed down, as if they did not have the expertise in carrying out the non-COVID tests.

For Dr. Lal PathLabs, our relentless focus has been on delivering high-quality diagnostics to customers at competitive rates, while offering them superior test experience with omnichannel approach. This is precisely in accordance with the internal strategy framed by us to reach out to more customers across different towns in India. Today, Dr. Lal brand has significantly strengthened presence on best-in-class quality of services offered across the gamut of test range that is second to none. Not only have we created an excellent lab network, but also invested extensively on the brand to create a lasting impact on the customers.

Patients trust us for the entire test experience from drawing the blood to receiving accurate reports in a time-bound manner. To streamline the overall brand experience for our patients, we have heavily invested in solidifying our technological infrastructure across the entire value chain. In fact, we have witnessed twice the increase in the IT spends over the past two years to create an asset that has cutting edge technology with enhanced focus on R&D capabilities.

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Lately, we have witnessed a very noticeable shift in the industry dynamics, favoring the organized chains due to consistent quality and superior service standards. Increased test intensity is also being driven by higher literacy rates, health awareness, better disposable income and an aging population. Dr. Lal PathLabs being the biggest pan -India play

er is well poised to benefit from this rapid shift and the acquisition of Suburban Diagnostics has only bolstered our presence further in some of the key clusters of the western regions. I believe we have built a robust business enterprise with a very strong interconnected lab network and the focus going ahead will be to stretch this huge infrastructure to deliver accelerated performance and enhanced value for all the stakeholders. Thank you very much.

I would now like to hand over the floor to Dr. Om

Om Prakash Manchanda: Thank you, Dr. Lal. Welcome, everyone, to Dr. Lal PathLabs Q2 & H1 FY23 earnings call. I hope that you and your loved ones are safe and healthy.

The diagnostics industry in the Indian healthcare context offers enormous growth potential even today and will continue to do so for the foreseeable future. I will talk to you today about the current trends, as well as strategic focus of Dr. Lal PathLabs.

First, let me share a bit of current trends , competition to start with. Post - COVID -19 pandemic, the Indian diagnostic industry has faced severe competition that is two pronged, one from the online aggregators and e - commerce players, and another one from hospital chains and pharma companies. This is a different kind of competition that we are witnessing today than earlier. This intense competition has resulted in some kind of price erosion in some of the key routine tests and wellness packages. Given this backdrop, LPL has further increased the spend on marketing and promotion and in technology to maintain its leadership position. The service delivery levels remain top -notch, thereby helping LPL gain competitive advantage over peers. On the other side, or the positive side, one advantage of organized competition has been that it has pushed the overall quality and service standards in the industry.

Now let me talk about general industry environment. Based on the publicly available information, it seems after witnessing a strong pent -up demand for diagnostics, following the ease of pandemic restrictions, the industry is nearing some kind of moderation in growth in some of the matured and highly penetrated metro markets. LPL is leveraging its expertise to penetrate deep across some of the untapped regions with high -growth potential. Overall, the sector is also seeing some pressure from diverse form of competition in the market, which is also transforming the industry value chain. However, in the recent months, we have seen some of the new entrants, who had been resorting to deep discounting have started taking up the prices of some of the bundled packages. Having said that, India remains a very large market with limited access to diagnostics, hence from a long - term standpoint, we believe the growth prospects appear promising.

Now a bit about our business. At macro level, we continue to witness favorable trends in sample collection growth on the non -COVID side of the business. As you are aware that last year had wild fluctuations on month - on-month basis for both COVID and non -COVID sales. Last year, Q2 and Q3 were very high base non -COVID quarters, while Q1 and Q4 were soft non-COVID quarters. Therefore, the best way to analyze the numbers is

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either on an annualized basis or on sequential terms. Sequentially, non-COVID business growth for the current quarter, that is Q2 over Q1 is more or less in line with what our business used to deliver in the immediate years

before COVID. There has been increased relevance of wellness and bundled tests, especially post the COVID outbreak. This has been driven by higher awareness around keeping good health and rightly managing underlying comorbidities. Swasthfit, which is our bundled testing offering has reached 20% in contribution to non-COVID revenues.

On the operations front, as we grow in scale, our teams are continually evaluating

and incorporating new tests into the menu, such that our patients can benefit from the latest innovation in technology and testing. This is driving sample traction both on the B2B aspect, as we sell directly for walk-in patients, as doctors take to prescribing and relying on such new tests. Over the past few years, we have meticulously developed a pan-India operation, reaching important population centers. We have strengthened home collection capabilities and revamped our digital assets at the front end. It remains a constant endeavor to densify our reach to adjacent clusters, and we have taken the route of creating hub labs and model reference lab to support higher momentum in sample volumes. In North and East, we are moving into the hinterland, and in the West, we are making inroads through our dual brand approach with Suburban. And in South, given our new reference lab at Bengaluru, we are driving sample collections across the regions.

With that, now I would like to invite our CEO, Bharath, to continue this conversation. Thank you.

Bharath Uppiliappan: I warmly welcome you all to this call today. I will take you through the business highlights. In Q2 FY'23, we served 7.2 million patients generating a revenue of INR533.8 crore, with a growth rate of 7.1% Y-o-Y. COVID and allied tests contributed only INR19.6 crore, which represents the lowest ever contribution at 3.7% of the overall quarterly revenue. Our non-COVID revenue of INR514.2 crore, registered a growth of 14.8% over Q2 last year. This growth has been led by patient volume of 8.6% Y-o-Y. Sequentially, that is Q2 FY'23 over Q1 FY'23, the revenue uplift rate for non-COVID business is 6.7%, which is near historical pre-COVID averages, which used to be around 8%. Last year, we had an uplift of 16.1% Q2 over Q1 in our non-COVID portfolio due to COVID pandemic led, non-COVID testing for tests like KFT, LFT, CBC, etc.. Hence, these numbers have been achieved in the context of a high base of last year of COVID-associated non-COVID tests, slip in seasonal and festive calendars, along with unprecedented rains towards the last fortnight of the quarter.

Now I'll take you through some of the key trends and directions in the business. Last quarter saw a significant uptrend in Swasthfit portfolio due to a continued market activation, family offer and other distribution excellence programs. Continuing on this trend, we are pleased to share with you that Q2 FY'23, we have achieved the highest-ever quarterly revenue of INR94.9 crore from our Swasthfit portfolio. After the launch of L-ACE, our Center of Excellence in autoimmunity, we are happy to launch our latest Center of Excellence L-CORD. L-CORD stands for Dr. Lal PathLabs Center of Excellence for Reproductive Diagnostics. Our mission via the Center of Excellence at LPL is to improve the medical outcomes in infertility, pregnancy and newborns. Issues like infertility have affected over 15% of Indian couples due to late marriages, stress and poor lifestyle. Over 1.7 million babies are born with both defects in India every year due to lack of

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awareness and affordability. This Center of Excellence will significantly benefit from a wide geographic footprint, strong clinician connect, comprehensive menu and sample aggregation capabilities.

Our expansion in South and Tier 2 plus cities continues to do well, and we are further strengthening these efforts. Recognizing the competitive landscape, we continue to be aggressive in our customer acquisition and

retention programs. The operating teams have continued to focus sharply on productivity initiatives to actively manage test mix cost to deliver, a healthy EBITDA margin. With that, I would like Ved to take you all through the financial performance.

Ved Prakash Goel: Thank you, Bharath. Good evening, everyone, and thank you for joining this

call today. I trust each of you and your families are safe and healthy. Please note that Q2 FY23 results include Suburban, hence are not comparable with previous year same quarter. Now I will share some of the financial highlights. We clocked the highest quarterly non -COVID revenue of INR514 crore, a growth of 14.8%. Non -COVID revenue grew by 19.5% to INR996 crore in first half FY'23. Though the non -COVID revenue increased by 14.8% in Q2, the reduction in COVID business by 61% as compared to last year, resulted in overall growth of 7.1%.

Total revenue came in at INR534 crore versus INR 498.4 crore last year same quarter. Revenue realization per patient for Q2 FY'23 is INR746, as against INR721 last year same quarter. The better realization is due to the higher contribution of Swasthfit, which has now reached to 20% of non -COVID business excluding Suburban. Normalized EBITDA after eliminating the impact of RSU and CSR for Q2 FY'23 is INR150 crore, as compared to INR152 crore reported in Q2 FY'22. Normalized EBITDA margin for Q2 FY'23 is at 28.1%. These margins, are inclusive of Suburban, which is relatively a low margin business. Normalized PBT after eliminating the impact of notional depreciation on account of Suburban acquisition, INR12 crore and one-time exceptional expenses of INR2 crore for Q2 FY'23 is INR117 crore. Normalized PBT margin is 21.9%.

Normalized PAT for Q2 FY23 is INR86 crore and normalized PAT margin is at 16.1%. Net cash and cash equivalent after adjustment of borrowings at the end of September '22 is INR419 crore. At last, a quick update on Suburban performance. Suburban revenue for Q2 FY23 is INR40 crore, of which non -COVID revenue is INR38 crore. Please note, this revenue is recorded on net basis due to transition to Ind AS. This is equivalent to INR55 crore, as per erstwhile accounting practices. Normalized EBITDA margin for Q2 FY'23 for Suburban came in at 17.6%. With that, I request the moderator to open the forum for Q&A. Thank you.

Moderator: We have our first question from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal: I have 3 questions. Firstly, Dr. Om, I wanted to know how was Q2 as per your own internal expectations because it was a monsoon quarter should have been seasonally stronger. I understand the festival came in the quarter also, and the base being a bit higher because Q1 last year was COVID impacted, but 6.5% revenue growth, ex of Suburban, is it in line with your expectations? I mean, you highlighted on competition, but how do you look way forward for this. This is my first question.

Om Prakash Manchanda: Well, you want me to answer now or wait for your second question?

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Rahul Agarwal: Yes. I mean, I can give you the question. So secondly, on Suburban, non -COVID was flat Q -o-Q. My sense was we were focusing on aggressive faster revenue growth because of new center openings here around Mumbai and Pune. So I'm not really sure why this number has stayed so low quarter -on-quarter. And thirdly, staff costs declined from first quarter to second quarter to INR91 crore, if any one-offs there, please. That's all.

Om Prakash Manchanda: Okay. I think the first question, this is a very good question is how do we see this number internally. I think what we are looking at is before COVID, there used to be a pattern on quarter -on-quarter. Let's say, if you look at '18 -19,

'19-'20, so normally, there are 2 moments when the uplift in our revenue line happens, one is Q2 and other is Q4. So Q2, usually, as Bharath mentioned in his comments is about 7.5% to 8% higher than Q1. And this number that we have is about 6.5%. We were cruising along well, I think end of September, I don't know, you know that Northern part of India had massive rains and that affected last week. But that's not probably an excuse as to why this

number is lower. But I would actually say slightly more or less in line with what we were doing earlier. But I think 6.5%, I could have been happy with, let's say about 7.5% is what I would say.

Now, why this number looks lower than year-on-year basis is mainly because Q2 of last year is highly bloated quarter and Q2 of last year versus Q1 of last year, there was jump of nearly 16%, which, to my mind, I think, is affecting this particular year-on-year number. And that's what I actually said in my commentary that try and look at for the full year basis, as to what the growth would be rather than on just 1 quarter. And going forward also, Q3 is also one such quarter, where the base is relatively higher, while Q4 is a soft quarter last year. Now second question about Suburban. Yes, internally, we would have expected slightly higher growth, but I think there are a few challenges that we are facing in stabilizing the asset. The new leadership is just stabilizing. There is some sort of cleaning up that's happening because there's a huge tail with a small little volumes here and there. There's a bit of rationalizing on our channel is happening, and that is what has led to this. But we are very confident that going forward, we should come back quickly on this.

Rahul Agarwal: Lastly, sir, on the staff cost, why has it come down quarter-on-quarter to INR 91 crore?

Om Prakash Manchanda: Ved, you want to answer that.

Ved Prakash Goel: Yes. So there is a charge of RSU. This was reduced by roughly INR 8.5 crore. That is where we are looking to employ cost down.

Om Prakash Manchanda: But that is not normalized, sorry, it's all-inclusive.

Ved Prakash Goel: All-inclusive.

Moderator: We have our next question from the line of Sriraam Rathie from BNP Paribas. Please go ahead.

Sriraam Rathie: Just one question. Sir on the margins, generally, we have been guiding like it should be around 25-odd percent. Looking at this quarter's number, I assume that, of course, Q2 is on the higher base, but Q2 is generally the highest in terms of margins. How should we look at going forward like, I

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mean, Q3, Q4 moving back to 25% or you would like to say that, I mean, probably it can be on the higher side now?

Ved Prakash Goel: So Sriraam, Ved here. So you are right that Q2 is always a higher margin quarter for us. And if we see pre-COVID levels, I think the same trend we have seen for last two quarters. As far as we are growing in this trajectory, I think we are able to maintain our margins going forward, which used to be pre-COVID level.

Sriraam Rathie: Okay. So reported EBITDA margin of somewhere around 26% odd is what we used to do pre-COVID that should.

Ved Prakash Goel: Yes. You see 26.9% and that too with Suburban.

Sriraam Rathie: Yes. That too with Suburban. I mean, assume that Suburban margins have improved. So ideally, we should be at this level, where we are right now, the

first half.

Ved Prakash Goel: Yes.

Sriraam Rathi: And just another thing is that I mean, then Q2 is, of course, the highest quarter for us. But I mean, generally, should we expect that Q3 and Q4 will move on the lower side in terms of revenue? Or this particular year, it could be different because already still we have not seen the pre-COVID level kind of growth in Q2.

Om Prakash Manchanda: No, no, no. Q3 is the lowest quarter in our portfolio. So Q2 is the highest actually. Normally, Q4 tends to be higher, but I think in the last 4 years, 5 years, I'm not seeing Q4 equal to Q2. Generally, I think I would say out of all the 4 quarters, Q2 is the highest. And that's not only true for us, but for the entire industry.

Sriraam Rathi: So I mean just on that only, I was asking that probably like in this year Q2, we have still not seen the full potential of growth, like we used to see in pre-COVID era. So I mean

in this year, can it be different that probably Q3 and Q4 may not be that weak versus Q2?

Ved Prakash Goel: I don't know. I'm not commenting on the numbers, but generally, winters, the business actually in healthcare, in general, gets very depressed. So I actually have my doubt that Q3 will look higher than or it will be definitely weaker than Q4, Q2 is what my sense, right?

Moderator: We have a next question from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just the first one, Dr. Om, on your opening remarks, you talked about moderation in growth for the industry. Maybe your number also reflects some of it, but also the contrary statement that organized is gaining share from unorganized. So just can you help us understand some commentators have used the word COVID fatigue from a consumer perspective that there has been too much testing and there is now lesser testing or wanting to go out, anything qualitatively you can share on the moderation of growth.

Om Prakash Manchanda: I think this moderation term, I picked up maybe by analyzing all the data that's available in public domain. And I noticed that growth rates of many players are definitely not in line with what market used to have earlier. I think that's where I was coming from, that are we seeing some kind of moderation.

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And then if I look at deep down the profile of various companies and their businesses mainly it is lying in metros. That's where the moderation is coming now. It could be a combination of a few things, which is very, very difficult for us to figure out. It could be competitive intensity because there are so many players coming in. Obviously, they are gaining shares from some of these established player could be one reason.

The other reason could be, I don't know that some kind of fatigue that has set in because so much of testing, everybody has gone through it's just a bit of a pause last 6 months, 7 months to 1 year. But I think overall, the contradiction that you are seeing in is that because our bundled packages are going up, there's general health awareness. Obviously, people know that they have to take charge of their health is where this comment came.

Shyam Srinivasan: Just coming to your point on Swasthfit. So we have grown 21% I think Y-o-Y. So is there tailwinds post-COVID that you're seeing Swasthfit grow the way that is? And what are some of the building blocks and enablers that you are putting in place, so that this growth can continue?

Om Prakash Manchanda: I think there is a change in consumer behavior that I'm noticing. Maybe I'll

ask my colleague to add on to this, if I miss out something, which I have always been saying, a lot of people used to call, this a preventive health checkup. So I don't think it is preventive health checkup. There may be some component of that, but definitely non-communicable diseases related customer base is now moving to bundled packages because it's a great value for money from a customer point of view. They see that they are able to get more tests done, and they want to scan themselves for a much wider portfolio of tests rather than just say only diabetes or thyroid, now in maybe sub INR 1,000, you can actually get lot of tests done. So I think there is a change in consumer behavior, who are seeing a much greater value for money.

The second thing is illness-related testing or high-end testing relation is 1:1. The doctor visit leads to testing. Then it again, they go back to doctor and come back to the lab. But in non-communicable diseases, usually, it is a one visit to the customer and there's a frequent visit to the lab. There's no in between visit to the doctors. So to that extent, it tends to behave like as if consumers are making their own decision or choice about testing. So I think that is driving this whole bundled

packages situation, right? You want to add something to this.

Bharath Uppiliappan: Just that like I mentioned in the opening comments, other things, which are coming up and what Swasthfit is really doing is one is around the family offer. So like Om said there is a consumer behavior change. Today, more and more people are willing to get their spouse or one of the other family members also tested on the same visit or let us say in a matter of 1 or 2 days. So we enable some tech solutions wherein family members can get tested within 1 or 2 days of the main person getting tested. And second, obviously, is that a lot of focus on distribution expansion for Swasthfit as a product, number of collection centers, etc. are selling that daily So the very usual way product companies sell that rigor is setting in, I guess.

Shyam Srinivasan: And my last question, if I may, is on the comment again, Dr. Om, on price erosion, realizations that you quoted this quarter don't seem to suggest. So are we breaking it down like non-Suburban realizations have eroded? So if you can explain that comment, please.

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Om Prakash Manchanda: It's a good question. So I think good that you asked this question, let me explain. ... Most companies are talking about revenue per patient. But if you look at revenue per test, that is where the erosion is happening because now the number of tests per patient is very sharply moving up in the entire industry. So when earlier, let's say, INR1,000 was realization for 5 tests and suddenly, Now you are doing more number of tests, maybe just increasing the price by INR200, not to the extent of INR1,000. So technically, revenue per patient is moving up, but per test is going down because now you're doing more tests per patient. That's what I meant by price erosion.

Shyam Srinivasan: So you are not alluding to competitive pressures here?

Om Prakash Manchanda: No, no, no.

Shyam Srinivasan: Okay. And your comment suggest that you have maintained prices

Om Prakash Manchanda: Per patient per test is falling. But the good news is that if you are able to actually maximize revenue per single visit, it's actually beneficial. But if it's on the base of multiple visits, then you have a problem, whereas Bharath mentioned that if one phlebotomist goes for a home collection, if you are able to collect more patient samples, a large number of family members, then actually it becomes a much more viable proposition.

Moderator: We have our next question from the line of Prakash Kapadia from Anived

Portfolio Managers. Please go ahead.

Prakash Kapadia: Yes. Couple of questions from my end. If you could give us some color into Swasthfit revenues. I think Bharath did mention about some geographical expansion in terms of increasing the reach. So what would be, say, Swasthfit revenues from top 5 cities for us as of now?

Bharath Uppiliappan: I'm sorry, I didn't carry that number today. But yes, I can pull this out and then we're going to send across.

Prakash Kapadia: Yes. And in a competitive, increasing competitive scenario, how critical are the 100 million patients, which we've serviced say, during the last 5 years? Because as the business works for us and we realize in terms of cross-sell, the scale, repeat purchase. So what is our endeavor and focus on these existing customers, which we serviced over the last 5 years?

Om Prakash Manchanda: So yes, Prakash, this is something we realize as a big opportunity. There's obviously work in progress and some has got executed, some is yet to get executed. We are not going to share some very specifics unless you get tested with us. You can see what we do in the background. But needless to say, we recognize this farming activity as a large source of revenue, and our teams are at work on that front.

Prakash Kapadia: So we are internally trying to upsell and cross-sell to some of these customers and ensure we get more per visit or some more family member test is what the endeavor would be?

Bharath Uppiliappan: Yes. So like Om mentioned, we are significantly upping our A&P expense in the direction and lot of them have got technology back ends built in now.

Prakash Kapadia: And lastly, from my side, Suburban said annualized revenues of around INR160 crore, we are around INR80 crore, as of H1. What would EBITDA

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margins look like from the current level. So at what scale does those EBITDA margins go to 20% plus for us or Suburban?

Ved Prakash Goel: I think, Prakash, I would probably not put a number right now because it's very early stage, as I have always been saying that our focus right now is to see if we can stimulate the top line growth than worry about EBITDA margins. We probably would look at maybe 2-3 quarters down the line, as to how we look at it. But I see this is a great platform for us to see if we can drive growth further. Unfortunately, this year, because the dependence of Suburban on COVID was extremely high, nearly half of the business was coming from COVID, and the entire industry has seen COVID sharply fall. So to that extent, Suburban is also adversely impacted. But we just need to go through this for a couple of quarters and then take a call on how do we look at the margin trajectory. But I think the immediate focus is to see that we are able to build infrastructure, and collection infrastructure in the city of Mumbai.

Prakash Kapadia: And post the acquisition, it's been almost a year, more than a year now. So where are we in terms of, say, the test menu, how many have we added in Suburban, some sense on the patient collection center. Has that Mumbai lab opened? Where are we on that journey for Suburban?

Om Prakash Manchanda: So maybe I'll request Shankha to talk about this because he is the one, who is driving this test..

Shankha Banerjee: So in terms of 3 specific things you said in terms of adding collection network, it's an ongoing process. We've been on the job, and we are adding network each quarter. I may not be in a position to share exact numbers right now. But maybe after 2 quarters or 3 quarters, we'll start sharing numbers..

And the new lab in Vidyavihar that we have set up a reference lab that we have started in terms of testing and stabilizing that lab internally. And all the required accreditations is what is under progress right now. I think as and when that gets over, you would maybe hear more about it in the marketplace.

Prakash Kapadia: And on the test menu, is that going to be now a very important addition to Suburban to drive growth for us?

Shankha Banerjee: Yes. That is a part of the plan. And yes, it will happen once, like I said, once these accreditations, etc., are completely done, then action on that end will also start happening. Also, we are looking at a very close integration between the Suburban and LPL test menu, leveraging both together.

Om Prakash Manchanda: So I think, Prakash, It's very clear in our mind. It's a reference lab for Dr. Lal PathLabs group companies in the city of Mumbai. This lab of Vidyavihar is not only for Suburban as a legal entity, but it is for the all group companies and which we believe is very important for us to go forward.

Moderator: We have our next question from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: So I have a clarification on the personnel costs. So does CSR and RSU expense is INR 63 million, right? So where is this INR 85 million setting?

Ved Prakash Goel: Sorry, Sayantan, so I've clarified that there is a RSU cost, not CSR, RS U cost, which is included in personnel cost is down because there is a low cost on account of RSU this quarter.

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Om Prakash Manchanda: This quarter charge from RSU is much less than what it was last year.

Ved Prakash Goel: Yes, yes.

Om Prakash Manchanda: And I think that is primarily in the share price, and that's why it fluctuates right?

Arvind Lal: The share price is one of the variables. There are other variables also. Share price is also one variable.

Sayantan Maji: And also in this quarter, so can you give a rough idea in terms of which were the regions, which grew at a faster rate compared to the corporate average. So for example, is North India growing at a lower rate compared to the East India?

Om Prakash Manchanda: Our core markets of UP, Punjab, rest of North, South, they all really grew at a very fast pace.

Moderator: We have our next question from the line of Sameer Gilani, an Individual Investor. Please go ahead.

Sameer Gilani: My question is more on your cash flow statement. There seems to be an increase of trade receivables by INR 92 crore, which is not a large number, but given the numbers we've seen in the past, it seems slightly large. Can you throw some light on this, please?

Ved Prakash Goel: Yes. So it used to be INR 85 crore kind of number. For this quarter, it is slightly high because of government outstanding, but still well within our DSO, which is 30 days.

Moderator: We have our next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : Om, just 2 questions. One is, (a) on Swasthfit. Since just switching back to the price competition that we talked about, I mean, just your thoughts on, isn't the bundled packages most relative to acute sort of testing more attuned to price erosion or the pressures around just discounting and all of that?

Om Prakash Manchanda: Sorry, I'm not sure I got your question.

Nitin Agarwal: So my question was when we consider bundled packages, versus acute testing, which happened in response to when a doctor sort of prescribes a diagnostic test, where the quality and the brand starts to become very important. I mean, a bundled test is an impulse sort of a purchase in some sense and are more prone to maybe pricing or the discounting that really the best offer that potentially a customer get?

Om Prakash Manchanda: No, no, no. Quality is equally important in this, sir, because I have also talked to my phlebotomists as well as our courier network. They have actually told me that, yes, it leads to certain trial generation, but people are still coming back and saying that, okay, we want to still go back to quality. Nobody will take chances just because somebody is selling at INR 100 lower. They would definitely want to try it out, but quality is important across the portfolio, just because a wrong result can also lead to anxiety. Otherwise, you may not have a problem, but you may have anxiety disorder. So I think quality is important all across. Yes. And I'm actually about firsthand feedback from my team, people, who actually go and collect the samples.

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Nitin Agarwal: Okay. And secondly, on Swasthfit, I mean, these are largely what home tests, which we did home collection, I mean, how important is home collection in this sort of bundled test versus the acute tests?

Om Prakash Manchanda: So I think I go back to the first question someone asked about generally what is happening in the industry. I think there is definitely a consumer behavior change, which is coming. One is that they are looking for value for money, which is leading to bundled packages, so that is whatever preventive health checkup bundled packages that we talk about. I think that's one change. Second is convenience is becoming a more sought-after sort of value by patients. And convenience essentially would mean is that can they access the brand online, can they pay online, so they don't want to come to the lab and wait for 40, 45 minutes. So I think with all that put together, home collection is one element of that convenience. The convenience could also be how quickly you deliver the report, how easy your website is accessible or app, etc.. So I would call that a convenience and the overall value is quite sought-after by the patients these days in addition to pricing and quality.

Nitin Agarwal: And sir, one more thing. We talked about we're not increasing some amount of price and competition, but our gross margin seems to be probably amongst the highest that we've done in a while, and this is despite some amount of rupee depreciation happening and we properly having some amount of imported raw materials. So how does one sort of tally that?

Om Prakash Manchanda: So I think mainly because Q2 generally, we all look at growth percentage. But if you keep mind away from that, in absolute terms, it's a very high quarter. So INR 534 crore in just 1 quarter is a very high number. So whatever our Q2 margins that you see, I don't think they are representative for the annual, for the real margin, right.

Nitin Agarwal: Yes. This quarter is non-COVID is highest revenue, INR 514 crore is the highest ever.

Om Prakash Manchanda: Highest ever and I think it will be the highest in any quarter of the year. So please do not look at the current margin, as a margin, which we will deliver for the year.

Nitin Agarwal: I was looking at the gross margin, but I understand the EBITDA margin because that's an operating leverage involved. I was more surprised by the expansion in the gross margins.

Om Prakash Manchanda: Gross margins, I mean, is it that high, Ved?

Ved Prakash Goel : It is high, little bit Om.

Om Prakash Manchanda: Sorry, I haven't seen that. Is it? I don't think it's.

Nitin Agarwal: No, no.

Om Prakash Manchanda: No, I don't think it's sharply high.

Nitin Agarwal: It's not that.

Om Prakash Manchanda: We will look into that. But at least my gut says that it won't be that high.

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Nitin Agarwal: And if I can take one last one. When we take a 3 year view from here on, what kind of gross margin level do you expect?

Om Prakash Manchanda: I think you are talking about gross margin being higher, mainly because last year COVID was higher, so COVID has lower gross margins. You're comparing with the last year number, right.

Nitin Agarwal: I was looking on a Q-o-Q basis.

Om Prakash Manchanda: Last year I'm looking at material cost % ex of Suburban, which is 23.2% this quarter versus 24.6% last year same quarter, so which essentially means gross margin this year is better than the last year.

Ved Prakash Goel: First half.

Om Prakash Manchanda: First half, sorry. I'm looking at both the quarters together. It's mainly because COVID component last year was very high, and the COVID had a lower gross margin than our rest of the portfolio. Now COVID has just gone away, so obviously, our gross margins look better.

Nitin Agarwal: Sorry, last one, if we take a 3-year view, what is the aspiration or rather what is your assessment when you look at the business, what is a good volume growth number, patient volume growth number that we should be sort of looking at?

Om Prakash Manchanda: It's very difficult to put that figure because industry is definitely in a state of flux. And on one side, I'm seeing some euphoria declining amongst competition. So we'll have to wait and watch. It's too early. Maybe next couple of quarters we'll have to see. I think this whole excitement, which COVID created in the financials is dying down. So my sense is, we will give a stable sort of rate after 2 quarters. So my best guess would be, I think you should wait and watch for another 2 quarters to see a real trend. But I think at a macro level, all the macros are in place in terms of diagnostics

is highly

underpenetrated India. Medicine has to become evidence-based, very large country, 1.4 billion people. It has to go to Tier 2, Tier 3 towns. I don't see any reason why industry growth rates should be on a long-term basis, if not higher, not the same as what we have seen.

From a volume perspective, I actually don't see any reason why industry should not grow. I think the challenge could be on the competitive intensity where some of these players continue to resort to cash burn, then I don't know how pricing would settle down, that is the only thing I would probably

watch out for. To my mind, if funding is drying up as we keep reading in the newspapers, we'll have to see after two, three quarters, what really happens and how they resolve. I think early signs are some of these players have started taking up the prices of the packages, which could be a sign that they have run out of steam to stay on those current price points.

Moderator: We have our next question from the line of Sameer Baisiwala from Morgan Stanley. Please go ahead.

Sameer Baisiwala: Sir, can you update us on your distribution expansion that is underway. I know you disclose the numbers at the end of the year, but we are seven months down, is it on course, Labs, PSCs and puts?

Bharath Uppiliappan: Yes, indeed, it is on track and I mentioned in the opening comment, that we are seeing good response on Tier 2 plus cities and on South also the

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geography and our expansion plans continue both on collection center, pickup points, as well as the lab network, no slowdown on this count.

Sameer Baisiwala: Okay. And sir, the second question is, you've been mentioning on your presentation deck, things like home ECG testing, radiology and gene testing. If you can talk a bit more about these, do you think these can become big in the overall portfolio? Or how do you view these?

Om Prakash Manchanda: I don't think this can become big. These are more filler tests, I would say. I don't think this is the belly of the business at all.

Sameer Baisiwala: Okay. That's fine. And final question from my side. How do you judge Suburban acquisition on ROI basis, given that you must have spent about INR1,000 crore plus and where the numbers are tracking right now?

Om Prakash Manchanda: I think we have looked at Suburban more from a geography or presence point of view, where Mumbai and Maharashtra are very significant markets. On our own organically, we may have had a very huge challenge in setting down, we have tried this last 10, 15 years, INR 160 crore and INR 170 crore of business, 80% is coming from Mumbai. I think we see this as a great platform in the life of Dr. Lal PathLabs only for times to come. So we are not looking at on a quarterly or a one-year basis. I know this year, some of this math will be little awkward given that covid has just gone away with half of the business, but so that's the life. And I really don't look at from or rather as a team, we don't look at it from three months or one year perspective. For us, we see this deal from a very, very long-term standpoint. And it also gives us ability to build, I think Prakash asked this question about central lab. If we're able to build a nice platform of very, very high-end wide test menu in Mumbai, I think it's a great thing for us too. On a small scale sometimes these investments really don't deliver, but at such a high base, this can really work well for us if you can turn this around.

Moderator: We have our next question from the line of Rishabh Tiwari from Allegro Capital Advisors. Please go ahead.

Rishabh Tiwari: The question is regarding the Non-COVID revenue growth, which is around 6.6% for this quarter. Could you please throw some light on how much of this is coming from volume and how much of special coming from price given that there is a price

erosion in the revenue for test while the RPP is going up?

Ved Prakash Goel: So let us say there is no price erosion per se, there is a bundle test and super specialty contribution moving up. So if you put back the numbers, half will be volume and half will be revenue realization, not a size increase, revenue realization moving up, actually going, so half, half.

Rishabh Tiwari: Okay, got it. Thanks. Just the last question, what was the contribution of Delhi NCR revenues in Q2? Since the opening remarks mentioned about focusing expansion in South and also the reference lab coming in Bangalore. So if could have a broad number, the contribution from Delhi NCR, I remember it was 35% last quarter a year?

Ved Prakash Goel: So same range, yes. It remains the same range.

Om Prakash Manchanda: I think the price erosion comment is not from our company stand, I think we have the general comment about industry because that's the way it has happened.

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Moderator: We have our next question from the line of Rishi Modi from Marcellus Investment Managers. Please go ahead.

Rishi Modi: So my first question is that our three year revenue CAGR, right, non-COVID, non-Suburban has been around 9% to 10%. So are we seeing some saturation out here? Or are we losing market share, like I'm guessing the industry might be growing much faster given post COVID as well. So just trying to understand, have we lost any market share?

Om Prakash Manchanda: See, we have done this analysis for all the companies who publish data and we are seeing at least our growth rate of the CAGR of 10%, it's still the highest growth, right. I don't see that we are losing our market share. So maybe it might just take a bit of time for it to come back to the same old trajectory, but slowly, slowly the business is inching upward, that's the point I was saying rather than looking at year-on-year see that how quarterly this trajectory is moving, it is more or less mimicking what it used to be earlier. I don't think that we are losing our market share at least if I look at the base on the published data. Now of course, there are large number of unlisted players, but we don't have access to the information that they have. But having said that, of course, they also are getting some business, so they must be nibbling away a little bit here and there, but I don't know whether that's a sustainable number for them, we'll have to wait and watch just our guess.

Rishi Modi: Right. And as you said right now that you're moving into Tier 2, Tier 3, so how do the productivity and profitability numbers stack up for Tier 2, Tier 3 versus a current year one numbers, like, what are you seeing out there?

Bharath Uppiliappan: So if you look at productivity, there are two types of productivity, one is throughput per outlet and then there is the entire ecosystem. The throughput obviously takes a bit of time more in the Tier 2, Tier 3 to build up. There are various types of market, brand strong market, brand weak market. On an overall level if you manage the expansion plan well enough, I think we'll continue to see the results as we are seeing now.

Om Prakash Manchanda: I think broad thumb rule is that, as you go away from the core markets, your margins are slightly lower. The trick lies in managing in a very calibrated manner, how we expand into these markets to manage your margins. Let's say, if our company was only Delhi NCR company, margins definitely would look much higher. They obviously get diluted as we go into the hinterland, I think that's a broad rule, right? We want to very, very carefully maneuver the balance between metro as well as Tier 2, Tier 3 towns and also how you manage your lab infra.

Rishi Modi: Got it. Okay. And so you mentioned on the call that you have doubled your tech expenses in the past few years. So where are you

investing in this tech like want to understand and what are you doing in the back end, if you could give some more clarity?

Bharath Uppiliappan: So on the front end, so you actually pointed out front and back end and the security layer, there are three different ways to cut it. On the front end, all our digital properties have been revamped, website, app, the whole experience has been revamped. On the supply chain side, we have put in lot of digital assets to monitor the sample flow. So we talked in the past about having a control tower. We talked about having AI /ML layer, analytics player, all those things are now in place. And at the back end and the infra side also

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there is a migration to cloud which is happening slowly and steadily. Most of our applications are now cloud hosted. Also there is a large investment around security, governance, data governance etc.. So host of initiatives just not about making a nice app, but the entire end -to-end front end and back end and middle in between also.

Arvind Lal: Rishi, this is Dr. Lal here.

Rishi Modi: Yes.

Arvind Lal: We were the first people to bring IT into the entire health care industry in 1986, so the experience which we have in IT and digitization nobody else has. So we are going around trading along very well and I think we are still the best.

Rishi Modi: Got it.

Bharath Uppiliappan: Also the partner ecosystem is also largely digitized now and a lot of programs are running on further strengthening those.

Rishi Modi: Understood, understood. Okay. Also, finally, I think Om touched upon the challenges of Suburban. If you could get into more depth on that, I think the two, three points were mentioned on Suburban, one was getting the partner network economics right. And then there was the team settling in. So if you could give more color on that?

Om Prakash Manchanda: I think to my mind, nearly half of the business was out of COVID last year, so that is settling now, declined to about 90% decline. I think that is one side, which we have to now manage because that has obviously dropped the throughput in the channel and we have to look at the whole viability. Second is, we have put and there is a change over from a promoter run set -up to a professional run set -up. So I think there are few HR things which are, of course, behind us. And third is putting this Vidyavihar lab, which is very critical to our success. As Shankha mentioned that we are in the process of getting accreditation as we launch that, this should fall in place. And third is, of course, Mumbai having said that is not a very easy market, otherwise, we wouldn't have done this, we would have done it our self. So , it's a highly competitive platform and I think we'll just find it out.

Rishi Modi: Right. So , on Suburban, right, you all have some basic radiology in the sub - urban centers . So like what's the plan? Are you all planning to continue with the radiology or and then maybe scale it up pan -India? Or are you all planning to continue being only a pathologist?

Om Prakash Manchanda: So, I think since you use the term radiology, we don't do CT/MR there, it's mainly x-ray, ultrasound and primarily if I look at they are more catering to the health checkup sort of a concept. I think as a company, we are more a pathology. I think it will be more in terms of building our collection network that's the way we have done in the LPL because that really gives you a decent scale. So our hope is potentially to build infrastructure in terms of collection network.

Moderator: We have our next question from the line of Arshita Jain from Nuva Wealth Management . Please go ahead.

Arshita Jain: So sir, in continuation to the last participant, so you mentioned the new management is still stabilizing this asset. So just wanted to understand, what

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are your internal expectations from this asset particularly the Suburban growth expectation?

Om Prakash Manchanda: So I think our internal expectation is, if we can build western region for combined business of Dr. Lal PathLabs Suburban together, that's how we will define success for our self. And when I say that our ability to really get synergies at the back end, so that both side labs are able to cater into the market and the front end, we continue to do a brand. So I think I would say western region business starts growing, that's how success will be defined for us.

Arshita Jain: No, I'm saying, you think this asset can grow much faster than the industry growth in the coming year?

Om Prakash Manchanda: I think so, here.

Arshita Jain: Okay. The second question is on the revenue contribution from the metros and Tier 1, what is it now? And what was it three years back? Any color on that?

Om Prakash Manchanda: So let's say, if I take Delhi NCR, three, four years back, it used to contribute nearly about 50% or 55%, now it is down to about 34%, in H1 FY23 excluding Suburban is that correct?

Om Prakash Manchanda: So we are spreading our reach very well outside Delhi NCR. So our dependence in metros over a period of time is reducing.

Arshita Jain: It will even include the Tier 1s also?

Om Prakash Manchanda: I don't know what you mean by Tier 1, are you talking about mini metros and Tier 1 or how do you define Tier 1 in fact?

Arshita Jain: Basically just wanted to know your contribution from Tier 2 to Tier 6 towns, Tier 2, Tier 3 towns right which is expanding simply?

Om Prakash Manchanda: Directionally, we are all going down the pop strata for our growth strategy, so whether if we go to Tier 3 or Tier 4, idea is to go down the pop strata. Let's say, you are in the South region, you are slightly still fighting the battle out in metros. So we will go to the next layer of towns. In North, we are obviously in Tier 3 right now. So I think I would say whatever markets we are, we are looking at next pop strata, which is down the pop strata.

Arshita Jain: Fair enough. And lastly Swasthfit, Swasthfit already is 20% of your non-COVID revenue, where do you think this contribution can move up in the coming quarters since you are seeing change in the customer or consumer behavior

Om Prakash Manchanda: I think that's a brilliant question and I think that's an overall trend in the industry. I would actually say that it will keep growing, I don't know what that number is because if you historically study 15, 20 years back, even these panels were not there, LFP, KFT, etc.. And somebody would have talked about this panel as a great story that time, today, now you're bunching these panels calling them bundle packaging, right? So I think directionally that's the way industry has moved from single test to a panel now, panels being bundled is now a big thing. So directionally, I would say it will keep growing.

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I don't know where it finally lands up and good news is that these are both packages where doctor intervention is relatively lesser

Arvind Lal: And Arshita, this is Dr. L al here. Don't forget that mortality and morbidity in India is 65% and hence these non-communicable diseases are not going to stop. Diabetes is going to grow, hypertension is going to grow, Cancer is definitely growing and the liver disease, kidney disease , where is the stoppage , there was a lull there because of the COVID because COVID took the front face because of the mortality, but the rest of the things have not disappeared.

Moderator: We have our next question from the line of Dheeresh Pathak from Wh iteOak Capital Management. Please go ahead.

Dheeresh Pathak: I was looking at you are saying that seasonally Q2 has done well. If I have a number of based on my mat

h, 6.7 million, so that is just 0.2 million increase in footfalls, which is a measure of nu mber of patients ex -COVID, ex - Suburban between Q1 and Q2, which in a pre -COVID time used to be anywhere between 0.4 million to 0.5 million extra patients we would get between Q1 and Q2. So from a footfall point of view, it's been softer versus what we woul d see in the pre -COVID time period, so I guess their numbers are right that I'm talking about.

Bharath Uppiliappan: So for Q2, the volume growth is 3.3%.

Om Prakash Manchanda: No, no, I think his question is, the way we are highlighting Q1 to Q2 , 6.5 value growth, he is saying volume growth is lower, right? Is that correct?

Dheeresh Pathak: Yes, yes. If you look at it just in terms of delta between the patient footfall between Q1 and Q2, on an absolute number, it is much lesser versus what we wer e doing with pre -COVID, so pre -COVID in FY'19 between Q1 and Q2, we did about 0.5 million extra patients, then in FY'20, which is obviously COVID hit in the last quarter, but if you see between Q1 and Q2 that was about 0.4 million. This year, if you take o ut Suburban, it is only 0.2.

Om Prakash Manchanda: Yes, so I think you have an observation. I think if you don't mind, we would definitely love to have an offline conversation on that, I don't have readily that data available, so I can't engage myself deeper into the conversation.

Dheeresh Pathak: But is that the right metric now . You think just looking more at patient footfall, so that is a measure that we should ...because more tests are getting bundled. The patient footfall also will be important metri c in your mind or value growth is more important?

Om Prakash Manchanda: No, no, actually all three I would say, I think there is a good point you're saying. I would say revenue, then revenue per patient, then number of patients, then I would say, number o f test per patient. I think all the four are important for us to actually see how we trade the business is going up.

Dheeresh Pathak: Okay. And sorry, go ahead, sir.

Om Prakash Manchanda: I said all these four metrics are important, yes. But having said that, I wouldn't worry about quarter -to-quarter variation, I'll tell you reason why. This year, let's take year in which Dengue incidence is very high, you will certainly see volume shooting up very sharply because one Dengue patient in a week's time might visit your lab 10 times, you understand what I mean? So, in a quarter, if the Dengue is not that high and certainly you'll see volume

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dropping. So I really won't worry about one quarter to another quarter, I will definitely look at this metric over a long er period as to how it's trending.

Dheeresh Pathak: Certainly. How do you characterize Dengue this quarter versus the past year?

Om Prakash Manchanda: I really won't have that data, but I think this year slightly Dengue has been little bit muted and in some places, it's averaging, I won't say it's unusually high.

Dheeresh Pathak: Okay. And what was the acquisition related amortization and what was the Ind AS adjustment?

Ved Prakash Goel: So in this quarter, INR12 crore amortization on account of acquisition.

Dheeresh Pathak: Okay. And what was the Ind AS is INR14 crore, IndAS adjustment between the EBITDA and the IndAS adjusted EBITDA?

Ved Prakash Goel: So it is not unusual, it's in line with the past trend only, so about 1%, 1.5% kind of yes.

Dheeresh Pathak: Okay. It is 2.9% of revenue it was last quarter, which was INR14 crore in absolute term, which is higher than 1%, I'm assuming it will be similar in absolute term versus last quarter, right?

Ved Prakash Goel: Yes, similar, so there is no change, the only thing in India rental cost is coming down in the form of rentals and depreciation, that's it.

Dheeresh Pathak: Okay. When you say 1%, it is higher than 1%, right, it is INR14 crore, which is 2.7%?

Ved Prakash Goel: I will just check, but it is.

Om Prakash Manchanda: He doesn't have that figure.

Dheeresh Pathak: Okay. I'll connect offline.

Moderator: We have our next question from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: So first question was on tax rate, for the full year, will it settle back down to the 25% or level that you normally have? Or will it stay elevated as you've seen in the first half?

Ved Prakash Goel: Yes, it will be similar, there is nothing change in there in terms of tax.

Prashant Nair: So similar as the first half of this quarter or similar to what you've been doing in the past?

Ved Prakash Goel: Yes, similar, I mean, in this first half also is similar, it's nothing which has changed.

Prashant Nair: All right. And so generally on the growth front. So when we look at industry growth, would it be fair to say that steady growth that you are doing in say in the Delhi NCR market where you are a leading player and have high market share, is a fair reflection of what the industry will be growing at? Or do you think you're still gaining some share in that market as well?

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Om Prakash Manchanda: In Delhi NCR?

Prashant Nair: Yes, so for example, my question is, if you're growing say 8%, 10% in Delhi NCR, is that a reflection of what industry growth would be for that market and similar markets like, say, larger markets, larger cities or each state has its own dynamics and growth rates could vary?

Om Prakash Manchanda: I think each city has its own dynamic because I think competitive intensity in general I'm seeing is high in four or five cities in the country, that's where the maximum noise is. You go down the pop strata, you may not hear some of these names. So I think it's a city level sort of a thing. And let's say, we all talk about certain pharma companies, we don't even hear them in northern part of India. So I think depending on where they are strong, the infra is strong, that's where they, I would say it varies from city to city, but definitely four or five big metros are where especially online players are more interested.

Moderator: We have a follow-up question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: I think in the initial comments, you mentioned something about introducing a new suite of test for pregnant women. I mean, if you can probably highlight little bit, put some more color on that? And I mean, what kind of opportunity do we see in this space?

Om Prakash Manchanda: So I think Bharath mentioned about that. Our view is that industry is getting sort of vertically split, you have bundle packages and then you have always high-end packages, right? Now that's where the segmentation would come in since all of you cover pharma space and you have things like Dermatology division, you have gynaecology, you have things like that. So similarly like we have launched this auto-immunity and this kind of segmentation segment focus would come. So this reproductive diagnostic is one such promising segment in the industry and that's what Bharath meant by that. So it's not about just pregnancy alone and I would say these are all life stage testing starting with puberty and motherhood and things like that. So I would club all that into a reproductive diagnostics.

Nitin Agarwal: And just one separate question, sir when you alluded the earlier point about expansion in sort of smaller towns and your comment that probably that growth comes at a slightly lower margin. So just picking a thought on it, is something like an absolute EBITDA per patient, is there a right metric to look at this business and is that a metric that we track internally also?

Om Prakash Manchanda: I think what I meant by that was basically it's a lower realization, you go down pop strata, it's affected by many factors like the ability to pay or in prescription habits of doctors, etc., if not like where somebody would go for INR2,000, INR3,000. So what I meant all that ability to pay lower down the pop strata and similar prescription behavior is very different than, let's say, the metro cities. And the profile of testing is also going to be different because they are more routine in nature which are low price. And compared to that unit economics, it's very difficult to get pathologists and at times we end up paying slightly higher salaries in smaller towns and in the metros. So it's all in all put together. And plus, you do not see. I said it's also revenue trajectory also doesn't pick up that sharply and thereby, the overall net impact is the margins are slightly depressed and what we would realize in metros, so scale is also very high in metros.

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Arvind Lal: Also, this is Dr. Lal here, also when you go away from the metros, you go into the other town, smaller towns, you are faced with lack of electricity, lack of potable water and so you're going to spend money on other things, including electricity, you are going to pay much more. And sometimes, we've even had to carry RO water all the way from Delhi to smaller towns. So nothing goes cheap, as you go actually deeper down, the cost of servicing that patient actually increases.

Nitin Agarwal: I mean sir link point as a proportion as we grow, as we grow more of volume growth is coming from the smaller areas, does that on a weighted average

basis create pressure on our EBITDA margins, so while absolute EBITDA come through what margin perspective it may just create problems rather pressure on the EBITDA margin?

Om Prakash Manchanda: So I think there are factors which are favorable, unfavorable. I think that is where the whole management lies, art of management lies and to best of our ability, we are trying to see how we don't dilute the margin. I think that's where you are seeing the numbers. But we are cautious also in our commentary because as you go down the pop strata , I think it's natural to believe that margins won't go up, but we are trying to see how we keep on sustaining our current level.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand over the call to management for closing comments. Over to you, sir.

Ved Prakash Goel: Thank you, everyone for being with us on this call today. I wish all of you remain safe and healthy. Looking forward to meet you next quarter conference call.. I would now request the moderator to close the call. Thank you.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

Call: Nov 2022

November 14, 2022

National Stock Exchange of India Limited
Exchange Plaza,
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Bandra Kurl a Complex, Bandra (E)
Mumbai – 400 051 BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
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Mumbai - 400 001

Subject : Financial Results Conference Call Transcript for Q 2 & H1 FY23

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Financial Results Conference Call Transcript of the Company for Q2 & H1 FY23.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Rajat Kalra
Company Secretary and Legal Head

Encl.: As above

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Dr. Lal PathLabs Limited
Q2 FY23 Earnings Conference Call
November 8, 2022

Call Duration ■ 1 hour 15 minutes
Management Speakers ■ (Hony) Brig. Dr. Arvind Lal – Executive Chairman
■ Dr. Om Prakash Manchanda - Managing Director
■ Mr. Bharath U - Chief Executive Officer
■ Mr. Ved Prakash Goel – Group Chief Financial Officer
■ Mr. Shankha Banerjee – CEO Suburban and other group companies
■ Mr. Rajat Kalra - Company Secretary and Head of Investor Relations
Participants who asked questions ■ Rahul Agarwal – InCred Capital
■ Sriram Rathi – BNP Paribas
■ Shyam Srinivasan – Goldman Sachs
■ Prakash Kapadia – Anived Portfolio Managers
■ Sayantan Maji - Credit Suisse
■ Sameer Gilani – Individual Investor
■ Nitin Agarwal – DAM Capital
■ Sameer Baisiwala - Morgan Stanley
■ Rishabh Tiwari - Allegro Capital Advisors
■ Rishi Modi - Marcellus Investment Managers

■ Arshita Jain - Nuvama Wealth Management
■ Dheeresh Pathak - WhiteOak Capital Management
■ Prashant Nair - Ambit Capital

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Moderator: Ladies and gentlemen, good day, and welcome to Dr. Lal PathLabs Q2 & H1 FY23 earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Nishid Solanki of CDR India.

Nishid Solanki: Thank you. Good afternoon, everyone, and welcome to Dr. Lal PathLabs' Q2 & H1 FY23 earnings conference call. Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal, Executive Chairman; Dr. Om Prakash Manchanda, Managing Director; Mr. Bharath U, CEO; Mr. Ved Prakash Goel, Group CFO; along with Mr. Shankha Banerjee, CEO of Suburban and other group companies; and Mr. Rajat Kalra, Company Secretary and Head of Investor Relations.

I would like to share our standard disclaimer here. Some of the statements made on today's call could be forward-looking in nature, and the actual results could vary from these forward-looking statements. A detailed statement in this regard is available in the results presentation, which was circulated earlier and is also available on the stock exchange website. I would now like to invite Dr. Lal to share his perspectives. Thank you, and over to you, Sir.

Arvind Lal: A very good evening and a warm welcome to everyone present on the call today. We are here to discuss Dr. Lal PathLabs Q2 & H1 FY23 earnings. I would like to take you all through some of the key initiatives and perspectives that have unfolded during this quarter.

Dr. Lal PathLabs has been very tactfully increasing its presence across the country, which is otherwise fragmented and dominated by unorganized players. According to news reports, there are approximately 3 lakh labs operational in India. During COVID-19, we also witnessed a new phenomenon of carrying out RT-PCR test by certain operators, who had never been in the diagnostic field earlier, but were able to pick up COVID samples, and we have learned that many such companies have now closed down, as if they did not have the expertise in carrying out the non-COVID tests.

For Dr. Lal PathLabs, our relentless focus has been on delivering high-quality diagnostics to customers at competitive rates, while offering them superior test experience with omnichannel approach. This is precisely in accordance with the internal strategy framed by us to reach out to more customers across different towns in India. Today, Dr. Lal brand has significantly strengthened presence on best-in-class quality of services offered across the gamut of test range that is second to none. Not only have we created an excellent lab network, but also invested extensively on the brand to create a lasting impact on the customers.

Patients trust us for the entire test experience from drawing the blood to receiving accurate reports in a time-bound manner. To streamline the overall brand experience for our patients, we have heavily invested in solidifying our technological infrastructure across the entire value chain. In fact, we have witnessed twice the increase in the IT spends over the past two years to create an asset that has cutting edge technology with enhanced focus on R&D capabilities.

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Lately, we have witnessed a very noticeable shift in the industry dynamics, favoring the organized chains due to consistent quality and superior service standards. Increased test intensity is also being driven by higher literacy rates, health awareness, better disposable income and an aging population. Dr. Lal PathLabs being the biggest pan -India play

er is well poised to benefit from this rapid shift and the acquisition of Suburban Diagnostics has only bolstered our presence further in some of the key clusters of the western regions. I believe we have built a robust business enterprise with a very strong interconnected lab network and the focus going ahead will be to stretch this huge infrastructure to deliver accelerated performance and enhanced value for all the stakeholders. Thank you very much.

I would now like to hand over the floor to Dr. Om

Om Prakash Manchanda: Thank you, Dr. Lal. Welcome, everyone, to Dr. Lal PathLabs Q2 & H1 FY23 earnings call. I hope that you and your loved ones are safe and healthy.

The diagnostics industry in the Indian healthcare context offers enormous growth potential even today and will continue to do so for the foreseeable future. I will talk to you today about the current trends, as well as strategic focus of Dr. Lal PathLabs.

First, let me share a bit of current trends , competition to start with. Post - COVID -19 pandemic, the Indian diagnostic industry has faced severe competition that is two pronged, one from the online aggregators and e - commerce players, and another one from hospital chains and pharma companies. This is a different kind of competition that we are witnessing today than earlier. This intense competition has resulted in some kind of price erosion in some of the key routine tests and wellness packages. Given this backdrop, LPL has further increased the spend on marketing and promotion and in technology to maintain its leadership position. The service delivery levels remain top -notch, thereby helping LPL gain competitive advantage over peers. On the other side, or the positive side, one advantage of organized competition has been that it has pushed the overall quality and service standards in the industry.

Now let me talk about general industry environment. Based on the publicly available information, it seems after witnessing a strong pent -up demand for diagnostics, following the ease of pandemic restrictions, the industry is nearing some kind of moderation in growth in some of the matured and highly penetrated metro markets. LPL is leveraging its expertise to penetrate deep across some of the untapped regions with high -growth potential. Overall, the sector is also seeing some pressure from diverse form of competition in the market, which is also transforming the industry value chain. However, in the recent months, we have seen some of the new entrants, who had been resorting to deep discounting have started taking up the prices of some of the bundled packages. Having said that, India remains a very large market with limited access to diagnostics, hence from a long - term standpoint, we believe the growth prospects appear promising.

Now a bit about our business. At macro level, we continue to witness favorable trends in sample collection growth on the non -COVID side of the business. As you are aware that last year had wild fluctuations on month - on-month basis for both COVID and non -COVID sales. Last year, Q2 and Q3 were very high base non -COVID quarters, while Q1 and Q4 were soft non-COVID quarters. Therefore, the best way to analyze the numbers is

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either on an annualized basis or on sequential terms. Sequentially, non-COVID business growth for the current quarter, that is Q2 over Q1 is more or less in line with what our business used to deliver in the immediate years

before COVID. There has been increased relevance of wellness and bundled tests, especially post the COVID outbreak. This has been driven by higher awareness around keeping good health and rightly managing underlying comorbidities. Swasthfit, which is our bundled testing offering has reached 20% in contribution to non-COVID revenues.

On the operations front, as we grow in scale, our teams are continually evaluating

and incorporating new tests into the menu, such that our patients can benefit from the latest innovation in technology and testing. This is driving sample traction both on the B2B aspect, as we sell directly for walk-in patients, as doctors take to prescribing and relying on such new tests. Over the past few years, we have meticulously developed a pan-India operation, reaching important population centers. We have strengthened home collection capabilities and revamped our digital assets at the front end. It remains a constant endeavor to densify our reach to adjacent clusters, and we have taken the route of creating hub labs and model reference lab to support higher momentum in sample volumes. In North and East, we are moving into the hinterland, and in the West, we are making inroads through our dual brand approach with Suburban. And in South, given our new reference lab at Bengaluru, we are driving sample collections across the regions.

With that, now I would like to invite our CEO, Bharath, to continue this conversation. Thank you.

Bharath Uppiliappan: I warmly welcome you all to this call today. I will take you through the business highlights. In Q2 FY'23, we served 7.2 million patients generating a revenue of INR533.8 crore, with a growth rate of 7.1% Y-o-Y. COVID and allied tests contributed only INR19.6 crore, which represents the lowest ever contribution at 3.7% of the overall quarterly revenue. Our non-COVID revenue of INR514.2 crore, registered a growth of 14.8% over Q2 last year. This growth has been led by patient volume of 8.6% Y-o-Y. Sequentially, that is Q2 FY'23 over Q1 FY'23, the revenue uplift rate for non-COVID business is 6.7%, which is near historical pre-COVID averages, which used to be around 8%. Last year, we had an uplift of 16.1% Q2 over Q1 in our non-COVID portfolio due to COVID pandemic led, non-COVID testing for tests like KFT, LFT, CBC, etc.. Hence, these numbers have been achieved in the context of a high base of last year of COVID-associated non-COVID tests, slip in seasonal and festive calendars, along with unprecedented rains towards the last fortnight of the quarter.

Now I'll take you through some of the key trends and directions in the business. Last quarter saw a significant uptrend in Swasthfit portfolio due to a continued market activation, family offer and other distribution excellence programs. Continuing on this trend, we are pleased to share with you that Q2 FY'23, we have achieved the highest-ever quarterly revenue of INR94.9 crore from our Swasthfit portfolio. After the launch of L-ACE, our Center of Excellence in autoimmunity, we are happy to launch our latest Center of Excellence L-CORD. L-CORD stands for Dr. Lal PathLabs Center of Excellence for Reproductive Diagnostics. Our mission via the Center of Excellence at LPL is to improve the medical outcomes in infertility, pregnancy and newborns. Issues like infertility have affected over 15% of Indian couples due to late marriages, stress and poor lifestyle. Over 1.7 million babies are born with both defects in India every year due to lack of

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awareness and affordability. This Center of Excellence will significantly benefit from a wide geographic footprint, strong clinician connect, comprehensive menu and sample aggregation capabilities.

Our expansion in South and Tier 2 plus cities continues to do well, and we are further strengthening these efforts. Recognizing the competitive landscape, we continue to be aggressive in our customer acquisition and

retention programs. The operating teams have continued to focus sharply on productivity initiatives to actively manage test mix cost to deliver, a healthy EBITDA margin. With that, I would like Ved to take you all through the financial performance.

Ved Prakash Goel: Thank you, Bharath. Good evening, everyone, and thank you for joining this

call today. I trust each of you and your families are safe and healthy. Please note that Q2 FY23 results include Suburban, hence are not comparable with previous year same quarter. Now I will share some of the financial highlights. We clocked the highest quarterly non-COVID revenue of INR514 crore, a growth of 14.8%. Non-COVID revenue grew by 19.5% to INR996 crore in first half FY'23. Though the non-COVID revenue increased by 14.8% in Q2, the reduction in COVID business by 61% as compared to last year, resulted in overall growth of 7.1%.

Total revenue came in at INR534 crore versus INR 498.4 crore last year same quarter. Revenue realization per patient for Q2 FY'23 is INR746, as against INR721 last year same quarter. The better realization is due to the higher contribution of Swasthfit, which has now reached to 20% of non-COVID business excluding Suburban. Normalized EBITDA after eliminating the impact of RSU and CSR for Q2 FY'23 is INR150 crore, as compared to INR152 crore reported in Q2 FY'22. Normalized EBITDA margin for Q2 FY'23 is at 28.1%. These margins, are inclusive of Suburban, which is relatively a low margin business. Normalized PBT after eliminating the impact of notional depreciation on account of Suburban acquisition, INR12 crore and one-time exceptional expenses of INR2 crore for Q2 FY'23 is INR117 crore. Normalized PBT margin is 21.9%.

Normalized PAT for Q2 FY23 is INR86 crore and normalized PAT margin is at 16.1%. Net cash and cash equivalent after adjustment of borrowings at the end of September '22 is INR419 crore. At last, a quick update on Suburban performance. Suburban revenue for Q2 FY23 is INR40 crore, of which non-COVID revenue is INR38 crore. Please note, this revenue is recorded on net basis due to transition to Ind AS. This is equivalent to INR55 crore, as per erstwhile accounting practices. Normalized EBITDA margin for Q2 FY'23 for Suburban came in at 17.6%. With that, I request the moderator to open the forum for Q&A. Thank you.

Moderator: We have our first question from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal: I have 3 questions. Firstly, Dr. Om, I wanted to know how was Q2 as per your own internal expectations because it was a monsoon quarter should have been seasonally stronger. I understand the festival came in the quarter also, and the base being a bit higher because Q1 last year was COVID impacted, but 6.5% revenue growth, ex of Suburban, is it in line with your expectations? I mean, you highlighted on competition, but how do you look way forward for this. This is my first question.

Om Prakash Manchanda: Well, you want me to answer now or wait for your second question?

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Rahul Agarwal: Yes. I mean, I can give you the question. So secondly, on Suburban, non-COVID was flat Q-o-Q. My sense was we were focusing on aggressive faster revenue growth because of new center openings here around Mumbai and Pune. So I'm not really sure why this number has stayed so low quarter-on-quarter. And thirdly, staff costs declined from first quarter to second quarter to INR91 crore, if any one-offs there, please. That's all.

Om Prakash Manchanda: Okay. I think the first question, this is a very good question is how do we see this number internally. I think what we are looking at is before COVID, there used to be a pattern on quarter-on-quarter. Let's say, if you look at '18-'19,

'19-'20, so normally, there are 2 moments when the uplift in our revenue line happens, one is Q2 and other is Q4. So Q2, usually, as Bharath mentioned in his comments is about 7.5% to 8% higher than Q1. And this number that we have is about 6.5%. We were cruising along well, I think end of September, I don't know, you know that Northern part of India had massive rains and that affected last week. But that's not probably an excuse as to why this

number is lower. But I would actually say slightly more or less in line with what we were doing earlier. But I think 6.5%, I could have been happy with, let's say about 7.5% is what I would say.

Now, why this number looks lower than year-on-year basis is mainly because Q2 of last year is highly bloated quarter and Q2 of last year versus Q1 of last year, there was jump of nearly 16%, which, to my mind, I think, is affecting this particular year-on-year number. And that's what I actually said in my commentary that try and look at for the full year basis, as to what the growth would be rather than on just 1 quarter. And going forward also, Q3 is also one such quarter, where the base is relatively higher, while Q4 is a soft quarter last year. Now second question about Suburban. Yes, internally, we would have expected slightly higher growth, but I think there are a few challenges that we are facing in stabilizing the asset. The new leadership is just stabilizing. There is some sort of cleaning up that's happening because there's a huge tail with a small little volumes here and there. There's a bit of rationalizing on our channel is happening, and that is what has led to this. But we are very confident that going forward, we should come back quickly on this.

Rahul Agarwal: Lastly, sir, on the staff cost, why has it come down quarter-on-quarter to INR 91 crore?

Om Prakash Manchanda: Ved, you want to answer that.

Ved Prakash Goel: Yes. So there is a charge of RSU. This was reduced by roughly INR 8.5 crore. That is where we are looking to employ cost down.

Om Prakash Manchanda: But that is not normalized, sorry, it's all-inclusive.

Ved Prakash Goel: All-inclusive.

Moderator: We have our next question from the line of Sriraam Rathie from BNP Paribas. Please go ahead.

Sriraam Rathie: Just one question. Sir on the margins, generally, we have been guiding like it should be around 25-odd percent. Looking at this quarter's number, I assume that, of course, Q2 is on the higher base, but Q2 is generally the highest in terms of margins. How should we look at going forward like, I

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mean, Q3, Q4 moving back to 25% or you would like to say that, I mean, probably it can be on the higher side now?

Ved Prakash Goel: So Sriraam, Ved here. So you are right that Q2 is always a higher margin quarter for us. And if we see pre-COVID levels, I think the same trend we have seen for last two quarters. As far as we are growing in this trajectory, I think we are able to maintain our margins going forward, which used to be pre-COVID level.

Sriraam Rathie: Okay. So reported EBITDA margin of somewhere around 26% odd is what we used to do pre-COVID that should.

Ved Prakash Goel: Yes. You see 26.9% and that too with Suburban.

Sriraam Rathie: Yes. That too with Suburban. I mean, assume that Suburban margins have improved. So ideally, we should be at this level, where we are right now, the

first half.

Ved Prakash Goel: Yes.

Sriraam Rathi: And just another thing is that I mean, then Q2 is, of course, the highest quarter for us. But I mean, generally, should we expect that Q3 and Q4 will move on the lower side in terms of revenue? Or this particular year, it could be different because already still we have not seen the pre-COVID level kind of growth in Q2.

Om Prakash Manchanda: No, no, no. Q3 is the lowest quarter in our portfolio. So Q2 is the highest actually. Normally, Q4 tends to be higher, but I think in the last 4 years, 5 years, I'm not seeing Q4 equal to Q2. Generally, I think I would say out of all the 4 quarters, Q2 is the highest. And that's not only true for us, but for the entire industry.

Sriraam Rathi: So I mean just on that only, I was asking that probably like in this year Q2, we have still not seen the full potential of growth, like we used to see in pre-COVID era. So I mean

in this year, can it be different that probably Q3 and Q4 may not be that weak versus Q2?

Ved Prakash Goel: I don't know. I'm not commenting on the numbers, but generally, winters, the business actually in healthcare, in general, gets very depressed. So I actually have my doubt that Q3 will look higher than or it will be definitely weaker than Q4, Q2 is what my sense, right?

Moderator: We have a next question from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just the first one, Dr. Om, on your opening remarks, you talked about moderation in growth for the industry. Maybe your number also reflects some of it, but also the contrary statement that organized is gaining share from unorganized. So just can you help us understand some commentators have used the word COVID fatigue from a consumer perspective that there has been too much testing and there is now lesser testing or wanting to go out, anything qualitatively you can share on the moderation of growth.

Om Prakash Manchanda: I think this moderation term, I picked up maybe by analyzing all the data that's available in public domain. And I noticed that growth rates of many players are definitely not in line with what market used to have earlier. I think that's where I was coming from, that are we seeing some kind of moderation.

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And then if I look at deep down the profile of various companies and their businesses mainly it is lying in metros. That's where the moderation is coming now. It could be a combination of a few things, which is very, very difficult for us to figure out. It could be competitive intensity because there are so many players coming in. Obviously, they are gaining shares from some of these established player could be one reason.

The other reason could be, I don't know that some kind of fatigue that has set in because so much of testing, everybody has gone through it's just a bit of a pause last 6 months, 7 months to 1 year. But I think overall, the contradiction that you are seeing in is that because our bundled packages are going up, there's general health awareness. Obviously, people know that they have to take charge of their health is where this comment came.

Shyam Srinivasan: Just coming to your point on Swasthfit. So we have grown 21% I think Y-o-Y. So is there tailwinds post-COVID that you're seeing Swasthfit grow the way that is? And what are some of the building blocks and enablers that you are putting in place, so that this growth can continue?

Om Prakash Manchanda: I think there is a change in consumer behavior that I'm noticing. Maybe I'll

ask my colleague to add on to this, if I miss out something, which I have always been saying, a lot of people used to call, this a preventive health checkup. So I don't think it is preventive health checkup. There may be some component of that, but definitely non-communicable diseases related customer base is now moving to bundled packages because it's a great value for money from a customer point of view. They see that they are able to get more tests done, and they want to scan themselves for a much wider portfolio of tests rather than just say only diabetes or thyroid, now in maybe sub INR 1,000, you can actually get lot of tests done. So I think there is a change in consumer behavior, who are seeing a much greater value for money.

The second thing is illness-related testing or high-end testing relation is 1:1. The doctor visit leads to testing. Then it again, they go back to doctor and come back to the lab. But in non-communicable diseases, usually, it is a one visit to the customer and there's a frequent visit to the lab. There's no in between visit to the doctors. So to that extent, it tends to behave like as if consumers are making their own decision or choice about testing. So I think that is driving this whole bundled

packages situation, right? You want to add something to this.

Bharath Uppiliappan: Just that like I mentioned in the opening comments, other things, which are coming up and what Swasthfit is really doing is one is around the family offer. So like Om said there is a consumer behavior change. Today, more and more people are willing to get their spouse or one of the other family members also tested on the same visit or let us say in a matter of 1 or 2 days. So we enable some tech solutions wherein family members can get tested within 1 or 2 days of the main person getting tested. And second, obviously, is that a lot of focus on distribution expansion for Swasthfit as a product, number of collection centers, etc. are selling that daily So the very usual way product companies sell that rigor is setting in, I guess.

Shyam Srinivasan: And my last question, if I may, is on the comment again, Dr. Om, on price erosion, realizations that you quoted this quarter don't seem to suggest. So are we breaking it down like non-Suburban realizations have eroded? So if you can explain that comment, please.

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Om Prakash Manchanda: It's a good question. So I think good that you asked this question, let me explain. ... Most companies are talking about revenue per patient. But if you look at revenue per test, that is where the erosion is happening because now the number of tests per patient is very sharply moving up in the entire industry. So when earlier, let's say, INR1,000 was realization for 5 tests and suddenly, Now you are doing more number of tests, maybe just increasing the price by INR200, not to the extent of INR1,000. So technically, revenue per patient is moving up, but per test is going down because now you're doing more tests per patient. That's what I meant by price erosion.

Shyam Srinivasan: So you are not alluding to competitive pressures here?

Om Prakash Manchanda: No, no, no.

Shyam Srinivasan: Okay. And your comment suggest that you have maintained prices

Om Prakash Manchanda: Per patient per test is falling. But the good news is that if you are able to actually maximize revenue per single visit, it's actually beneficial. But if it's on the base of multiple visits, then you have a problem, whereas Bharath mentioned that if one phlebotomist goes for a home collection, if you are able to collect more patient samples, a large number of family members, then actually it becomes a much more viable proposition.

Moderator: We have our next question from the line of Prakash Kapadia from Anived

Portfolio Managers. Please go ahead.

Prakash Kapadia: Yes. Couple of questions from my end. If you could give us some color into Swasthfit revenues. I think Bharath did mention about some geographical expansion in terms of increasing the reach. So what would be, say, Swasthfit revenues from top 5 cities for us as of now?

Bharath Uppiliappan: I'm sorry, I didn't carry that number today. But yes, I can pull this out and then we're going to send across.

Prakash Kapadia: Yes. And in a competitive, increasing competitive scenario, how critical are the 100 million patients, which we've serviced say, during the last 5 years? Because as the business works for us and we realize in terms of cross-sell, the scale, repeat purchase. So what is our endeavor and focus on these existing customers, which we serviced over the last 5 years?

Om Prakash Manchanda: So yes, Prakash, this is something we realize as a big opportunity. There's obviously work in progress and some has got executed, some is yet to get executed. We are not going to share some very specifics unless you get tested with us. You can see what we do in the background. But needless to say, we recognize this farming activity as a large source of revenue, and our teams are at work on that front.

Prakash Kapadia: So we are internally trying to upsell and cross-sell to some of these customers and ensure we get more per visit or some more family member test is what the endeavor would be?

Bharath Uppiliappan: Yes. So like Om mentioned, we are significantly upping our A&P expense in the direction and lot of them have got technology back ends built in now.

Prakash Kapadia: And lastly, from my side, Suburban said annualized revenues of around INR160 crore, we are around INR80 crore, as on H1. What would EBITDA

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margins look like from the current level. So at what scale does those EBITDA margins go to 20% plus for us or Suburban?

Ved Prakash Goel: I think, Prakash, I would probably not put a number right now because it's very early stage, as I have always been saying that our focus right now is to see if we can stimulate the top line growth than worry about EBITDA margins. We probably would look at maybe 2-3 quarters down the line, as to how we look at it. But I see this is a great platform for us to see if we can drive growth further. Unfortunately, this year, because the dependence of Suburban on COVID was extremely high, nearly half of the business was coming from COVID, and the entire industry has seen COVID sharply fall. So to that extent, Suburban is also adversely impacted. But we just need to go through this for a couple of quarters and then take a call on how do we look at the margin trajectory. But I think the immediate focus is to see that we are able to build infrastructure, and collection infrastructure in the city of Mumbai.

Prakash Kapadia: And post the acquisition, it's been almost a year, more than a year now. So where are we in terms of, say, the test menu, how many have we added in Suburban, some sense on the patient collection center. Has that Mumbai lab opened? Where are we on that journey for Suburban?

Om Prakash Manchanda: So maybe I'll request Shankha to talk about this because he is the one, who is driving this test..

Shankha Banerjee: So in terms of 3 specific things you said in terms of adding collection network, it's an ongoing process. We've been on the job, and we are adding network each quarter. I may not be in a position to share exact numbers right now. But maybe after 2 quarters or 3 quarters, we'll start sharing numbers..

And the new lab in Vidyavihar that we have set up a reference lab that we have started in terms of testing and stabilizing that lab internally. And all the required accreditations is what is under progress right now. I think as and when that gets over, you would maybe hear more about it in the marketplace.

Prakash Kapadia: And on the test menu, is that going to be now a very important addition to Suburban to drive growth for us?

Shankha Banerjee: Yes. That is a part of the plan. And yes, it will happen once, like I said, once these accreditations, etc., are completely done, then action on that end will also start happening. Also, we are looking at a very close integration between the Suburban and LPL test menu, leveraging both together.

Om Prakash Manchanda: So I think, Prakash, It's very clear in our mind. It's a reference lab for Dr. Lal PathLabs group companies in the city of Mumbai. This lab of Vidyavihar is not only for Suburban as a legal entity, but it is for the all group companies and which we believe is very important for us to go forward.

Moderator: We have our next question from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: So I have a clarification on the personnel costs. So does CSR and RSU expense is INR 63 million, right? So where is this INR 85 million setting?

Ved Prakash Goel: Sorry, Sayantan, so I've clarified that there is a RSU cost, not CSR, RS U cost, which is included in personnel cost is down because there is a low cost on account of RSU this quarter.

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Om Prakash Manchanda: This quarter charge from RSU is much less than what it was last year.

Ved Prakash Goel: Yes, yes.

Om Prakash Manchanda: And I think that is primarily in the share price, and that's why it fluctuates right?

Arvind Lal: The share price is one of the variables. There are other variables also. Share price is also one variable.

Sayantan Maji: And also in this quarter, so can you give a rough idea in terms of which were the regions, which grew at a faster rate compared to the corporate average. So for example, is North India growing at a lower rate compared to the East India?

Om Prakash Manchanda: Our core markets of UP, Punjab, rest of North, South, they all really grew at a very fast pace.

Moderator: We have our next question from the line of Sameer Gilani, an Individual Investor. Please go ahead.

Sameer Gilani: My question is more on your cash flow statement. There seems to be an increase of trade receivables by INR 92 crore, which is not a large number, but given the numbers we've seen in the past, it seems slightly large. Can you throw some light on this, please?

Ved Prakash Goel: Yes. So it used to be INR 85 crore kind of number. For this quarter, it is slightly high because of government outstanding, but still well within our DSO, which is 30 days.

Moderator: We have our next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : Om, just 2 questions. One is, (a) on Swasthfit. Since just switching back to the price competition that we talked about, I mean, just your thoughts on, isn't the bundled packages most relative to acute sort of testing more attuned to price erosion or the pressures around just discounting and all of that?

Om Prakash Manchanda: Sorry, I'm not sure I got your question.

Nitin Agarwal: So my question was when we consider bundled packages, versus acute testing, which happened in response to when a doctor sort of prescribes a diagnostic test, where the quality and the brand starts to become very important. I mean, a bundled test is an impulse sort of a purchase in some sense and are more prone to maybe pricing or the discounting that really the best offer that potentially a customer get?

Om Prakash Manchanda: No, no, no. Quality is equally important in this, sir, because I have also talked to my phlebotomists as well as our courier network. They have actually told me that, yes, it leads to certain trial generation, but people are still coming back and saying that, okay, we want to still go back to quality. Nobody will take chances just because somebody is selling at INR 100 lower. They would definitely want to try it out, but quality is important across the portfolio, just because a wrong result can also lead to anxiety. Otherwise, you may not have a problem, but you may have anxiety disorder. So I think quality is important all across. Yes. And I'm actually about firsthand feedback from my team, people, who actually go and collect the samples.

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Nitin Agarwal: Okay. And secondly, on Swasthfit, I mean, these are largely what home tests, which we did home collection, I mean, how important is home collection in this sort of bundled test versus the acute tests?

Om Prakash Manchanda: So I think I go back to the first question someone asked about generally what is happening in the industry. I think there is definitely a consumer behavior change, which is coming. One is that they are looking for value for money, which is leading to bundled packages, so that is whatever preventive health checkup bundled packages that we talk about. I think that's one change. Second is convenience is becoming a more sought-after sort of value by patients. And convenience essentially would mean is that can they access the brand online, can they pay online, so they don't want to come to the lab

and wait for 40, 45 minutes. So I think with all that put together, home collection is one element of that convenience. The convenience could also be how quickly you deliver the report, how easy your website is accessible or app, etc.. So I would call that a convenience and the overall value is quite sought-after by the patients these days in addition to pricing and quality.

Nitin Agarwal: And sir, one more thing. We talked about we're not increasing some amount of price and competition, but our gross margin seems to be probably amongst the highest that we've done in a while, and this is despite some amount of rupee depreciation happening and we properly having some amount of imported raw materials. So how does one sort of tally that?

Om Prakash Manchanda: So I think mainly because Q2 generally, we all look at growth percentage. But if you keep mind away from that, in absolute terms, it's a very high quarter. So INR 534 crore in just 1 quarter is a very high number. So whatever our Q2 margins that you see, I don't think they are representative for the annual, for the real margin, right.

Nitin Agarwal: Yes. This quarter is non-COVID is highest revenue, INR 514 crore is the highest ever.

Om Prakash Manchanda: Highest ever and I think it will be the highest in any quarter of the year. So please do not look at the current margin, as a margin, which we will deliver for the year.

Nitin Agarwal: I was looking at the gross margin, but I understand the EBITDA margin because that's an operating leverage involved. I was more surprised by the expansion in the gross margins.

Om Prakash Manchanda: Gross margins, I mean, is it that high, Ved?

Ved Prakash Goel : It is high, little bit Om.

Om Prakash Manchanda: Sorry, I haven't seen that. Is it? I don't think it's.

Nitin Agarwal: No, no.

Om Prakash Manchanda: No, I don't think it's sharply high.

Nitin Agarwal: It's not that.

Om Prakash Manchanda: We will look into that. But at least my gut says that it won't be that high.

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Nitin Agarwal: And if I can take one last one. When we take a 3 year view from here on, what kind of gross margin level do you expect?

Om Prakash Manchanda: I think you are talking about gross margin being higher, mainly because last year COVID was higher, so COVID has lower gross margins. You're comparing with the last year number, right.

Nitin Agarwal: I was looking on a Q-o-Q basis.

Om Prakash Manchanda: Last year I'm looking at material cost % ex of Suburban , which is 23.2% this quarter versus 24.6% last year same quarter, so which essentially means gross margin this year is better than the last year.

Ved Prakash Goel: First half.

Om Prakash Manchanda: First half, sorry. I'm looking at both the quarters together. It's mainly because COVID component last year was very high, and the COVID had a lower gross margin than our rest of the portfolio. Now COVID has just gone away, so obviously, our gross margins look better.

Nitin Agarwal: Sorry, last one, if we take a 3 -year view, what is the aspiration or rather what is your assessment when you look at the business, what is a good volume growth number, patient volume growth number that we should be sort of looking at?

Om Prakash Manchanda: It's very difficult to put that figure because industry is definitely in a state of flux. And on one side, I'm seeing some euphoria declining amongst competition. So we'll have to wait and watch. It's too early. Maybe next couple of quarters we'll have to see. I think this whole excitement, which COVID created in the financials is dying down. So my sense is, we will give a stable sort of rate after 2 quarters. So my best guess would be, I think you should wait and watch for another 2 quarters to see a real trend. But I think at a macro level, all the macros are in place in terms of diagnostics

is highly

underpenetrated India. Medicine has to become evidence -based, very large country, 1.4 billion people. It has to go to Tier 2, Tier 3 towns. I don't see any reason why industry growth rates should be on a long -term basis, if not higher, not the same as what we have seen.

From a volume perspective, I actually don't see any reason why industry should not grow. I think the challenge could be on the competitive intensity where some of these players continue to resort to cash burn , then I don't know how pricing would settle down , that is the only thing I would probably

watch out for. To my mind, if funding is drying up as we keep reading in the newspapers, we'll have to see after two, three quarters, what really happens and how they resolve. I think early signs are some of these players have started taking up the prices of the packages, which could be a sign that they have run out of steam to stay on those current price points.

Moderator: We have our next question from the line of Sameer Baisiwala from Morgan Stanley. Please go ahead.

Sameer Baisiwala: Sir, can you update us on your distribution expansion that is underway. I know you disclose the numbers at the end of the year, but we are seven months down, is it on course, Labs, PSCs and puts?

Bharath Uppiliappan: Yes, indeed, it is on track and I mentioned in the opening comment, that we are seeing good response on Tier 2 plus cities and on South also the

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geography and our expansion plans continue both on collection center, pickup points, as well as the lab network, no slowdown on this count.

Sameer Baisiwala: Okay. And sir, the second question is, you've been mentioning on your presentation deck, things like home ECG testing, radiology and gene testing. If you can talk a bit more about these, do you think these can become big in the overall portfolio? Or how do you view these?

Om Prakash Manchanda: I don't think this can become big. These are more filler tests, I would say. I don't think this is the belly of the business at all.

Sameer Baisiwala: Okay. That's fine. And final question from my side. How do you judge Suburban acquisition on ROI basis, given that you must have spent about INR1,000 crore plus and where the numbers are tracking right now?

Om Prakash Manchanda: I think we have looked at Suburban more from a geography or presence point of view, where Mumbai and Maharashtra are very significant markets. On our own organically, we may have had a very huge challenge in setting down, we have tried this last 10, 15 years, INR 160 crore and INR 170 crore of business, 80% is coming from Mumbai. I think we see this as a great platform in the life of Dr. Lal PathLabs only for times to come. So we are not looking at on a quarterly or a one-year basis. I know this year, some of this math will be little awkward given that covid has just gone away with half of the business, but so that's the life. And I really don't look at from or rather as a team, we don't look at it from three months or one year perspective. For us, we see this deal from a very, very long-term standpoint. And it also gives us ability to build, I think Prakash asked this question about central lab. If we're able to build a nice platform of very, very high-end wide test menu in Mumbai, I think it's a great thing for us too. On a small scale sometimes these investments really don't deliver, but at such a high base, this can really work well for us if you can turn this around.

Moderator: We have our next question from the line of Rishabh Tiwari from Allegro Capital Advisors. Please go ahead.

Rishabh Tiwari: The question is regarding the Non-COVID revenue growth, which is around 6.6% for this quarter. Could you please throw some light on how much of this is coming from volume and how much of special coming from price given that there is a price

erosion in the revenue for test while the RPP is going up?

Ved Prakash Goel: So let us say there is no price erosion per se, there is a bundle test and super specialty contribution moving up. So if you put back the numbers, half will be volume and half will be revenue realization, not a size increase, revenue realization moving up, actually going, so half, half.

Rishabh Tiwari: Okay, got it. Thanks. Just the last question, what was the contribution of Delhi NCR revenues in Q2? Since the opening remarks mentioned about focusing expansion in South and also the reference lab coming in Bangalore. So if could have a broad number, the contribution from Delhi NCR, I remember it was 35% last quarter a year?

Ved Prakash Goel: So same range, yes. It remains the same range.

Om Prakash Manchanda: I think the price erosion comment is not from our company stand, I think we have the general comment about industry because that's the way it has happened.

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Moderator: We have our next question from the line of Rishi Modi from Marcellus Investment Managers. Please go ahead.

Rishi Modi: So my first question is that our three year revenue CAGR, right, non-COVID, non-Suburban has been around 9% to 10%. So are we seeing some saturation out here? Or are we losing market share, like I'm guessing the industry might be growing much faster given post COVID as well. So just trying to understand, have we lost any market share?

Om Prakash Manchanda: See, we have done this analysis for all the companies who publish data and we are seeing at least our growth rate of the CAGR of 10%, it's still the highest growth, right. I don't see that we are losing our market share. So maybe it might just take a bit of time for it to come back to the same old trajectory, but slowly, slowly the business is inching upward, that's the point I was saying rather than looking at year-on-year see that how quarterly this trajectory is moving, it is more or less mimicking what it used to be earlier. I don't think that we are losing our market share at least if I look at the base on the published data. Now of course, there are large number of unlisted players, but we don't have access to the information that they have. But having said that, of course, they also are getting some business, so they must be nibbling away a little bit here and there, but I don't know whether that's a sustainable number for them, we'll have to wait and watch just our guess.

Rishi Modi: Right. And as you said right now that you're moving into Tier 2, Tier 3, so how do the productivity and profitability numbers stack up for Tier 2, Tier 3 versus a current year one numbers, like, what are you seeing out there?

Bharath Uppiliappan: So if you look at productivity, there are two types of productivity, one is throughput per outlet and then there is the entire ecosystem. The throughput obviously takes a bit of time more in the Tier 2, Tier 3 to build up. There are various types of market, brand strong market, brand weak market. On an overall level if you manage the expansion plan well enough, I think we'll continue to see the results as we are seeing now.

Om Prakash Manchanda: I think broad thumb rule is that, as you go away from the core markets, your margins are slightly lower. The trick lies in managing in a very calibrated manner, how we expand into these markets to manage your margins. Let's say, if our company was only Delhi NCR company, margins definitely would look much higher. They obviously get diluted as we go into the hinterland, I think that's a broad rule, right? We want to very, very carefully maneuver the balance between metro as well as Tier 2, Tier 3 towns and also how you manage your lab infra.

Rishi Modi: Got it. Okay. And so you mentioned on the call that you have doubled your tech expenses in the past few years. So where are you

investing in this tech like want to understand and what are you doing in the back end, if you could give some more clarity?

Bharath Uppiliappan: So on the front end, so you actually pointed out front and back end and the security layer, there are three different ways to cut it. On the front end, all our digital properties have been revamped, website, app, the whole experience has been revamped. On the supply chain side, we have put in lot of digital assets to monitor the sample flow. So we talked in the past about having a control tower. We talked about having AI /ML layer, analytics player, all those things are now in place. And at the back end and the infra side also

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there is a migration to cloud which is happening slowly and steadily. Most of our applications are now cloud hosted. Also there is a large investment around security, governance, data governance etc.. So host of initiatives just not about making a nice app, but the entire end -to-end front end and back end and middle in between also.

Arvind Lal: Rishi, this is Dr. Lal here.

Rishi Modi: Yes.

Arvind Lal: We were the first people to bring IT into the entire health care industry in 1986, so the experience which we have in IT and digitization nobody else has. So we are going around trading along very well and I think we are still the best.

Rishi Modi: Got it.

Bharath Uppiliappan: Also the partner ecosystem is also largely digitized now and a lot of programs are running on further strengthening those.

Rishi Modi: Understood, understood. Okay. Also, finally, I think Om touched upon the challenges of Suburban. If you could get into more depth on that, I think the two, three points were mentioned on Suburban, one was getting the partner network economics right. And then there was the team settling in. So if you could give more color on that?

Om Prakash Manchanda: I think to my mind, nearly half of the business was out of COVID last year, so that is settling now, declined to about 90% decline. I think that is one side, which we have to now manage because that has obviously dropped the throughput in the channel and we have to look at the whole viability. Second is, we have put and there is a change over from a promoter run set -up to a professional run set -up. So I think there are few HR things which are, of course, behind us. And third is putting this Vidyavihar lab, which is very critical to our success. As Shankha mentioned that we are in the process of getting accreditation as we launch that, this should fall in place. And third is, of course, Mumbai having said that is not a very easy market, otherwise, we wouldn't have done this, we would have done it our self. So , it's a highly competitive platform and I think we'll just find it out.

Rishi Modi: Right. So , on Suburban, right, you all have some basic radiology in the sub - urban centers . So like what's the plan? Are you all planning to continue with the radiology or and then maybe scale it up pan -India? Or are you all planning to continue being only a pathologist?

Om Prakash Manchanda: So, I think since you use the term radiology, we don't do CT/MR there, it's mainly x-ray, ultrasound and primarily if I look at they are more catering to the health checkup sort of a concept. I think as a company, we are more a pathology. I think it will be more in terms of building our collection network that's the way we have done in the LPL because that really gives you a decent scale. So our hope is potentially to build infrastructure in terms of collection network.

Moderator: We have our next question from the line of Arshita Jain from Nuvama Wealth Management . Please go ahead.

Arshita Jain: So sir, in continuation to the last participant, so you mentioned the new management is still stabilizing this asset. So just wanted to understand, what

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are your internal expectations from this asset particularly the Suburban growth expectation?

Om Prakash Manchanda: So I think our internal expectation is, if we can build western region for combined business of Dr. Lal PathLabs Suburban together, that's how we will define success for our self. And when I say that our ability to really get synergies at the back end, so that both side labs are able to cater into the market and the front end, we continue to do a brand. So I think I would say western region business starts growing, that's how success will be defined for us.

Arshita Jain: No, I'm saying, you think this asset can grow much faster than the industry growth in the coming year?

Om Prakash Manchanda: I think so, here.

Arshita Jain: Okay. The second question is on the revenue contribution from the metros and Tier 1, what is it now? And what was it three years back? Any color on that?

Om Prakash Manchanda: So let's say, if I take Delhi NCR, three, four years back, it used to contribute nearly about 50% or 55%, now it is down to about 34%, in H1 FY23 excluding Suburban is that correct?

Om Prakash Manchanda: So we are spreading our reach very well outside Delhi NCR. So our dependence in metros over a period of time is reducing.

Arshita Jain: It will even include the Tier 1s also?

Om Prakash Manchanda: I don't know what you mean by Tier 1, are you talking about mini metros and Tier 1 or how do you define Tier 1 in fact?

Arshita Jain: Basically just wanted to know your contribution from Tier 2 to Tier 6 towns, Tier 2, Tier 3 towns right which is expanding simply?

Om Prakash Manchanda: Directionally, we are all going down the pop strata for our growth strategy, so whether if we go to Tier 3 or Tier 4, idea is to go down the pop strata. Let's say, you are in the South region, you are slightly still fighting the battle out in metros. So we will go to the next layer of towns. In North, we are obviously in Tier 3 right now. So I think I would say whatever markets we are, we are looking at next pop strata, which is down the pop strata.

Arshita Jain: Fair enough. And lastly Swasthfit, Swasthfit already is 20% of your non-COVID revenue, where do you think this contribution can move up in the coming quarters since you are seeing change in the customer or consumer behavior

Om Prakash Manchanda: I think that's a brilliant question and I think that's an overall trend in the industry. I would actually say that it will keep growing, I don't know what that number is because if you historically study 15, 20 years back, even these panels were not there, LFP, KFT, etc.. And somebody would have talked about this panel as a great story that time, today, now you're bunching these panels calling them bundle packaging, right? So I think directionally that's the way industry has moved from single test to a panel now, panels being bundled is now a big thing. So directionally, I would say it will keep growing.

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I don't know where it finally lands up and good news is that these are both packages where doctor intervention is relatively lesser

Arvind Lal: And Arshita, this is Dr. L al here. Don't forget that mortality and morbidity in India is 65% and hence these non-communicable diseases are not going to stop. Diabetes is going to grow, hypertension is going to grow, Cancer is definitely growing and the liver disease, kidney disease , where is the stoppage , there was a lull there because of the COVID because COVID took the front face because of the mortality, but the rest of the things have not disappeared.

Moderator: We have our next question from the line of Dheeresh Pathak from Wh iteOak Capital Management. Please go ahead.

Dheeresh Pathak: I was looking at you are saying that seasonally Q2 has done well. If I have a number of based on my mat

h, 6.7 million, so that is just 0.2 million increase in footfalls, which is a measure of nu mber of patients ex -COVID, ex - Suburban between Q1 and Q2, which in a pre -COVID time used to be anywhere between 0.4 million to 0.5 million extra patients we would get between Q1 and Q2. So from a footfall point of view, it's been softer versus what we woul d see in the pre -COVID time period, so I guess their numbers are right that I'm talking about.

Bharath Uppiliappan: So for Q2, the volume growth is 3.3%.

Om Prakash Manchanda: No, no, I think his question is, the way we are highlighting Q1 to Q2 , 6.5 value growth, he is saying volume growth is lower, right? Is that correct?

Dheeresh Pathak: Yes, yes. If you look at it just in terms of delta between the patient footfall between Q1 and Q2, on an absolute number, it is much lesser versus what we wer e doing with pre -COVID, so pre -COVID in FY'19 between Q1 and Q2, we did about 0.5 million extra patients, then in FY'20, which is obviously COVID hit in the last quarter, but if you see between Q1 and Q2 that was about 0.4 million. This year, if you take o ut Suburban, it is only 0.2.

Om Prakash Manchanda: Yes, so I think you have an observation. I think if you don't mind, we would definitely love to have an offline conversation on that, I don't have readily that data available, so I can't engage myself deeper into the conversation.

Dheeresh Pathak: But is that the right metric now . You think just looking more at patient footfall, so that is a measure that we should ...because more tests are getting bundled. The patient footfall also will be important metri c in your mind or value growth is more important?

Om Prakash Manchanda: No, no, actually all three I would say, I think there is a good point you're saying. I would say revenue, then revenue per patient, then number of patients, then I would say, number o f test per patient. I think all the four are important for us to actually see how we trade the business is going up.

Dheeresh Pathak: Okay. And sorry, go ahead, sir.

Om Prakash Manchanda: I said all these four metrics are important, yes. But having said that, I wouldn't worry about quarter -to-quarter variation, I'll tell you reason why. This year, let's take year in which Dengue incidence is very high, you will certainly see volume shooting up very sharply because one Dengue patient in a week's time might visit your lab 10 times, you understand what I mean? So, in a quarter, if the Dengue is not that high and certainly you'll see volume

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dropping. So I really won't worry about one quarter to another quarter, I will definitely look at this metric over a long er period as to how it's trending.

Dheeresh Pathak: Certainly. How do you characterize Dengue this quarter versus the past year?

Om Prakash Manchanda: I really won't have that data, but I think this year slightly Dengue has been little bit muted and in some places, it's averaging, I won't say it's unusually high.

Dheeresh Pathak: Okay. And what was the acquisition related amortization and what was the Ind AS adjustment?

Ved Prakash Goel: So in this quarter, INR12 crore amortization on account of acquisition.

Dheeresh Pathak: Okay. And what was the Ind AS is INR14 crore, IndAS adjustment between the EBITDA and the IndAS adjusted EBITDA?

Ved Prakash Goel: So it is not unusual, it's in line with the past trend only, so about 1%, 1.5% kind of yes.

Dheeresh Pathak: Okay. It is 2.9% of revenue it was last quarter, which was INR14 crore in absolute term, which is higher than 1%, I'm assuming it will be similar in absolute term versus last quarter, right?

Ved Prakash Goel: Yes, similar, so there is no change, the only thing in India rental cost is coming down in the form of rentals and depreciation, that's it.

Dheeresh Pathak: Okay. When you say 1%, it is higher than 1%, right, it is INR14 crore, which is 2.7%?

Ved Prakash Goel: I will just check, but it is.

Om Prakash Manchanda: He doesn't have that figure.

Dheeresh Pathak: Okay. I'll connect offline.

Moderator: We have our next question from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: So first question was on tax rate, for the full year, will it settle back down to the 25% or level that you normally have? Or will it stay elevated as you've seen in the first half?

Ved Prakash Goel: Yes, it will be similar, there is nothing change in there in terms of tax.

Prashant Nair: So similar as the first half of this quarter or similar to what you've been doing in the past?

Ved Prakash Goel: Yes, similar, I mean, in this first half also is similar, it's nothing which has changed.

Prashant Nair: All right. And so generally on the growth front. So when we look at industry growth, would it be fair to say that steady growth that you are doing in say in the Delhi NCR market where you are a leading player and have high market share, is a fair reflection of what the industry will be growing at? Or do you think you're still gaining some share in that market as well?

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Om Prakash Manchanda: In Delhi NCR?

Prashant Nair: Yes, so for example, my question is, if you're growing say 8%, 10% in Delhi NCR, is that a reflection of what industry growth would be for that market and similar markets like, say, larger markets, larger cities or each state has its own dynamics and growth rates could vary?

Om Prakash Manchanda: I think each city has its own dynamic because I think competitive intensity in general I'm seeing is high in four or five cities in the country, that's where the maximum noise is. You go down the pop strata, you may not hear some of these names. So I think it's a city level sort of a thing. And let's say, we all talk about certain pharma companies, we don't even hear them in northern part of India. So I think depending on where they are strong, the infra is strong, that's where they, I would say it varies from city to city, but definitely four or five big metros are where especially online players are more interested.

Moderator: We have a follow-up question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: I think in the initial comments, you mentioned something about introducing a new suite of test for pregnant women. I mean, if you can probably highlight little bit, put some more color on that? And I mean, what kind of opportunity do we see in this space?

Om Prakash Manchanda: So I think Bharath mentioned about that. Our view is that industry is getting sort of vertically split, you have bundle packages and then you have always high-end packages, right? Now that's where the segmentation would come in since all of you cover pharma space and you have things like Dermatology division, you have gynaecology, you have things like that. So similarly like we have launched this auto-immunity and this kind of segmentation segment focus would come. So this reproductive diagnostic is one such promising segment in the industry and that's what Bharath meant by that. So it's not about just pregnancy alone and I would say these are all life stage testing starting with puberty and motherhood and things like that. So I would club all that into a reproductive diagnostics.

Nitin Agarwal: And just one separate question, sir when you alluded the earlier point about expansion in sort of smaller towns and your comment that probably that growth comes at a slightly lower margin. So just picking a thought on it, is something like an absolute EBITDA per patient, is there a right metric to look at this business and is that a metric that we track internally also?

Om Prakash Manchanda: I think what I meant by that was basically it's a lower realization, you go down pop strata, it's affected by many factors like the ability to pay or in prescription habits of doctors, etc., if not like where somebody would go for INR2,000, INR3,000. So what I meant all that ability to pay lower down the pop strata and similar prescription behavior is very different than, let's say, the metro cities. And the profile of testing is also going to be different because they are more routine in nature which are low price. And compared to that unit economics, it's very difficult to get pathologists and at times we end up paying slightly higher salaries in smaller towns and in the metros. So it's all in all put together. And plus, you do not see. I said it's also revenue trajectory also doesn't pick up that sharply and thereby, the overall net impact is the margins are slightly depressed and what we would realize in metros, so scale is also very high in metros.

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Arvind Lal: Also, this is Dr. Lal here, also when you go away from the metros, you go into the other town, smaller towns, you are faced with lack of electricity, lack of potable water and so you're going to spend money on other things, including electricity, you are going to pay much more. And sometimes, we've even had to carry RO water all the way from Delhi to smaller towns. So nothing goes cheap, as you go actually deeper down, the cost of servicing that patient actually increases.

Nitin Agarwal: I mean sir link point as a proportion as we grow, as we grow more of volume growth is coming from the smaller areas, does that on a weighted average

basis create pressure on our EBITDA margins, so while absolute EBITDA come through what margin perspective it may just create problems rather pressure on the EBITDA margin?

Om Prakash Manchanda: So I think there are factors which are favorable, unfavorable. I think that is where the whole management lies, art of management lies and to best of our ability, we are trying to see how we don't dilute the margin. I think that's where you are seeing the numbers. But we are cautious also in our commentary because as you go down the pop strata , I think it's natural to believe that margins won't go up, but we are trying to see how we keep on sustaining our current level.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand over the call to management for closing comments. Over to you, sir.

Ved Prakash Goel: Thank you, everyone for being with us on this call today. I wish all of you remain safe and healthy. Looking forward to meet you next quarter conference call.. I would now request the moderator to close the call. Thank you.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

Call: Feb 2023

February 08, 2023

National Stock Exchange of India Limited
Exchange Plaza,
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Bandra Kurl a Complex, Bandra (E)
Mumbai – 400 051 BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai - 400 001

Subject : Financial Results Conference Call Transcript for Q 3 FY23

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Financial Results Conference Call Transcript of the Company for Q3 FY23.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Rajat Kalra
Company Secretary and Legal Head

Encl.: As above
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Dr. Lal PathLabs
Q3 FY23 Earnings Conference Call Transcript
February 02, 2023

Call Duration • 1 hour 22 min
Management Speakers • Dr. Om Prakash Manchanda - Managing Director
• Mr. Bharath U - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer
• Mr. Shankha Banerjee – CEO Suburban and other group companies
• Mr. Rajat Kalra - Company Secretary and Head of Investor Relations
Participants who asked questions • Dheeresh Pathak - WhiteOak Capital Management
• Rahul Agarwal – InCred Capital
• Prakash Kapadia – Anived Portfolio Managers
• Prakash Agarwal – Axis Capital
• Cyndrella Carvalho - JM Financial
• Shyam Srinivasan – Goldman Sachs
• Sameer Baisiwala - Morgan Stanley
• Aashita Jain - Nuvama Wealth Management
• Sayantan Maji - Credit Suisse
• Tushar Manudhane – Motilal Oswal
• Tanmay Gandhi - Investec
• Nitin Agarwal – DAM Capital

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Moderator: Ladies and gentlemen, good day, and welcome to the Dr. Lal PathLabs Q 3 FY23 earnings conference call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Nishid Solanki of CDR India. Thank you. And over to you, Mr. Solanki .

Nishid Solanki: Thank you. Good evening , everyone, and welcome to Dr. Lal PathLabs Q 3 FY23 earnings conference call.

Today , we are joined by senior members of the management team , including Dr. Om Prakash Manchanda -- Managing Director; Mr. Bharat H U -- CEO; Mr. Ved Prakash Goel -- Group CFO; and Mr. Shank ha Banerjee -- CEO, Suburban and Other Group Companies ; and Mr. Rajat Kalra -- Company Secretary & Head of Investor Relations. (Hony) Brig. Dr. Arvind Lal will not be able to join the call today due to some personal engagements.

I would like to share a standard disclaimer here. Some of the statements made on today's call could be forward -looking in nature and the actual results could vary from these forward -looking statements. A detailed disclosure in this regard is available in the 'Results Presentation', which has been circulated to you and also available on stock exchange websites .

I would now like to invite "Dr. Om Prakash Manchanda to share his Perspectives ."

Thank you. And over to you, sir.

Dr. Om P Manchanda : Thank you, Nishid. Welcome everyone to Dr. Lal PathLabs Q 3 FY23 Earnings Call .

I hope you and your loved ones are safe and healthy.

I'll start by taking you through some industry landscape and key internal developments.

"The Industry Landscape "

Industry at large is experiencing operating deleverage in this financial year due to sharp decline in COVID and COVID -related revenue. The decline in COVID business has been to the extent of 80 % to 85% compared to last year in general. While it was not very clear last year, but deeper analysis also suggests that even non -COVID revenues were favorably as well as unfavorably impacted in some quarters due to COVID, thereby resulting in extremely uneven quarter numbers in FY22. This brings me to a point that for meaningful analysis on top line and bottom line one needs to look at CAGR pattern of non -COVID sales over three to four years on an annualized basis , that is the comparison with pre-COVID times. Also, one needs to look at trajectory building on sequencing pattern that is quarter -on-quarter trends over the last three to four quarters instead of year -on-year basis.

The second is industry also experienced in these couple of years , entry of many players coming from pharma , hospitals and even e -commerce as well. Most large players in the industry also haven't taken meaningful price increase in the last five to six years. On the contrary, one thing is that bundling of tests may have led to higher realization per patient but has resulted into effective decline in realization per test. So, far, this has been possible due to operating leverage , mainly driven by higher number of tests per patient. Though it would be very interesting to see how this pattern takes shape, given very high inflation scenarios in times to come.

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Now, a few developments at our end. Some of you may be aware that we have launched a state -of-the-art reference lab in Mumbai. This move is in line with our long-term strategy of driving the West region as a long -term growth pillar.

This is the first private lab in West India to have BSL III, bio containment lab, inclusive of mycobacteriology, mycology and molecular biology departments. The combined in

fra between L PL and Suburban put together now stands at 50 labs in the state of Maharashtra, and out of which , 20 labs are in Mumbai . The central lab at the top of this pyramid gives us a huge competitive edge in the West region.

"Portfolio ":

Our portfolio mix comprising super speciality tests and bundled test offering under the brand name , "Swasthfit" is witnessing strong response as it offers a comprehensive solution to the patients. It reflects a large understanding of the patient's diagnostic needs, ability to cross -sell and up -sell, and the value sought by patients . Contribution from Swasthfit now stands close to 19% to 20% of our overall sales.

"High -End Test Portfolio" :

As an organization, our vision is to be the most trusted healthcare partner, enabling healthier lives. To surpass this, we continue to strive in technologies and launched newer tests that continue to help early and accurate diagnosis. We have invested in Second Electron Microscope at our National Reference Lab in Delhi.

We are also creating specialty verticals to provide sharper focus on certain identified segments. In last couple of years, we have created three verticals; #1, Genevolve focuses on genomics; #2, L-CoRD focuses on reproductive diagnostics; #3, L-ACE focuses on auto-immune disorders. And there are many more such initiatives in the pipeline.

I want to talk about the "Tech Piece" :

There are a lot of conversations around changing patient needs, and businesses becoming digital. Over the last three years, we have significantly enhanced our digital capabilities. Our new consumer app today has a very good rating on android and also custom built and integrated 1x view across all channels.

With that now, I would like to invite our CEO – Bharath to continue the conversation forward. Thank you.

Bharath Uppiliappan : Thank you, Om. A very good evening to each one of you present here. I warmly welcome all of you to our Q3 FY23 Earnings Call. I will take you through some of the "Operating and Business Highlights of our Company."

In Q3 FY23, we served 6.5 million patients, registering a total revenue of Rs. 489 crore . COVID and allied test declined 80% and contributed to only Rs.11 crore , that is just 2% of the overall revenue in Q3 FY23 versus 12% in last year's same quarter. Our non -COVID revenue at Rs.478 crore, registered a growth of 9.1% in Q3 FY23 over the last year's same quarter. Patient volume registered a growth of 4% and RPP is up by 5%. Test per patient also moved up from 2.59 in Q3 FY22 to 2.66 in Q3 FY23.

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As you would have noticed versus past trends, this quarter has seen some noteworthy interplay between patient visits, test per patient and test mix. Q3 FY22 base events impacting various KPIs on a YoY basis. Hence, it is better to analyze the numbers on a three -year CAGR basis.

For the organic business, we are seeing an improving trend in our three -year CAGR on a sequential basis, which has moved up from 9.4% in H1 FY23 to 10 .8% in Q3 FY23. Patient three -year CAGR volume growth in Q3 FY23 was at 8.3%. We continue to take measures to further accelerate the growth rates. We have also significantly scaled up our activation for L-CoRD which is a center for excellence for Reproductive Diagnostics .

During Q3, we organized 500 -plus activities across India on a single day, which is an achievement in itself. We continue to see traction in L-ACE in our Autoimmunity Center of Excellence . Bundled test programs , Swasthfit continues to do well and its contribution continues to be in the range of 19% to 20% of the non-COVID business. We continue to invest in our digital initiatives to improve patient journeys for our B2C business as well as for our B2B partners. We are seeing very encouraging trends and we'll build on these in the coming quarters as well. Overall, we remain steadfast in our commitment

to the strategic agenda and execution focus.

With that , I would like to invite Ved to take you all through the Financial Performance. .

Ved Prakash Goel : Thank you , Bharath . Hello, everyone , and thank you for joining this call. I trust each of you and your families are safe and healthy.

I will share some of the "Key Financial Highlights" :

Our non-COVID revenue for Q3 FY23 is at Rs.478 crore, a growth of 9.1% . YTD non-COVID revenue grew by 15.9% to Rs.1 ,474 crore. Total revenue for Q3 is Rs.489 crore against Rs.497 crore last year's same quarter . This decline of 1.5% is due to a fall in COVID business by 80% as compared to last year. If you remember , COVID contributed 12% in Q3 last year, while it is nearly 2% now.

On a YTD basis , COVID contribution has fallen sharper from 21% to just 3% now. YTD total revenue came at Rs.1 ,526 crore versus Rs.1 ,602 crore last year same period . Revenue realization per patient for Q 3 FY23 is Rs.754 as against Rs.747 last year same quarter . Better realization per patient is due to the test mix and higher contribution of Swasthfit which is 19% now .

Normalized EBITDA after eliminating the impact of RSU and CSR for Q3 FY23 is Rs.122 crore as compared to Rs.119 crore last year's same quarter . The normalized EBITDA margin for this quarter is at 25%. Normalized PBT after eliminating the impact of notional depreciation on account of consolidation of Suburban for Q3 FY'23 is Rs.89 crore. The normalized PBT margin is 18% . The normalized PAT for Q3

FY23 is Rs.66 crore and the normalized PAT margin is at 14 %. Net cash and cash equivalents as on 31st December 2022 are Rs.489 crore.

To conclude while we are focused on driving top line , our initiative continues in the direction of cost optimization as well by using digital tools and technologies.

With that , I request the moderator to open the forum for question -and-answer.

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Moderator: We will now begin the question -and-answer session. The first question is from the line of the Dheeresh Pathak from WhiteOak Capital. Please go ahead.

Dheeresh Pathak: Sir, on Suburban , the patient count for the quarter, the revenue and EBITDA , if you can provide ?

Ved Prakash Goel : So, Suburban , this quarter has revenue of Rs.37 crore, out of which Rs.35 crore is non-COVID. And EBITDA for this quarter is about 7%, which is low , because we have launched this reference lab , in this quarter there is an impact so all the expenses related to the lab are also factored in. Generally , Q3 is having lower margins overall as compared to other quarters.

Dheeresh Pathak: Number of patients for Suburban ?

Ved Prakash Goel : Let me come back to you for the Suburban patients.

Dheeresh Pathak: So, adjusted for Suburban , it seems the three -year patient count CAGR is running at mid -single digit. You said, that look at three -year numbers. So , is this the right metric to look at or no?

Dr. Om P Manchanda: You're saying three -year number for value and volume , both you are talking about or?

Dheeresh Pathak: No, I'm just talking about volume right now.

Bharath Uppiliappan: If I use number of patients as a measure of volume , for December quarter, the patient CAGR on an ex-COVID , ex-Suburban basis seems to be running at mid-single digit.

Dr. Om P Manchanda: 8.3% , yes, you're right.

Bharath Uppiliappan: It's even lower , it's not even 8.3 5%. If I make some assumptions for Suburban you might be including Suburban there , right.

Dheeresh Pathak: 8.3% excluding Suburban ?

Bharath Uppiliappan: 8.3% excluding Suburban volume .

Dr. Om P Manchanda: Sorry, your question was about Suburban volume , this quarter is 0.3 million .

Dheeresh Pathak: Maybe will take number separately. I'm getting a different look. But anyways, this 7%-8% volume CAGR , how are you interpreting this versus the baseline growth we used to have earlier

this is lower than that so how should we as investors interpret this?

Dr. Om P Manchanda: One thing is fairly clear if you look at growth rates from FY20 till now, this is definitely soft by a couple of percentages. That number is clear. Now, is it because of this disruption we've had due to COVID or is it because of higher competitive intensity ?

Bharath Uppiliappan: COVID patient, non-COVID volume.

Dr. Om P Manchanda: Basically, two factors I would say are very important. And my sense is maybe both are valid. As we mentioned earlier also the purchase frequency in this business is at least once a year, that's an average we see in our business. So , because of deep discounting and aggressive promotion, we may have had a trial generation done by

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a lot of patients, but eventually repeat purchase would happen at least after one year.

So, we will actually know in a longer period of time as to how it is. But slowly, slowly, I think a very small sort of upward movement. Because our dependence on Delhi NCR is very high and Delhi NCR is slightly lower growth in overall, I think that's where it's not showing up. But it's very clear that definitely a couple of percentage points are softer than what we used to have.

Dheeresh Pathak: Is there some cohort analysis that you might have done or you can do that let's say pre-COVID , what percentage of patient volumes would be new to Dr. Lal , and what percentage would be our old patient cohorts who are coming after one year, two years , three years versus what is happening now , let's say in FY20, your certain percentage of volumes would come from old customer cohorts , and as of now, what percentage is it meaningfully lower now versus what it used to be in pre-COVID?

Dr. Om P Manchanda: I think that's a great question and I think we spend a lot of time getting deeper into this. Unfortunately, the data is not that clean, because we get revenues from all streams , one number is being used for multiple patients. So , it becomes extremely challenging for us to do this analysis. However, we do it on a small sort of data set. But again, it's very challenging to figure it out. I think what is coming out is very clear that the frequency of visits to us from a patient is I think 1.3 or 1.4 . If I look at total number of patients that we have, number of times they come , average is just about

1.3, 1.4. Again, we have to then slice it further, people who are in a chronic disease, they obviously must be coming much more frequently than somebody who has this dengue event in three years just comes once, so you don't have that frequency. So, I think a lot of these efforts are being done. Unfortunately, I probably don't have anything which I can make a sort of firm statement as of now. But we are improving as we go along. As we mentioned, we have spent a lot of time and effort on our tech thing. We have now a data lake built in terms of all the data from various channels being put together in one place. So, maybe I think two, three, four quarters down the line we will have a clearer number.

Moderator: The next question is from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal: Just two questions. Firstly, related to what the earlier participant asked. Your own understanding of what really happened in nine months is obviously clear that mature markets are going lower, overall, the non-COVID is softer, my channel check also tells me that patients are actually doing a lot of self-medication and hence, the visits are also lower post-COVID era, I don't know how to read this. But to get back to anywhere between 10%-13% of non-COVID patient walk-ins, is there anything we could do additionally where we can get to that number or do we have to wait with the time and then further understand, do we grow at 15%, or not, what's the answer to that?

at?

Dr. Om P Manchanda: I think that's a great question. The answer to this is based on my sort of understanding is, one will have to widen the footprint, because this is a very asset-light business, the supply side is abundant, you will always have somebody just entering the business, whether it's a hospital side or pharma or even smaller players. So, technically, somebody having a very high market share in one market is probably a difficult thing to achieve. So, you need to widen the footprint. And that footprint will have to be done both organically and inorganically. I don't know, we'll have to wait for results for overall other companies and understand the industry trajectory. But my sense is the industry is going to face similar pressure elsewhere. So, one may see lot of inorganic opportunities coming up in the next couple of years. So, keeping that in mind, I think for us, West and South are two big gaps, and West, we fill it up, we are very aggressive on this with the launch of reference labs. So, answer number

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one to your question would be that we have to continuously keep on widening our geographical presence. The second thing is that I think we'll have to probably do is look at our business model slightly differently than what we have done in the past. It's very clear in our trends that patients or consumers are looking for a higher level of convenience, those days have gone, where people will come to a lab and stand in the queue and wait for 30-35 minutes for their turn to come and give blood sample. So, they are expecting collection to be done near them, which could be in the format of a collection center or a home collection, etc. See, in the past, labs were used not only from a supply side but also from a demand side of getting more walk-ins into the lab. That also added to the overhead structure. I think that model also will have to undergo a change. So, where we put more centralized labs and then put a lot of spokes in the format of collection centers. I think these are two things we'll have to do. In terms of pricing, etc., that's the market for us to decide, because you can't command prices the way you want them. So, we'll have to see how the competitive intensity settles down with time. But, from a volume perspective, these two things will have to be done. And third is in terms of clear differentiation is trust. In the last, at least one month I have traveled the markets and met a lot of doctors. To my mind, trust remains number one thing on their mind. So, this aggressive promotion in this business does not go down too well. My reading is that eventually a person who commands trust of medical fraternity will eventually last.

Rahul Agarwal: But my sense is Dr. Lal has always been the leader in all three factors and you've been doing that, you've been widening footprint, your brand value I think is the highest in the country, and in terms of convenience, I think you have the best app ratings are good, you are anyway investing for last three years in digital. But, incrementally what do we do? I think we're already doing this, right?

Dr. Om P Manchanda: But if you see performance amongst all the published data, which I have compared with the last four years, we are the best performer in any case. I know the numbers may be a little soft, but we are still number one in terms of performance.

Rahul Agarwal: Lastly, on Suburban, in terms of integration and opening of centers, we obviously need to have higher throughputs there. Where have we reached, and what's the next 12 months' plan, could you just elaborate, please?

Dr. Om P Manchanda: I think on Suburban, one setback that we've had is that this business was highly dependent on COVID, 50% of the turnover last year came from COVID. All of us knew that it is going to go down, but probably one didn't expect

that there would be

a sharp fall to the extent of 85%. The business had added some cost to it to serve the entire turnover. But, as the COVID business slipped away and one got really struggling in terms of managing these costs. So, that is one challenge we have faced. In terms of your question on transition, I think our team now is well in place, we have Shankha Banerjee who's our CEO. So, I think that the transition more or less is stabilized. As I mentioned to you, we have launched "Vidyavihar Lab," that's a big move. All our experiences of the past when we launched Rohini Lab in 2010 in Kolkata in 2018, and now we also launched in Bangalore and now Mumbai. We feel that this infrastructure of central lab will definitely add to this and I traveled Mumbai in the last one month, twice, thrice met many doctors. I think our overall visibility in the market, especially amongst medical fraternity has gone up, because Suburban was seen as outsourcing high-end test to other labs. Now, it is seen as people are going to outsource to this lab. I think that's very clearly communicated. And we'll continue to do that, we have done a lot of activities, we're doing that same thing in Pune as well. So, Vidyavihar is a big initiative we feel for Suburban. And we've also seen that the combined infrastructure of LPL and Suburban put together is very, very strong. I think 50 labs in the state of Maharashtra and 28 in Mumbai, including of course, there are a few HL Ms that we have in Mumbai. Our infrastructure is well in

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place. We just need to convert that into numbers. I'm pretty hopeful that we are well placed to take it on. And also, I think one must keep in mind that my experience is these numbers don't move quarterly that sharply, but over a period of two to three years, we will definitely see a difference in positioning.

Moderator: Next question is from the line of Prakash Kapadia from Anived Portfolio. Please go ahead.

Prakash Kapadia: Couple of questions from my end. If I were to look at growth drivers for FY24, is non-Delhi, NCR market and Suburban the key metrics which we have to evaluate for top-line growth?

Bharath Uppiliappan: So, Prakash, the way we are looking at the business for FY24 is to say that we will grow back in Delhi NCR strongly in the sense what I mean is related to what its recent performance has been. So, we'll bring it back to the original pre-COVID level growth. We are seeing some initial early signs of all the activation we have done in Delhi NCR, and hopefully touchwood in another three, four months' time, we'll get far more confidence to say, yes, it is really coming back on track. So, that's firing up combined with the rest of India firing back as usual. We're not saying, we are discarding one geography or focusing only on one geography. But one key task we have taken is to see the revival of Delhi NCR and we are seeing some early signs on that.

Dr. Om P Manchanda: The way I look at it, there are three clear buckets that we have in our business; one is the maintenance markets, then there are growth markets, then there are emerging markets. The entire belt of East, rest of North which consists of UP, Punjab, Haryana, Himachal, Rajasthan, they are really a big growth market for us. While we will revive Delhi NCR, but still my sense is NCR growth will not be in line with our overall company growth. We will push this up. But I would put that market as from a maintenance perspective. Growth would be East, UP, Bihar, Rajasthan, Punjab, Haryana and then there are emerging markets, I would put, South and West in that, where it will take definitely a year time for us to really put them back into a growth market, but right now, we will invest more in infrastructure in select pockets like Maharashtra and a couple of other places.

Prakash Kapadia: In larger cities, give

in that competitive intensity still remains high, is a strategy for a specialized test menus and focusing on certain high-end tests add to this growth?

Dr. Om P Manchanda: Yes, then there are two distinct segments. One segment is becoming direct-to-consumer, where this whole preventive, lifestyle, chronic illness, all that is falling in, because that's a segment which sometimes does not depend on prescription or if at all it depends on prescription is one prescription leading to multiple visits to lab. And then the other one is, is a very institutional hospital pick-up business, which is high-end business. I get a sense that is not going to be direct-to-consumer. I think one will have to work with the medical fraternity and also maybe companion diagnostics work with pharma companies as well to see how we look at the high-end portfolios, which

are very, very specialized like I just mentioned, electron microscopy, or say, for example, cytogenetics, or say, for example, gene sequences, etc., So, I think those are the tests which will have to be promoted through high-end test that we talked about. We have launched these things like Genevolve, and we've launched L-CoRD. So, we are building a center of excellence around some of these tests, as you have in pharma companies, various verticals, same way we are trying to build a category or a center of excellence around this.

Prakash Kapadia: You mentioned about the reference lab and given that infrastructure is ready in Maharashtra and Mumbai, so, for Suburban, what's going to drive this growth,

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because if we look at existing customers, you're obviously have at a larger COVID base in Suburban, now, will it be the existing customers, will it be new customer addition, will it be partially some of these high-end test which Suburban can do given that now the reference lab has come, so, if you could give us some insights on Suburban plans that will be helpful?

Dr. Om P Manchanda: Maybe I can ask Shankha to address this but let me put my thoughts. I clearly see three areas. One is widening footprint here. If I study Suburban business, it's mainly come from Andheri, Borivali, western track side areas. It does not have very strong presence in South Mumbai. LPL system has a lab in Dadar. So, if we can actually do one lab and testing for multiple brands, in this case, two brands, you have easily widened your footprint. Similarly, I'm seeing that in other markets like Pune as well. So, clearly, bringing some synergies between the infrastructure of LPL and Suburban, that gives us both cost side synergy as well as revenue side, because it gives you a widening sort of presence in the market, that's number one. Number two is Suburban in earlier avatar was outsourcing a lot of high-end tests to other large companies. Now, the whole thing is flipped upside down that now people start outsourcing it to Suburban. And Suburban is positioned as a central Lab, which actually is doing the entire test menu. It is well supported by NRL in Delhi, but it is giving a report on Suburban letterhead. So, that's the way it is. It is a (CAP) College of American Pathologists accredited lab now. So, it is positioned as a ultimate lab, which is doing every test. We don't say no to any test anymore. That's the second part of our strategy. Third I think is Suburban is very well positioned on the corporate health checkup, which we are here to evolve the strategy around this, but I think that's a big strength Suburban as a brand has in the city of Mumbai, that it is seen as a comprehensive health checkups. Let me elaborate this a little more. Health checkups, the way it's been promoted by all pathology companies, it is only path tests, right? Health checkup also has some element of imaging inbuilt into this, let's say, stress test, TMT, you want to do x-ray, you want to do upper abdominal,

lower

abdominal, ultrasound, then only it becomes a complete health checkup, by the way, which just by doing 30 tests and a pathology is still not a very comprehensive health checkup. To my mind, real health check up would be the patient undergoes some imaging tests as well, like say, for example, as I mentioned, the x-ray of chest, ultrasound and things like that. I think Suburban has that infrastructure in Mumbai, which we will figure out how do we leverage that. And that's a unique strength this company has. Shankha, do you want to add something?

Shankha Banerjee: No, I think, Om kind of said the whole detail. I think the key here would be, we will do B2C, B2B both as well as existing geography and new geography. So, if you think about it, it's basically a 2x2. With the S DRL launch in place, we are going to be on all four, actions will be there, but I think results in some of these quadrants some will be quicker, some would be slightly longer wavelength, but yes, actions will be on all four to drive growth.

Prakash Kapadia: So, the customer segment, Shankha, will be slightly easier to scale I guess, because the higher end or the comprehensive packages have pretty big hospitals, also in Mumbai, which are offering some of these, so, I would guess that's the right timeline to think of scale for that business, right?

Shankha Banerjee: Like I said, B2C, B2B as well as new and existing geography. So, now B2C, whether it's direct consumer as you're suggesting, so, obviously, that also plays into how we want to deploy our money. So, yes, it will be a mix of all and you will see various intensities in all the quadrants, but like I said, some of them will start delivering revenues immediately, and some of it will after some time.

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Dr. Om P Manchanda: Maybe I want to add one more point on the Suburban. Interestingly, we are also tweaking our business model a little bit here. Suburban in earlier avatar was very

dependent on its own infrastructure ; so, it had its own PSC own collection center managed by this one. Now I'm seeing is in the last one year, I think our team has done a tremendous job of building franchisee infrastructure which brings in a lot of entrepreneurial energy in the system, where some of these owners whom I met in the last one month, they are actually very active on just doing very local marketing and doing some health checkups and camps, etc. , That actually brings you to the point that the way Dr. Lal PathLabs is built on a franchisee infrastructure, because that entrepreneurial energy really takes it up. So , I'm seeing those early signs in Suburban as well , I think that's another change we want to bring in there.

Moderator: Next question is from the line of Prakash Agarwal from Axis Capital . Please go ahead.

Prakash Agarwal : My question is on pricing , you mentioned that pricing will be largely static. But the competition has lower pricing . You mentioned growth drivers being widening footprint and doing more test services, etc. , Is there a thought on any pricing that we can play as well?

Dr. Om P Manchanda: Yes, I think that's a great question. My sense is that at the current level of growth in the industry, one may not have the option to absorb these costs but to pass them on to the market. I also foresee a rebalancing of pricing, because in the past, a lot of high-end tests were actually , if I may use the word , 'subsidized' by routine tests , some bit of that rationalization will take place. My personal view is that industry has reached that point . Especially for smaller players going to be very tough. Either they compromise with the margins or they increase the prices. And that's why I mentioned that it'll be very interesting to see how price value equation moves in next two, three years.

Prakash Agarwal: Will we take pricing as our strategy?

Dr. Om

P Manchanda: Our strategy would be , I think , we will just wait and watch how the growth trajectory happens , but we will be open to considering that as well as we go along.

Prakash Agarwal: Second question for Ved is on other expenses . There seems to be a lot of cost optimization , gone down QoQ, YoY despite having so many activities which was mentioned in the opening remarks . So, is there a one-off here or these are the new levels of costs ?

Ved Prakash Goel : Prakash, last year if you remember, we had some one-off transaction cost which was the last year same quarter , so that is one . Having said that, there are two things happening . While we are investing more in IT, digital and marketing, at the same time, we are optimizing costs in some of these areas, where we can leverage our existing infrastructure. As Dr. Om was mentioning that more franchisees have fixed overhead by opening more and more labs. So, there are other levers which we are taking to optimize cost and balancing , somewhere we are increasing , somewhere we are optimizing .

Prakash Agarwal: That would also be the answer for QoQ decline, because there was not extra-transaction cost, no?

Ved Prakash Goel : Yes. But if you see, again, the quarter is not representative, because, again, there is some ups and downs on some margins .

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Dr. Om P Manchanda: We've spent a lot of time on margins , just look at on YTD basis is ranging between 25% to 26% before RSU and CSR. And if I go back year, '19, year '18 and '17 pre-COVID times , I think IndAS adjustments also to do all that , it used to range between 25%, 26%, we are somewhere there only this time. In the last two years, FY'21, FY'22, because of that operating leverage, our margins just went up. So, broadly that's where we are. Now, I think the key question which was asked earlier is that "Can you sustain this going forward without the price increase?" To me, we'll have to probably wait and watch, but we will definitely be open to taking price increase if required.

Prakash Agarwal: On the South market, you mentioned in the past it would be largely organic . In this call, I heard that you would look at both the options organic and inorganic and would that be on the back of the cash of Rs.400-plus crore or what is it?

Dr. Om P Manchanda: No, that was actually a general comment I was making in terms of growth opportunity. So, because of the fragmented nature of the market, and the global experience that we have , inorganic opportunities do come up, especially when the industry undergoes margin pressure. Since any opportunity that comes in the South for inorganic will definitely be welcome , but right now our organic efforts are on . But, obviously, we don't want to pay a very high valuation to these places just because you're desperate to acquire. So , that's why we're building up now organic platform ourselves. If something attractive comes on our way, we'll definitely be open to doing

that here. But this inorganic , organic is a general comment, that opportunity will always be there , that's how these businesses globally are also built that way.

Moderator: The next question is from the line of Cyndrella Carva lho from JM Financial . Please go ahead.

Cyndrella Carvalho: Ved sir, i s there any one -off expenses related to Vidya vihar Lab we have opened , and if so, can you quantify ?

Ved Prakash Goel : There is no material one -off in this on account of Suburban or this lab. Having said that, because this is a big reference lab , we have some expenses which is going to come for some time on account of this lab.

Dr. Om P Manchanda: So, Suburban P&L, broadly speaking , is undergoing one mega change which did not exist earlier, which is that our infrastructure is becoming more franchised. And since we have to do revenue share, that line item is growing . Is it correct , Shank ha? Sha

nkha Banerjee: Yes.

Dr. Om P Manchanda: So, that line item in the short term will have an adverse impact , but as the growth picks up with time, which we are very confident it will do , will definitely get normalized over a period of time . I think that's one add -on cost. S uburban also in relative term has a higher manpower cost compared to let's say LPL comparison when we do. Given the low turnover base , it's justifiable . But it has an infrastructure, which is slightly different from a PathLabs infrastructure. As I mentioned to you , it has x -ray, ultrasound, and a manning model, which is suited for a corporate health checkup. So, we want to aggressively drive that part of the business as well so that we are able to reduce as a percentage of o ur manpower cost. I think these are two m ajor costs that exist on that P&L. Consumable -wise, I think it's very similar to what we have in LPL. Is it correct?

Shankha Banerjee: Yes.

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Dr. Om P Manchanda: I think these two costs , one is manpower , other is franchising revenue share are the two costs w hich are actually comparatively higher than LPL P&L, but those are required to actually drive growth. So , 99.9% , our efforts ar e on driving top line .

Ved Prakash Goel : Rental is also high .

Dr. Om P Manchanda: Rental probably will come down because as we have started this Vidya vihar Lab , some of the testing facilities will get consolidated.

Cyndrella Carvalho: If we look at the overall inorganic opportunities coming to our way, can you help us understand the valuation parameters , are they coming back to the pre-COVID level or still you see some valuations still being higher, or the expectations of COVID still continuing -?

Dr. Om P Manchanda: Expectation s of COVID will continue. At least FY '23 to be out of the equation and then we will see what happens in FY '24.

Cyndrella Carvalho: Do you have any understanding on cluster base in the emerging market s and the growth?

Moderator: Sorry to interrupt . Your voice was breaking . Next question is from the line of Sh yam Srinivasan from Goldman Sachs . Please go ahead.

Shyam Srinivasan : Just the first one on the RPP. I heard a number of 754. I'm just doing non -COVID revenue of 480 divided by six non -COVID patients, it seems to be 800. So , is there some clarity around what is 754 if you can do it ? Also, question two is on , if we could get the total patients and total samples also will be helpful .

Ved Prakash Goel : Shyam, non -COVID. RPP is 744, while 754 is in total including COVID since y ou are mentioning it.

Shyam Srinivasan: I'm just trying to look for, like what is the trend on non -COVID? It seems to be 478 by six is 800. So , what's happening there? Just an extension of the previous participant question on pricing in non-COVID revenues . I think you mentioned five percentage points up for 754 , but I wanted to see whether the trend is different for non-COVID revenue ?

Ved Prakash Goel : If you see 6.4 million non-COVID patient s for this quarter with a RPP of 744. And if we see volume growth is 3.6 on non -COVID basis, which is a difference of (+5%) which is due to m ix and some channel that makes primarily because of Swasthfit , some high -end test has increased , and some bit of Suburban also, because Suburban is also having higher realization per patient.

Dr. Om P Manchanda: Shyam, first g et the numbers , right. So, you have Rs.478 crore of non -COVID turnover this quarter, right? And then you have 6.4 million patients of non -COVID.

Shyam Srinivasan: Sorry to interrupt you , Om, but the presentation say s 6 million , so which is where I calculated the number , I'm saying Slide #6.

Ved Prakash Goel : It is rounded off . Sorry.

Dr. Om P Manchanda: Anyway, the number is 6.4 which is rounded off to 6, 0.4 is a decimal point. Let's just

expand this question. So, 478 divided by 6.4 gives us 74.4. But it is still a 5.3% revenue per patient gone up, which is slightly unusual, we have not seen this kind of

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jump in the past. So, let's just try and understand this better. So, the one factor is Suburban realization is higher than LPL realization. So, when you combine, this has pushed this up, that's number one. Number two is Swasthfit, which has been a reason consistently for the last four, five quarters because the contribution from Swasthfit is going up. Third is higher end test to some extent, the contribution is also on the rise, which has a higher realization per patient. And finally, last year base has a very high dengue component, which is actually a very low realization number, because a lot of people once somebody is a dengue patient, you follow it up a lot of platelet tests, which are very, very low-value tests. That is also one of the reasons why you're seeing lower volume growth because last year has a very high volume due to dengue. So, that explains a little bit of that. But, going forward, we do believe as the Swasthfit goes up, as the high-end test component goes up, we will see a slight firmness on this number as we go along.

Moderator: Next question is from the line of Sameer Baisiwala from Morgan Stanley. Please go ahead.

Sameer Baisiwala: Om, how do you differentiate between your four big labs national and regional just in terms of capacity, in terms of complexity of the test, some thoughts would be great.

Dr. Om P Manchanda: There are many ways people look at reference labs. One way to look at is test menu that how many you were doing before; how many you are doing now. So, let's say, our ability to do the number of tests in Mumbai, we have seen going up from 700 to 2,800. So, that's one way to look at it. Another way to look at it is that some uniqueness that you have in the lab like I talked about this BSL -III facility, which is a very unique thing, is not in the region, electron microscope that we have added, it becomes a big news in the town that this sort of equipment has come, that's another way to look at it. But I think, from an economics perspective, one best way to look at is the following: If I get let's say, my hub is Lucknow, if I get 100 samples every day, how many actually remain within Lucknow, and my dependence on NRL is let's say 10%. As I keep reducing that number of dependencies on NRL, which is Delhi, that means I'm creating an independent ecosystem and it can be managed within that region itself. So, our way of looking at it is that keep on reducing the dependence on big lab in Delhi and make every cluster a very independent thing and then you are able to run it like a P&L for that region, is what we want to achieve. It has two or three advantages. One is the medical fraternity likes it, because then they feel very close to the doctor sitting in the lab. And many times they pick up the phone and talk to the doctor as well. That becomes a little bit of a comfort factor for especially a high-end doctor that lab is within my reach. Second is turnaround time actually gets improved, because the sample doesn't have to hop at two different locations. So, it just hits Lucknow and gets tested. And that gives me the ability to actually go deeper into Tier-3, Tier-4 towns, let's say for example, if you're familiar with the UP, you can think of towns like Shahjahanpur, Sultanpur, Mirzapur, which otherwise sitting in Delhi, you will not be able to think about those towns as you will be able to think once you're sitting in Lucknow. So, that's the way we look at central lab or reference lab format. I would actually call this as a hub lab more than even how do I make sure that all my samples get tested there. So, we do some kind of analysis. I think there are some tests, turnaround time runs in days as well, reports are not given next

day. Those

departments like cytogenetics, HLA typing, or genomics or fish testing, common people like you and me won't even understand some of these tests. But they are actually just 5% to 10% of the portfolio. It actually doesn't even make sense to do in the region, because human component in these tests is very, very high. And at times, doctors want the report to grade by one particular doctor. So, for them, the lab is not important as important is who's signing that report.

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Sameer Baisiwala: When you're talking about a volume growth three-year, four-year CAGR, I also just quickly take your channel growth over a three-year CAGR 6%-20%, and 14% for your labs, PSCs and PUPs. So, looks like the volume growth is massively tracking behind the distribution channel that you are creating. So, just your thoughts on that?

Dr. Om P Manchanda: We spent a lot of time analyzing why our volume growth is lower than value this time. I think analysis is throwing up, last year had a very high level of dengue incidence.

Dengue actually brings a lot of platelets. And somehow this volume, what we're talking about are not real patients, they're all patient visits. So, one dengue patient comes within a span of at least 10, or 12 days, some six, or seven times.

Sameer Baisiwala: Om, this is three-year CAGR; 2019-2022. So, I thought I'll just let you know.

Dr. Om P Manchanda: Yes, I think that's very clear. I think that somebody asked this first question that definitely volume growth is a couple of percentage points softer than what it used to be earlier.

Sameer Baisiwala: No, relative to the growth in your distribution channels, I'm trying to ask.

Bharath Uppiliappan: That is the additional Suburban which has caused a sharp blip.

Dr. Om P Manchanda: I don't think I understood. So, you are saying our channel growth rate is higher than the volume growth?

Sameer Baisiwala: Exactly.

Dr. Om P Manchanda: It has grown faster, but our volume has not kept pace with that.

Sameer Baisiwala: No, way behind, Om, your PSCs have grown at 20% CAGR, there is no Suburban in my number, 20% CAGR of PSC growth versus some mid-single digit volume growth and likewise PUP?

Bharath Uppiliappan: Sameer, as you go down the pop strata, we have been talking of going to Tier-2, Tier-3, Tier-4, throughputs take a lot of time to build through. I think so we are putting the infrastructure in place and over a period of time we will see this even out. So, on the PSC side, that is a story. On the lab side, on the organic business, if you remove Suburban extra addition, I think our lab growth has been broadly in line if not lower than that of the volume growth.

Dr. Om P Manchanda: Sameer, one of the reasons which we will have to probably validate this, see, what we are seeing I think the numbers are not in public domain. Our contribution from collection centers has sharply moved up in the last couple of years. And that primary shift has happened from a lab. So, let's take one lab, which is doing 100 samples per day, now is shifting to at least 10 PSCs. So, the same, let's say, 100, earlier was coming from one lab, now, it actually comes from 10 PSCs. You understand what I mean? So, you may have added more infra, but your number of patients have remained the same. I am just thinking as we're talking, I'll have to probably go back and do this analysis. But that's one thought that comes to my mind, maybe creating this issue.

Sameer Baisiwala: And this thought is confirmed by the significant drop in the footfalls in the labs?

Dr. Om P Manchanda: Yes, I think COVID has actually done this very clearly. And I'm seeing that, in general, patients do not really want to come to overcrowded places anymore if they have the

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option to go to the nearest collection center. The comfort is very high with

th Dr. Lal

PathLabs because they know it's the same infra. Because this can only happen to a lab which you trust. So, let's say I stay in Gurgaon and I live on Sona Road, lab is a five kilometer away, I would rather say instead of going to the lab, can I actually either call that person home or can I go to the nearest collection center. So, that tendency is very clear in the consumer behavior now.

Sameer Baisiwala: Om, have you tested the pricing power in B2C channel?

Dr. Om P Manchanda: I think honestly, we've also been little cautious of ourselves given the competitive intensity. But as we go along, we'll have to really test it out. Because so far, we were able to maintain our margins within this range of 25% - 26%. We didn't want to push it up further. We also became cautious and also there was a period of this COVID time and very uncertain times. And another problem is B2C is the following there are certain pockets, B2C is not uniformly spread business across the country. So, you don't want to create lopsidedness in your system that you load one market because of a higher B2C components and not do it another. But in a calibrated manner, we are definitely open to now looking at price rationalization as we go along. That will include even not only B2C but also B2B as well.

Moderator: Next question is from the line of Aashita Jain from Nuvama Group. Please go ahead.

Aashita Jain: Sir, could you please talk a bit about your current infrastructure in South India market and the expansion plans maybe in terms of your labs and collection centers?

Bharath Uppiliappan: So, in South of India, you must look at infrastructure from a three-year perspective.

In the South, we started off with about 11, 12 labs, now we're running at 23 labs. And second is on the collection centers. We hardly had any collection centers, we have a lot of company centers, and a lot of pickup business. Today, we run about 250-plus collection centers and this network will obviously build up into the future. Even on the pickup side, a lot of hospitals have been added, so that traction is going on. But we still have a very less presence in South compared to what we have in West or East, North east obviously we are ahead. So, we made significant progress, but I

think there is still miles and miles to go further.

Dr. Om P Manchanda: Yes, we have now labs in place like Tirunelveli, Madurai, Pondicherry, Kakinada and Kurnool. So, all these came up in the last one, one and a half years. We have some of these towns which I'm pretty familiar with these towns because I worked as ASM many years back. But we have now started having labs in these places.

Aashita Jain: Any expansion plans for which you spend for the next three years?

Bharath Uppiliappan: We put a central lab with a good amount of capacity. So, the immediate task is to get the client base up and get the sample base up into these towns and we'll keep expanding labs, there is no particular number I want to give on this call. But expansion is constantly being evaluated across the country.

Dr. Om P Manchanda: I think let me put it this way. Let's say South as a region, will we look at that as a direct -to-consumer market or more as a high-end institutional business? I think clearly answer is, we want to look at this as a high-end business market, which is more reaching out to hospitals, smaller labs, so which clearly brings us to the point that we may not build a very large front-end infrastructure, because that may put a lot of overheads into the P&L because there are a lot of local competition, local labs entrenched for many, many years, so, we don't want to be up against that. We by the way enjoy good equity amongst medical fraternity. So, for us, the model

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essentially would be not to proliferate as much front-end infrastructure as we invest behind high-end test, that

that would be more back-end central lab kind of infra is what I would think would work.

Aashita Jain: Secondly, on Swasthfit, when I look at some of the packages and your competitors are offering them and attractively priced it, so in your view, what will be your strategy to compete and grow this particular vertical, Swasthfit basically?

Bharath Uppiliappan: Even before we began the Swasthfit journey, there were competitors selling packages cheaper than us. And even after the competitors, newer players come in with cheaper packages, our Swasthfit contribution only moved up. So, I think the issue is not about only price in this category, there are a lot of other variables of access, trust, timely delivery, etc., which play into it. So, pricing is a consideration, but not the only consideration. So, we have the plan to grow the Swasthfit portfolio, the bundled test portfolio, and that continues. That is the reason why this quarter also we got a contribution of 19% or so on the Swasthfit portfolio. Just to repeat, price is not the only lever, there are multiple other levers. So, it's not a commoditized product the way one typically tends to imagine it to be.

Moderator: The next question is from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: You were mentioning Tier-2, and Tier-3 towns. So, it's a deliberate strategy and you're following. Is it because of your slower growth into Tier-1 cities? And what would be the contribution from metros and Tier-1 cities for us today approximately?

Bharath Uppiliappan: You're saying, is there a growth rate difference between metro cities and lower cities, right?

Sayantan Maji: Yes, for example, as we talked about Bangalore, then Chennai, Hyderabad, so, should we not focus on these larger markets in Tier-1 cities and then venture into Tier-2, Tier-3 towns or is it because that you want to venture into those towns because of lower competition?

Bharath Uppiliappan: No-no, there are two ways to look at it. One is that in the Northern, eastern part of the country, it is a go down the pop strata game. In the southern market, we want to put reference lab and also go to key medical hubs, where there is volume and generate equity base over there. So, you must look at our strategy slightly differently in an emerging market to a stronghold market to a dominant market. So, that's way I would call it out.

Sayantan Maji: In East India, do we see our base maturing or do we still see mid-teens kind of a growth in that geography?

Bharath Uppiliappan: So, East India is definitely not a mature market yet, there is still miles to go. All the markets of East India, be it Bihar, Jharkhand, Odisha or Northeast, all remains potential. There is a lot of work to be done within large cities, there is also work to be done in going down the pop strata with access. So, I think the East India game is still a lot of ways to go.

Sayantan Maji: What could be the Ind-AS benefit for this quarter? And what is the percentage of revenue from both sample collections in the B2C segment for us?

Bharath Uppiliappan: The home sample number is about 9%-10%.

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Ved Prakash Goel : IndAS is even -steeper, because it's not new , it's also in the base , so there is no difference.

Sayantana Maji: Would it be around 2% of base approximately?

Ved Prakash Goel : Maybe more 2.5%.

Moderator: Next question is from the line of Tushar Manudhane from Motilal Oswal. Please go ahead.

Tushar Manudhane: I was referring to the profitability of Suburban . Currently, this is at 7% and the kind of restructuring which we're doing , and in addition to that, we have Vidya Vihar Lab as well. So, to what extent this margin can improve say over next two to four quarters ?

Ved Prakash Goel : Tushar, as I said, this quarter is not representative . So, on a yearly basis,

s, definitely ,

it is going to improve from here . But again, the focus right now to invest in the business and drive top line. So , we are not cutting costs right now , but it will be improving from here.

Dr. Om P Manchanda: We want to achieve one thing which is that model which was driven by own infra , moving it to franchisee is one sort of a headwind which is going to come on the P&L because we have to do a revenue share, right , and we believe that is important for us to drive growth. So, I think that is some bit of a phase manner we'll have to see. I think FY'24 focus is just to see if we can simulate the top line growth which we're seeing some green shoots and I want to make sure that next 12 -15 months we just drive for the top line. In the process , given our experience because of operating leverage in this business, I think margins should improve directionally

Tushar Manudhane: Currently, how would you be owned and franchisee , just to clarify ? And then to what extent do you want to take it as a part of the strategy ?

Shankha Banerjee : So, currently maybe a 1:2 ratio for own versus franchisee that we have achieved as of now in terms of a number of infrastructure outlets, revenue . It could be even higher for own versus franchisee; I'm just talking about infra numbers . And going forward that number could go as high as 1:4, 1:5 .

Tushar Manudhane: Lastly, on the way of expanding to Tier-2 cities , and you're looking at this franchising model , so, broadly the other expenses, at what rate can be considered to be growing for the next couple of years ?

Shankha Banerjee: For overall business , you're asking or Suburban ?

Dr. Om P Manchanda: Yes, overall business.

Ved Prakash Goel : I missed that question.

Dr. Om P Manchanda: On overheads, how do you see growth growing over a period of time ?

Ved Prakash Goel : Tushar, I think overheads, obviously there is inflation, which is happening everywhere , especially new talent hire.

Dr. Om P Manchanda: I would rephrase this . I think our EBIT DA margins is between 25 % , 26% range, right , and there is a certain growth projection for next year. Our first attempt would be how

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do we remain within this range . I think clearly industry has reached a stage where you not only have to manage your top line, but you have to manage all three lines, middle line, bottom line, top line . So, we'll have to really work very hard in managing the entire P&L shape. So, I think somebody asked an earlier question, "Would you be looking at price increase?" Probably , the answer is yes if we are not able to observe this cost . "Would we be looking at reorganizing our infra in terms of increasing productivity ?" Again, we have done that quite a bit in the past, we'll continue to do that as well. So , that's the way we look at our costs. A lot of automation has taken place in the last two years, maybe there are certain process changes we'll have to do where manual component has to be pulled out, we'll have to look at automation. So , this effort we're just staying in this race will continue. We'll have to see how we move quarter -on-quarter basis.

Moderator: Next question is from the line of Tanmay Gandhi from Investec . Please go ahead.

Tanmay Gandhi : Sir, while we are talking about the competition, we said that routine tests are actually subsidizing the high-end test side, and at that point of time, we said that we are watching out the pricing situation and might take some price cuts, right , and while talking about the cost, we said that we will probably pass on the cost inflation, right. So, how do we reconcile that?

Dr. Om P Manchanda: No, I think the point that I was making is that there are a lot of these higher -end tests that have very high manual components inbuilt in that , while routine tests are very machine -driven tests, let's say biotechnology tests,

etc. , there, the productivity is very

high. But let's say the histopathology department , where we read these biopsies,

manual components and it is also read by top notch doctors, their cost is very high. So, traditionally, what has been happening is that, since this biopsy business or histopathology business was very small, one didn't bother too much about margins, but it was like a real brand positioning, because it really mattered from an image perspective. But now these businesses are becoming really large, so, one has to look at the entire price structure keeping that in mind. And secondly, on the routine test, as you all know, competitive intensity is very high, we can't keep on just loading that part of the business to look at. So, I think there's a bit of rationalization effort, which is what we are trying to do.

Tanmay Gandhi: Any sense about how would our realization be, in that case?

Dr. Om P Manchanda: I don't think if you're trying to build your model, there's a material change as of now, you should worry about. I think when we go along, we'll probably share with you a little more. I don't think we should tweak our model saying that there is some big change that's going to happen. It's going to be a very, very measured sort of way of rationalizing.

Tanmay Gandhi: Our non-COVID RPP is Rs.744 and total RPP is 754. But if I remember correctly, in the last few quarters, we mentioned that COVID realizations are dilutive. So, is there any change in pricing or other change?

Ved Prakash Goel: Tanmay, it is because of mix, which is a higher test and Swasthfit and Suburban also. So, all put together is where the difference is.

Tanmay Gandhi: No sir, I'm actually comparing the total RPP at the company level and non-COVID RPP. If there is any difference, that means that COVID realization is accretive, right?

Bharath Uppiliappan: No-no, it is a test mix and the geography mix change.

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Dr. Om P Manchanda: Total RPP is 754, non-COVID is 744. So, what has led to this higher Rs.10?

Shankha Banerjee: Because of the allied component, Om, in that.

Dr. Om P Manchanda: So, actually COVID has two buckets. One is RT-PCR, the other is allied testing. So, RT-PCR is dilutive and the other one is taking it up, right, that's what is making difference but Rs.10 is a little high?

Shankha Banerjee: Because now the RT-PCR component of the COVID has gone down.

Bharath Uppiliappan: Significantly down.

Shankha Banerjee: It's more of the allied.

Dr. Om P Manchanda: So, that is what has led to this, yes.

Bharath Uppiliappan: COVID allied testing mix.

Tanmay Gandhi: For the industry in general, right, earlier, I think as per the industry reports, the general understanding was that diagnostics is a (+12%) growth for the industry right.

But given the current competition scenario and the quick scale up which these competitors have seen and the way they are expanding their testing infrastructure, their revenue base, do you think that the growth trajectory could change?

Dr. Om P Manchanda: Definitely, no doubt. Let's look at five years back and now, competitive intensity amongst organized players have gone up sharply, because today you have brands which are being advertised heavily which are reaching out, they're building chain labs. And obviously, if they are there in the market, they are obviously gaining shares, etc., at the cost of earlier players. Now, I think one interesting thing one has to see is that a lot of deep discounting has taken place in the last year, year and a half, and purchase frequency is not that high in this business. So, one will have to actually see the thickness of this customer base over a period of time. I think many of these players have been in the market for maybe two to three years. We'll have to see how they actually stabilize in the year coming which is

FY '24, it will be very important to see that. We are also seeing gradual improvement in our business as well, it's not that we are not seeing that, right.

Bharath Uppiliappan: So, like we mentioned, Om, for the organic business, the H1 growth rate was 9.4 CAGR, it has moved to 10.8 in Q3.

Dr. Om P Manchanda: On a longer-term. There is a bit of recovery. I think the good news is that so much of action, which is taking place, market is going to grow in terms of business; Tier-2, Tier-3 towns growth rate would be there. So, we'll have to just see. While we are defending in one place, we are also making sure that we grow in other place, I think it's a combination of both.

Tanmay Gandhi: But when we say that the lower growth is also attributable to competitive intensity, but even for our Swasthfit growing very fast, right, earlier it used to be 15%, and currently it is?

Dr. Om P Manchanda: But it's not all incremental, it's also cannibalized our individual routine test, right, so, a lot of shift is happening. While the contribution of Swasthfit may have gone up, it sounds like 20% extra growth has come, it's not come as incremental, because a lot

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of these individual tests are now being clubbed into , people are availing bundle d packages which is much greater value for money for patients.

Tanmay Gandhi: Within this 20% Swasthfit contribution , how much would be the wellness piece , any broader number?

Bharath Uppiliappan : No, there is no way we can differentiate where someone boards a Swasthfit with fever or had chronic diabetes or came in for annual health checkup , there is no way of determining that instance in our system capability currently. So , I guess we are unable to differentiate and this is what we have said in the past as well .

Dr. Om P Manchanda: Yes, it's very difficult to tag that this turnover is due to wellness or this turnover is because even chronic patients are also upgrading themselves to bundle d packages.

Moderator: The next question is from line of Nitin Agarwal from DAM Capital Advisors . Please go ahead.

Nitin Agarwal : On the bundle d packages, two questions. One is a), proportion of Swasthfit , is it kind of uniform across various markets or this package is more skewed in certain markets ?

Bharath Uppiliappan : The proportion varies, but you must look at a proportion not only at a geography level, but a channel level of geography , because each geography also has a different channel mix and Swasthfit is only through our B2C channel or a collection center channel. So , if you look at within the channel , within the geography perspective numbers, yes, there are still wide variations across the country. And there is one insight we're using to get geographies to get to the same level as the national average, or within well -performing geographies, there are centers which are not doing as well to get those up.

Nitin Agarwal: In these geographies which you are doing well would be typically traditional geographies of North or the newer geographies where the proportions are higher ?

Bharath Uppiliappan : So, it's actually city specific . So, it's got a lot of detailing around that. So, I can't make a broad -brush statement, but North is definitely a much stronger Swasthfit market for us given the distribution reach we have and the consumer awareness about it also.

Nitin Agarwal: Secondly, from a business perspective, are these bundle d packages equivalent from EBITDA profile perspective for the firm or are they dilutive, more accretive?

Bharath Uppiliappan : RPP is significantly higher. The net of servicing costs, there should be a higher EBITDA -accretive profile.

Nitin Agarwal: Just trying to think through now on the comment, because as a proportion this has grown much faster for us over the last few quarters , and EBITDA margins have kind of

stayed around there, or maybe somewhat under pressure , the usual acute testing business, the profitability would have been under more pressure than higher pressure than the overall business , is that a fair way to look at it ?

Bharath Uppiliappan : So, there has been a lot of other costs inflation, and that is one thing we haven't taken a price increase, because we have been able to manage higher realization, offset some other portions of the business through Swasthfit and a portfolio management approach.

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Nitin Agarwal: Secondly, in terms of the volume s slowdown you've had this quarter or in the last couple of quarters, it is again broad -based across various geographical clusters or there are particular geographies where the relative volumes slowdowns are more pronounced ?

Dr. Om P Manchanda: I think Delhi NCR is a little more , because in the base, we had very high incidence of dengue last year. So , in relative terms, other geographies are better than Delhi NCR because of a very high base of dengue last year.

Moderator: Ladies and gentlemen , that was the last question for the day. I now hand the conference over to the management for closing comments.

Ved Prakash Goel : Thank you, everyone, for being with us on the call today. I hope we are able to address your queries. If you have any more questions, please feel free to reach out to our Investor Relations team, CDR India , or me , we'll be happy to clarify our thoughts. Thank you once again . I would now request the moderator to close the call.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

Call: May 2023

May 16 , 2023

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Conference Call on Financial Results for Q 4 & FY23

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q4 & FY23 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
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Dr. Lal PathLabs
Q4 & FY23 Earnings Conference Call Transcript
May 11 , 2023

Call Duration ■ 1 hour 06 minutes
Management Speakers ■ (Hony) Brig. Dr. Arvind Lal – Executive Chairman
■ Dr. Om Prakash Manchanda - Managing Director
■ Mr. Bharath Uppiliappan - Chief Executive Officer
■ Mr. Ved Prakash Goel – Group Chief Financial Officer
■ Mr. Shankha Banerjee – CEO Suburban and other Group Companies
Participants who asked questions ■ Rahul Agarwal – InCred Capital
■ Prakash Kapadia - Anived Portfolio Managers
■ Bino Pathiparampil - Elara Capital
■ Lavanya Tottala - UBS Securities (India)
■ Prakash Agarwal - Axis Capital
■ Aneesh Deora - Nomura Financial Advisory and Securities
■ Nitin Agarwal - DAM Capital Advisors
■ Cyndrella Carvalho - JM Financial Institutional Securities
■ Tanmay Gandhi - Investec Capital Services
■ Yogesh Tiwary - Arihant Capital Markets
■ Praveen Kumar - Acuitas Capital Advisors
■ Zaid Munshi - Concept Investwell

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Moderator: Ladies and gentlemen, good day, and welcome to Dr. Lal PathLabs Q4 FY23 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Siddharth Rangnekar of CDR India. Thank you. And over to you, sir.

Siddharth Rangnekar: Good evening, everyone, and welcome to Dr. Lal PathLabs Q4 & FY23 Earnings Conference Call.

Today, we are joined by senior members of the Management Team, including (Hony) Brig. Dr. Arvind Lal – Executive Chairman, Dr. Om Prakash Manchanda – Managing Director; Mr. Bharath Uppiliappan – CEO; Mr. Ved Prakash Goel – Group CFO; and Mr. Shankha Banerjee – CEO, Suburban and Other Group Companies.

I would like to point out a standard disclaimer here. Some of the statements made on today's call could be forward-looking in nature and actual results could vary from these forward-looking statements. A detailed disclosure in this regard is available in the 'Results Presentation,' which is available on stock exchange websites.

I would now like to invite Dr. Lal to share his perspectives. Thank you. And over to you, sir.

Dr. Arvind Lal: Thank you, Siddharth. A very good evening and a warm welcome to everyone present on this call. We are here to discuss Dr. Lal PathLabs Q4 FY23 Earnings and I would like to take you all through the "Key Developments and Updates" during the period under review.

India requires quality diagnostics of scale. Over the years, we have tracked the gradual evolution of the way doctors and patients manage disease and health. Accurate diagnostics is increasingly contributing throughout the treatment process, including health management, disease detection, prognosis, diagnosis, treatment planning and post-treatment monitoring. We therefore continue to invest in creating an efficient technology-backed network that can align with the growth in sample volumes.

To start with, I'm delighted to share the Dr. Lal Path Labs has clocked 15.5% growth in the non-COVID revenue in FY 23. We have proudly served about 27 million patients in FY23. This has come on the back of a sharp focus on core operations across the country including Tier-2 and Tier-3 cities and investments in digital infrastructure.

We have continued to make progress across our three strategic pillars that is geographical expansion, creating a unique and fresh-size portfolio and augmenting the technology infrastructure for the future.

The highlight was the unveiling of our Mumbai Reference Lab at Vidyavihar in January this year. This has immediately resulted in the increased testing of the high-ended test and it is also the first private lab in West India to have a BSL Level -III biocontainment lab. This speaks leaps and bounds of our commitment to offer our best-in-class services to our patients by bringing accurate diagnosis at reasonable prices to the fore.

We also added a second electron microscope at our National Reference Lab in Rohini in Delhi.

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At this point, I would like to remind you that Dr. Lal Path Labs already have the world's largest histopathology testing center or load, processing up to 1,400 biopsies a day, and also the largest kidney biopsy testing center. We were the first private lab in South Asia and Southeast Asia to have an electron microscope way back in 2016. Furthermore, I am pleased to share that we continue to work towards enhancing our technological capabilities. We have begun implementing and upgraded lab information system and advanced tools like artificial intelligence and data mining. These have enabled us to achieve enhanced

operational efficiencies and quicker turnaround time or TAT.

We continue to add new tests to our test menu and diversify our reach to higher franchise share of patient service centers.

Our processes are tuned to drive trust towards our brand and therefore I think we are uniquely placed to both drive and benefit from the expansion in consumption of diagnostic services going forward.

Thank you very much and I would now like to hand over the floor to Dr. Om. Over to

you, Dr. Om.

Dr. Om P Manchanda: Thank you, Dr. Lal. Welcome everyone to Dr. Lal PathLabs Q4 & FY 23 earnings call.

I hope that you and your loved ones are keeping safe and healthy .

I'll share a few of the industry and business insights that I see emerging based on FY23 results. During the entire FY23 in quarterly commentaries, I've always been highlighting that we should annualize instead of quarterly trends. Now, since the financial year is over, the entire analysis becomes a bit more meaningful as we have full year numbers in front of us.

Let's have a quick look at FY23 :

While FY22 was operationally very challenging due to very high incidence of delta and subsequent overhang of travel -related test requirements and various waves like Omicron, etc.,

In contrast to FY22 , we experienced FY23 turned out to be financially very challenging for the entire industry due to the following reasons :

1. Nearly 80% to 90% decline in COVID and COVID -related revenues. The higher the contribution of COVID testing to the total revenue , bigger the challenge.

2. Inability to rollback capacities and manpower and reagent stocks due to unpredictability of the future COVID demand pattern.

On the other hand, FY23 also experienced an uneven quarterly base and coupled with aggressive competitive activities further made it difficult for all of us in understanding the underlying growth trends .

Now since FY23 is over , I'm clearly seeing few trends. So, my take is the following :

1. Bundled packages have become a way of life in our industry . Contribution of Swasth fit to LPL total revenue has further gone up to 22% in Q4. This means we are doing a higher number of tests per patients and that's very clearly visible in our sample volumes as well .

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2. CAGR trends based on this is our internal estimates of few large players.

Their data is in public domain. That also include their inorganic assets and residual FY23 COVID revenues . Over four years suggest that some of these players have experienced all put together about growth rate of 10 -11%. This may not be reflective of entire industry growth, but if I look at all the data put together in public domain , I'm seeing CAGR of 10 -11%.

3. After a long gap , we saw in Q4 some of the players have taken price increase and we also did a little bit in the month of February , we will talk about that .

4. As the pressure on margins buildup in the industry, especially on smaller and mid -sized players , we do see consolidation opportunities emerging in times to come ; however , valuation expectations and unavailability of quality assets will make this journey a little difficult.

All suburban diagnostics as a platform we are now set for the next phase of integration . As Dr. Lal mentioned that we built a state -of-the-art reference lab in Vidyavihar in the month of January this year .

Second is, as all of you know, that COVID contributed nearly 50% of the total revenue of the Suburban in FY22. It is now down to just 6% in FY23. The stage is definitely set for the next phase wherein we plan to bring in now back -end synergies between the parent company, LPL, as well as Suburban Diagnostics and go further aggressive in Mumbai and Western region .

In the end , I would say the brand that has the trust of the patients and medical fraternity , holistically invested in network creation , infrastructure scale up , patient services, etc. , will

sustain and grow in the long run .

With that now I would like to invite our CEO – Bharath to continue this conversation.

Thank you and over to you , Bharath .

Bharath Uppiliappan: Thank you , Om . A very good evening to everyone present here and I warmly welcome you all to our Q4 FY23 earnings conference call.

I will now take you through some of the operating and business highlights of our company :

In Q4 FY23, we registered 6.3 million patient visits, generating a total revenue of Rs.491 crore . COVID and allied test declined 83% and contributed to only Rs.11 crore , that is just 2% of the overall revenue.

During the full financial year, we registered 26.9 patient visits and generated a total revenue of Rs.2,017 crore .

You would remember last year, in Q4, we had a pronounced omicron wave of COVID with high testing volumes comparable to the end of delta wave. As a result, test mix profile shifted significantly. Hence for analysis purposes, one must look beyond YoY

growth and use other metrics like sequential and three -year CAGR numbers .
Our non-COVID revenue at Rs.480 crore registered a YoY growth of 14.4% and followed historical sequential trends between Q3 and Q4 .

The test per patients also moved up from 2.48 in Q4 FY22 to 2.76 in Q4 FY23, representing a 11.2% growth in non -COVID test conducted . Underlying non-COVID patient visits in the organic business grew near 8%.

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As you are aware we had taken some price corrections in the month of February '23. Same has got settled and has been absorbed by various segments of our customers. According to our estimate, this price correction has had a 1.7% impact in this quarter and 2.5% on an ongoing basis.

We have been focused on four strategic programs building bundle test portfolio , Swasth fit, super specialty business , enhancing digital capabilities and geographic expansion including serving Tier-3 and 4 cities .

Like Om mentioned , bundle test portfolio continues to do very well and it contributed nearly 22% of Q423 of non -COVID business.

Overall , Swasthfit portfolio generated a revenue of Rs.370 crore in FY23. Super specialty portfolio led by Genomic Testing Division, Genevolve , Center of Excellence for Reproductive Diagnostics, LCoRD and also center for autoimmunity excellence LACE is becoming similar to the Swast fit portfolio in size and growth rate.

Our expansion program aimed at reaching the interiors of the country in Tier-3 plus towns or Bharat, continues unabated. We now have 277 labs at the year end, including 70 labs at Tier-3 towns like Siwan, Murshidabad in East and Sultanpur in the North .

We continue to invest in scalable and profitable digital programs , combining medical excellence and leveraging digital and analytical capabilities to improve patient journey.

I would like to talk about three digital properties that were recently launched and scaled up . Today, namely Chips, recommendation, engine and patient wallet . Our Chips customer health improvement plans is designed to engage our patients digitally with personalized digital diagnostic interventions. Further, we have better recommendation engines that share these specific tests at the point of sales. All these programs combined with the patient wallet will provide personalized , scalable , digital customer relationship management program. We are receiving good customer traction on this program since their launch.

We believe we have the right set of building blocks, which we have put in place in the marketplace with a few more in the pipeline and these we believe will help us deliver industry -leading growth rates.

With that, I would like to invite Ved to take you all to the financial performance . Over to you , Ved .

Ved P. Goel: Thank you, Bharat h. Good evening, everyone and thank you for joining this call. I'm now sharing some of the "Key Financial Highlights " for Q4 & FY23 :

Our non -

COVID revenue for Q4 FY23 came in at Rs.480 crore versus Rs.420 crore last year same quarter , a growth of 14.4%, whereas FY23 non -COVID revenue came in at Rs.1,954 crore versus Rs.1,691 crore last year, a growth of 15.5%. Total revenue for Q4 FY23 came in at Rs.491 crore against Rs.486 crore last year same quarter, a growth of 1.1%. Total revenue for FY23 came in at Rs.2,017 crore versus Rs.2,087 crore in FY22.

Revenue realization per patient for Q4 FY23 is Rs.774 against Rs.728 last year same quarter, an increase of 6% , led by price increase, test mix and higher contribution of Swasthfit.

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Normalized EBITDA after eliminating the impact of RSU , CSR and one time exceptional expenses for Q4 FY23 came in at Rs.131 crore and Rs.528 crore for FY23. Normalized EBITDA margin for Q4 is at 26.6% and 26.2% for FY23 .

Normalized PBT after eliminating the impact of notional depreciation on account of consolidation of suburban financials and one time exceptional expenses for Q4 FY23 came in at Rs.102 crore versus Rs.94 crore in last year same quarter and Rs.400 crore for FY23 versus Rs.494 crore in FY22.

Normalized PBT margin is 20.8% for Q4 FY23 and 19.8% for FY23.

Normalized PAT for Q4 FY23 came in at Rs.76 crore versus Rs.73 crore in Q4 FY22 and Rs.297 crore for FY23 versus Rs.369 crore in FY22. Normalized PAT margin is at 15.5% for Q4 and 14.7% for FY23.

Cash and cash equivalents as on March '23 is Rs.838 crore. Net of borrowing, it is Rs.601 crore.

I'm pleased to share that our efforts toward optimizing cost and increasing the operational efficiency by using the technology and digital tools has resulted in reducing our debtor outstanding and inventory to 28 days now. So, one side, we are doing efforts on front-end patient side as Bharat h mentioned that digital or technology has given the edge. Another side on the operational side, we are taking technology and tools to optimize the cost.

Finally, we are pleased to share that the Board of Directors has proposed a final dividend of Rs.6 per share. With this, the total dividend for FY23 is Rs.12 per share.

A final dividend will be paid after the shareholders' approval in the upcoming AGM.

This brings me to the conclusion of my opening remarks, and now I would request the moderator to open the forum for Q&A. Thank you.

Moderator: We will now begin the question-and-answer session. The first question is from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal: Two questions. Firstly, I noticed revenue mix largely shifting to outside Delhi NCR.

So, obviously other geographies are doing better. B2B is a bit higher YoY and Swasthfit is obviously growing faster. All of this combined, Om, the question for you that does this result in margin dilution for fiscal '24 YoY?

Dr. Om P Manchanda: We don't think so. Our margins, if you look at are nearing our pre-COVID level, so the COVID impact is out. So, even in FY24 also, we believe that we should be able to sustain a similar level of gross margin.

Rahul Agarwal: In spite of these three things gaining revenue share, outside Delhi NCR B2B and Swasthfit?

Dr. Om P Manchanda: Yes, you see, outside Delhi NCR, one must realize that as a scale picks up, there are markets which actually mimic like Delhi NCR. And if you notice in the last four to five years, our expansion through lab network is actually not that fast that it used to be earlier. So, because most of it is now building through hub labs where our tendency is now to open more collection points and labs. So, I think we are actually optimizing the lab cost, in such a way that our margins don't get diluted. So, that's the way we see it. And in any case, if you look at the earlier portfolio mix, we always had some markets which are operating on a lower margin, but our rest of north now

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has fairly scaled up, where margins are

pretty healthy there. There is some variation, it's not material enough, so that's the way I would look at it.

Rahul Agarwal: Secondly, on the growth side of it, obviously, starting first quarter of fiscal '24, the base is supported because we no more have very large COVID revenues. Life coming back to normal, obviously that helps in getting normal diseases and hence normal testing for non-COVID should pick up. Employee attrition is generally low now in favor of digital lab what I understand from my channel checks. And obviously growth outside Delhi NCR is faster than Delhi NCR. So, combined of all of this, do you see 13%,-14% revenue growth next year?

Dr. Om P Manchanda: So, I think instead of me putting a number, clearly I do believe that we entered in FY23, a much more difficult outside environment than in FY24. As you outlined it very well, pressure on attrition is very low, our ability to take price increase there we have demonstrated that COVID pressure, should we roll back capacity or not, should we buy some more stocks, etc., I think now that is also behind us. So, the good news is that we are now entering into FY24 with a cleaner slate with the less sort of headwinds compared to what we saw last year. I think overall it augurs well. Now coming to growth rate, it's very difficult for the number right now, but I definitely do believe that we should do better than what we've done in previous year.

Rahul Agarwal: Just a bookkeeping, 26.6 million patients non-COVID per full year, could you break down that into Dr. Lal and Suburban separately for FY23, 26.6 million patients.

Ved P. Goel: So, Rahul, this Suburban was not fully baked-in in this year. So, probably these numbers may not give the right picture. So, at this point I think this is the consolidated number we have –

Om P. Manchanda: For us, if you look at suburban, so far, our tendency was to show separately because in some quarters we didn't have a base. So, now, since that sort of issue is no more with us, we want to operationally see it as if like both parent and suburban they go hand-in-hand together. So, we would not want to actually not highlight suburban separately, that's the way we look at it.

Rahul Agarwal: Can I get it for fourth quarter in that case, that's a YoY comparison, right?

Ved P. Goel: I'll come back to you.

Om P. Manchanda: That number I think immediately not available with us. We'll come back to you.

Moderator: Thank you. The next question is on the line of Prakash Kapadia from Anived Portfolio Managers. Please go ahead.

Prakash Kapadia: A couple of questions from my end. There have been price hikes in the industry by

new age players. Any change in competitive landscape we are witnessing and given that growth rates in industry have generally come down post -COVID , so any acceleration or any trends we are witnessing or sensing from unorganized or shift from regional players for national players like us ? We've also taken some price hikes in our portfolio. So, if you could help us understand the impact of revenues for FY24 and is it in specialty, is it in packages, is it in specific geographies, if you could highlight that? And the third question is the endeavor to drive organic growth, what should be the key drivers for that in FY24 and beyond? So, obviously Suburban scale is one of the factors, but how is non-metro contribution shaping up , how is new patient addition happening , new test menu, so some of these points will be helpful if you can clarify ?

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Bharath Uppiliappan: Hi, Prakash, this is Bharath here. Let me start with the first question on how the competition intensity is shaping up in the industry, right? So, there are two, three ways to look at it. One is that the incumbent players and let's say some of the new traditional players , model players, let us say for sake of any other better word , continue

to intensify be competitive in the marketplace. So, there is no let down on that account. So, including new entrants, regional lab chains, etc ., all continue to compete very aggressively in the marketplace like in the past. What we are seeing is that some of this new age e-Com-based Google advertising, Facebook advertising - based companies because of maybe funding pressure or because they're not able to see the traction themselves are scaling down, let us say deep discounting because patients are also now realizing that the deep discounting are actually a clickbait operation whereas they finally end up paying the money , they pay similar money to what they pay to one of the incumbent players . So, I think it is all catching up and the industry is a bit of a transitional phase , everyone's trying to innovate and try and do the best they can. So , that is the way to look at the competitive landscape today , but yes, the advertising intensity has mildly come down, at least from a (ATL) above the line perspective. Digital media competition still continues very intensely. So, I think that's the way to -

Prakash Kapadia: So, Bharath , effectively, you're saying this would take time and there is no major shift. Maybe the pace has decreased, but it remains as competitive as it was.

Bharath Uppiliappan: Yes, . It is as if the world has come to a new age, new world wherein it's all less competition and the things back. It is still intensely competitive. All of us fighting for a share of the business. But yes, some segments of competition have kind of come down. So, the second question revolves around, just remind me on -

Prakash Kapadia: It was about price hikes, which we have taken. So, is it, for specialty portfolio , is it specific geographies and the impact of that if any on next year and going forward.

Bharath Uppiliappan: So, we have not taken any price increase on routine test , the most commonly ordered tests including packages. So, that is the first statement I'd like to make. We have taken price increase on what is really esoteric test, high end test done in reference lab only and so on and so forth. And I talked about that in the opening speech of 1.7% impact for this quarter and 2.5% estimated going forward, assuming the mix holds up. So, it is not geography -specific , it is pan India basis and really tests which are less frequently ordered , so consumers and patients don't feel the pinch of it. The good news is that all B2B partners somewhere the samples predominantly come from, have largely accepted the price increase. We are not finding any major impact or push back as a result of this , because it's been long overdue, and they also realize that they have also taken up prices. So, I think it has gone well how execution was planned for. And on the third question of growth levers going forward , Dr. Lal covered this, Dr. Om covered this , I also covered this , there are three or four ways to look at it. One is geographic expansion in Tier-3, Tier-4 cities, along with strengthening our metro city presence either in wherever you operate in North east or also including Mumbai. So, there is a geographic expansion plan. Second is differentiated channel management programs. We talked about that I think in the last quarter also on how do we manage B2B separately then B2C . So, a lot of work is going on channel management programs and driving growth through channel partners. The third obviously we spoke about today of high-end testing , either the genomic testing or the centers of excellence on reproductive diagnostics or autoimmunity. Those are all key ways to differentiate your business while delivering value to all stakeholders. The fourth is obviously getting the tech stack up to shape , so that we constantly innovate to provide a wow ! factor to the patients , saying something new has happened . So, we spoke about the chips program today

, we spoke about the recommendation

engine and also the wallet. So, it's a combination of what we have done traditionally,

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which is geographic expansion combining with obviously Swasthfit and super specialty portfolio and layering a tech component to that.

Moderator: The next question is from the line of Bino Pat hiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil I: Just a quick clarification. When you say the price increases that you took will have an impact of 2.5% , is that on your overall revenue or on just the portfolio on which you have taken price increases ?

Bharath Uppiliappan: On the overall revenue .

Bino Pathiparampil: What are your plans for addition of labs in FY 24 and FY '25?

Bharath Uppiliappan: So, our lab expansion program continues the way it has always been , about 10 to 15 labs predominantly in Tier-3, Tier-4 cities spread across northeast and some portions of south and west.

Moderator: The next question is from the line of Lavanya Tottala from UBS Securities (India) .

Please go ahead.

Lavanya Tottala : I just wanted to understand the margins for suburban in th is quarter just because last quarter we have seen a meaningful hit ?

Ved P. Goel: So, normalize d EBITDA margins for Suburban for this quarter is about 11.2% and for full year it is 12%.

Lavanya Tottala : So, we are still seeing the impact of the new franchise opened on the margin fr om suburban ?

Dr. Om P Manchanda: I think on Suburban, if you go back pre-COVID times, that was the trajectory of this business in any case. I think during COVID times we experience d very high leverage because Suburban has e xperienced very sharp drop of COVID revenue this year from nearly Rs.113 crore , it has fallen to just Rs.9 crore . The P &L shape is very similar to let's say what it used to be pre-covid time. But the important thing is that this has a platform of nearly Rs.155 crore of revenue , 80% of that comes from city of Mumbai. And now with the sort of support of centralized lab and LPL parent support , we are basically hoping to drive this both revenue synergies as well as the cost side synergies. We realize higher m argins, but this is the margin which we used to have pre-COVID times.

Ved P. Goel: And these margins, please note that we have launched this big reference lab in Mumbai. So, this quarter all the impact of that reference lab has also come.

Lavanya Tottala : One more thing on the exceptional expense that you have seen in the current quarter . What is it related to ? I just wanted to understand the tax rate guidance that you give for next F Y24.

Ved P. Goel: So, this exceptional item is one large amount receivable from BMC , Mumbai Municipal Corporation. As per policy , we have provided , but we are hopeful that this money will come to us very soon. And second , a small amount which has been provided on account of COVID inventory, which is going to expire. So, these are the exception item s.

Lavanya Tottala : On the tax rate?

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Ved P. Goel: We have a normal tax rate which is about 25% for the corporate. There is nothing which is exceptional here.

Moderator: The next question is on the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal: First question is just trying to understand the price increase . You mentioned it is for specialized test. How has been the experience versus the competition , has the competition followed you or have you lost some business because of price hike or it is status quo and it is value -accretive ?

Bharath Uppiliappan: Two, three things . One is that this price increase has gone relatively well. We were skeptical in the b eginning, but I think execution was near perfect and communicating to customers on why this price increase and in the context of the overall tenure of the price increase, this seem to be logical and reasonable to them. Number two , w

e

explain ed the impact. That's about 2.5% on an ongoing basis on the overall portfolio given the mix of the portfolio on w hich this price increase has been taken. And we also have not taken price increase on the routine portfolio o r the most commonly ordered test portfolio or the health packages or the bundle test packages. Now, with competition follow, each company will follow its own policies, procedures, priorities, etc., But given the inflationary situation, the lowering of COVID volumes, I do expect that everyone will take pr ice increase as per their areas of strength and what they think they should be doing . So, yes, the industry price table should start to move up.

Prakash Agarwal: And the price increase would be only for your specialized, which is about 30 %- 35%

or lower in terms of total volume , value?

Bharath Uppiliappan: So, it's on value basis, it is about nearly 50% of the portfolios on which we have taken.

Prakash Agarwal: 50% you claim is on your specialized testing menu ?

Bharath Uppiliappan: It is not only specialize d, specialized plus I said less frequently ordered tests and so on.

Prakash Agarwal: And the second question was trying to understand the competitive environment and the B 2B segment. Now , we hear that a lot of new players are getting very aggressive especia lly on the B2B side. And your progress on the South, please?

Bharath Uppiliappan: On the B2B side, my response is that , yes, it is as competitive as ever. We have actually put in place multiple programs to better manage this channel and segment this chann el like we do for patients. So, we have a B 2B channel program, which is very intense and very well appreciated . And in fact, B2B is one of our fast growing channels today as well compared to historical averages. So , we're very pleased with the progress of this whole approach of channel management program on the overall revenue growth rates. On South also, our progress continues ; we grew South at a reasonable pace compared to the company growth rate , we grew ahead of the company growth rate significantly. So, I think , yes, we'll continue to build deeper penetration in the South and the geographies that we currently operate in.

Prakash Agarwal: So, B2B you would have grown is what you're saying ? You've done very well.

Bharath Uppiliappan: Good growth rates on B2B.

Prakash Agarwal: And margin accretion or flattish?

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Bharath Uppiliappan: So, if you manage the mix well and the cost structure well and there is also scope to negotiate the back -end cost, so I think it plays out well enough. Also, as the volume grows a s testing efficiencies come in, we're able to amortize lot of control, calibration cost, etc.,

Moderator: The next question is from the line of An eesh Deora from Nomura Financial Advisory and Securities . Please go ahead .

Aneesh Deora : Sir, I'm looking at the quarter -on-quarter sequential data of 4Q over 3Q. So, while the realization per patient has improved and even the number of tests per patients and the number of samples process have shown an upt ick, but if you see the number of patients , that is the patient volume, they have shown a decline over 3Q which was supposed to be a seasonally weaker quarter over a strong 4Q. So, can you share your thoughts around this decline and is it pointing out any particular trend that you're seeing that w e have a bearing in the coming periods?

Bharath Uppiliappan: So, on patient visits, I don't think we should get swung by quarter -on-quarter movements. There's lot of base issue last quarter, this quarter, last year, this year , etc., Like I mentioned in the opening comments , our underlying patient visit growth rate is near 8%, number of tests performed is at 11 .2% and revenue is obviously higher.

Moderator: The next question is on the line of Nitin Agarwal from DAM Capital Advisors . Please go ahead .

Nitin Ag arwal: Two que

stions. One is on the operational bit. We've seen some reduction in employee cost on a YoY and QoQ basis. Likewise, if I adjust for the one -off that you mentioned about Rs.7-odd crore , other expenses are down . So, what would we attribute to these reductions given the fact that business has been growing ?

Bharath Uppiliappan: So, while Ved can give you specific s at an overall level, Ved in this opening comment talked about use of technology in automating a lot of processes, etc. , So, that has given us a large headcount , what I would call leverage . But specifics around the rest of the equation , Ved, you want to take on?

Ved P. Goel: Nitin, so this is primarily because of RSU charge, which is lower than last year and that is the precise reason for compensation or employee cost. So, this is one . Other expenses obviously we had a lot of programs done in last year where efficiency has been created , as in my opening remarks also I've said lot of use of technology tools we have created and duplication of the repetition of the work which can be used by technology tools. So, those have given us some advantage and that's why we have got some advantage on other expenses.

Nitin Agarwal: And secondly, since we have the full year numbers and you have some more clarity on the post -COVID trends, what is a realistic ballpark range that one should work with in terms of the patient testing volume growth for us ?

Ved P. Goel: Volume means patient you are saying?

Nitin Agarwal: Yes, patie nt volume going forward, I mean on a directional basis, it's a late single digit, early double digit, mid double digit , I mean, what is the realistic number given the industry dynamics we can work with ?

Dr. Om P Manchanda: See, it's very difficult to put a figure, but I think the first question was asked is that we do believe that FY24 numbers should really be better than last year figures. I think all sort of variables are pointing towards that. We will not have that sort of

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competitive scenario as we experienced last year. And as you mentioned that since COVID is not there, actually in one-way, the good news is that we're done with it in one year itself, otherwise it would have been something which is base effect and now we can completely focus on non-COVID business. I think the only thing we are now carrying with us is I think Rs. 63 crore of COVID and allied business in our base. We don't know, it's highly unpredictable, it can actually be as good as zero as well, it could be same. But I think if I take this out, we definitely are hopeful of improving our performance in FY24.

Nitin Agarwal: Typically, from a seasonal perspective, is Q4 expect supposedly a better quarter for the diagnostic business than Q3?

Bharath Uppiliappan: No, no, I mentioned in the opening speech saying that sequentially Q3 and Q4 are similar for us.

Dr. Om P Manchanda: I think we've also made this comment in the past, but I think for the last two, three years, I'm seeing that these variations come due to infections. And for some reason I find that even though following the pattern because Q3 is no more a healthy season anymore, even in Q3, also we are having lot of infections. So, in some way Q3 actually has moved up while Q4 may have been similar. So, that's why the difference you see is much less than before. Let's see how it happens this year, because this COVID thing has always been there, right? In fact, this whole respiratory infection in general has been there in the population. I think let's just wait out for FY24, but you're right, Q3 is generally a softer quarter, but we have not seen that trend in the last couple of years.

Nitin Agarwal: Since you mentioned the point of infections we've seen very high intensity of incidence rather of infections. The whole of FY23 which is reflected in I guess the pharmaceutical sales of anti-infectives and all the c

old and cough preparations, I

mean in your assessment any tailwind for our business also?

Dr. Om P Manchanda: Actually, respiratory infections in general do not really directly lead to very high jump in testing. In fact, during this month of April and even in March also, we were talking about COVID, etc., We didn't see direct sort of jump in COVID testing. But I think one phase which normally tends to go up in situation like this is CRP which is inflammatory marker. So, that is one test we have seen a very significant jump in Q4, right? So, CRP is one test we saw a big jump.

Nitin Agarwal: And lastly, on the realization per patient, obviously with the bundling that we are doing, that's sort of going up. We are about 780 or there about for this quarter. I mean how does one look at this number, is there a significant scope for appreciation from these levels on this number?

Dr. Om P Manchanda: I don't think so. I think there are a few things that have gone into the base - I is of course bundle packages now contribution going up to 2.2%. I don't foresee a very sharp jump again in the contribution. I think it should stabilize around this number.

Suburban in general has a higher realization than Dr. Lal PathLabs. So, that also has gone in for the full year impact. I think whatever little will come now is due to price increase that we've taken in the month of February. Maybe that will have some marginal impact. Mix may not change. My gut says that we may not see a very sharp jump on this in FY24.

Moderator: The next question is from the line of Cyndrella Carvalho from JM Financial Institutional Securities. Please go ahead.

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Cyndrella Carvalho: So, if we are looking at the volume growth, you in your opening remark quite interestingly highlighted the Mumbai lab. We know our earlier labs whenever they have come, they have given us a good traction. So, what do you think about the Mumbai lab, how it will contribute to our overall patient growth, like, how should we relate this, if you can help us understand this?

Shankha Banerjee: So, the Mumbai lab has been launched in January. The thing is it could be a slightly longer wavelength compared to the other reference labs because from the Suburban point of view, it's kind of starting a completely new vertical so to say in terms of the specialized business, and also trying to get a completely new customer client profile to be built under the portfolio that we are currently doing. So, from that point of view, yes, from the last few quarters, it will definitely have better impact in terms of patient growth, but it may not be exactly comparable in terms of the things which we saw in East Calcutta or Bengaluru, it might take a slightly longer wavelength.

Cyndrella Carvalho: In your opening remarks, you also mentioned about a slightly higher cost base that

we carried for the entire FY23 . So, now, if we look at our cost base versus the growth that we have seen, at least the normalization that we have seen in F Y23, do you think the upcoming year F Y24, '25 this cost base will support us on the growth and you should see some operating leverage also ?

Ved P. Goel: That's the idea , because one is launching this reference lab, which will obviously give advantage and what Shank ha is saying on B2B side . Because all the infrastructure is in place and I think you also have visited that lab. Now we have to leverage this infra . In our view , the efforts is more on top line but similarly the operating leverage will also come on the bottom line. So , I believe that margins from here should improve.

Cyndrella Carvalho: If we try and understand the overall marketing scenario today, you highlighted that there is some sluggishness , there is intensity continues, we have taken specialized price increases . But the specialized price increase should reflect across the entire year, right, I mean

n, we just started in month of Feb, is that understanding correct?

Bharath Uppiliappan: Yes, indeed.

Moderator: The next question is from the line of Tanmay Gandhi from Investec Capital Services . Please go ahead.

Tanmay Gandhi: Sir, first question is on sequential growth, right . If we historically look at our sequential growth in 4Q , it had changed between 3 % to 5%, right , but this time it was flat. So , anything worth highlighting the re?

Bharath Uppiliappan: We couldn't hear your question properly. Could you repeat yourself if you don't mind ?

Tanmay Gandhi: So, my question is that, historically if we look at our sequential growth for 4Q, it has ranged between 3 % to 5%, right , but this year it is flat. So, anything particular would like to highlight here ?

Om P. Manchanda: That's what I actually mentioned. Your observation is right . It's not 3 % to 5% , normally it's about 2% , and the 2% actually is neither here nor there , so sometimes depending on the infection rate is flat . But I'm actually seeing this trend for the last 2 years , even I think if I remember in the previous year also we saw the same thing. So, my hypothesis is that now it's just evened out in terms of infection rates. Earlier Q3 was seen as a healthy quarter. I think now because of air pollution, etc ., all these things, Swasthfit which is more screening packages , it's become a little bit of even. So, looks like now the pattern that I'm seeing is the Q2 is the highest, all other three quarters are very similar, Q 1, Q3 and Q4. That's the observation that I have for the last couple of years. Let's see in FY24 what happens because at least we will not have this whole variable of COVID , at least it will be more an illness space. But your observation is right , this time we are not seeing a sequential jump which we normally see. This is not the first time I was noticing, even last year also , I did notice that.

Tanmay Gandhi: We understand that year -on-year growth for this year is not that relevant because of volatile base . But sir, if we try to extrapolate our sequential growth, which we have seen during the last four quarters to F Y24, then it roughly translates to a mid to high single digit growth. So , is that the right way to look at it ?

Dr. Om P Manchanda: No, no, I don't think . We have done 15 .5% in non-COVID, but it also has four quarters of Suburban compared to two quarters previous year, right? So, the normalized growth will be how much for this year?

Ved P. Goel: 10%-11%.

Dr. Om P Manchanda: Yes, we definitely hope to have better performance this year. It won't be single digit.

Tanmay Gandhi: On Swasthfit, so we have been saying that largely it has peaked out and you don't see any sharp jump from this level, right? And historically, our revenue growth was largely driven by our patient and sample growth . But this year it was also driven by higher realization per patient. Going forward , as we expect some moderation in Swasthfit expansion , do you see again our revenue growth would be driven by volume and volume growth has actually come down , right?

Bharath Uppiliappan: So, yes, our efforts will always be to drive the patient visit growth , and there are various techniques we are using to get better on that count . But as a result of bundling , the test per patient is also moving up significantly , about 11 %-odd in Q4 . And that trend, Swasthfit further grows, it will reflect in the numbers . But our intrinsic effort is to acquire new customers and service our existing customers in a better fashion.

Tanmay Gandhi: A related question , in Q4, did we have any one -off kind of growth in in tax -related packages for Swasthfit ?

Bharath Uppiliappan: In the last two, three years I think in Q4 Swasthfit picks up because of this tax advantage. It's a normal scenario. It's in the base as well. But this year growth has been

still better than the previous year led by better distribution, better understanding from patients, acceptance, etc., So, it is more than tax.

Moderator: The next question is from the line of Yogesh Tiwari from Arihant Capital Markets. Please go ahead.

Yogesh Tiwari: So, my first question is post-COVID, what would be the growth of the in-vitro diagnostic industry?

Om P. Manchanda: Normally, IVD industry, we refer to the reagent suppliers. I presume you're asking for pathology, people like us, right?

Yogesh Tiwari: Yes.

Om P. Manchanda: So, as I mentioned to you the data, which is available in public domain, I just added up at least four, five companies data. In the last four years CAGR I have removed this impact of COVID years which is FY21 and FY22. The growth rate that I'm seeing
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is about 10-11% CAGR over a three year or four-year period. I personally feel that it should sustain. One variable which probably is going to get added because in the last four years CAGR may not have very high pricing impact. But since our ability to take price increase is now coming back, let's see how other players do, but my reading is it will definitely be in this way going forward.

Yogesh Tiwari: And sir, what would be the total size of the industry if you can share your thoughts?

Om P. Manchanda: There is no published data available. Normally, we refer that large players contribute about 15% of the value. So, I think one crude method would be to divide the Rs.

5,000 crore by 0.15 and that gives Rs. 30,000-35,000-40,000 crore, which is essentially the private lab business and then you add this to our hospital lab business, which is there. So, my sense is it should go up to Rs. 60,000 crore, something like that. That's one of the ways to look at it.

Yogesh Tiwari: Which you mentioned, this is for the Indian domestic, right?

Om P. Manchanda: Yes, India domestic market.

Moderator: The next question is from the line of Praveen Kumar from Acuitas Capital Advisors. Please go ahead.

Praveen Kumar: I had a couple of questions. The first question was on a comment that Dr. Om had made in his opening remarks. He had mentioned that bundled packages have become a way of life. So, just thinking about the comment a little bit more, see, in terms of bundle packages, the decision-making point could be perceived to shift to the patient instead of the doctor. And since the Doctor Connect is a major complete advantage for Dr. Lal, how do you perceive this panning out in terms of your competitive advantage?

Dr. Om P. Manchanda: I think that's a great sort of a question. My reading is if it is only limited to preventive health checkup, yes, decision-making is with consumer or the patient. Since lot of chronic disease patients also are shifting to bundle pack... they actually upgrade to these packages you are going to see better value for money. So, even illness patients are also in bundle packages. So, the doctor's decision-making is still there. Only thing is that the doctors also have started prescribing now some of these packages as a part of the normal prescription.

Praveen Kumar: I had a second question, which was more on the investor presentation, the financials in particular. What I wanted to understand was the company declares normalized EBITDA margins where few items are excluded, one of which is the employee stock compensation or RSU based compensation. So, I just wanted to understand the rationale for excluding this. Is it that you perceive these to be more one-off kind of expenses because if not, if you perceive that these are more ongoing nature of compensation-based practices, then in one way or the other, the companies bear in the cost for that, right. So, shouldn't it be part of the overall EBITDA itself, not a specific normalized EBITDA?

Ved P. Goel: So, Praveen, there are two reasons. One is of course this

is non-cash item, it is not

really where the cash outflow from the Company. And second, this charge varies depending on the price of the share because if we are granting ESOP at a time where the price differentiates, so probably this fluctuates the charge. So, these are the reasons why we are showing separately.

Praveen Kumar: But just to understand that, if I look at your past annual reports and other financial disclosures, this cost has varied between one to one and a half percent kind of a
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number, right? So, just wanted to understand that it seems to vary in that range. So, shouldn't that be part of this non-normalized EBITDA?

Dr. Om P. Manchanda: You have both the figures in any case; you have a normalized and if you see EBITDA figures also you can see that. But you're right, there is a baseline number which will stay here, which could be, I think one, one and a half percent is really a good assumption.

Moderator: The next question is in the line of Zaid Munshi from Concept Invest well. Please go ahead.

Zaid Munshi : My question is related to the hospital base. Do we have any plans to expand in partnering with the hospitals for their diagnostic part , as we don't disclose the numbers how many of hospitals we operate ?

Bharath Uppiliappan: So, we have said in the past that we continue to look at HLM as and when we find a very strategic or a geographic fit available to us, because expanding into a hospital lab management like a normal retail business can lead to a lot of other issues like outstanding debtors and so on. So, it is best to do it with lot of consideration rather than do it on a mass scale. So, we are selective about the HLMs we enter into contracts with, but yes , we do HLM .

Zaid Munshi: So, currently main focus is on independent labs and not on hospitals?

Bharath Uppiliappan: Yes, yes . So, I think that is the main focus . Once in a while we get an opportunity , we'll do HLM , no problem at all.

Moderator: The next question is from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: So, my first question is on home sample collection. So, I just want to check what are the current trends that you're seeing after normalization in post -COVID outbreak , do you see patients coming back to your centers and a percentage of revenue from home sample collection , has it come down and currently how much percentage of B2C revenues are coming from home sample collection ? In case this is something which is sustaining even after COVID , then what are the steps that we're taking to be competitive especially versus the integrated digital health platforms ?

Bharath Uppiliappan: So, two, three points. One is that during COVID I think the predominant mode of operation itself was home collection. So , the number was very, very high ; I think close to 20%-odd those days. But pre -COVID this number of home collection was about 5%. Today , I think what we are seeing is let us say about 8 % , 9% of home collection, which is a better trend than what we had in the pre -COVID days , but significantly down obviously from the COVID days. So , that is one way to look at the numbers. Second is, the underlying thing is about the patients choosing a lab of trust. It is not about convenience . Convenience is one of the factors involved in it, but they want to look at labs, etc., and we continue to enjoy a very favored position on that count. So , we are seeing our home collection revenues in line with this thought of us.

Sayantan Maji: This 8 % to 9% is of B2C or of the total revenue?

Bharath Uppiliappan: Of I think the total revenues here.

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Sayantan Maji: In that you have mentioned of variable model . Can you just explain it a little bit like what is this variable model which grows with business volume?

Bharath Uppiliappan: When did we speak out variable model.

Sayantan Maji:

So, in slide number 25 of the investor deck, it says that in the home collection, it says that it's a variable model which grows with business.

Ved P. Goel: This is one way of hiring people, which is fixed pay and another way is you pay as per pickup. So, it is not a fixed charge, like our own center versus franchisee center , it is similar to that like , we instead of hiring people giving fixed contract, it is per pickup base the payment.

Sayantan Maji: Basically , the phlebotomist is entirely dependent on the number of samples he or she picks up in a day ?

Ved P. Goel: Right.

Sayantan Maji: And the second question is a bookkeeping one. So, now that FY23 is over, what was the total Ind AS impact on EBITDA or Ind AS benefit to the EBITDA ?

Ved P. Goel: So, it is about two and a half percent roughly which is on account of Ind AS.

Moderator: Ladies and gentlemen, that would be our last question for today. I now hand the conference back to the management for the closing remarks. Thank you and over to you.

Ved P. Goel: Thank you , everyone for being with us on this call today. I hope we were able to address your queries. If you have any more questions or queries, please feel free to reach out to us or our Investor Relations team, CDR India and we will be happy to clarify your thoughts. Thank you once again . I would now request the moderator to close the call .

Moderator: Ladies and gentlemen, on behalf of Dr. Lal PathLabs , that concludes this conference.

Thank you all for joining us and you may now disconnect your lines.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

Call: Aug 2023

August 02 , 2023

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Conference Call on Financial Results for Q 1 FY24

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q 1 FY24 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 15:

Dr. Lal PathLabs Limited
Q1 FY24 Earnings Conference Call Transcript
July 27, 2023

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Bharath Uppiliappan - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer
• Mr. Shankha Banerjee – CEO – Group/ Associate Companies
Participants who asked questions • Rahul Agarwal – InCred Capital
• Aneesh Deora - Nomura Financial Advisory and Securities
• Tanmay Gandhi - Investec Capital Services
• Mehul Sheth - Axis Capital
• Cyndrella Carvalho - JM Financial Institutional Securities
• Aashita Jain – Nuvama Securities
• Vignesh Iyer - Sequent Investments
• Omkar Kamtekar - Bonanza Portfolio
• Karan Vora - Goldman Sachs
• Dheeresh Pathak - White Oak Capital

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Moderator : Ladies and gentlemen , good day, and welcome to the Q1 FY 24 Earnings Conference Call of Dr. Lal PathLabs.

As a reminder all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call please signal the operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Nishid Solanki of CDR India.

Nishid Solanki : Good evening, everyone and welcome to Dr. Lal PathLabs ' Q1 FY24 Earnings Conference Call.

Today , we are joined by senior members of the Management Team including (Hony) Brig. Dr. Arvind Lal - Executive Chairman , Dr. Om Prakash Manchanda - Managing Director , Mr. Bharath Uppiliappan – CEO , Mr. Ved Prakash Goel - Group CFO along with Mr. Shankha Banerjee – CEO of Suburban and other Group Companies.

I would like to share our standard disclaimer here. Some of the statements made on today's call would be forward -looking in nature, and the actual results could vary from these forward -looking statements. A detailed statement in this regard is available in the Results presentation which has been circulated earlier and also available on the stock exchange websites.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspectives.

(Hony) Brig. Dr. Arvind Lal : A very good evening today everyone. I welcome you all to Dr. Lal PathLabs ' Q1 FY 24 Earnings Conference Call. I would commence by shedding some light on the evolving market situation and the progress that we have made.

India's diagnostic industry remains vibrant presenting abundant opportunities in the future. In contrast to other nations India's healthcare expenditure relative to its GDP is reasonably low. However, a shift towards evidence -based medicine has come of age after the pandemic. The industry is undergoing a transformation from unorganized to organized more so since the onset of the pandemic. This positive shift continues and is attributed to multiple reasons including higher awareness and increasing preference of customers for branded players it offers superior quality and efficiency.

I am glad to share with you all that International Newsweek magazine has recognized Dr. Lal PathLabs as the 15th of the 100th Global Most Loved Workplaces 2023 across the world. There are only two other Indian companies on the list which were ranked much below us.

As a leader in the diagnostic industry, we have consistently thrived to deliver superior patient experience by expanding our test menu and penetrating deeper in to our core markets as well as foraying into newer markets. The synergy between Suburban Diagnostics and Dr. Lal PathLabs' brand has further enhanced , helping us garner incremental shares in our high potential Western market. We are constantly focused on expanding our presence to reach a larger number of customers , especially in the Tier-III and Tier-IV cities . While doing so we remain committed to continuously improving our testing capabilities, quality, and patient experience.

We have implemented Total Lab Automation or TLA which is supplemented with digital intelligence. We have also reduced the requirement of aliquots or what are sub-samples by 93% by using inbuilt advanced sorting logic and aliquoting

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algorithms which resulted in saving of more than 1.3 million aliquots and more than 5,200 tons of plastic waste in our National Reference Lab at Rohini, New Delhi. This underlines Dr. Lal PathLabs' strong commitment towards sustainability and creating a greener environment.

Our investments in strengthening the digital infrastructure has started yielding positive results as anticipated . This will help us in consistently meeting patient expectations on several parameters while

driving volume growth. Given our strong operating model and brand affinity in both B2C and B2B segments , I see the company continue to gain significant market share.

Thank you very much . I would now like to hand over the floor to Dr. Om Manchanda. Over to you, Om.

Dr. Om P. Manchanda : Thank you Dr. Lal. Welcome to Dr. Lal PathLabs' Q1 FY 24 earnings call. I shall share the broad industry trends.

The first one is at an overall industry level, the contribution from COVID testing has further declined. We are now mainly left with COVID allied testing and this sub -group within COVID sales has a very strong overlap with non -COVID testing. Therefore, tracking and reporting sales with and without COVID separately does not make sense anymore. Therefore, we have also removed this chart from the Investor Deck where we were showing these numbers separately. The good news is that in some

ways this reduces the complexity and helps us analyze business trends better. Second point is the Suburban Diagnostic now enters into Phase II of integration where we have started looking at common lab infrastructure between the two brands that is Dr. Lal PathLabs' and Suburban.

Swasthfit continues to ride the momentum of preventive health checkup sales. The contribution has further gone up in this quarter. That is showing a strong positive impact on test per patient as well as higher realization per patient. The added advantage of this increase in contribution is also seen in simplified operations.

Widening our footprint both in terms of depth that is going deeper into Tier-II, Tier-III and Tier-IV towns as well as in terms of width which is both through inorganic as well as organic will continue to be the central theme of growth strategy of Dr. Lal PathLabs.

With that, now I handover to Bharath for further update on Q1 FY24 results. Over to you, Bharath.

Bharath Uppiliappan : A very good evening to everyone present here. I warmly welcome you all to Q1 FY 24 Earnings Conference Call.

Operating and Business Highlights of the Company

In Q1 FY 24, we recorded a revenue of Rs. 541 crore which is a growth of 7.6% over last year Q1, and the non-COVID revenue grew by 9.7% YoY. The COVID RT-PCR testing revenue declined by 81% in Q1 FY 24 to Rs. 2.3 crore from Rs. 12 crore in Q1 FY 23.

Revenue per patient for Q1 FY 24 was Rs. 789, this was sequentially higher versus the preceding quarter by 1.9% led by high contribution of Swasthfit, Delhi NCR region and full quarter flow through of price adjustments.

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In Q1 FY 24, we recorded 6.9 million patient visits, which is flattish at an overall level led by an unusually high base in Q1 last year. However, net of COVID-related testing we grew patient volumes sequentially by 8%. Also, on a four-year CAGR basis, the patient volume growth for Q1 FY 24 stood around 7% for the organic business.

We also recorded a 6.1% growth in samples, which stood at 19.3 million in Q1 FY24. The tests per patient also moved up significantly from 2.6 in Q1 FY 23 to 2.8 in Q1 FY24, representing a 7.0% growth.

Update on Strategic Initiatives

The first one is on increase in marketing efforts. In this quarter, on the organic business side, we launched a new marketing campaign "Bharat Ka Vishwas" with quantifiable reasons to believe why millions of patients and doctors trust Dr. Lal PathLabs. We believe that this campaign over a longer period of time will help pivot the conversation to the underliners of this category and not just on pricing and discounting.

This combined with our digital led initiatives like chips, recommendation engines, supported with on-ground activation programs are driving growth in a strong hold markets of North and East. Delhi NCR region also recorded strong growth rates in Q1 FY 24. In the city of Mumbai and Pune, under Suburban, we have launched a strong direct-to-consumer program supported by Thematic above-the-line campaign.

Second strategic initiative revolves around our portfolio. Our bundled test portfolio Swasthfit continues to perform well and I am happy to share with you that we have registered the highest ever quarterly revenue from Swasthfit of Rs. 112 crore in this quarter with a contribution of 22% to the total revenue excluding Suburban.

Our Super Specialty business continues to do well with strong growth rates supported by new initiatives. We are also creating variants of common tests like Lipid Profile and making it medically more relevant to patient sub-segments like diabetics and heart related ailments. Higher Delhi NCR contribution and above average growth rate in Swasthfit and Super Specialty coupled with reinvigorating our routine portfolio have positively contributed to revenue per patient and margin profile for the quarter.

Number three, strategic initiative revolves around geographical expansions. As stated in our previous call's strategic expansions in Tier-III and Tier-IV cities of the country continues as per plan. In addition to the three labs, we opened new labs in the preceding quarter at Siwan, Sultanpur, and Murshidabad, we have commenced operations at four of our Greenfield labs in Jind in Haryana, Una in Himachal Pradesh, Tamluk, and Hooghly in West Bengal. These continue the right set of building blocks coupled with the Pan India presence and the digital infrastructure that will help us deliver industry-leading growth rates for a long period of time.

With that, I would like to invite Ved to take you all through the financial performance. Over to you, Ved.

Ved P. Goel : Hello everyone and thank you for joining this call. I trust each of you and your family

are safe and healthy.

Key Financial Highlight for the Quarter

Our non-COVID revenue for Q1 FY 24 came in at Rs. 528 crore versus Rs. 482 crore last year same quarter, a growth of 9.7%. Total revenue for Q1 FY 24 came in at Rs. 541 crore against Rs. 503 crore last year same quarter, a growth of 7.6%.

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Revenue realization per patient for Q1 FY 24 is Rs. 789 as against Rs. 727 last year same quarter, an increase of 8.6% led by price increase, test mix and higher contribution of Swasthfit.

Normalized EBITDA after eliminating the impact of RSU and CSR for Q1 FY 24 came in at Rs. 154 crore as compared to Rs. 125 crore in Q1 FY 23. Normalized EBITDA margin for Q1 FY 24 is 28.4%.

PBT for Q1 FY 24 came in at Rs. 117 crore versus Rs. 81 crore in last year same quarter. PBT margin is 21.7% for Q1 FY 24. PAT for Q1 FY 24 came in at Rs. 84 crore versus Rs. 58 crore in Q1 FY 23. The PAT margin is at 15.4% for Q1 FY 24. Cash and cash equivalent as on June 30th is Rs. 877 crore, net of borrowing it is Rs. 731 crore.

I am pleased to share that our continuous efforts towards optimizing material costs and improving operational efficiency has enabled us to invest more into A&P & Technology.

At last, we are pleased to share with you that the Board of Directors of the Company have approved an interim dividend of Rs. 6 per equity share.

This brings me to the conclusion of my opening remarks and now I would request the moderator to open the forum for Q&A. Thank you.

Moderator: The first question is from the line of Rahul Agarwal from Incred Capital. Please go ahead.

Rahul Agarwal: Firstly just your thoughts Dr. Om on the quarter, 10% revenue growth I think both YoY and four-year CAGR look similar. So, what went well and what did not versus what you expected?

Dr. Om P. Manchanda: So, let me first talk about what did not do well rather than what we did well, what did well. One is I think we would have loved to see a slightly higher volume growth, that's something we felt short. But I think what did well on the number of samples growth or test growth, because test per patient has gone up very sharply.

Second is contribution

from Swasthfit continues to do well and I think this is a very strategic sort of initiative from our side, we believe that trust is very important in diagnostics and more and more people do health checkups and as healthcare awareness goes up, this segment will continue to show growth and that's a very big positive sign. I think COVID part in any case was declining and we are happy that it's almost going away. Life becomes simple to analyze the trend so that I think is a positive part.

On the financial side I am sure you would have seen that our margins are much better than what we expected. So, I think these are all your takeaways as far as results are concerned.

Rahul Agarwal: Got it, yes because I was thinking 10%, I thought we should do better in Q1, not because it was a very strong season for us but just purely because Q-o-Q we were seeing improvements into non-COVID so it looks like last quarter versus this quarter there has hardly been any improvement on non-COVID that's what it feels like, is that correct understanding.

Dr. Om P. Manchanda: So, I personally think I have repeatedly been saying that these are gradual improvements that happen in our space. So, certainly, overnight jump one should

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not expect. But I think the good news is trajectory is moving upward because there was a time when we were a little concerned about hyper-competition and whether we can sustain this growth, but I think overall now signs are very positive as we go forward.

Rahul Agarwal: And secondly on the operating expenditure, looks like that's not grown in proportion to business growth which is good news and hence margins are up. I also understand that generally the first half of Dr. Lal has higher margins and then through the year it comes down second half. But any specific efforts you would like to highlight on cost savings with some details?

Ved Prakash Goel: Yes, so Rahul there are two major initiatives one is on the material cost side which obviously there is an impact of mix and portfolio change. And second is obviously we are looking at how to optimize this cost because this is the biggest cost in our P&L. So, this helped us in this quarter. And the second is last year in the same quarter we had some provision against debtors which is not there in this quarter. So,

that is another reason .

Rahul Agarwal : What was the quantum for that provision?

Ved Prakash Goel : It was about Rs. 5 crore , this was the BMC provision we had made against recovery for Suburban.

Rahul Agarwal : And lastly on Suburban , how did that do during the quarter , some color on whatever you could share net of IndAS revenue, patient volumes, our revenue per patient , and overall business , you are expanding into South Bombay, what's really happening on Suburban ?

Dr. Om P. Manchanda : So, I think Suburban what we are doing now is seeing what kind of synergies we can draw between the two companies and two brands. So, we are looking very carefully at some synergies that can come out of the lab infrastructure that exists between the two companies. So, I think that's the update we have.

Secondly in terms of marketing activities , we are looking more aggressive than before in Mumbai. We are trying to see how we can improve our B2C contribution in Mumbai market.

Rahul Agarwal : Any topline number and volume number you could share please.

Dr. Om P. Manchanda : Yes, I think right now I would say volume growth is slightly muted there, but we will probably share as we go along.

Moderator : Thank you. The next question is from the line of Aneesh Deora from Nomura Financial Advisory and Securities . Please go ahead.

Aneesh Deora : Firstly, on the margin , so it was 27% for this quarter , the EBITDA margin which is higher than the full year guidance of 25%. So, does this guidance level still hold and how should we think about margins for the coming

quarters?

Ved Prakash Goel : So, this is Ved , again as Rahul mentioned previously , generally we have first half where margins are higher than second half, so we must wait, but we are holding that statement that we should maintain our pre -COVID margins going forward.

Aneesh Deora : And what is the reason for the margins being higher in the first half of the year for Dr. Lal?

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Dr. Om P. Manchanda : So, the first half usually our revenue if you look at the trends historically, they are always higher. And we have also seen the mix of tests also drifted more toward certain kind of tests where gross margins are higher.

Aneesh Deora : And secondly , we have implemented the price increases which is being adequately reflected in the revenues per patient which have inched up quarter -on-quarter , but when we look at the revenue per test, that has largely remained flat. So, how do you think about this and what are the dynamics around it, like one would ideally expect that the revenue per test would also inch up proportionately , but that has not happened so any thoughts around this.

Bharath Uppiliappan : So, revenue per patient sequentially there was a positive impact of about 1.2% led by price flow. Revenue per test is a mix of multiple things I am not sure how I should respond , but give me some time if it comes up later in the queue I will

Dr. Om P. Manchanda : I will probably add to this. See the revenue per test will continue to be lower because as a contribution of Swasthfit keeps moving up, because Swasthfit has lot of tests in it you will always see that situation , but I think we have to look at economics from an overall standpoint because we don't mind revenue of Swasthfit going up because the marginal cost on additional test is very low. But we can pass on that benefit through bundle packages to the patient, in that case, you will always see not only for us I think the industry itself is seeing that the revenue per test is going down , but revenue per patient is going up.

Aneesh Deora :: And just a clarification you mentioned that four-year CAGR for the organic business was 7%, right if I got that correctly, just want to clarify.

Ved Prakash Goel : Yes, there was patient growth.

Dr. Om P. Manchanda : So, volume we were talking about.

Moderator : Thank you. The next question is from the line of Tanmay Gandhi from Investec. Please go ahead.

Tanmay Gandhi : My question is again on the operating expenses so if we look at the sequential growth, so revenues have grown by almost 10%, but still our other expenditures have degrown by 5%. So, is there any one-off in the last quarter base or is there any postponement of expenses which is there in this quarter.

Ved Prakash Goel : No, so Tanmay as I mentioned there was one provision, we took in the last year that was the only one item. And there is no postponement of the expenses in this quarter.

Tanmay Gandhi : Last year you mean Q4 FY23?

Dr. Om P. Manchanda : Q1 of last year.

Tanmay Gandhi : No, I am talking about sequentially, so even sequentially it has declined by 5%.

Ved Prakash Goel : Sequentially also.

Tanmay Gandhi : So, provision was there in Q1 as well as Q4, is that right?

Ved Prakash Goel : Yes in Q4 .

Tanmay Gandhi : And can you share margins for Suburban, have they also improved?

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Dr. Om P. Manchanda : Margin for Suburban also has improved , if you do just non -COVID versus non - COVID, right Yes margins have improved there also .

Tanmay Gandhi : Can you share the numbers?

Dr. Om P. Manchanda : I think the range is between about 11% to 13%, exact numbers we don't have , but it's in that range about 11% to 13%.

Moderator : Thank you. The next question is from the line of Mehul Sheth from Axis Capital.

Please go ahead.

Mehul Sheth : First question is around the gross margin so does your gross margin also had some benefit of that price increa

se that you have taken in few of your tests?

Dr. Om P. Manchanda : Yes, that's right , it has.

Mehul Sheth : Can you quantify, mean how much was because of that price increase?

Ved Prakash Goel : It is about 3% which is for the full year.

Dr. Om P. Manchanda : Yes, so what we are seeing is that there is a 3% upside to the revenue due to price increase. So, whatever benefit goes down to gross margin one can calculate that.

Mehul Sheth : And one more question on your expansion side so do you have any near-term target which you are focusing more on the West and the South now to expanding collection centers there . So, do you have any near -term target, how much center you want to add?

Dr. Om P. Manchanda : So, I think our near -term target for the West region is how do we create synergy between Dr. Lal PathLabs network as well as Suburban network. So, currently we are looking at , we have mapped the lab locations for both companies and identifying gap areas so we will gradually fill these gaps. On an overall company basis usually, we put about 10 to 15 labs so that effort will continue on the organic side. And we will continue to look at white spaces for our collection centers. So, in general , normally we have seen about 10% odd of infrastructure growth that happens in our system. So, that really turns out to be about 20-odd labs every year.

Mehul Sheth : And just a last question to Ved, can you give break up the Suburban sales number at least, how were total sales, COVID and non -COVID sales breakup if you have them handy ?

Ved Prakash Goel : So, Suburban revenue is Rs. 37 crore this quarter. And largely it is a non-COVID, very small portion was COVID .

Dr. Om P. Manchanda : Of which about Rs. 36 crore is non-COVID, Rs. 1 crore is COVID.

Moderator : Thank you. The next question is from the line of Cyndrella Thomas Carvalho from JM Financial Institutional Securities . Please go ahead.

Cyndrella Carvalho : Dr. Om you mention ed certain disappointments that you were experiencing largely on the volume side, could you elaborate more on that statement . I mean is it something to do geography wise or is it a certain segment of the business that you were expecting to be better?

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Dr. Om P. Manchanda : See the thing is if you look at th e patients that we report, technically they are not unique patient s, these are all patient visits . So, that's how historically we have been tracking this number. And we have always had a challenge of tracking unique patients because patients in our s ystem come from various channels. They come through walk -ins, they come through collection centers, they also come through pick - up points, it's very difficult to have one identifier , a patient ID and then we have so many unique patients.

But what we are fi nding is that patient visits probably have been less this quarter. And it led us to ask many questions in our mind, one was we took a price increase, is that impacting it. We did this analysis for those tests where we had taken a price increas e, we didn't find any variation between though that portfolio which has been experiencing pric e increase versus the other one where we have not taken price increase. In fact, the growth rate for both sides is the same. So, that means we have ruled out that price increase has impacted it.

We also saw whether the competition is impacting it, we did this Tier wise analysis we again found that it's probably uniformly across this thing. We also found that we are n ot very sure it's a hypothesis whether because the bundle packages are going up, whether it's impacting the patient visits frequency, we still don't know that. I think the strong sort of reason that we can find out is that last year I think no n-COVID numbers are also impacted by COVID itself which is very difficult to identify for us. And the base of last year is much higher if you have that number with you, cl

ose to

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Bharath Uppiliappan : 25% of the patients are in Q1.

Dr. Om P. Manchanda : Yes, in Q1, so I think right now probably the only analysis that we have is the last year base is much higher that probably has impacted this volume growth. We will have to wait and see for this quarter and maybe two quarters later. We are very hopeful on an annualized basis we will see a smart recovery on the volume side, I think that's the only thing I can probably say right now.

Cyndrella Carvalho : Dr. Om I said what will drive this volume recovery for us, what are the efforts we are investing in which gives you the confidence that we will be able to report higher volumes?

Bharath Uppiliappan : So, like we talked about in the opening speech I think we are very focused on new customer acquisition programs, it can either be a physical hunting program or even a digital acquisition program. So, that is really what we are looking at as one stage. Second is around the whole infrastructure expansion program. So, we announced new labs, we are putting up collection centers in the regular space and we can accelerate that whole area. So, that is going to be a second lever of getting new customers in. And third is obviously improving our customer service levels as we move forward as well. So, that's taken as increases and visit frequency increases and recommendation by doctors also increase. And the Super Specialty portfolio also will continue to make more inroads in acquiring new patients propagating those tests. So, I think within these four levers which we have, we should be able to see a smart recovery like what Dr. Om mentioned in the full year term.

Cyndrella Carvalho : And one last question in terms of the cost if we look at the cost base, I think like all the participants are asking how we make the investments but somehow the cost base has remained steady, so how should we look at it?

Dr. Om P. Manchanda : I think structurally the P&L shape is changing in two to three aspects, one is if you plot data of the last sort of 10 to 12 quarters, our rental cost is directionally going

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down. And one of the reasons for that is a lot of business is shifting to collection center network. People are not coming to as walk-ins but rather they are going to nearby collection center, so that's one change that we are observing.

Second is we are seeing a very volatility in the mix change. I think one of the reasons probably last year our reagents cost was high because COVID testing was very high and that led to higher reagents cost, that receipts we are probably going back to the lower sort of the consumption cost. So, that second change that I am observing. Going forward you should see two heads going up #1 A&P and we are constantly going to increase this number, gradually I think as we get lot of confidence on margin recovery as we have seen in this quarter, we will try and plough back a lot of money back into the sort of A&P. The second area is IT cost, we have seen a sharp increase in the last three to four quarters and we will reassess the requirement, if required we will invest more in IT infra as well.

Moderator : Thank you. The next question is from the line of Aashita Jain from Nuvama. Please go ahead.

Aashita Jain : My question is more on the industry side, if I look at the price hikes that you have taken and several of your peers have also taken in semi-specialized and high-end kind of test. Will it be possible for us to take price hikes going forward in line with the inflation at least in these high-end tests, what is your take on that?

Dr. Om P. Manchanda : So, I think at a macro level I can only say that there is a bit of a rebalancing, this is going to happen on pricing. Traditionally my experience has been on high-end tests that were actually cross-subsidized by routine tests in the past. As the competitive intensity in

the routine is increasing, so I think some of these high-end prices will get automatically corrected because while sometimes the realization per patient high-end is higher, gross margins are not as high as they are in the routine tests.

I think first my sense is this industry is going to experience a lot of rebalancing of pricing. And another thing one must realize that the human component on high-end test is much higher, routine tests are highly automated, machine driven, that's why you see a lot of competitive intensity on routine, you don't see that much on high-end. How we can do it, yes, this is our first attempt in February this year; we will gradually sort of wait and watch and see how we go forward, but I think one should be able to correct it on a period basis.

Aashita Jain : My second was on the industry growth in general so recently in one of the CRISIL reports they have pegged the diagnostic industry growth at about 5% to 7% for the next five years. And this number was around 10% last year. So, just wanted to know your thoughts on this, how do you see industry growth panning out for say next three to four years? And where organized players like you stand in that?

Dr. Om P. Manchanda : I haven't seen this report, but I would be surprised if the industry growth rate falls to this level, because to my mind Tier-II and Tier-III markets are just opening up from a diagnostic perspective and penetration of diagnostics in this industry on a sort of qualitative basis I can say it's pretty low. So, I would be surprised if this number is true, but I will look up to this report and see what it is. This has come out recently.

Aashita Jain : So, in one of the annual reports of the healthcare major, I think they have reported this number 5% to 7% diagnostic drop .

Dr. Om P. Manchanda : And there is a reference to the CRISIL report, is it?

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Aashita Jain : Yes, so checked with the Company it was from CRISIL that is what I understand .

Dr. Om P. Manchanda : Well, my own personal view is I think it's quite a low ball, I don't think so. While it may not be still as high as it used to be in the past, I think this to me looks low.

Aashita Jain : And just lastly on the volume side I mean there are a lot of questions from that, if I look at your non -COVID sample growth it's somewhere around 6%, which is still lower than what you have been guiding around 8% to 9% and again lower than what the pre -COVID level. So, what is the trend that you are seeing on the volume side, are the volumes picking up gradually ? How should we see this going forward?

Dr. Om P. Manchanda : Actually, volume growth number that we have been talking about is, these are patient visits. And sometimes there is a spike also that happens in volumes, because as we enter into high sort of vector borne diseases like malaria, chikungunya or dengue etc. one patient actually visits multiple times during fever season. And certainly when you have a non -season , these visits tend to drop also very sharply.

So, I would wait and watch, I wouldn't get perturbed by these numbers, but I do believe that there should be gradual jump in these volumes. And one of the analysis that we have done is sequentially how it has done, I think that it has done better, right, from a Q4 to Q1 about there is a jump of 8% in volume and on an CAGR basis for the last three or four years, it's about 6.5%.

I think it's just that Q1 of this year versus Q1 these numbers look very low. So, I will probably hold on to this, but I think as I made an opening comment, I think it's lower than what we expected, so we will have to wait and watch for 2024.

Moderator : Thank you. The next question is from the line of Vignesh Iyer from Sequent Investments. Please go ahead.

Vignesh Iyer : Just want to understand on the EBITDA side of it, I mean is this number a more sustainable number and can we assume a more sustainable

number going forward
on the EBITDA side of it?

Ved Prakash Goel : So, as I mentioned earlier somebody else has also asked this question, so this is normally first half and we have higher margins, but full year I don't think this kind of margins we are guiding. What we are saying is at least we are confident that we can maintain our pre -COVID level margins for the full year.

Vignesh Iyer : On the tax rate side , what would be a blended tax rate for FY2024 , I mean are we shifting to something like 25% ?

Ved Prakash Goel : Yes, we don't have any advantage , I mean normal tax rate is 25% which is applicable to us.

Vignesh Iyer : No, like we have paid around 29% if I am not wrong for the quarter, right ?

Ved Prakash Goel : That was mainly because of the notional depreciation you have seen, but if you see the standalone, it is about 25%.

Moderator : Thank you. The next question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar : So, firstly my question is with respect to Swasthfit , over the long term what kind of proportion of the total revenue do we target for the 22%, how much of the total blended share could be contributed by Swasthfit?

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Bharath Uppiliappan : So, there can be multiple applications for Swasthfit from what we call a bundled test offering portfolio. We can create bundles for many other disease conditions , we can get faster diagnosis out. So, I think, one way to look at it it's a great thing we will grow this portfolio only.

Second is that say we will vary this and hence try to grow varied portfolio. So, yes, the future looks very bright, I don't want to put a number but between 20% to 25% in the near term should be reasonably possible as per our estimate.

Omkar Kamtekar : So, we are also looking to create other bundled tests so that those also contribute towards the growth, that would be the right thinking?

Dr. Om P. Manchanda : Yes, let's look at from the market perspective there is a need to create this bundling. I think bundling has also become much more affordable now. And doctors are also suggesting LFT/ KFT together which they were earlier prescribing separately, patients also want to do it together . In fact, I have seen request coming even for high -

end test bundling as well and see whether they can diagnose this early.

One which I keep saying the core problem our industry solves are two, first one is we must diagnose it accurately and second, we must diagnose it early. I think accuracy wise the industry has done great job; quality labs have come up. This early piece is being driven by multiple factors it could be genetic testing, it could be bundling, because you can do the entire testing in one go and diagnose it early. And I think patients in both the medical fraternity and patients are seeing great value in this because bundling is much cheaper than ordering individual tests.

Omkar Kamtekar : And lastly just a clarification, in the previous question when you were mentioning the Tier-II and Tier-III growth, I was not able to hear so what was the growth number what you were expecting from this Tier-II and Tier-III?

Dr. Om P. Manchanda : So, I was responding to someone mentioned about industry is going to grow only about 5% to 7% my thing was that I haven't seen the report myself, but my own gut says that it is too low, while one should not see quarterly numbers, but over a long period of time this whole Tier-II and Tier-III market is yet to open up.

Omkar Kamtekar : So, what would be the number if you could just have rough estimate?

Dr. Om P. Manchanda : Very early to put that thing, because the only way I can look at it is that you and I see the same numbers, 1.4 billion people and if you draw a pyramid I think most of this industry is still lying in top end of the market which is metros and mini-metros but it is still to go down, especially from organized industry point of view, while unorganized

sector may still be existing in Tier-II and Tier-III towns, but I think organized industry still has to make lot of influence in these markets. So, it is more a qualitative comment well I don't have any number to which I can.

Omkar Kamtekar : And lastly, so there is one problem you generally face in Tier-II and Tier-III or lower cities that generally patients would only try to go to a diagnostic center where the doctor has recommended. So, you may go to a specific lab or a specific technician to get their report. So, how will this be overcome?

Dr. Om P. Manchanda : No, I fully agree with you, there is a challenge. And I think 20 years back that was challenging even in cities as well. So, I think as gradually more brand awareness picks up, affordability goes up, and people would be willing to pay a brand premium for this as the overall cost component goes down, OPD prices go up. So, I think some of these practices would tend to go away, and then branded players will be able to succeed, that's probably I can, I think it will happen, but you are right there are challenges, this market is not there just for us to grab, just like that.

Moderator : Thank you. The next question is from the line of Rahul Agarwal from Incred Capital. Please go ahead.

Rahul Agarwal : One question Dr. Om was how we analyze retail businesses, we look at same store sales growth, I was just trying to apply that to diagnostics. So, let's say you had a franchisee network of 3,000 roughly at the end of March 2020 so will it be fair to say that that network is still growing on revenue or most of the topline growth which we are seeing is coming from the new networks which was added over the last three years, any analysis there please?

Dr. Om P. Manchanda : It's growing across, in fact you will be surprised our Delhi NCR has shown better growth while Delhi NCR has the same store, because there is hardly any scope to grow further outlets. So, technically if I look at this geography in the same store as a concept it is still showing growth. So, I think it is across, the profile of growth may be different in these markets. The same store growth may be coming more because people are shifting to bundling, people are shifting more preventive while it may be less in other markets, but there is growth happening in both the sectors.

Rahul Jain : I will move on to the second question, it was on cash flow. My sense is our cash profit for the quarter was about Rs. 120 crore as I understand the gross cash you have reported is Rs. 877 and the debt on the books is about Rs. 146 crore. If I do a math net cash is about Rs. 730 so it looks like there is hardly any CAPEX done in the quarter, is that understanding correct?

Ved Prakash Goel : Yes, you are right.

Rahul Jain : And lastly Delhi NCR you were saying that has improved and the growth is better, what's the revenue share here for the quarter?

Ved Prakash Goel : So, it's about 32% overall.

Rahul Jain : Including Suburban right?

Dr. Om P. Manchanda : Yes, all put together it is consolidated one figure, so we are not going to talk about non-COVID separately, one number all companies as one. All put together 32% is from Delhi NCR.

Moderator : Thank you. The next question is from the line of Karan Vora from Goldman Sachs. Please go ahead.

Karan Vora : My first question is on broadly, so you highlighted that from the competitive intensity point of view the market has kind of stabilized. But is the competitive intensity back to pre -COVID levels or it's still high. And from where is it coming from? Is it still the online players or you are also seeing it from the hospital players?

Also, in addition to that, in the Delhi NCR region you mentioned kind of a higher growth, but do we think that now that we have reached a sizable scale in this region, going back to pre -COVID volume growths would be difficult?

Id be difficult?

Dr. Om P. Manchanda : Let me first answer competition from before COVID and post -COVID whether competitive intensity is same or it has gone up, answer yes, it has gone up. So, but definitely it has come down from the recent past, but it has settled down at a much higher level. So, I would say it is a new normal that has settled down. Now where is Page 14 of 15:

it coming from, it came from three sides one was of course this new age players, private equity funded , second was hospitals and third was some of the pharma companies.

So, I think as the new normal is settling down I would put hospitals as number 1 sort of a competitive threat to industry or rather which can change the whole construct.

So, I think hospitals are getting more aggressive. Can they be Pan India, I have my sort of a doubt on that, but obviously in regional or city level, they have brand equity, and they are trying to leverage that which is what has changed.

The second question I think you had was on Delhi NCR growth -- so Delhi NCR growth though it has gone up, but I don't think it's sustainable what you we have seen in Q1 but Delhi NCR being a very sort of a high market share, market for us, so I don't see this growth would be in line with our overall blended growth for the portfolio. So, I think growth contribution from Delhi NCR will be lower than the rest of India as we go forward while a slightly better growth that we have seen in this quarter it will just be an exception.

Karan Vora : Another question on Suburban, so you mentioned in the call previously as to you are in the Phase II of the integration journey, so just if you can throw a bit more color on how many phases do you plan to completely integrate Suburban into and what is the timeline for that, like will it take a year or maybe more than that? And by when can you expect the margins to cross 20% levels?

Dr. Om P. Manchanda : When we acquired about a year and a half back that time our focus was to build Suburban as a separate company, as a separate team and separate unit. And we tried to do that. We have put out certain growth figures, unfortunately we didn't see those numbers coming in. So, essentially the idea was can we just drive topline and not worry about rest of the stuff, but then slowly we realized as the COVID receded we suddenly found the margins came down very sharply than what we had seen during COVID time.

And the growth rate was definitely not as high as what we expected, then we quickly have changed our gear saying that not only does this asset has to give us revenue synergy , but it also we have to look at cost synergies as well. And when the cost synergy started falling in place then we looked at two phases, Phase I was Can we look at our corporate cost of Dr. Lal PathLabs, can that structure provide some overriding support to this asset like Finance, HR, Marketing, IT etc. so that was Phase I.

And Phase II is when we went down to operations, we found that if we look at rationalizing our lab infra we can derive some synergies out of this , so currently we are in that phase. I think the last phase would be when you bring front-end synergies as well like the sales team becoming one and things like that which to my mind will happen as we go along, but right now Phase I and Phase II is the focus.

Karan Vora : And any guidance on the timeline for all of this to happen?

Dr. Om P. Manchanda : I would play it by the year and see how our integration happens, because the integration of lab ops is not that simple, because both the companies operate on different lab installation systems, different bar -coding system. So, I would not put any timeline right, but I would want to get it right the first time , slow and steady.

Moderator : Thank you. The next question is from the line of Dheeresh Pathak from White Oak Capital . Please go ahead.

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Dheeresh Pathak : I came in late

so my apologies, but the way I am looking at the result is that non -COVID revenue grew 10% and the patient count is flat 7 million to 7 million. So, everything is coming from realization increase, now there is price increase and there is mix impact, so can you break this 10% non -COVID increase in realization into

price increase that you have taken on like -to-like and mix?

Dr. Om P. Manchanda : So, I think we have spoken enough on this maybe you were not there when we talked about this, but just to give a split of 10%, 3% is on account of price increase, balance is on account of mix. And if you take out COVID volume then the growth rate is about 2.5% on volume. Volume is if you take away COVID numbers out of that, but since you asked about non-COVID 10% break up right.

Dheeresh Pathak :: Yes.

Dr. Om P. Manchanda : So, non-COVID , whatever the number is to 3% is on account of price, balance is about 2.5% on volume and balance is the mix.

Moderator : Thank you. The next question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar : So, my question is with respect to what will be the next geography for expansion that we are looking for, one? And secondly the cash that is in the books of Rs. 877 crore , will that be used for any inorganic expansion over the next one or two years?

Dr. Om P. Manchanda : I couldn't hear your first question, but I did hear your second one, the cash on the books yes , I think the primary objective is to utilize it for growth, both organic and inorganic. Since our business organically does not require too much capital, we will primarily use this for inorganic and we are still digesting our last acquisition that we did in Mumbai . So, as this stabilizes as the new opportunity comes up, we will always be open to look at inorganic opportunities. The first question I am sorry, I couldn't hear that, you were asking something about Tier II or Tier-III.

Omkar Kamtekar : Which are the regions are we looking to expand in the Western region further or we are going for South region , on the national front which region are we next focusing on for expansion?

Dr. Om P. Manchanda : South and West so there is no doubt, I think every call that we talk about South and West are gap areas for us, we will try and grow in these areas.

Moderator : Thank you. That was the last question. I would now like to hand the conference over to the management for closing comments.

Ved Prakash Goel : Thank you everyone for being with us on this call today. I hope we were able to address your questions.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

Call: Nov 2023

November 08 , 2023

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Conference Call on Financial Results for Q2 & H1 FY24

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q2 & H1 FY24 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above

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Dr. Lal PathLabs Limited
Q2 FY24 Earnings Conference Call Transcript
November 2, 2023

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Bharath Uppiliappan - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer
• Mr. Shankha Banerjee – CEO Suburban and other Group Companies
Participants who asked questions • Rahul Agarwal – InCred Capital
• Rishi Mody – Marcellus Investment
• Prakash Kapadia – Anived Portfolio
• Aneesh Deora - Nomura
• Nitin Agarwal – DAM Capital
• Lavanya Tottala – UBS Securities
• Jainil Shah – JM Financial
• Karan Vora – Goldman Sachs
• Punit Pujara - Helios
• Rahul Jeewani – IIFL Securities

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Moderator Ladies and gentlemen, good day and welcome to Dr. Lal PathLabs ' Q2 FY24 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference, please signal an operator by pressing (*)then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Nishid Solanki of CDR India. Thank you. And over to you, Nishid.

Nishid Solanki : Good evening , everyone and welcome to Dr. Lal PathLabs ' Q2 FY24 Earnings Conference Call.

Today we are joined by Senior Members of the Management Team, including (Hony) Brig. Dr. Arvind Lal - Executive Chairman; Dr. Om Prakash Manchanda - Managing Director; Mr. Bharath Uppiliappan - CEO; Mr. Ved Prakash Goel - Group CFO along with Mr. Shankha Banerjee - CEO of Suburban and other Group Companies.

I would like to share our standard disclaimer here. Some of the statements made on today's conference call could be forward -looking in nature, and the actual results could vary from these forward -looking statements. A detailed disclaimer in this regard is available in the results presentation which has been circulated to you earlier and also available on the Stock Exchange website s.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspectives.

Dr. Arvind Lal : A very good evening and a warm welcome to everyone present on the call. We are here to discuss Dr. Lal PathLabs ' Q2 FY24 Earnings. I would like to take you through the progress that we have made, and initiatives outlined to sustain our performance trajectory.

Dr. Lal PathLabs has been meeting diagnostic requirements of the nation for several decades. And next year we shall be celebrating 75 years of our existence. Consequently, both patients and physicians regard our brand as a trusted healthcare partner. Toda y we are known for Quality and Accuracy, Accessibility and Affordability to meet the testing needs of all our patients and referring doctors throughout the country.

Our initiatives towards developing a model of Excellence and Diagnostics have not only earned us numerous accolades but have also helped us gain the confidence of the people of India as underlined in our new marketing campaign, "Bharat Ka Vishwas."

Our distinctive grasp of intricate market dynamics and omni -channel presence sets us apart as we aim to branch out in underserved Tier -III and Tier -IV markets. This along with sharp focus and leveraging a strong digital infrastructure will differentiate us as we achieve major milestones going ahead on a network rollout. We have implemented a new custom -built logistics solution and this product with enhancements will help us to serve our customers better.

The expected synergies between the two brands, Dr. Lal PathLabs and Suburban Diagnostics , are becoming stronger. And we now have a stake of significance in

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the western market. Suburban performance is improving day -by-day, and especially after the opening of the Reference Lab in Mumbai. The approach here is to combine our capabilities and serve as many patients as possible to offer them super -specialty, high -quality tests at affordable prices with quick turnaround. Our next phase of growth will be led by a combination of factors. The continuous shift from unorganized to organized is one of them. This will be supported by meticulous execution of strategy to further add to our network scale and by elevating our servic e standards through best -in-class operating processes and technological advancements.

The diagnostic sector in India exhibits substantial prospects for future development. And I promise that Dr. Lal PathLabs will be at the forefront of this opportunity as consumers move towards more reputable brands that offer

enhanced quality and best-in-class testing experience.

Thank you very much. That concludes my initial thoughts. I would now like to hand over the floor to Dr. Om. Thank you. Over to you, Om.

Dr. Om P. Manchanda: Welcome everyone to Dr. Lal PathLabs Q2 FY24 Earnings Call. I will talk to you about the Company 's strategic priorities and throw some light on the current business trends.

At the outset, I would like to mention that COVID and COVID allied contribution to the business has fallen now just 2%. Therefore, in our headline figures, we are not

reporting it separately. To further enhance the understanding of the overall P&L, we are also moving away from the term 'Normalized EBITDA' and will only be reporting one EBITDA figure as per IndAS that is net of ESOP and CSR expense.

Key Business Trends

For the last four quarters, LPL consolidated revenue growth trends both in value and volume terms are showing a healthy and steady rise. Last time we crossed the milestone of Rs. 600 crore in a quarter that was Q1 of FY22. But that was primarily driven by a 36% contribution coming from COVID and COVID allied tests. In the current quarter again, we have crossed this milestone, but this time COVID and COVID allied contribution is just under 2%. On total revenue basis, four years CAGR trend, that is from FY20 onwards for the current quarter, the growth rate is 13.2%.

Our strategic focus continues to be on similar lines, and I will just mention some of these areas that we are focusing on.

I) I think the biggest focus for us is to go deeper. As you all know that we have a strong brand franchise in Northern Eastern market. And there is a tremendous amount of focus on Tier -III and Tier -IV towns and go wider in Southern and Western markets.

II) The second priority for us is to enhance productivity and service levels to maintain a sustainable competitive advantage. And as you know, that as a team, we continue to look for running processes which are more efficient and that show up in our margins as well.

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III) We are driving participation of our partners like Collection Centers to drive growth. As you know, the contribution of Collection Center business over a period of time has now become 50%.

IV) We continue to have an enhanced focus on high -end tests in our portfolio.

V) Continue to build 'Suburban' in key markets like Mumbai, Pune and Goa.

VI) Keep focus on Preventive Healthcare Tests portfolio that is Swasthfit and LPL as well as Preventive Health Check Ups portfolio in Suburban.

VII) And lastly, leverage technology to drive process efficiencies and drive marketing programs.

That's it from my side. Thank you. And over to Bharath now to continue the discussion.

Bharath Uppiliappan : I warmly welcome you all on this call today. And wish you and your family members a joyous and healthy festive season ahead. I will now take you through the business and operating highlights.

Operating and Business Highlights of the Company

I am pleased to share with you that we delivered a robust quarter of revenue and profit growth while making good progress on our strategic growth agendas.

In Q2 FY24, we achieved a revenue of Rs. 601 crore, a growth of 12.6% in total revenue over Q2 last year. Net of RT -PCR test, our growth of revenue for Q2 FY24 is 14.4%.

In Q2 FY24, we served 7.5 million patients representing a growth of 5.2% over last year, and net of RT -PCR the patient growth for this quarter is 7.7%. You will notice that our patient growth is significantly higher than our Q1 FY24 numbers.

In this quarter, we tested 21.2 million samples representing a growth of 11% over last year. And net of RT -PCR, the sample growth for Q2 FY24 is 12%.

The RPP for Q2 FY24 is at Rs. 798, a growth of 1% over Q1 FY24 that is sequential, mainly due to mix.

It is also pertinent to point out that Suburban Diagnostics registered an encouraging

start

to its growth journey by registering a growth of 10.4% excluding RT -PCR in Q2 FY24.

The strong performance in Q2 FY24 is mainly on account of five key factors:

(1) Market activation and execution across all geographies including that of Suburban. Our D2C program has begun to gain traction and so has our key account management program

(2) Our investments in 35 hub labs across the country are bearing results now. And this has enabled us to process samples with better tech driven by automation.

This has enabled us to gain market share in these markets

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(3) Our expansion program in Tier -III Plus towns continues to show encouraging results. On the back of this response, we are planning to accelerate this journey

by opening 20 plus labs in Tier -III plus towns. This will obviously be backed by a strong collection center network expansion and market activation program
(4) As you all know, we had launched a marketing campaign "Bharat Ka Vishwas" with the aim of fostering trust and convenience for both doctors and patients. I am glad to share that we have started witnessing favorable outcomes, further solidifying the trust in our brand among healthcare professionals and patients
(5) Our product portfolio focus continues to do well. 'SwasthFit' together with ProSelf at Suburban Diagnostics delivered a robust growth of 25% during the quarter, and it currently constitutes 21% of our total revenue. The revenue from this portfolio bundle test during Q2 FY24 is at Rs. 125 crore enroute to Rs. 500 crore annual run-rate revenue

Our focus on growing super-specialty business across various therapies and investments in building digital infrastructure has played a key pivotal role in driving incremental volumes and attracting more patients. During this quarter, we have gone live with our new custom built, feature rich logistics app.
On the P&L side, our continuous efforts towards optimizing operational efficiency across all cost line items and reimagining the value chain has led to an improvement in profitability and gives us the headroom to invest in growth opportunities.
Overall, we are moving the right levers to optimally set a growth trajectory that will give us sustainable growth.

With that I would like to invite Ved to take you through all the financial performances. Over to you, Ved.

Ved Prakash Goel : Good evening, everyone and thank you for joining this call today. I am pleased to announce that we have reported robust performance this quarter, sharing some of the key highlights for the quarter's first half.

Key Financial Highlight for the Quarter

Revenue for Q2 FY24 came in at Rs. 601 crore against Rs. 534 crore last year same quarter, a growth of 12.6%. In the first half FY24, total revenue is Rs. 1,142 crore Vs. Rs. 1,037 crore last year, a growth of 10.2%. Revenue realization per patient for Q2 FY24 is Rs. 798 as against Rs. 746 last year in the same quarter, an increase of 7% led by price increase, test mix and higher contribution of SwasthFit. EBITDA for Q2 FY24 came in at Rs. 178 crore as compared to Rs. 144 crore in Q2 FY23. EBITDA margin for Q2 FY24 is 29.6% Vs. 26.9% in Q2 last year. In first-half FY24, EBITDA is Rs. 324 crore Vs. Rs. 261 crore same period last year with a margin of 28.4% Vs. 25.2% last year same period.

PBT for Q2 FY24 came in at Rs. 152 crore Vs. Rs. 103 crore in last year same quarter. PBT margin is 25.3% for Q2 FY24 against 19.3% for Q2 FY23. In first-half FY24, PBT is Rs. 270 crore Vs. Rs. 184 crore last year same period with a margin of 23.6% against 17.8% last year.

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PAT for Q2 FY24 came in at Rs. 111 crore Vs. Rs. 72 crore in Q2 FY23. PAT margin is at 18.4% for Q2 FY 24 against 13.6% for Q2 FY23. In first-half FY 24, PAT is Rs. 194 crore Vs. Rs. 131 crore last year with a margin of 17% against 12.6%.
EPS in Q2 FY24 is Rs. 13.2 Vs. Rs. 8.6 in Q2 FY23. With this, EPS

for first-half

FY24 is Rs. 23.1 against Rs. 15.6, last year.

As mentioned by Dr. Om, we are simplifying the reporting of profit numbers and accordingly all the above profit numbers are after CSR, RSU and Notional Depreciation because of consolidation of Suburban.

Net cash and cash equivalent as on September 23 is Rs. 780 crore.

Inventory holding is now at 31 days. And debtors outstanding is 14 days of total sales and 28 days of credit sales.

ROCE for the year is at 25.3%.

Total CAPEX for first half is Rs. 27 crore, primarily on account of new infra and investment in technology.

Once again, our continuous efforts toward improving operational efficiency has led to improved profit margins and thus provide greater opportunity to invest in underpenetrated markets, brand awareness and future technologies.

That brings me to the conclusion of my opening remarks. And I would now request the moderator to open the forum for Q&A.

Moderator : Thank you very much. We will now begin the question-and-answer session.

The first question is from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal : I have three questions, two on business and one more structural. Firstly, the new slide which you put in the presentation on Tier-III markets. Just wanted to know,

how do you define Tier -III markets and below, in your mind, how do you classify that? And if you could elaborate a bit more on the accelerated program for these markets, because my sense is, as revenue share increases and centers take a bit of time to ramp up how does the incremental margins look like?

Bharath Uppiliappan : Mr. Rahul, I didn't get your second part of the first question, but the answer of Tier - I, Tier -II, Tier -III as per Government of India classification, as defined for I think Census and HRA and so on and so forth. So, we just follow the same classification.

Rahul Agarwal : The second part of the question essentially was, you mentioned about 20 plus labs getting opened into the Tier -III markets, just wanted to understand as the center when these labs ramp up, incremental margins how should we look at that, because I think if you do that and the revenue share further increases from here from 34%, how do the consolidated margins behave?

Dr. Om P. Manchanda: I actually would say that this is a typical business model which we have always followed. We open 15 - 20 labs every year and each lab that we open it further proliferates Collection Center and pick -up point in that area. So, I would say that it's not going to make any difference, this kind of model is already baked in into our

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margins. However, I do take your point that these centers would be in some of these Tier-III towns, but I really don't see materially margin getting impacted, because some of these places our cost structure is also equally very competitive. It's much lower than what we normally end up incurring in metros. So, if at all, there is something in the short term, but materially I don't think it will change the overall trajectory of the margins.

Rahul Agarwal : Secondly, on the revenue per patient I think obviously pricing mix -wise Swasthfit and all have contributed, but anything from the Delhi NCR market looks like your core must have done better this quarter. Is that true and how much was Delhi NCR overall business?

Dr. Om P. Manchanda: I think Q2 as everybody knows that it's a high sort of fever season. And fever is actually all across. We have seen the entire, this year, we have seen across India itself, but most of it is from Northern markets like UP, Bihar Delhi NCR included. So, it is from everywhere, especially September month. So, that jump has come all across.

Rahul Agarwal : And how much would be Delhi NCR now?

Dr. Om P. Manchanda: Delhi NCR contribution this quarter is 31%. Actually this, it used to hover around 34%. And now this year we have seen UP

and Uttarakhand markets have done far better. But we have seen growth everywhere.

Rahul Agarwal : And lastly this cash again is getting accumulated about Rs. 800 crore, I think will make much more money in the second half. Any thoughts, how do we deploy? I know you have been looking for assets into South India, you have been mentioning that in prior calls, but any thoughts?

Dr. Om P. Manchanda: Yes, I think our view on cash utilization has always been, is a three -pronged one of course we continue to invest in the business. Organically that requirement is not very high. We will continue to pay dividends as per our policy. And of course, we will keep scouting for acquisitions. But we probably would be a little more sort of value -driven and unless we find quality assets at the right price, we probably will be careful doing acquisitions, of course in order of priority South is definitely number one because the contribution from this market is the lowest. So, that's the way we will look at it. Right now , there is nothing that we can share with all of you.

Moderator : The next question is from the line of Rishi Mody from Marcellus Investment Managers. Please go ahead.

Rishi Mody : So, a quick bookkeeping question, first, could you just tell me what is your non - COVID revenue the way you all used to report excluding RT -PCR, COVID allied and D -dimer and all those tests?

Dr. Om P. Manchanda: It's Rs. 589 crore, out of Rs. 601 crore .

Rishi Mody : So, Rs. 12 crore is COVID revenue this quarter.

Dr. Om P. Manchanda: Non-COVID is Rs. 589 crore and COVID and allied is Rs. 12 crore as we were reporting it earlier.

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Rishi Mody : Dr. Om, in last quarter's call you mentioned that you are all expecting a smart recovery by the end of this year. Could you elaborate on what you mean t by smart recovery, and what growth are you all looking at?

Dr. Om P. Manchanda: See recovery in the sense that we were, I don't know what I mentioned last time, but recovery, essentially, I mean on volume growth as well as value both together.

And if you see the last four quarters' trends we were in a negative trajectory for volume. Now, we are actually coming into a very positive trajectory. And if I take out the RT-PCR volume, I think volume on non-COVID, pure non-COVID looks better. And we are hopeful that in times to come this will further go up. And in value terms also we have seen trajectory moving up. Of course, it's contributed by little bit due to price increase as well, but that is what I probably mean that in the growth rates are slowly inching upwards.

Rishi Mody : And like how much do you think we will be able to take this notch -up in terms of volume value?

Dr. Om P. Manchanda: See value as I mentioned in my opening comments, if I look at the last four years CAGR, we are now seeing about 13% growth for the quarter. There was a time when it had fallen below this number, I think in the early teens, we are now almost getting that number, I would say.

Rishi Mody : My third question is to Bharath, Bharath in the PPT comment that you have put in, you mentioned that Suburban has begun gaining traction. Could you shed more light here about what has happened over the last year? What are you seeing today which is making you comment that Suburban is gaining traction?

Bharath Uppiliappan : So, I think a lot of action has happened over the last one and a half years or so which includes stabilization of asset, customer base re-confidence gaining. In fact, last quarter, I think when we had met we kind of shared with you that we have put up a very aggressive marketing program in the city of Mumbai, led by ATL Digital and the full suite on that. And I think this quarter we followed up with a very strong below-the-line activation program as well. So, a lot of what I would call efforts are being taken on building the Suburban brand in the western market. And we are seeing t

he first green shoots of the growth trajectory for the business.

Rishi Mody : What do you think how big is it going to become over say one to three years as part of Lal's revenue, if you had to put a number there?

Dr. Om P. Manchanda: Maybe just to continue with the earlier question of yours. I think two – three fundamental shifts that have taken place for us in Suburban, number one Suburban as our business was more own asset driven, I think slowly, we have shifted towards a franchisee driven business. That is one change that has happened.

We also went through a bit of high employee turnover, which also has stabilized quite a bit. And that is what Bharath meant by a lot of effort has gone into stabilizing the ship as it changed hands.

I think the most important thing has been that we piloted few marketing programs and we are seeing some response to some of these things. So, we know fairly reasonably well know that what really works and what doesn't work.

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Now coming back to your second question, it's very clearly right now three-market approach Mumbai is number one, and second is Pune and then third is Goa. And you know that within Maharashtra, Mumbai is the largest market and there is no published data, but I think it's a fairly big market. And if we get our formula right, that's where we presume that we should actually be doing well. And for us, it's a very strategic market because our LPL presence in the Western region has been less. So, if it really works well, hopefully Suburban should become a meaningful sort of contributor to the total business.

Rishi Mody : And we are giving up the Madhya Pradesh market, if I remember Suburban had some operations there as well.

Dr. Om P. Manchanda: Yes, so I think there were one or two labs. Indore, Jabalpur and Rewa, I think that we are giving up, because it doesn't make sense for Suburban to put effort there because LPL is fairly strong in these markets. It is just a maintenance sort of thing, but really investment is primarily in these three cities and whatever spillover in rest of Maharashtra is.

Moderator : The next question is from the line of Prakash Kapadia from Anived Portfolio Managers. Please go ahead.

Prakash Kapadia : Dr. Om, if I were to take a mid-term view given that most of the competitive intensity seems behind us, would it be right to say there could be a 10% patient volume growth and with a bit of product mix and realization change, we should be able to grow revenues at 14% - 15% over the next three to four years, is that the right way to envisage the future?

Dr. Om P. Manchanda: Prakash, I wish I could give these numbers. I would hesitate to actually talk about some guidance. What all I can say is that as an organization we are fairly well spread out across the country. And as I mentioned in my opening comments that we want to go deeper and wider, I think this strategy of going deeper in to the

Northern Eastern Market is on its way. Today, there are 75 districts of UP, a very large population I think 22 - 23 crore of population we are number one and way ahead of the number two guy is very far behind us. So, as the country's economy grows, and especially UP, we are very well placed to actually gain a lot of market in these places, including Bihar, Orissa, the rest of North, you name I think our spread is probably one of the best. And I think we know what is really required to really go deep, that's one thing that we are doing.

Second is we continue to put effort to get wider and wider. I think as our footprint grows --. And the second thing is also we are trying to look at the business model which remains competitive so that we don't resort to, and our cost structure is competitive so that we can compete on price, etc. While some of this intensity may have slowed down a bit, but we still believe that intensity will continue to remain because this sector has always been

competitive from unorganized sector, smaller

labs it's just that now the organized competition is growing. So, the market will continue to grow because as evidence-based medicine grows, preventive health check-ups grow I think the market will continue to grow, we just have to look internally and see that our competitive advantage remains intact. And then hopefully, we should be able to achieve what we have set out for ourselves.

Prakash Kapadia : This time around you have given the contribution from Tier -III and beyond in terms of revenue contribution. So, is it fair to say North and East would be a large part of

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this Tier -III revenue. And could you take us through the journey in these Tier -III cities and beyond. Do you have basic packages? Is it largely B2B? Or it is Swasthfit and awareness seems to be increasing there also?

Bharath Uppiliappan : So, Prakash this Tier -III story is not new, we have always talked about this, I think for last more than two years in my recollection. There have always been constant efforts put up over here, on this front. In these Tier -III markets what we have is a classical suite, which we do is to first to start to put Collection Centers. As we build Collection Centers, we begin to realize what kind of tests sell etc., which are predominantly routine. But also, what we are now realizing is slightly higher in tests also come in, especially in Gynae and infectious diseases kind of portfolio. And, as we densify these clusters, we then identify the distance from the nearest other lab we have and start to populate those. And that is the reason why we arrived at this number which we just talked about, of putting 20 plus new labs in these towns, because we already have a Collection Center network, and this allows us opportunity to further densify and go down deeper as Dr. Om mentioned in his opening speech.

So, I guess this we have been doing for quite some time, it is just that it is coming to the fore from a sizable opportunity perspective now and we are talking more about it.

Prakash Kapadia : And would it be fair to say, B2B is still a larger portion, but now we are seeing traction on the consumer side.

Bharath Uppiliappan : So, Prakash, we have talked about this many times in the past, we continue to believe we are a n omni -channel player, we are not restricted to B2B, B2C, we will pull in all levers because there is an interplay between all the channels. A B2B patient today may walk in B2C tomorrow; B2C may go to a hospital tomorrow. So, it's important for us to kind o f do an omni -channel play and not just be focused on one or the other channel. So, given geography, we look at the healthcare ecosystem and the patient or the consumer ecosystem and try and build our offering around that.

Prakash Kapadia : So, it's a mix of both in this Tier -III and beyond.

Bharath Uppiliappan : Yes, of course there can never be either only B2B or only B2C .

Prakash Kapadia : My worry was, it should not be B2B driven, because that seems to be more price conscious .

Bharath Uppiliappan : No, so we obviously manage the mix judiciously.

Prakash Kapadia : And lastly, on Suburban is there an app launch planned in Suburban? Have we taken some pricing changes, if any in suburban to grow in Western and specifically Mumbai, because there seems to be a lot of consumer campaigns which we see in and around Suburban now as compared to earlier. So, some road ahead for Suburban, if you could share.

Bharath Uppiliappan : So, I think we have just begun the journey on building or let us say further strengthening the consumer franchise and the patient franchise along with the doctor network, which will support Suburban. So, it is going to be a long journey, but we are committed to making that happen. So, once again, in the case of the

focus market for Suburban, the full suite of omni-channel play will be put into say wh

at you are seeing is evidently what is apparent in the outside which is the B2C plan, but there is also a strong B2B program also being put in play.

Moderator : The next question is from the line of Aneesh Deora from Nomura. Please go ahead.

Aneesh Deora : Regarding the Dengue related cases, so the news article says that the Dengue outbreak in this particular season was much more pronounced and more so in the Delhi NCR and the North India region. So, would you be able to sort of quantify the Dengue -related revenues that let's say Dr. Lal would have seen in this quarter as against the same quarter in the previous years, any quantification there would help.

Bharath Uppiliappan : So, on Dengue, I think we saw September month having a larger kind of play. And we saw this not only in Delhi NCR, but across North and East geographies. This has been a past trend as well. However, this time what we did was, further, you know, we have this bundle portfolio on fever panel also. And that kind of really took off this season, because we had a panel, one for three fevers and one for five fevers. And this is really what caught people's, the patient's imagination. Because at one go, they could have got tested for the relevant fever without having to go through in a sequential format which they were doing in the past. So, bundling here again, helped us.

And I also talked about the hub lab program so most of our hub labs are equipped with ELISA testing, which is the gold standard for Dengue testing, Dengue . I think we leveraged that infrastructure, the Collection Center, the new Digital Logistics app to kind of significantly improve our service delivery in the marketplace.

Aneesh Deora : Could you may be as a percentage of revenues what were the Dengue related revenues in this quarter? Could you just maybe throw some light there?

Dr. Om P. Manchanda: We have not been separately giving these numbers. But for your better understanding, let me tell you every second quarter of every year has a fever season. So, whatever jump you see in this quarter, one would have already seen the last year also same quarter. But still for you to know, what is that jump, normally, I have seen that variation is maybe half a percent or 1% plus, minus on a total quarter figure. So, while the season is very high, at times this is not more than 1% extra or lower or higher that you see. Exact numbers, we normally don't share it because it creates more complexity about these figures, but otherwise, it is there last year also in the same quarter base as well.

Aneesh Deora : Secondly, I was looking at the gross margin levels. So, the gross margin for Q 2 has come to 79.6% which seems to be a historically high number, I mean, the highest. So, we just wanted your thoughts around what is driving this gross margin, are price hikes helping this or are there any softening of raw material prices also. And how should we sort of think about this number going forward? What would be the sustainable levels?

Ved Prakash Goel : So, this is both in fact, one is, of course, what you are saying is price impact and second operational efficiency has also led to this higher gross margin, because the consumption cost which was like suppose earlier was in the range of 22% now came down to 20% or so. So, this is because of certain tests mix also and operational efficiency like Bharath mentioned hub labs and where we are consolidating, and a lot of operational efficiency has come in consumption so both

price as well as operational efficiency. And I am hopeful that this consumption cost is stabilizing here.

Aneesh Deora : So, would these 79% - 80% odd gross margins be the sustainable margins to work with, going forward?

Ved Prakash Goel : So, let's suppose, if we have to remove the price impact then obviously this will be higher than earlier.

Dr. Om P. Manchanda: I just want to add one more thing to this questi

on, I think one is probably what Ved is saying there maybe because volume growth is not that high as a value growth so that may be helping this margin as well. Second, mix plays a very important role, we have usually seen in Q2, our mix is very drifted toward routine test where the gross margin is generally very high. So, as you move into Q3 and Q4 routine tests actually will drop, and high -end tests actually go up where the gross margins are not that high. So , don't take Q2 gross margin as a representative number for the year. That's the only request we have. I think what you should look at is what the

annual figure would be, that maybe a representative figure.

Moderator : The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : On the gross margin level just sort of going back to the question. Now, how much further scope do we have with respect to whatever, there is obviously a meaningful reduction we are getting in gross margins this year given the way trends are. I mean, do we have a meaningful scope still left to further squeeze out efficiencies in the gross margin level?

Ved Prakash Goel : No Nitin, so as I mentioned that we are looking that this is stabilizing, I don't think you should factor any further improvement on this. So, whatever we are showing now it is not even representative for the year, as Dr. Om mentioned, because there is some mix change in this quarter which is impacting this gross margin. So, on a sustainable basis I think we should say about 74% to 75%, kind of margins.

Nitin Agarwal : And secondly, on the overhead costs given the fact that we made a lot of references, we are focusing on Tier -III markets on a growth basis going forward. I mean, what implications does it have for our operating costs? I mean, obviously, lot of the growth is going to come from the franchise route, the fee -for-collection as a percentage, probably at around 14% continues there. But how should we think about the other overhead costs when we try to go out in the smaller areas?

Ved Prakash Goel : I think as Bharath mentioned earlier see one is this channel obviously, we have higher contribution. Now 45% -46% of business is coming from the CC. And obviously as a percentage it is inching up and so as a percentage of this revenue share is going up. But this is not because we are increasing the margins of the CC. This is because of mixed change I mean the contribution of these channels.

Dr. Om P. Manchanda: I will probably take a little time on this question, because it's important to answer this. See if you open a lab in a Tier -III town broad cost of running that lab would not be my sense is Rs. 50 to Rs. 70 lakh in a year. If we are able to generate Rs. 2.5 crore of turnover and 30% of that is roughly Rs. 75 lakh is what normally we will end up paying to a Collection Center if we open there, that's our normal way of

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growing it. So, the moment we open a one lab in a Tier -III town, the moment you reach 2.5 you are virtually breaking even in terms of Collection Center Vs. the lab. So, to my mind, as I was answering Rahul's question earlier in the short term you might see a bit of a change up and down, but over a period of time, it is a way of growing our business. But what this lab would do for us is that it will help us to open further, more Collection Center as we go deeper in Tier -IV towns. So, for us, it's a way of life, I don't think one should look at it as a dilution of the margins.

Nitin Agarwal : The other point essentially is that in terms of when we look at the overall cost structure, we have done a phenomenal job over the years including this year, the way trends are in terms of managing our costs. While I presume will be a continuous exercise in the firm to keep out efficiencies I mean, I am just wondering, how much further scope do we have on an overall level to take out further cos

t efficiencies?

And the reason why I talk about that is because in the past we have talked about EBITDA margins essentially stabilizing at about 25% - 26% levels. We seem to be now again trending reasonably higher from beyond those levels given where H1 has been.

Dr. Om P. Manchanda: That's mainly because of operating leverage, the moment you touch a number of Rs. 600 crore your EBITDA margin will improve. So, I think as a team you have to find out ways and means of increasing top -line, rest everything will fall in place.

Ved Prakash Goel : Just to add Nitin as I mentioned in my opening speech maybe this is the opportunity where we can invest in the business. And even if we are improving efficiency, operating leverage is coming so, it is giving opportunity where we will do the investment in the business, whether it is technology, whether it is newer geography and so on so forth. So, I don't think this 29% or 28% EBITDA margin is a representative margin for the year.

Moderator : The next question is from the line of Lavanya Tottala from UBS Securities. Please go ahead.

Lavanya Tottala : Just one thing on the margins which you were discussing. So, now that with increasing share of bundled services like with improving sales per patient, shouldn't our margins be better than what it was before with operating leverage, it should sustain, right.

Ved Prakash Goel : As I mentioned this, maybe because operating leverage is very high in our business as Dr. Om mentioning Rs. 600 crore revenue will always give very high margins. But this is again an opportunity for us whenever we are getting improved margins

we will invest back into the business. So, I don't think we should factor in these kinds of margins for a longer period.

Lavanya Tottala : So, the sustainable levels should be at what level?

Ved Prakash Goel : I think around 26% we should see that these are more sustainable margins.

Lavanya Tottala : I was just asking because with increasing bundled services even in this quarter with a fever and all the bundled services will give us better operating leverage. So, that's why I was checking if the sustainable levels will be higher?

Dr. Om P. Manchanda: Actually, bundled services generally now is stabilizing at 21% - 22% of the total revenue. So, I think it will probably stay here for some time is what my sense is.

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Lavanya Tottala : Just can you help me with the revenue and margin numbers for Suburban this quarter, I don't know if it was earlier covered.

Ved Prakash Goel : Suburban revenue for the quarter is Rs. 42.7 crore , total revenue for the quarter.

Lavanya Tottala : This was last year, how much?

Ved Prakash Goel : It was Rs. 40 crore total, but if we out COVID out of that, it was Rs. 37.5 crore.

Lavanya Tottala : And margins?

Ved Prakash Goel : Margins is around 13% but again, as we always mention that right now the focus is to improve top -line and invest in the business. Obviously, operational efficiency or some of the backend synergies are coming so gradually that margins will improve.

But right now, the focus is on the top line.

Moderator : The next question is from the line of Jainil Shah from JM Financial. Please go ahead.

Jainil Shah : My first question is about the competitive intensity. So, is there some more room for improvement in the current scenario? And can you throw some more light on what trends we are seeing? And especially how is the competitive scenario from hospitals in the Northern region? Also, if you can talk about the scope for further price hikes over the next two to three years?

Bharath Uppiliappan : I think the competitive intensity, you know, if you cut it in multiple segments there are varying kind of degrees. So, if you look at online players obviously the extra burn, it seems, has come down significantly and they have also taken up price increases. So, both A & P for them has come down in my assessment.

nt. But they also

moved up pricing and some of them have publicly talked about the same.

If you look at our regular competitors, I think we all continue to be as competitive as in the past, if not more. And I think that is a way of life for all of us. So, I think what you must do is to address competitive intensity segment by segment and not just take an overall macro view, but yes, the online intensity has come down a tad and we are also improving our mix and our offering. Our D2C program has really done well. And we continue to gain traction on our Direct -to-Consumer program as well.

As far as price increases are concerned, at this stage we don't want to make any comments on that. We are comfortable where we are, when we are focused on driving the volume growths up. And we will figure this out if and when we have to, but our current focus is on driving volumes are very clear .

Jainil Shah : Just coming back on the competitive intensity side. So, which particular segment is hurting us the most? And what would be the current share of revenue from aggregators to our total top -line?

Bharath Uppiliappan : Our share of revenue from aggregators is very minuscule , single digits if at all or lower single digit at the very best not significant at all.

Jainil Shah : Which particular competitive segment would be hurting us the most or has impacted us over the last one or two years?

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Bharath Uppiliappan : In the last two years back I think if you look at a two year history in the past then online was very clearly the one which was really kind of what I would call giving us a bit of a run, because we were also not prepared for that kind of scenario. But if I look at today, we are very well equipped internally as a team to be able to handle some of these challenges. So, one part is about competitive intensity but the second is about organization's learning ability to be able to implement stuff, which will protect us in the future as well. On that metric, we have moved up significantly.

Dr. Om P. Manchanda: I would also add one more thing to what Bharath just said, I think more than hurting what has happened is that this new age player actually were bringing sort of mega trend the way customers were availing diagnostic services. While we had started home collection, but they started using tech to really order booking, paying online, etc. I think that 's something in the last three years, our teams have done a

tremendous work to really catch up. So, we are now fairly well tech company like any other company. So, I think that is one change that I have seen in the last three to five years. But otherwise, yes, the cash burn and competitive intensity in terms of advertising that has always been there, but now to some extent is moderating.

Jainil Shah : My second question is on the EBITDA margin, you are guiding for around 26% sustainable EBITDA margins. With operating leverage playing out in Suburban as well as Dr. Lal isn't it pretty conservative, because for the last probably two years we have been having digital spends as well. How much can the new lab investments probably offset the operating leverage? So, basically, how much reinvestment would there be in the business?

Dr. Om P. Manchanda: There are inflationary pressures, there is a cost of hiring, growing up, a lot of tech talent is required in the system, so all other costs are also going up. And I think many of you have asked this question as we go to Tier -III, Tier -IV towns, what happens to the cost structure, of course, that will have an impact and, in any case, we have not resorted to price increase. So, I rather would maintain the competitive advantage in terms of pricing and operate within range bound margins so that we are able to actually address a larger sort of a market segment. And not always resort to price increases.

So, our attempt has always been to stay very competitive, run the busi-

ness

efficiently and see that we are able to address all market segments. So, that's also driven, it's not that we can't increase margins, but I think it's important to see that can this Rs. 600 crore number move up very sharply when we are well positioned to gain much higher market share, because this industry is so fragmented while we may be the India's largest diagnostic company, but our market share still would be single digit so, how we go up is what probably our focus is.

And by the way, having higher margin you are also attracting more competition. So, you might as well be a range bound and then increase our top -line.

Moderator : The next question is from line of Karan Vora from Goldman Sachs. Please go ahead.

Karan Vora : Basically on volume growth can you highlight what is the patient volume growth for Suburban and Dr. Lal core business. So, just trying to get some sense as to whether most of the growth for Suburban is coming from volumes, or it is also price that is a meaningful factor, that is my first question.

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Bharath Uppiliappan : Our total volume growth for the quarter is 5.2%. However, since COVID is going away RT -PCR test has gone to a large extent. So, if we remove only RT -PCR, our growth for the quarter is 7.7% including Suburban.

Karan Vora : What would be Suburban's volume growth, Suburban base last year to Suburban this year and ex -Suburban the core business volume growth?

Dr. Om P. Manchanda: The numbers are not readily available, so we probably can offline share .

Karan Vora : My second question would be so on margins you previously highlighted to 26% as a sustainable range. So, with Suburban kind of ramping up over the medium term, so basically, should we assume that incrementally over and above 26% you would be investing back in the business or that flow through could still come in and we can probably see year -on-year margin progression to some extent.

Dr. Om P. Manchanda: I think directionally we will invest back into the business. See these fluctuations in the margins actually happen on a quarter -on-quarter basis. And I think, when we see them in Quarter 3 call, you will actually find a different picture. So, that's why, in the first half our margin is generally higher than the second half. So, you will see that trajectory going down. So, I think over the period, on an annual basis, we will try and manage this margin. And then if there is a tendency for it to work, we will invest back into the top -line, because for us growth is most important.

Moderator : The next question is from the line of Punit Pujara from Helios. Please go ahead.

Punit Pujara : Bharath, in your comments in the PPT you mentioned that the Key Account Management program has been showing some traction. So, could you elaborate a bit on that? And while you do that, could you just spell out the B2B revenue contribution of the consolidated topline in H 1 FY24?

Bharath Uppiliappan : The Key Account Management Program is aimed at our pickup point business, which is a B2B business, where we are trying to create segments and then give a solution for each of the segments in a very customized form. And even within those segments Tier -III customers there is an A, B, C kind of classification and give differential service. This has met with a lot of success, because we have been able to solve that particular sub -segment's problem in this very specific manner, rather than try and build an omnibus kind of solution. So, segmentation of B2B clients is

really what the Key Account Management program tries to do. Obviously, this is at a very rudimentary stage. We can go a lot higher on this count and we will polish up this program as we move forward. On the B2B volume, I think the number of contributions is around 25% to 30% give or take.

Punit Pujara : 25% to 30% as a percentage of revenue in first half of this Fiscal Year , correct?

Bharath Uppiliap

pan : Yes, give or take.

Punit Pujara : My second question is for Dr. Om, so it's almost two years now since we consolidated Suburban. So, what would have been the key learnings and some of the mistakes that you would want to avoid going forward while you prioritize inorganic acquisitions especially for the South region.

Dr. Om P. Manchanda: One learning is in one of event one must be careful, don't get carried away, we didn't realize the COVID will suddenly fall so sharply, so I think that's one. I think the second learning is that we must really know what is very strategic, for us

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Bombay, Maharashtra was very strategic. There are times when we get leads from other markets where we are already very strong so we can't afford to pay that kind of valuation that we have done in Western markets, because we are already very strong in the Northern and Eastern markets. Sometimes people tend to benchmark the same value because they expect the same valuation , so one then needs to be careful on that. And I think the third is some of the integration challenges that we faced post the acquisition that we would be really more sort of aware about this and try and prepare it before we do the transaction.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Securities. Please go ahead.

Rahul Jeewani : Now on this four -year value figures which you called out as 13%, if I dissect these numbers on an ex -Suburban basis then our ex -Suburban value figure is somewhere around 10.5% and if I also back calculate the volume growth for us on an organic basis that is still trending around 7%. So, both in value as well as volume figure our growth is still below pre -COVID level. So, pre -COVID FY17 to FY20 our volume growth was trending around 12% - 12.5% kind of a number which was driving a mid-teens kind of growth for us. So, we are still very far off as far as that mid-teens growth on an organic basis is concerned. So, can you call out in terms of why we are still not back to pre -COVID level growth ?

Dr. Om P. Manchanda: Yes, I think you are absolutely right, it's not only for us I think overall industry if you look at, I think the numbers that are in the public domain, so everyone's growth is below what they were trending before. But all I am saying directionally it is improving, but otherwise you are right, we are definitely not in line with what we used to grow earlier, which is true for the entire industry. I think the numbers of other companies that are in public domain same trajectory is there.

Rahul Jeewani : So, any reason in terms of whether organized players, because you have been pointing out that the competitive intensity in the space is coming down, we have taken price increases as well. So, with these two factors working in our favor, still the growth is not back to what we were seeing earlier. So, anything in terms of structurally which could have change or which is changing due to which the growth at an industry level seems to have slowed down?

Dr. Om P. Manchanda: I can give a slightly my own answer based on the experience that I have had. I think this industry has seen a lot of growth for organized players, because there was a shift from unorganized to organized in Metro, right in the last let's say 15 - 20 cities. And a lot of players have come in this area, the market has saturated. Now the next phase of growth is actually going to come from the next layers of town which is Tier - II, Tier -III, Tier -IV. Those towns actually we will see the same sort of thing, but they are in transition because doctor influence is still very high, direct -to-consumer is just taking shape. So, in my view it's maybe a market in transition but overall potential for the country is huge, because we are a 1.4 billion population country. So, my sense is that a lot of us grew up in large metros to start with and the base has grown bigger for many o

f us. So, that's why you will see a slowdown in growth.

And also, inability to take price increase due to competition etc., all that has contributed to a bit of a slowdown. But as we go deeper into Tier -II, Tier -III, Tier -IV towns I am hopeful that this market will again crack open in favor of organized players.

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Rahul Jeewani : Just one related question to that aspect, so you are talking about growth in some of these smaller markets. So, how do you see the healthcare infra in the smaller markets because what we hear from some of the other healthcare players is that the availability of doctors in Tier -III, Tier -IV towns is very limited, due to which if you look at the pharma companies many of these pharma companies approach the smaller markets towards say generic or a retail led distribution business rather than a doctor led approach. So, can the healthcare infrastructure in these smaller markets be a hindrance to your growth in these smaller towns?

Dr. Om P. Manchanda: So, let's start step by step first of all is there a need for healthcare . The answer is a big yes in these markets. Now earlier affordability could have been a challenge, but I think in relative terms as the country's economy grows, affordability is also growing up. I think the challenge would be basically access, I think ecosystem will find its own solution to cater to this market. I have a very strong sense that distribution of health infra is going to go down to pop -strata. I think Government is also giving a big push. I am very hopeful that investment also will go in that area because the market is probably getting ripe for it to be topped.

My other thing is access also will get improved through tech solutions. I see a lot of things that are happening in telepathology, teleradiology, tele -consultation. So, I think a lot of these solutions will come which will provide access to these Tier -III, Tier-IV towns. So, overall, I think everybody is eyeing these markets. So, even in hospitals also I think they may be done with the metros now they are going to look into Tier -III, Tier -IV towns.

Overall, I think healthcare will attract a lot of investment in these. Yes, I agree with the shortage of manpower is probably getting resolved through tech solutions, let's see how it goes but I am pretty hopeful that health infra will grow in these time s pan.

Moderator : Thank you. Ladies and gentlemen, we will take this as the last question for today. I now hand the conference back to the management for their closing comments.

Thank you. And over to you.

Ved Prakash Goel : Thank you everyone for being with us on this call today. I hope we have satisfactorily addressed all your queries. If you have any more questions, please feel free to reach out to us. Thank you once again and a very Happy Diwali and upcoming festivals too. I would now request the moderator to close the call. Thank you.

Moderator : Thank you very much. Ladies and gentlemen, on behalf of Dr. Lal PathLabs that concludes this conference.

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Sub: Transcript of Conference Call on Financial Results for Q3 & 9M FY24

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q3 & 9M FY24 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
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Dr. Lal PathLabs
Q3 FY24 Earnings Conference Call
February 01, 2024

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Bharath Uppiliappan - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer
• Mr. Shankha Banerjee – CEO Suburban and other Group Companies
Participants who asked questions • Prakash Kapadia – Anived Portfolio
• Rahul Agarwal – InCred Capital
• Rahul Jeewani – IIFL Securities
• Omkar Kamtekar – Bonanza
• Punit Pujara - Helios
• Prashant Nair – Ambit Capital

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Moderator: Ladies and gentlemen , good day and welcome to the Dr. Lal PathLabs' Q3 FY24 Earnings Conference Call .

As a reminder , all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference , please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Siddharth Rangnekar of CDR India. Thank you and over to you sir.

Siddharth Rangnekar : Good evening, everyone and welcome to Dr. Lal PathLabs ' Q3 FY24 Earnings Conference Call.

Today we are joined by the Senior Members of the Management Team including (Hony) Brig Dr. Arvind Lal – who is the Executive Chairman , Dr. Om Prakash Manchanda – Managing Director , Mr. Bharat h Uppiliappan – CEO , Mr. V ed Prakash Goel – Group CFO along with Mr. Shank ha Banerjee – CEO of Suburban and the other Group Companies.

I would like to commence by stating the standard disclaimer here. Some of the statements made on today 's call could be forward -looking in nature and the actual results could vary from these forward -looking statements. A detailed disclaimer is also available in the results presentation that has been circulated to you earlier and is also available on the stock exchange website.

With that , I would like to invite (Hony) Br ig Dr. Arvind Lal to share his perspectives.

Dr. Arvind Lal: Good evening , ladies and gentlemen and a warm welcome to all participants on the call. We are gathered here today to delve into the Q 3 FY24 earnings of Dr. Lal PathLabs. I wish to begin by sharing my thoughts on the evolving market dynamics and the progress that we have made.

India 's diagnostic sector is undergoing a rapid transformation . Increasing burden of non-communicable diseases as well as new emerging and existing communicable diseases , increased adoption of evidence -based treatment and a growing emphasis on preventive healthcare diagnostics are expected to sustain the growth momentum . While the competition in this space has increased , the established pan-India chains like Dr. Lal Pat hLabs are strategically positioned to exceed industry growth , progressively expanding their market share compared to both organized and unorganized players. This is possible due to the continuous improvement in scale achieved through our deep penetration in Tier-2 and Tier-3 towns and offering the largest portfolio of tests and services at competitive prices.

Dr. Lal PathLabs has continued to be the preferred choice for customers seeking latest high-end testing and accurate diagnosis , over the last 75 years. Yes , this is our platinum year. For example , we have achieved a milestone in Histocompatibility and Transplant Immunology testing with Next Generation Sequencing or NGS by carrying out 600 runs of HLA-NGS with 27 ,000 samples analyzed for Bone Marrow and Solid organ Transplantation across India , Africa , Bangladesh and Nepal. We have also taken the lead in establishing 'donor derived cells free DNA ' as a non -invasive monitoring test for solid organ transplantation and have recruited 475 patients for post-transplant monitoring of which 222 patients are for kidney , 70 patients for heart and 18 3 patients for lung transplantation.

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To maintain affordability, we have predominantly retained our pricing mix , barring minor adjustments in prices to account for cost inflation in special ity and super special ity tests. Through this , we aim to cater to a larger patient base , solidifying our position as the leading diagnostic brand in the country.

Our performance going ahead will be guided by expanding our presence in the underpenetrated Tier-3 and Tier-4 markets , coupled with the targeted focus on high -potential markets in the west and south region

s. This is in addition to extensively

utilizing our digital infrastructure where we have made investments across AI , data analytics , machine learning etc. Not only will this enhance the overall patient experience and provide tailored solutions but will also accelerate the volume trajectory .

In the western markets, through Suburban diagnostics and especially its Reference Lab we are emerging as a formidable player , aiming to achieve rapid scale by outpacing industry growth. Our synergies are beginning to unfold, and we are on course to fortify our position in this market.

That concludes my initial remarks. Thank you and I would now like to hand it over to Dr. Om . Over to you , Om.

Dr. Om P. Manchanda: Welcome to our call today . I shall walk you through the evolving industry scenario and progress on strategy .

In the current financial year , at an industry level , whatever the impact of Covid -19 testing was on the revenue side has all gone away . We have also experienced the

same, therefore we stopped reporting the split since last few quarters. It is now only one figure that we report, thereby making the analysis easy and simple. Covid -19 also created an unevenness on growth pattern both on revenue and volume. That pattern is now getting smoothed.

Our business has delivered double-digit top-line growth rates on Y-o-Y basis both in the current quarter as well as on YTD basis. We have significantly improved our operating and net margins profile driven by efficiencies of scale and productivity initiatives. Our efforts continue to look for expansion in Tier-3 and Tier-4 towns through organic means of growth. Our teams have carefully identified pockets and the markets that will drive future growth for us.

Industry experienced bundling of routine tests as a new consumer trend. We led this trend and successfully built a brand – Swasthfit. The revenue contribution from Swasthfit has now stabilized, ranging between 19% to 21% of our total revenue. The next wave of bundling, we believe, would be in the specialized test as that could be medically more efficient for clinicians.

Our investments in technology and digital at the infrastructure level, at the front end and in the interactions with patients are also translating into salience. We are pushing ahead with some of the brand and chosen geographies, while meticulously executing a network and test menu expansion agenda. With that, I would like to hand over to Bharath to continue the conversation. Over to you, Bharat.

Bharath Uppiliappan: I warmly welcome you all on this call today. I will now take you through the business and operating highlights.

I am pleased to share with you that we have delivered yet another quarter of double-digit total revenue growth and a healthy margin profile. In Q3 FY24, we clocked a revenue of Rs. 539 crore representing a growth of 10.1% by serving 6.7 million

patients and testing 18.6 million samples, representing a sample growth of 7.1% over last year.

The RPP for Q3 FY24 is at Rs. 807, a growth of 1% over Q2 FY24 mainly due to mix.

Further, to accelerate the patient volume and revenue stream as a team we are sharply focused on putting into play a white space expansion plan. Last quarter, we shared that we have put in place the program for 20+ new labs expansion. This plan is progressing on track and parallel to the creation of collection centers and pickup point ecosystem is also tracking well.

During COVID, a trend of acquiring and serving patients digitally emerged, popularly known as D2C. I am pleased to share that we have in a focused manner developed end-to-end D2C capabilities starting from digital marketing to lead generation to servicing the patients at home. We are now expanding this D2C program to Tier-2 and Tier-3 cities as well.

Our key product programs like Swasthfit contributing to about 20% of revenue. Med

ical centers of excellence like LACE and LCoRD continue to fortify their position amongst the medical community and patients.

In addition, digitally enabled initiatives like CHIPs and recommendation engine continue to make meaningful contribution in enhancing patient loyalty and stickiness. Overall, we are moving the right levers to optimally set a growth trajectory that will give us sustainable growth.

With that, I would like to invite Ved to take you all through the financial performance. Over to you, Ved.

Ved Prakash Goel: Hello everyone and thank you for joining this call.

I am sharing the key Financial Highlights for Q3 and 9 months:

- Revenue for Q3 FY24 came in at Rs. 539 crore against Rs. 489 crore last year same quarter, a growth of 10.1%. YTD revenue for 9 months is Rs. 1,681 crore Vs Rs. 1,526 crore in last year same period, a growth of 10.2%.
- EBITDA for Q3 FY24 came in at Rs. 141 crore as compared to Rs. 113 crore in Q3 last year, registered a growth of 24.6% with EBITDA margin of 26.1%. 9 months EBITDA is Rs. 465 crore Vs Rs. 374 crore, last year same period with a margin of 27.6%.
- PBT for Q3 FY24 came in at Rs. 116 crore, registered a growth of 51.6% with PBT margin of 21.5%. 9 months PBT is Rs. 386 crore with a margin of 22.9%.
- PAT for Q3 FY24 came in at Rs. 82 crore Vs 54 crore last year same quarter, registered a growth of 53.3% with PAT margin of 15.3%. 9 months PAT is Rs. 277 crore with a margin of 16.4%.
- EPS in Q3 FY24 is Rs. 9.8 with a growth of 53.2%. 9 months EPS stood at

Rs. 32.9.

- Cash and cash equivalent as on December '23 is Rs. 957 crore. Net of borrowing it is Rs. 853 crore.

At last, we are pleased to announce that the board of director has approved a second interim dividend of Rs. 12 per share. With this, total dividend for the year 2023 -24 stood at Rs. 18 per share.

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That brings me to the conclusion of my opening remarks, and I would now request the moderator to open the quorum for Q&A.

Moderator: The first question is from the line of Prakash Kapadia from Ani ved Portfolio Managers Private Limited.

Prakash Kapadia: A couple of questions from my end, diagnostic industry growth has definitely slowed down from earlier pre-Covid days. So, what is our sense, is it customer fatigue? Is it something else which is affecting volume growth for the sector? We have obviously grown at 10%. But this is on a 9% base of last year. So, any sense you could give us for the industry?

Secondly, if I were to envisage higher growth from hereon for FY25 and beyond, so for us what would drive that? Is it Suburban scale? Is it increased contribution? Is it newer geographies where we have been pretty aggressive in Tier-3, Tier-4 cities where we have an early mover advantage? Can specialized test increase? As you know the price rise base is also now anniversary is happening in February so, what will drive growth from here on? And lastly, on Suburban what is the game plan? Because now centers are added, labs have opened, Reference Lab is done, the integration of infrastructure is done. So, can we expect much higher growth in Suburban from here on or not? Those are my questions.

Dr. Om P. Manchanda: You have asked 2-3 questions. So maybe I will first pick the industry part then probably give the other growth and Suburban questions to my colleagues later. I think on the industry front, you are absolutely right that the growth rates of all the companies which are the figures are in public domain. If I add it up, industry growth rates don't seem to be adding up to what we were having pre-Covid levels. I think what you just said is very right. In fact, we are industry-leading growth because double digits many of the other players we see are in single digits.

Prakash Kapadia: Very low single digits.

Dr. Om P. Manchanda: Very low single digit. Yes, and I think in fa

ct our number is actually all inclusive. If

you take their numbers all-inclusive then it is even contrasting now. I think earlier all of us were attributing this to new-age competition. We were thinking that probably it's losing out to that even. They also seem to be in some difficult situation though I don't have any data to support that. But I think as we travel in the market and overall, in general, that intensity which was there 2-3 years back is not there anymore.

The question arises what is going on? One hypothesis that we have probably been that fatigue factor that you talked about is during COVID times there's a tremendous amount of testing and a lot of people went for health checkups etc.

There is a bit of a pause because this is not a want that you want to get tested every now and then. It's a bit of a need base and then suddenly you say listen I just got myself tested last year. Then can I give it a pause because there's too much testing and healthcare and all being talked about. So, there could be one qualitative reason but there is nothing that supports this argument. Second also could be a lot of these bundling things that we have done, like say for example we were discussing earlier within us is that this fever panel is one test and one visit. Now in the earlier years, this fever panel may have been multiple visits because you test for dengue and then you test for malaria you test for something else. Now you want to get everything tested in one sample itself. So that probably could be there could be a correlation.

While one has seen in the industry revenue per patient going up, but volume growth is not that high. I think revenue per patient for most organizations has been significantly higher, but volume growth has not been. So, could the bundling be causing this less frequency of visit? The third hypothesis could be that many of the

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hospitals have woken up to this idea of how they keep their captive business intact and not let this leak out to private players. At least this is someone's phenomena that I've seen in larger cities where probably some of the hospital chains have become a little more aggressive in retaining their OPD business for diagnostics as well. So, I think on the whole this is what probably I can think of. Now I think the million-dollar question is that going forward do we see the softness, or do we see this reviving, my personal view and I think I've always maintained that is that diagnostic is the important part of any medical practice. Without this, actually to my mind there is no

medicine. And these days wherever I go without prescribing a test, doctor s really don't shift to medicine. I think the next phase of growth actually lies as we go down the pop setup. A lot of action has been in the diagnosis in the larger cities. I think we just need to go down and probably trigger more shift from unorganized player , trigger more sort of prescription habits of higher order test. So , I think that is where the industry growth lies and we have been talking about this Tier-3, Tier-4 towns now for last three -four quarters and we have separately started tracking this data. So , we are putting a tremendous amount of focus in these towns , technically speaking going deeper into our core markets. The idea is to really increase market access , get more patients by getting deeper and wider is where we are . Now other questions related to Suburban, and I think you asked something else as well. I 'll hand it over to Bharat h to respond.

Shankha Banerjee : On the Suburban question, I think like you rightly said that the reference lab has been set up. It 's almost a year. The infra expansion of CC is a ctually not only expansion we are also looking at strengthening the way that channel is being run. I think quite a lot of work on that has happened and to cut the long story short yes , growth rates will

improve in the future. That is really what we are working towards. There was something on Dr. Lal PathLabs . I think Bharat h will take that question.

Bharath Uppiliappan : That Dr. Om address ed on Tier-3 and Tier-4.

Prakash Kapadia: So, in terms of the growth drivers if any I was just trying to understand , will Tier-3, Tier-4 be the biggest driver or will specialized test drive this , increased worse contribution looks unlikely to majorly drive growth from here on or it 's going to be Suburban scale which will grow now at a much faster pace. So , the overall revenues could be significantly higher or much better than what we are currently trending is what I was trying to understand .

Bharath Uppiliappan: So, I think what we have said so far is that we will significantly scale up a Tier-1, Tier-2 presence led by Suburban obviously in Mumbai and some parts of Maharashtra.

It's a w ide space for us. We will build an aggressive presence in Tier-1, Tier-2 cities in that fashion. The second thing we have said so far is that Tier-3-4 is where Dr. Lal PathLabs in the core markets will focus to drive its growth from. From a product point of view , Dr. Om mentioned in his opening speech that apart from doing bundling on Swasthfit we are now looking at bundles on the higher end specialty tests also. So , you must see as product innovation continuous pipeline as other source of growth.

In my opening remark , I mentioned D2C as a capability we have built in. So , we are building multiple other capabilities in the company to drive the channel growth including PUP, key account management etc. So , there are three verticals , the Geographic Expansion is one vertical , Channel is one and Product is the other one.

Prakash Kapadia: You are saying all of this should at least directionally looks like we should head for higher growth.

Bharath Uppiliappan: Yes, we are trying the best we can to get the numbers moved up from where it is.

Prakash Kapadia: Yes, despite a low base.

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Dr. Om P. Manchanda: No, low base for Q3 always remains low base. So , I don 't think that is because Y-o-Y is the best way to look at. In fact , this quarter number by the way is very similar to what we did in Q 1 and if you look at historical trend Q1 and Q3 being very equal. So , to that extent it 's not below.

Prakash Kapadia: I'm saying industry , obviously doctor has slowed down. What I was referring to the low base was last year in the base of Q3 there was a 9% growth in non -Covid revenues. That is what I meant. From that perspective, despite being 9% non -Covid revenues , our overall revenues are up 10%. I 'm just trying to figure out what is that missing link because everything seems to be there but it 's not translating one for the industry. Obviously , we are doing better than the industry but looks like there is a large potential and the scientific based or evidence -based testing because anywhere you talk , go talk to friends , fraternity without a report no doctor is willing to give a medicine. So somewhere I was trying to understand what I am missing .

Moderator: The next question is from the line of Rahul Agarwal from InCred Equities.

Rahul Agarwal: I have four questions. I 'll try to be very quick. Firstly , on this Tier-3, Tier-4 cities. Dr.

Om basically the management commentary around this has obviously increased as you also alluded that last 3-4 quarters , we have been highlighting this. I wanted to understand what does management really expect in terms of the change in business when we started tracking this ? There are two things here , one is purely in terms of operations , how does it change your daily routine in terms of the entire effort of capital allocation and sales force being focused on Tier-2, Tier-4?

Secondly in terms of financials as analysts and shareholders what should we focus on when this business actually sees higher growth in Tier-3, Tie

r-4? It could be

revenue mix, it could be more B2B, it could be higher working capital, it could be lower margins, anything. And related question to that also was that the doctor influence in these cities according to me should be even tougher to break in Vs. what we see in metros. So, your thoughts please.

Dr. Om P. Manchanda: So, I think one of the hypotheses that we have is that our strong areas are UP, Bihar, Orissa, the rest of north like Punjab, Haryana etc. We have a great sort of franchise in larger cities. And if you look at the population of this block, I'm told UP alone is nearly 20+ crore. It's a huge market and there are 75 districts and 335 tehsils. Even in all the districts we still don't have labs. And these districts actually they don't fall in big sort of population numbers. So, the idea is how do you go deeper in these markets and there's so many businesses lying there. So, this Tier-3, Tier-4 idea comes that if we take larger test menu to district headquarters, so that will give us the ability to go deeper.

If you look at it, Government medical infrastructure also operates something like that. You have a district hospital which have very large patient flow coming in and if you analyze that flow, actually it's all building it up from ground up which is like Tier-3, Tier-4 villages etc. So, we just want to actually follow that pattern and go deeper and leverage our strength in these places because our brand is very strong. Now obviously this Tier-3, Tier-4 strategy will not work in the South and West market because there even in metro and Tier-1 town itself our presence is weak. So that clearly falls in this bucket of looking at inorganic opportunities. That's the whole idea of Tier-3, Tier-4. Now I think your next question was how do you look at from working capital perspective, operations perspective and infrastructure? I think from a financial perspective, I don't see any difference between what we do otherwise and higher folks on Tier-2, Tier-3. I know the realization would be lower as we go down the pop setup, so would also be the cost as well because many of these tests that will come from these places will be routine in nature. And routine testing is not in the range of 20%-21% of the revenue but it's more in the range of 15% odd. So, to my mind the

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financial profile of the P & L and the balance sheet may not change. Operationally yes, it's a bit of a challenge for us because you are technically managing a network of business which is much more spread out than what it is today. But that I think has been our differentiation as well because that is why we have grown the way we've grown in the past mainly because our management capability of managing remote infrastructure and do it in a quality manner. So, I think operationally there is a bit of a change for us because we will have to really get the management more decentralized than running it from the corporate office is what I can think of.

Rahul Agarwal: Anything on the doctor influence?

Dr. Om P. Manchanda: Doctor influence, I think that piece is where I would say we will have to use the whole D2C approach and brand building approach. My personal experience has been as we expanded out of Delhi NCR, went into other markets, it's definitely a long-drawn process but over a period of time if we have a reason to communicate with the consumers which is more in the form of service, more in the form of health checkups, I have a sense that once brand crosses a particular critical mass, it becomes difficult for doctors to de-sell you because if everybody is using Dr. Lal PathLabs if some doctor says don't go there then doctor runs the risk of losing his or her own credibility. So, I think we know the entire brand-building process. It takes a bit of time but that is what we will try and do as we go forward. But obviously we will have to

offset the

doctor influence and build a Direct-to-Consumer brand.

Rahul Agarwal: One more thing was this 32% to 34% of total top line which is coming from these markets. I see that number in a very narrow band over the last 4 years. I don't have the 9-month number but when I look at 19-20 what you have disclosed, it's not really moved ahead. So, anything different you are doing this time around that this number, will that actually track higher going forward?

Dr. Om P. Manchanda: I think maybe this is the way our internal number is right now. We are giving more focus in the system. That is why it's coming. I think the first stage is for us to know where we are. The second is if the growth rate in Tier-3, Tier-4 is higher than other places, the answer is definitely, yes. Now, I think our teams are aiming that how do we accelerate their growth. So that's the whole idea. I think it just tells us that we have a long way to go. Yes, you are absolutely right that it may not have moved much, it's still moving within the narrow range, but it needs to move up. That is what the whole idea is.

Rahul Agarwal: Secondly, I wanted to discuss one new topic with you and the team because this business is very largely dependent on franchisee. I don't know if the metric called Revenue per Franchisee really matters and I will give you the context. What I'm

thinking is because the growth is really not picking up as what we all want to mid-teens. And my understanding is Dr. Lal has a very decentralized franchisee network which basically means that each franchisee is running at max three centers maybe nationally. Would it be a good idea to track revenue for franchisees? That is one question. And secondly would it be a good idea to basically aggregate and have larger franchisee partners so keep them motivated, compete better, commit more capital and effort to basically get the growth higher?

Bharath Uppiliappan: We have a metric called Revenue per Franchisee and we track it internally and our regional teams do that because underlying is the profitability of the franchisee. We are very conscious of that. Second thing I want to just say that is that we have got about 300 to 400 odd multi-franchise operators with us. So typically, it works well in larger cities where we can put up same person and put up three four shops and then kind of supervise them. Beyond that, since we have a lot of other smaller town operations, one person doing multiple centers from a quality of talent management bandwidth etc. that becomes difficult for him. And that is our observation. We

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encourage multi-year franchising which is we haven't stopped people from taking multiple centers. We got people who have gone up to 6-7. Our view is that beyond 7 it becomes difficult to manage, the geographic dispersion, people rotation etc. But yes, it is an important angle of how we build franchising in the future.

Rahul Agarwal: So, it does not really make any difference and there is no thought internally that we should do this kind of things to basically get growth up. It doesn't really help, right?

Bharath Uppiliappan: We track profitability which is a key metric and as long as we manage that then we are okay with that.

Rahul Agarwal: And the revenue per franchisee trend over the last post-Covid last 3 years, how is that behaved, according to me it should have been declining but your thoughts?

Bharath Uppiliappan: No, it doesn't decline because of the bundling effect, the bundle test they make more money on that. So, on a retained revenue basis they are making more money.

Rahul Agarwal: Seasonally Third Quarter has never seen such high gross margins. I think when I look at the past 4-5 years history, Q3 generally you see a drop in gross margins Q-o-Q, what was different this time?

Ved Prakash Goel: No, as Dr. Om mentioned in the opening speech, the operatin

g efficiency and

obviously the mix is also which sometimes create that. You are right in the past, there may not be that kind of margins. But yes, what we are doing is, right now probably you have seen which has improved some gross margin this time.

Dr. Om P. Manchanda: No. I think just to add to what Ved said is that this year definitely a lot of initiatives we took to improve efficiencies on both purchase, consumption, consumables and all that has come and baked in these numbers that is also showing up.

Ved Prakash Goel: But sequentially if you see Rahul, it is not very different from Q2-Q3.

Dr. Om P. Manchanda: Revenue was down, so he was expecting lower gross margins, right?

Rahul Agarwal: Yes, I was expecting a lower number.

Dr. Om P. Manchanda: You should be happy.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Securities.

Rahul Jeewani: Now if we look at our value growth for both the Third Quarter and 9M, our value growth has been 10%. But if we disaggregate that between volumes and realization, then the volume growth both for the Third Quarter and 9M seems to be between 2.5% to 3% kind of a number. The volume growth in our portfolio both for the Third Quarter and the 9M has been lower only. So, can you comment in terms of slim growth settling over the next four quarters and are you planning to take another round of price increase?

Bharath Uppiliappan: Let me take the second question. We are not planning a price increase as of now, but we keep the options open. On the volume front yes, it has been a bit softer than what we had anticipated. What we are trying to do is to revise the volume growth by going deeper in the geographies of Tier-3, Tier-4. We're trying to strengthen the Suburban business so that Mumbai, which is a Tier-1 city we start to get a better presence and get a bit of medical equity also going by the new centers of excellence. So, there are a host of initiatives we are taking as a team to drive the volume growth up and we are hopeful that this will start to turn around in some time.

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Rahul Jeewani: If you are not planning to take price increase this year and if we don't see much of an acceleration in volume growth next year, do you think that we can sustain this current growth going into next year?

Dr. Om P. Manchanda: No, I think that's mathematics. So, the idea is our pricing is the last resort. We don't want to use pricing as a lever to show higher revenue growth. We definitely want to make sure that the volume is the lever to show higher revenue growth. There's no

doubt about that and we don't want to also get into a situation where we keep increasing prices and start losing our competitive advantage. We just took price increase last year. As Bharat h mentioned , that as of now there's no plan . If the situation demands if we are not able to cover up our cost through higher gross margin, then we'll consider this . Because last year price increase was not taken on the entire portfolio. It was only taken on the part of the portfolio. We still have a possibility of taking other part of the portfolio, but we will use that as a last resort because we want to stimulate volume growth. That is very clear in our mind.

Rahul Jeewani: Since you pointed out that you are trying to accelerate your growth by ramping up Suburban , so can you call out the growth for Suburban this quarter both in terms of value and volume? I'm assuming that the value growth in Suburban would be higher than the 10% number which we have delivered at an overall company level.

Dr. Om P. Manchanda: I think Suburban though Shank ha mentioned. But last time also I think I talked about this. We are actually doing a very strategic shift in Suburban . The shift is basically driving growth through channel collection centers. And we are also doing a strategic shift from our B2B doctor driven b

usiness to Direct to Consumer . So, there are early green shoots where we are seeing this strategy is working. I think you will have to give us a little bit of few more quarters to actually start sharing these numbers in much more detail. Right now , I think numbers are still subdued but we are very confident that this whole franchisee network driven Direct to Consumer business should work for us and will give us long term sustainable advantage. Because I think Rahul or somebody asked about this doctor influence , we want to really come out of that mode and drive direct to consumer growth. So , give us few more quarters to talk about Suburban as we go along.

Moderator: The next question is from the line of Omkar Kamtekar from Bonanza.

Omkar Kamtekar: First question that I wanted to ask was what was the Free Cash Flow generation for the quarter and for the Nine months ?

Ved Prakash Goel : We have Rs. 853 crore net cash in our books and for 9M we have added about Rs. 250 crore roughly.

Omkar Kamtekar : What is the FCF generated for the quarter.

Ved Prakash Goel : For the quarter it is roughly Rs. 116 crore.

Omkar Kamtekar : If we calculate the Revenue per Patient , the revenue per patient for the year YTD 9 months is Rs. 796, so let's round it up to Rs. 800. Other than the change in the mix , what are the drivers that we can see that this revenue per patient can increase?

Because it has been very steadily increasing. It's now close to Rs. 800. What do you think it will take to reach maybe Rs. 850 and higher levels?

Dr. Om P. Manchanda: I think there are some things that have helped as you rightly mentioned , one is Swasthfit package , price increase , mix change , all that has contributed to this. Now going forward, we clearly foresee that this bundling of high -end tests is a low-hanging opportunity for us, and I think should drive Revenue per Patient. But I won't change
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this number as much as the volume growth because I know if I go down the pop strata, go to smaller towns that may pull this number down as well. So, as a metric I think to me this could be a side effect rather than the main effect that we will probably drive. Our focus essentially would continue to remain on two things is that how do we get more number of tests per patient and how do we get more patients? That's it. Rest I think this P & L will take care of itself.

Omkar Kamtekar : On that point, a nswering in the previous question you said that we are focusing more on the B2C segment and trying to get the customer more aware about our offerings .

Dr. Om P. Manchanda: Let me correct this. I think the whole B2C or D2C idea is how do we keep the brand as top of mind. I would actually say that we want to keep the top -of-mind awareness of Dr. Lal because as and when the need arises the person should think of Dr. Lal PathLabs rather than any other brand. And that can be served through many other channels. It can be served through even hospital also. No problem. It can be served through a smaller lab also as long as the person is very keen to have a report from Dr. Lal Path Labs in his or her access.

Omkar Kamtekar : So, it's like we are trying to position ourselves as the product leader , service leader . So, with that in mind are we looking at , for example with respect to the other players that are there in the industry what they are doing is they are entering into contracts or maybe MoUs with these smaller town Tier-2, Tier-3 hospitals , district hospitals , etc., and they can get a captive audience right from Day1 . The volumes are there for them because the people are there right from Day1 everybody is going to these specific hospitals in their regions. So, is that something of a route that we may go down? Maybe we can even approach Government hospitals for that matter so that the volume that we have been missing could be brought on board. Is that something

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at could be taken up?

Bharath Uppiliappan : Let's put it this way that we will look at all channels as long as we are assured of let us say the volume and also the payments coming in through . Government contracts we evaluate very carefully because there are multiple angles to be taken care of. So, we have been kind of present in all the areas where the Government hospitals are. We do have contracts with the Government on many fronts. So as long as we look at the total commercial package and it makes sense for us, we will definitely do those agreements and sign-ups. But if they don't , then we will rather walk away from those opportunities.

Dr. Om P. Manchanda: I think we need to be careful about , are we managing the optics of volume growth or are we exploring long term sustainable advantage? If we are to manage optics of volume growth there are multiple ways to get this volume so we can actually tomorrow run a huge sugar camp and you will have la khs and lakhs of customer coming in. So, I don't think that is what we want to do. We want to make sure that whatever we do is sustainable and profitable.

Omkar Kamtekar : On the cash flow question , we have approximately Rs. 800 odd crore worth of cash in the books. Any color with respect to any acquisition in the pipeline or maybe something that is on the block. Not that I ask you to disclose it but are we looking for any specific acquisition or something so that we can utilize this cash?

Dr. Om P. Manchanda: I agree with you actually our strategy is fairly clear which is that we want to have market access and that can come through both organic which is what Tier-3, Tier -4 we keep talking about , will come through organic means in our strong markets and other markets it's all M &A. I think the last M &A that we did was Suburban 2.5 years back and our thesis is that okay if you do 10 M&As, 3 -4 may not work but the other 4-5-6 which will work will take care of the failures. I think the last 3 years' valuations had gone through the roof and expectations of even private players was in line with Page 12 of 14

the sort of public market multiples which we can not give. And I think now with a smoothening of curve, and I can see the numbers of other companies as they are getting declared. My sense is that the P &L of many of these companies will come under a bit of stress. And hopefully this whole M &A piece should open up and we are open to doing M &A in those markets where we need it.

Omkar Kamtekar : Have we scouted any candidates ?

Dr. Om P. Manchanda: Right now , I don't think we have anything that can be shared with anyone, but efforts are always on.

Moderator: The next question is from the line of Punit Pujara from Helios Capital .

Punit Pujara: If I look at margins on a 9M basis be it gross margins or EBITDA margins so gross margins are expanded by 2% points and EBITDA margins 3% points. And Dr. Om, I remember in the last call you mentioned that having higher margins you also attract more competition. So, the question I'm coming to is, is there room for you to reinvest these margins to accelerate volume growth which I believe comes out from the call that's a key metric you want to improve.

Dr. Om P. Manchanda: Sorry I missed the last line.

Bharath Uppiliappan : Is there a way to reinvest the extra margin?

Dr. Om P. Manchanda: I think we have done that piece quite a bit. While I don't know whether line-wise items are visible to you or not . We have invested in 2-line items aggressively. One is our A&P which has significantly gone up what we have done in the last couple of years.

Ved Prakash Goel : And IT expense.

Dr. Om P. Manchanda: There was one more thing I think on the technology front we are actually a tech company. A lot of investment has gone in these areas. Maybe one area that probably we would like to invest is go deeper and spend on bit of a Lab infra in Tier

-3, Tier -4

that we have repeatedly talked about on this call because sometimes it's very difficult to actually know what all these efficiency efforts will lead to since we believe we have a buffer and so we will start investing. And I think some of those efforts have already started in the second half of this financial year. But to reassure you that we want to invest for growth. We are not here to just maximize the margins and slow down. The idea is to really reinvest some of this profitability back into the top-line growth and that too more on volumes.

Punit Pujara: Dr. Om, you also mentioned about driving a durable growth which in my opinion is through number of patients increasing but on 9M basis , patients have increased by 2.5% odd and your number of tests have grown by 8% which implies that this bundling of test is working really well. But organic patient volume growth in your mind will that be a right metric for that durable growth that we just referred to?

Dr. Om P. Manchanda: Actually, I know we have always talked about patient growth , volume growth. These are actually not patients. They are actually patient visits. And what we are little unsure about that whether all the efforts of bundling , etc., or fever panel , etc., are leading to reduction in patient visits. You understand what I mean?

Punit Pujara: Yes.

Dr. Om P. Manchanda: So that piece actually we are not able to establish through data because it is not a regular purchase. Patient comes to us only when that person needs us. So sometime

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that patient can drop off. It's not that the patient has dropped off because he or she has gone to competition. Patient has dropped off because that patient now is cured and does not need us and will come back maybe after 2-3 years whenever the need arises. So, these are all patient visits. So, I think we are also parallel ly tracking. We are not in a position to share that data because we do not have the capability to get the exact number. But from a trend perspective we are constantly looking at what's happening to unique patients rather than looking at just patient visits. And also, please remember these patient visits as we are talking about , I think 3 quarters back were also in a trajectory where it was negative growth by the way . We may have forgotten this. So, to that extent actually we are seeing , I have the data in front of me. Q2 FY23 -2.3% , Q4 FY23 -4.8% , Q1 FY24 -0.9%. I think on the back of that negative growth having these 3 quarters positive also is a positive sign I would say. So, we are improving. We are still not there but I think there is a slight improvement on these numbers.

Dr. Om P. Manchanda: And things have done exceedingly well on sample growth. So, this growth of revenue is not only driven by pricing . It is driven by samples growth which is more testing also.

Punit Pujara: This Tech investments that you referred to will enable you to track this unique patient's visit in a better way I believe .

Dr. Om P. Manchanda: Only in one segment because some segments like which comes through hospitals , which comes through even collection center s also a limited way, but which is coming directly to us that we can track definitely.

Moderator: The next question is from the line of Prashant Nair from Ambit Capital.

Prashant Nair: Dr. Om, I think you partially addressed this but if you could elaborate a bit more .

When you talk about maybe increasing lab infra to target Tier-2, Tier-3 cities , is there any way you can quantify either number of labs , or if you could even give qualitative color on will these labs be similar to what you currently run in the bigger cities , or will they kind of have slightly more limited test menu , will the number of spokes they can support or number of connection centers they can support be higher , lower than what you have currently? I mean is the nature of the network likely t

o change or is it just

adding the way you have been adding so far?

Dr. Om P. Manchanda: If you study our infra , we went through a stage where there used to be Satellite Labs and there was a Central Lab, if you go back 10 years . And now we are sitting on a format where there's an in between layer called Hub labs. These Hub labs are more similar to Central Lab but they are still not Central Lab. So, I think it's more a colonization of that cluster where you have put already a Hub lab, let's say , in a city like Varanasi or city like Kanpur. Now you want to go deeper in that cluster with satellite labs. Obviously , the answer to your question is that this will have a limited or may not be that wide a menu but as the volume picks up the menu will also expand. These labs will act more as a feeder to the big lab that we have in the region. So, it will have a limited menu.

Prashant Nair: Is there any target in terms of setting up XX number of labs .

Dr. Om P. Manchanda: We are in the process of identifying those markets .

Bharath Uppiliappan : We have done 20 , announced 20 now .

Dr. Om P. Manchanda: We have already said that we will do 20 labs , by when will that be ready?

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Bharath Uppiliappan : March -April.

Dr. Om P. Manchanda: By March -April, 20 more will be ready.

Bharath Uppiliappan : We are actually in the process.

Dr. Om P. Manchanda: 20 labs are right now in the works and by May when we come for the next results , those labs will be up and running. While having said that we are also doing planning for next year because we want to identify the potential towns for us and the support system for these labs and then maybe at the next meeting we'll be in a better position to highlight those. But I think directionally , you should get an idea that we are expanding our market access going deeper.

Moderator: Ladies and gentlemen , we will take that as our last question. I would now like to hand the conference over to the management for closing comments. Over to you sir.

Ved Prakash Goel : Thank you , everyone , for being with us on this call today. I hope we are able to answer all your points satisfactorily. Please feel free to reach out to us in case you have any further questions. Thank you once again.

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Call: May 2024

Classification: Internal
May 15, 2024

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q4 & FY24 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q4 & FY24 Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 16
Classification: Confidential

Dr. Lal PathLabs Limited
Q4 FY24 Earnings Conference Call
May 10, 2024

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Shankha Banerjee - Chief Executive Officer (Designate)
• Mr. Ved Prakash Goel – Group Chief Financial Officer
Participants who asked questions • Binay Singh - Morgan Stanley
• Amey Singh – JM Financial
• Kunal Randeria – Axis Capital
• Nitin Agarwal – DAM Capital
• Pranav Chawla – Antique Stock Broking
• Prakash Kapadia – Spark PWM
• Anshul Agarwal – Emkay Global
• Jainil Shah – JM Financial
• Shaleen Kumar – UBS Securities India
• Dheeresh Pathak – Whiteoak Capital
• Sumit Gupta – Centrum India

Classification: Confidential Moderator: Ladies and gentlemen, good day, and welcome to Dr. Lal PathLabs Q4 FY24 Earnings Conference Call. As a reminder, all participant lines will be in the listen - only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Siddharth Rangnekar from CDR India. Thank you, and over to you, sir.

Siddharth Rangnekar: Thank you, Michelle. Good afternoon, everyone, and welcome to Dr. Lal PathLabs Q4 FY24 Earnings Conference Call. Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal, Executive Chairman; Dr. Om Prakash Manchanda, Managing Director; Mr. Shankha Banerjee, CEO (Designate) and Mr. Ved Prakash Goel, Group CFO.

I would like to share that some of the statements made on today's call could be forward-looking in nature and actual results could vary from these results and forward-looking statements made. A detailed statement in this regard is available in the results presentation, which has been circulated to you and is also available on the stock exchange website.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspectives. Thank you, and over to you, Dr. Lal.

Dr. Arvind Lal: Thank you, Siddharth. Good afternoon, and a warm welcome to all participants on the call. We are here to discuss the Q4 FY24 earnings performance of Dr. Lal PathLabs. I would like to commence by discussing my insights into the changing market dynamics and the advancement achieved at Dr. Lal PathLabs.

As we approach our 75th/Platinum anniversary this year, Dr. Lal PathLabs continues to be a trusted healthcare partner, renowned for its commitment to quality, accuracy, accessibility and affordability. Our marketing campaign, "Bharat Ka Vishwas", exemplifies this dedication and reinforces our position as a cornerstone of nation's diagnostic landscape.

Our relentless pursuit of excellence in diagnostics has earned us numerous complements and the confidence of patients and physicians alike. We remain focused on deepening our presence in underserved Tier -3 and Tier -4 markets, while leveraging our robust digital infrastructure to differentiate ourselves and achieve significant milestones in network rollout. The recent introduction of a custom-built logistics solution further enhances our ability to serve our customers better.

The integration between Dr. Lal PathLabs and Suburban Diagnostics continues to yield promising results in the Western market, where we have established a growing presence. We are making targeted investments in both marketing and process efficiencies to strengthen the brand while elevating the patient experience. Our reference lab in Mumbai has begun to receive increased volumes of various clusters across Maharashtra for both routine and semi specialized tests. Our collaborative approach is enabling us to combine inherent strengths and provide patients with a wide array of tests across multiple categories with speed and precision.

Looking ahead, our growth trajectory will be fueled by the ongoing shift from unorganized to organized, more awareness on accurate diagnosis as well as focus on wellness portfolio, among others. We remain committed to expanding our network and elevating our service standards through operational excellence and technological advancements. Despite increased competition, we are strategically

Classification: Confidential positioned to deliver sustained growth and further consolidate our market share, driven by our comprehensive test portfolio and competitive pricing strategy.

That concludes my initial remarks. Before I hand over, I would like

to extend a note

of appreciation for Bharat and wish him well in his future endeavors. As you know, we will be having Shankha Banerjee, leading the business as CEO, going forward, and he is here today. I welcome him into this role.

Thank you. I would now like to hand over the floor to Dr. Om Manchanda. Over to you, Om.

Dr. Om P. Manchanda: Thank you, Dr. Lal, and welcome all of you to our call today. I shall walk you through the evolving industry scenario and progress in our strategy.

I'm happy to announce that we have achieved double-digit top line growth on a year-on-year basis. This growth has been accompanied by a significant jump in our operating and net margins, primarily driven by efficiencies of scale and productivity initiatives.

Our strategic focus continues to remain widening and deepening the brand's presence geographically, thereby driving sustainable volume and value growth.

There is also a sharp focus on expanding into Tier -3 and Tier-4 towns organically in LPL core markets. And in the rest of North India, we have made good headway with this strategy. Additionally, we are also establishing 20 new hub labs under the DLPL brand name to broaden our reach. As the year progresses, we shall undertake to add more such hub labs.

We have identified key growth areas, aiming to strengthen our presence in Mumbai and in select parts of Maharashtra via Suburban. In order to boost momentum in Suburban Diagnostics itself, we are promoting higher sampling and enhanced branding.

Our 'Swasthfit' brand, now a consistent revenue driver, comprising 22% to 24% of the total revenue is being extended to many other markets. We are moving towards bundling specialized tests as well. The medical centers of excellence that we have developed are scaling well, rendering better patient engagement and outcomes. Being guided by a strategic focus on sustainable volume growth, these initiatives would attract patients from unorganized labs, encouraging high and specialized test prescriptions.

Our investments in technology and digital infrastructure are yielding tangible results, enhancing our brand visibility and patient interactions. We continue to push ahead with brand initiatives in select markets, while also executing a strategic agenda for network and test menu expansion.

We remain vigilant of the market dynamics and evolving the regulatory frameworks, adapting our approach to ensure compliance and sustainability.

As we press forward, maintaining a robust financial footing is paramount. With a focused approach towards cost optimization and strategic investments, we aim to uphold our financial metrics and cost structure.

I Now hand over to Shankha to delve deeper into our strategic initiatives and operational highlights. Over to you, Shankha.

Shankha Banerjee: Thank you, Om. I extend a warm welcome to all participants joining this call today. I will proceed to share the business and operating highlights with you.

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Classification: Confidential I am happy to share that we have closed the year on a strong performance at both top line and profitability. In Q4 FY24, we achieved a revenue of Rs. 545 Crore, marking an 11.1% growth. This was made possible by serving 6.5 million patients and testing 19.1 million samples, reflecting a sample growth of 8.1% compared to the same period last year.

Our Q4 FY24 revenue per patient stands at Rs. 833, a 3.2% growth over Q3 FY24, primarily driven by favorable mix management. In order to intensify our efforts in key clusters of Western and Southern India, we are expanding our lab infrastructure. This is being executed in a time-bound manner and alongside, we are building a robust ecosystem of collection centers and pickup points.

Our flagship bundle program – 'Swasthfit', continues to gain healthy traction, contributing to approximately 24% of total revenues in Q4. Building on this success, we would

continue to expand its reach and develop programs to sustain the growth trajectory. Furthermore, medical centers of excellence, such as LACE and LC oRD, are solidifying their positions within the medical community and among patients. Leveraging our digital infrastructure, which includes advanced AI and data analytics, continues to drive efficiency and elevate the overall patient experience. Technology lies at the heart of our operations, and we are exerting every effort to maximize the use of our assets to achieve sustainable and profitable growth.

To conclude, I would like to say that we are moving in the right direction. Our performance going ahead will be fueled by expanding our reach, enhancing our digital capabilities and focusing on product innovation and value-driven offerings. Thank you for your continued support and trust in our journey.

That concludes my opening thoughts. I would now request Ved to take you all through the financial performance. Over to you, Ved.

Ved Prakash Goel: Thank you, Shankha. Good afternoon, everyone, and warm welcome. I'm sharing some of the key financial highlights for Q4 and full year FY24. Revenue for Q4 FY24 came in at Rs. 545 crore against Rs. 491 crore last year same quarter, a growth of 11.1%. Revenue for the full year came in at Rs. 2,227 crore v/s Rs. 2,017 crore in FY23, a growth of 10.4%. Revenue per patient for Q4 is Rs. 833 and full year is Rs. 806 v/s Rs. 750 in the previous year, a growth of 7.5%.

Sample per patient for Q4 is 2.91 and 2.83 for the year, a growth of 4.7% and 5.3%, respectively. EBITDA for Q4 FY24 came at Rs. 145 crore v/s Rs. 116 crore in Q4 FY23, registered a growth of 25.1% with an EBITDA margin of 26.5%. For full year, EBITDA came in at least Rs. 609 crore v/s Rs. 490 crore in FY23, registered a growth

of 24.4% with an EBITDA margin of 27.4%. PBT for Q4 FY24 came in at Rs. 120 crore, registered a growth of 44.1% with a margin of 22%. FY24 PBT is Rs. 505 crore, registered a growth of 47% with a margin of 22.7%.

PAT for Q4 FY24 came in at Rs. 86 crore v/s Rs. 57 crore last year same quarter, registered a growth of 50.8% with a margin of 15.7%. Full year FY24 PAT is Rs. 360 crore -- sorry, Rs. 362 crore, registered a growth of 50.3% with a margin of 16.3%. EPS in Q4 FY24 is Rs. 10.1 and Rs. 43 for the year with a growth of 49%. Cash and cash equivalent as on March'24 is Rs. 960 Crore. Net of borrowing, it is Rs. 877 crore. Our receivables and inventory have further improved. Now, the DSO as on March 31, 2024, is only 25 days of credit sales and inventory is at 30 days. We still have a negative working capital of 26 days.

At last, we are pleased to announce that the Board of Directors has approved a dividend of Rs. 6 per share. With this, total dividend for the year stood at Rs. 24 per share.

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Classification: Confidential As mentioned by Dr. Om, we are committed to continue our efforts to optimize cost, increase productivity and enhance efficiency through tech-driven initiatives. With this, I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A.

Moderator: The first question is from the line of Binay Singh from Morgan Stanley. Please go ahead.

Binay Singh: I had 3 questions. Firstly, on the industry growth and competitive intensity. Any sort of update from what you shared last call, any changes that you see there in? The second question I have is on the Suburban side. If you could talk a little bit about the growth you are seeing, and fair to believe that the initiatives you talked about sampling promotion, brand building, these are more recent. What kind of traction you expect going ahead?

And lastly, the number of tests that you report in the presentation. We see a sharp drop in the tests under radiology from 1,947 to around 1,400 in this quarter. So any read-through from that?

Dr. Om P. Mancha

Dr. Om P. Mancha: Okay. This is Om here. Let me take the first question. And maybe the other 2 questions, I'll ask Shankha and Ved to address. So, coming to industry growth. Based on the sort of trend analysis that I have done in the last 3 years, I'm beginning to feel that probably COVID was impacting base not just due to COVID alone, I think some of the non-COVID sales in the previous years was also influenced by COVID. As the COVID has gone away, I think that FY24 base for the entire industry will be completely cleaned up, and you won't have any impact of either COVID or non-COVID sales influence due to COVID.

My personal view is our industry would see better growth in FY25 than what we have seen in FY23 or even FY24. Second part of your question was on this competitive intensity. In terms of number of players, I don't see anybody going down or getting out of the business. Whatever had to happen has already happened during COVID time.

So now, whosoever has been in the business, my sense is they'll continue. But I think the level playing field is becoming a little more, a little better compared to, I think, some of the players have already taken price increases. So the deep discounting or if I were to call, predatory pricing, I think that is now not there in the market. So overall, I would say it's a positive news as we go forward into FY25.

Shankha Banerjee: So right, there was a question on Suburban. So this is Shankha here. So yes, Suburban growth traction on revenue side definitely looking up. I think our Q4 revenue growth is inching towards double digits. We are very close to double-digit growth now on Suburban. And a lot of these patient acquisition activities, I think which Om mentioned, as well as some other future improvements that we have done, should ensure that we are able to not only sustain this growth trajectory, but maybe also build upon it going forward. You want to take the point on tests?

Binay Singh: Sorry, this is double digit as in early double-digit or high double-digit growth?

Shankha Banerjee: No, no. So this is just reaching double digits. It's just sub double-digit level, yes.

Binay Singh: Anything on market share of Suburban in Mumbai?

Shankha Banerjee: So market share is obviously a very difficult number. I think in terms of assessing what the overall market potential itself is, we don't have any official data on that. But one thing which we know is that Suburban as a brand, will be the third largest brand in Mumbai as a city.

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Classification: Confidential Ved Prakash Goel: Right. Binay on the number of tests, we have rationalized some of the tests, which

were not in use. So that's why we have taken away from the list duplication.

Dr. Om P. Manchanda : The same thing that what we discussed the other day. Yes. I think this rationalization is primarily done because there are a lot of medical terms, which are actually very overlapping in nature. So some of these cleanup we've done. It's not that our portfolio has shrunk, but it's just that cleanup in our nomenclature.

Binay Singh: Great and also congratulations on almost completing 75 years, big milestone. Congrats on that.

Moderator: The next question is from the line of Amey Chalke from JM Financial.

Amey Chalke : Congrats on a good set of numbers to the management. The first question I have is, is it possible for us to gain volume growth for Delhi and rest of the North India region. And second, in Delhi we are seeing a lot of hospital chains coming back to their own lab business. There is a captive business as well, but they're also venturing into the outside market. Are you seeing the impact of this activity on your business?

Dr. Om P. Manchanda: Okay. I think on overall Delhi NCR business, we have seen a lot of positive traction this year. In fact, if at all, there is one part of the business, which has done well for us, is Delhi NCR in FY24 .

I think your point is valid. Some hospital lab chains have come into this business . My reading is that whatever gains they had to make, they probably have already made because of their own customer base while they'll continue to remain in the business.

But to my mind we are seeing a much higher growth than what we have experienced in the previous 3 -4-5 years. So overall, I would say that we are well positioned in Delhi NCR especially given that Swasthfit business is doing pretty very well in this space.

Amey Chalke : Sure. And the second question I have is on Tier-3 contribution which is around 34%.

Is it possible for us to give example of the cities what you consider in the Tier-3? And the second thing is that we see in the labs as a proportion of total labs around 40%, but the PUP number seems to be on the higher side, the collections number seems to be on a higher side. So . is the business model a bit different, you need to invest much more on the collection center side when you go for the Tier-3?

Shankha Banerjee: There is a list of cities which is their Tier-3 plus. If you remove the top 108 -odd cities of the country rest of them are then classified as Tier-3 and Tier-4. So that classification is something which we have taken from whatever the government of India does the classification on. It's publicly available data.

The second question which is on the model. So the collection network primarily which is linked to all the lab infrastructure is franchisee -driven. So to that extent there isn't a heavy company investment in terms of building the network, but what it definitely does is that in the Tier-3 cities and below we have a much larger geographic footprint from which the lab actually collect samples. And therefore, we see a larger a number of collection or larger collection network associated with the labs which are operating in Tier-3 or below.

Amey Chalke : Is it because of the scale of the margins of this business will be similar to our company average or how that will be?

Shankha Banerjee: I think we have seen that and you would have seen that over the last 3 years to 4 years the contribution of the Tier-3, Tier-4 markets have increased or improved in our overall mix. However, we've been able to sustain our EBITDA profile. So we have

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Classification: Confidential obviously worked out modalities in which the overall company margins is not going to be impacted adversely.

Dr. Om P. Manchanda : So just to add to what Shankha is saying I think the trick lies in the way you model your business and manage the logistics of it. So idea is that how you really align these Tier-3, Tier 4 cities to the nearest lab is how we are confident of managing the margins.

Amey Chalke : Thank you so much. I will join back the queue.

Moderator: The next question is from the line of Kunal Randeria from Axis Capital. Please go ahead.

Kunal Randeria: Sir just going through some of the operating parameters like the number of labs, the pickup points and so on. So , at a consol level you have added let's say three labs in the last 2 years and around 1,000 -odd pickup points. But if I were to look at Tier-3 centers there you have added maybe 15 labs and maybe 1,600 or 1,500 pickup points. So , it basically means that you're scaling back in your Tier-1, Tier-2 cities. So while I understand you're doubling down on Tier-3, but scaling back is something which I don't completely understand this, is this competition or is it maybe some inefficiencies which have crept in and you are just trying to sort of rationalize it going forward?

Dr. Om P. Manchanda: No, I think the way you should look at it is the following ; because having more labs

technically is not a demand generation activity. It's the ability to service the demand. So most Tier-1 cities we already have labs. In some places, we will have multiple labs also. In some places, we'll have HLMS as well. It's

only Tier-3 and below where there's a huge gap, we don't even have labs at all. That's why in terms of expansion of our lab infra you will always see that more labs are coming in Tier-3 by default itself because as per the government definition I think Tier-1 has about 8-9 cities and then Tier-2 have around 98 cities. So all other towns are falling in Tier-3 and below. So those are the places where we don't have too many labs. So that's why you will see expansion coming only in Tier-3. So it's not about scaling back in Tier-1, but it's more about the gap that exists is only Tier-3 and below.

Shankha Banerjee: And I think specifically on this point of net add of 3 whereas you see an add of 15 in Tier-3. Yes, there has been a bit of an overlap rationalization that has been done, but it's kind of onetime. It's not any scaling back. It's more of a onetime rationalization to see that the overlaps are reduced.

Kunal Randeria: Sure. And the same kind of logic applies on the pickup points also because there also seen a number of pickup points seems to have gone down in Tier-1 and 2's?

Shankha Banerjee: Pickup points, again, we've got a dynamic way of looking at it because sometimes if a pickup point is not giving us regular samples for a period of time, we kind of deactivate them in our system. So, there is no scaling back so to say. I think these numbers of our pickup point acquisition activities in Tier-1 and Tier-2 continue to be operational.

Dr. Om P. Manchanda: I think broadly this chart of Tier-3 plus 1 should read as if like we are deepening our presence in the market. That's the way one should look at. Now the play between CC pickup points and labs, it's a very dynamic process mainly because sometimes our intention is always to maximize throughput per lab. And once the throughput per lab is maximized then it's a huge positive on all sides. You will expand your test menu. You can operate 24/7. You can provide the courier pickup 24/7. You connect many roads. So it's like if you get higher throughput per lab, a lot of other parameters just fall in place. So I think that number is more always constantly optimizing operations. But the reason why we probably are giving the chance of Tier-3 plus is

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Classification: Confidential just to say that we are not just a few city dependent company, we are widening and deepening our presence across the country.

Kunal Randeria: Sure, sir. And just carrying forward on that. Any particular region, not Tier-3, any particular region you would like to call out which has surpassed expectations and where would you look to expand further?

Dr. Om P. Manchanda: It's mainly Northern part of India. I would say UP, Bihar. Because most of the cities in UP, if you look at the population is just ballooning beyond over 22-23 crore. Now such a large population base is I think if you pick cities, most of these cities are in the category of Tier-3 and below. Maybe just say Lucknow, Kanpur, etc. And that is where our presence is just growing. And this State is a huge state for us, and I think it shows good sign of growth. I would call out this particular State.

Kunal Randeria: Got it, sir. And just one more, if I can. Swasthfit contribution is 24% of revenue is probably the highest, I think, what is the kind of headroom you see over here?

Dr. Om P. Manchanda: It's a very interesting, we are currently in the process of identifying because virtually, every quarter we are seeing this contribution inching upwards. We're trying to figure it out as to what is really happening. My hypothesis there could be 2-3 factors that are leading to this. One is patients have seen greater value for money because obviously, it makes sense to go for a bundled package, we can do more tests at a much lower price. Maybe medical doctors are seeing greater value because then they don't need to ask for a follow-up test in one go, they get multiple te

sts and their

ability to diagnose what the underlying cause for any illness is there. Third could also be the way consumer behavior is changing. Because most of these now, even if they come to the lab or they book online, I have a feeling it's easy to book a bundled test. You just press a button, and it all gets done or repeat the order. All that is probably happening. So there is a sense that I get is a combined factor of all these 3 leading to higher percentage of contribution from Swasthfit.

We also believe, I think I've spoken about this in the earlier calls, is that as the bundling goes up, it is having ramification of two other parameters. One is the number of tests per patients are going up because bundled packages have more tests. I'm also getting a sense that footfall is also falling. Because once you get these tests done in one go, then you don't need to come back again because all the tests you've done in one visit itself which to some extent commercially is not a bad idea

because our overhead consumption is servicing the patient is highest.

It's not in the testing that is more, but it's more the servicing the patient visit. I think we should look at a volume component both together, number of tests per patient as well as number of patient footfall. I think combined together is where the volume should be looked at. And it's an interesting sort of the thing that is emerging. Let's see how it goes, but we are very confident that Swasthfit is going to grow further.

Moderator: We'll take the next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Just taking forward your previous comments. This bundled tests, obviously, moving up the overall realization per patient. I mean, where do you think this number really ends up? At what stage it will start to, in your assessment stops growing? Because we've had a consistent Q-o-Q growth on this number for the last several quarters now.

Dr. Om P. Manchanda: There are three distinct segments in the market. One is pure screening -screening, where there's no illness and people are just getting themselves checked to know whether there is a problem or not. If that factor is dominating and driving this number up, that means the market is expanding. The other is a lot of people who are suffering

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Classification: Confidential with NCD which are chronic diseases like diabetes, high cholesterol or some other illnesses, where they need to go for regular checkups. Those are basically upgrading from an individual test. Even there is also no problem. Let these bundled tests grow because many of these NCDs are interconnected also.

Third area is which is an illness driven or communicable diseases where things like typhoid, malaria, dengue, these are all seasonal flu, etc. To my mind, these are three distinct segments. If screening and NCDs are there, then this number, actually, my sense is can even go 30 %-40% as well. So I think it will continue to grow as we go forward. And also, I think as the bundling goes up, it's a greater value for money. I think that we are also hovering around price points of between Rs. 1,000 - 2,000 which is just a ticket size which is easy to take out from the pocket. This may just become a new norm in the industry.

People will just get to know, I think I mentioned to you earlier also, if you trace the history over the last 20 years, it started with a single test, then it became like a panel test, like LFT, KFT, they're also technically bundled tests for a particular organ. What the next stage has been the bundling of these panels only, LFT, KFT clubbed together. So this is, to my mind, is a new way of life, and I think this will become a norm in the industry. I think your specific question is to what extent it can go? Anybody's imagination. My sense is it can be 30 %-40% of the portfolio as well.

Nitin Agarwal: And sir, I guess, based upon

your comment, probably in the past, you were tracking the patient volume growth number more closely, given the way the dynamics in the market are playing, probably you think sample growth, probably the more apt number to look at as things are.

Dr. Om P. Manchanda: Sorry to interrupt you, I would say both.

Nitin Agarwal: Okay. And secondly, on the cost part of it. We've had a pretty sharp jump up in our gross margin this year. I mean how much scope do we have for that to squeeze out more, especially at the gross level, contribution level?

Shankha Banerjee: I keep asking this question to Ved as well. So Ved should answer.

Ved Prakash Goel: So I think it's pretty high. I mean 80% roughly is gross margin. I think what we can do is, in my view, we can sustain here. But beyond this, I don't think there is a room to improve further, especially on the gross margin. But I think we can sustain between this 78% to 80% kind of margin.

Moderator: The next question is from the line of Pranav Chawla from Antique Stock Broking Limited. Please go ahead.

Pranav Chawla: I just wanted to ask what is our regional mix for the fiscal. We used to highlight the split between Delhi NCR, South, West, International, East and North. Can you provide this?

Ved Prakash Goel: Delhi NCR contribution is about 31% for the quarter.

Pranav Chawla: Sir, for the fiscal?

Ved Prakash Goel: 31%, for the full year as well.

Dr. Om P. Manchanda: Yes, I'll tell you, 31% Delhi NCR, 31% Rest of North, 15% is East, 15% is West, South is 6%, and other 3%. Others will constitute international and CTL.

Pranav Chawla: International as well. Sir, and what is our B2B, B2C mix for the fiscal?

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Classification: Confidential Dr. Om P. Manchanda: It is always estimate, right?

Ved Prakash Goel: Retail, including home collection, is about 32% for the year.

Dr. Om P. Manchanda: But that is our own infra. There is also a retail component in collection center.

Ved Prakash Goel: For franchise it's about 37%, which is collections center.

Dr. Om P. Manchanda : For full year?

Ved Prakash Goel: Yes.

Dr. Om P. Manchanda : I can come back on that, 1 second . The retail component of direct -to-consumer, our own infra is very clear. But the retail component of our collection center is generally we take an assumption that 50% of their business would be direct -to-consumer and balance 50% would be a pickup business.

Ved Prakash Goel: That exact split, we don't have.

Dr. Om P. Manchanda: We don't have that, but we normally tend to assume that number and add it up.

Pranav Chawla: Okay. Perfect. Sir, just can you provide us a color on how the competition has been in a core market of Delhi NCR? And how the region of Mumbai has been doing? How Suburban is growing? Even if you can share the quarterly revenue number of Suburban, that would be great.

Shankha Banerjee: I think, firstly, to answer Suburban. I think there are quite a lot of revenue and patient growth activities that one is doing. I think , we talked about a lot of sampling activities, patient acquisition, offline, online activities that are happening. Also, we are expanding geographic testing, footprint for Suburban to get into newer pockets and tap into those opportunities. Also, running a lot of clinician connect programs to get better prescriptions and conversion programs there as well. Also beefing up the sales team, creating some specialized sales team structures to get better extraction from B2B clients as well. So there are multipronged activities currently on for driving revenue growth, and those things are going to be sustained over the next few quarters.

Dr. Om P. Manchanda: Just to add to what Shankha said, now it's been nearly about more than 2 years. We have more than 2 years. We mentioned about Suburban , it went through 2 phases.

Phase 1 was basically to stabilize the ship because it is just changing hands from

promoter to us. And then that piece was done. I think now the second piece was actually getting the strategic building block in place. If you recall, a major shift that we've had is the franchising of collection centers. That's the model which LPL follows. And we find that entrepreneurial energy in franchising is extremely high. And we've used that to drive a lot of sampling, a lot of camp activities in various colonies of Mumbai.

And I don't know if you guys must have seen in Pune as well. And I think now with Shankha moving as CEO in LPL, we are now going to integrate this company much more with the parent as well where a lot of back -end support operation will be driven from here. So, we are very confident that all the building blocks of future growth for Suburban are in place and we are going to be driving this business from the center. Now early days, but good sort of numbers are there last quarter. It is definitely driving a lot of confidence in this. And I'm very, very confident because as a brand, as a single largest brand in one city, there are not too many of them in this country. And the suburban, 80% of its business comes from just one city of Mumbai. And that's a huge, huge sort of advantage platform for us to expand from here on. And it also has a brand which lends very well to its sort of a direct -to-consumer. It's brand awareness

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Classification: Confidential is fairly high compared to many other small time pathology labs. So I'm fairly confident that going forward, Suburban would turn out to be a good asset.

Pranav Chawla: Okay. Sir, then can you just give us some broad color on Delhi NCR region , like how is the competitive intensity, pricing, realization, etcetera, just for Delhi NCR?

Shankha Banerjee : Competitive intensity in Delhi NCR hasn't really changed too much v/s what I think we spoke about in the last quarter. So I think a base level of competitive intensity is always there. We haven't seen any sharp up move or down move in terms of competitive intensity when it comes to Delhi NCR, at least not in this last quarter.

Moderator: The next question is from the line of Prakash Kapadia from Spark PMS.

Prakash Kapadia: I have two questions. If I look at the Swasthfit contribution. It has been increasing. It has been doing well for us. So is it currently just a few top cities or metro specific, is there a scope to take this beyond some of the top cities? That's my first question.

And secondly, if I look at where the industry is, given that some of the price increase has happened, competitive intensity has stabilized or decreased.

So from year on, if I were to build a scenario of doubling our patient count of 27.5, 28 million over the next few years, what are the key variables which will get us there?

Is it convenience? Is it accuracy? Is it increased genomics contribution? Is it doctor advocacy, if you could give some direction on those parts will be helpful?

Shankha Banerjee: Right. Okay. Thanks for the question. So on Swasthfit, it is not a metro phenomenon for us. I think over the last few years, we have been consciously expanding and driving Swasthfit into multiple geographies. And that maybe one of the underlying reasons why we are seeing consistent growth, and we will continue to do that. So

that expanding the footprint for Swasth is something which we are actively working on and it's a continuing process. So that point is there. On the other question on where and how will the future growth come from? So yes, there will be, like we said, the patient and samples, there are two measures of volume that we see. I think both of them are what we are really eagerly looking forward to be driving, although there is a price increase taken by competition and there is this sense or feeling that intensity is low. We believe that competitive intensity underlying intensity is still what this industry has been used to or has been

seeing in the past continues.

So we are not really looking to a price increase driven growth. We continue to believe in driving our growth through patient and sample volume addition and mix improvements.

Prakash Kapadia: I get to the point of price increases, not the route. But in terms of the patient volume growth, I'm just trying to envisage a scenario what will that one or two variables be? Because in terms of branding, we are one of the oldest companies. In terms of cost per sample, we are amongst one of the best. So why is that volume growth not really coming through for us? So, is it going to be some of the newer tests, which will contribute more or some specific convenience factor from a consumer standpoint? I was more trying to understand that.

Dr. Om P. Manchanda: Okay. This is Om here. I don't think we should really get hung up on just the volume number, okay. As I mentioned to you, I think there's a lot of interplay between test per patient and the footprint. The volume number that we are talking about technically is a footprint. It's not a unique patient. I think the question that you asked is a very great question in terms of what is the variable which we probably need to address to look at the entire market. My view is that any category, if you pick up, they're always bucketed into three price points: One is the premium end, mass market and the discount end.

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Classification: Confidential Right now, I think Dr. Lal PathLabs fuddles in all these three segments. As we go forward, we'll have to carefully segment the market and design our proposition in such a way that premium customer gets premium service, but at a premium price as well. Discount customer gets discounted service at a discount price. Now getting it offered under same names sometimes is very, very difficult to achieve that in the same market.

To me, I think we'll have to probably look at that. That is where, somebody used to ask me that Suburban, are you going to keep it separately, the answer is yes. It has multi-brand strategy and also different services and at different price points. I think we are reaching at that stage where market will have to address these things as we go forward.

Moderator: We'll take the next question from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: So, considering in the last quarter, we have seen some upward pricing action from online players as well as other national chains. Do you believe that our volume growth is a result of this pricing action since you have not taken any hikes and follow-up on that would be then could these be our sustainable volumes considering that we are not looking to take a price hike in the next few quarters?

Dr. Om P. Manchanda: It's early days, and some of these competition that you are talking about, our friction with these guys is not really head on, except in few cities. The markets where we are present, actually, we don't really compete too much with them. The early sort of a sign of increase in volume that we are seeing maybe 2. A). of course, base effect because last year, volume growth has not been that much. Second could also be that, which I'm not fully sure, but I still will make that point. I get a sense that last two years, non-COVID volume also was being influenced by COVID. As that has gone away now, we are actually in a clean base, and we are seeing now original non-COVID volume, so to call it. I think from here on, we'll have to probably wait it out for another quarter before we start making any sort of view on this. But definitely, directionally, I'm seeing a better volume growth than what we have seen on a year basis.

Anshul Agrawal: Got it. And another question on competitive intensity, Dr. Om, if I may. So, considering that one of the other listed regional chain is getting very aggressive in Pune. How do we aim to protect or expand our market share or suburban's market

share in the Pune region?

Are there any thoughts around this? Any strategy?

Dr. Om P. Manchanda: First of all, I think market is very large. So, it's that organized players, put together is not even 15% of the total market. So, there is a lot to gain for all of us from unorganized space. And if I were to guess the name of this company, which is

probably they are more an integrated business of radiology and pathology compared to us, which is just pathology. So, right now, our business model is to actually play more in pathology, which is more hub-and-spoke model, low capex, high ROCE v/s, let's say, take integrated model. While both models are well proven, there's nothing wrong with the other side as well. But I would say the market is large enough for all of us to stay.

Anshul Agrawal: Sure. Just one last question from my end. Any sense on what would our SSG volume growths would be for the current year and the contribution from the new centers?

Ved Prakash Goel: I think, Anshul, we don't track like this because like Dr. Om was mentioning hub and spoke, so we have geography or a cluster where we have lab is one. But we have collection center, and we have pick points around that. So as a cluster or geography, we track but as a same-store, we don't look at that way. But separately if you see Delhi as a political state, probably we are not expanding that much, which is growing,
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Classification: Confidential let's suppose high single digit. That is where you can look at, and rest is coming from the new infra or expansion of higher growth in the newer markets.

Anshul Agrawal: Got it. Very clear. Just one last question from my end for Ved -ji probably. Just as you mentioned that gross margins would remain range-bound. Any thoughts around EBITDA margins, given our expansion plan?

Ved Prakash Goel: I think we are saying pre-COVID level margins, which is around 26% EBITDA margin, where we are confident that we can maintain.

Moderator: The next question is from the line of Jainil Shah from JM Financial.

Jainil Shah: My first question is on our guidance for next year. So, we alluded to our growth being higher in FY25. So just wanted to understand how much will be volume-driven and how much will be price-driven and how much growth are we budgeting for suburban here?

Dr. Om P. Manchanda: I think the way we look at our numbers is that internally, the view is that we should probably not take a price increase as things stand today. If we don't do that, I would say that we will probably better our growth slightly than what we have delivered in FY24 without the price increase. The next question is, where is it going to come from? It's going to be a combination of both volume as well as number of tests per patient, and maybe a mix also because we are seeing higher end tests, also being prescribed more. So, our internal sort of desire is to beat the last year growth without the price increase.

Jainil Shah: Sure. And would you say that FY24, at least 4Q, we would not be having any COVID revenues? So essentially, we've grown at around 14% on a non-COVID base?

Dr. Om P. Manchanda: So, right now COVID was not there, that is fine. But what is the other thing you said 14%.

Ved Prakash Goel: So full year, growth is 10.4%.

Dr. Om P. Manchanda: So I think the best way to look at the following. We are exiting the year, which is Q4 at 11.1% revenue, okay, which is primarily without any COVID impact, either in the base or in current year, right. More or less. Now this 11.1% has an advantage of one month where there was no price increase, has a disadvantage of base of two months, which is Feb and March. If we continue without the price increase, we are confident that we should be on this number or slightly better.

Jainil Shah: And just wanted full year revenue and EBITDA for suburban?

Dr. Om P. Manchanda: This year?

Jainil Shah: FY

24, yes.

Ved Prakash Goel: This year, we did Rs. 164 Crore revenue for suburban.

Jainil Shah: And EBITDA would be? Or we are incurring losses there because of the selling lab?

Ved Prakash Goel: 17% EBITDA margins.

Dr. Om P. Manchanda: No, that is for the quarter. Full year?

Ved Prakash Goel: Full year is about 12%-13%.

Dr. Om P. Manchanda: Full year is 13%, Q4 is 17%.

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Classification: Confidential Moderator: The next question is from the line of Shaleen Kumar from UBS Securities India.

Shaleen Kumar: Sir, just a clarification, right? Most of my questions have been answered. So, Ved sir has been saying that our margins should be pre-COVID level 26%, right? But you have already delivered more than 27%. And I understand, let's say, gross margin remains where they are, but there is definitely going to be operating leverage. Now two things can happen from here, either your margins will expand or probably you said you want to invest in growth, which means that growth expansion should happen, right? So, is my understanding right that you have decided to invest in growth and then the growth uptick will happen while margin may remain where they are, with a lag, that's okay, but that's the idea, right?

Dr. Om P. Manchanda: I think Ved was explaining gross margin, there's hardly any scope left now for gross margins to improve. And your point of operating leverage is definitely valid that if we grow from here organically, you may see expansion of EBITDA margins. But we want to really plough it back into the market going to Tier-3, Tier -4 towns, where maybe pricing could be , may not be exactly the way Tier-1 pricing is, where our cost structure may be very different. But we will try and invest more behind growth rather than expanding our margins .

Shaleen Kumar: Which is fine. Sir, my question was that, that will reflect in our growth, maybe with a lag over a medium term, right? So that's the way we should look at it, right? And the main part of that will come in .

Dr. Om P. Manchanda: Yes, ultimately, because there are so many white spaces in the market, our company has to focus on growth. There's no doubt about that.

Shaleen Kumar: Which is fine with us, sir. I just wanted to have a clarification on that.

Dr. Om P. Manchanda: No, no. It's very clear. And that would also include M&A as well. So , it's not that , while it may be a bit of a time since we did last M&A, but that has not been our eyes are not on inorganic. We will also search for inorganic.

Shaleen Kumar: Would you actually answer my second question, because you're already close to Rs. 1,000 Crore of cash. And I mean, you will be generating roughly Rs. 400 Crore of cash on average. So is this something, which we can expect in the near term in terms of the organic?.

Dr. Om P. Manchanda: I don't know, right? Let's just see how it goes. Right now, there's nothing that we can share with you. But definitely, it's part of our stated strategy.

Moderator: The next question is from the line of Pranav Chawla from Antique Stock Broking.

Pranav Chawla: I think we highlighted we plan to add around 20 hubs going forward. So , can you allude us, what time line and what CAPEX kind are we looking at? And what regions this will be in?

Dr. Om P. Manchanda: See, our average capex for this normally is about Rs. 50-60 Crore , which is a combination of investments in IT, investments in instruments, investments in labs.

So I would say that will continue. It's about Rs, 60-odd Crore of normal CAPEX . Most of these expansion on opening labs primarily been in Northern part of India and rest of north, essentially where our brand is strong. That is where we are seeing better traction for the business for our lines. I think that's where we will go.

Pranav Chawla: Perfect. And sir, what markets we'll be looking for inorganic expansion? Because I think Calcutta, East, we have been t

rying to grow organically and still the size of the business is pretty small. Same is the case with I think West is also now at 15%. So what target markets we'll be looking at?

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Classification: Confidential Dr. Om P. Manchanda: I think the answer is in these numbers only, so just 31 Delhi NCR, 31 rest of North,

15 East, 15 West, South is only 6. So these numbers are telling me where I should go. So obviously, South is one, definitely number one priority. It's not that we can't do M&A in other markets as well, but sometimes it becomes very challenging because with same surveys, a lot of friction that happens on the ground in terms of teams, in terms of brands . It's always better to go in those markets where brand presence for Dr. Lal PathLabs is weaker. And in any case, we are not doing these M&As to ramp up our top line, we are doing it more as a market entry strategy. So it is natural for us to look at in those places where our business is weak, which, to my mind right now it looks like South is the place.

Pranav Chawla: Sir, any large players or large competition in these markets like that we may explore, like number two, number three players .

Dr. Om P. Manchanda: No. I think we are indifferent size. My sense is we look at even not just region, we also look at just city level entry as well. So we are open to even smaller size acquisitions as well. So there's nothing that holds us back. In fact, our PUPL, we haven't talked about it much. But places like Indore, places like Baroda, we've actually bought individual labs, which was just about Rs. 8-10 crore as well. Now since the number is small, they don't come up on the radar for us for discussion. But we are open to doing those tuck -in acquisitions as well.

Moderator: The next question is from the line of Dheeresh Pathak from White Oak Capital.

Dheeresh Pathak : I'm sorry if you already answered this sir, but for the full year and for the quarter, if you can just give how much growth came from pure realization increase, like -to-like realization increase, not the effect of mix?

Dr. Om P. Manchanda: Taking out the mix impact. You mean due to price?

Dheeresh Pathak : Just from price increase, not coming from any mix effect or anything. Just your price increase, how much growth came for the full year and for the quarter?

Shankha Banerjee : Just pure price, we got realization benefit of 3.5% for full year.

Dr. Om P. Manchanda : On a year basis, but it's lesser in Q4.

Shankha Banerjee : In Q4, it is lesser. But on a full year basis, 3.5% is pure increase driven, which is for full year.

Dheeresh Pathak : And this is annualized in this year? This got annualized in Q4 now, right?

Shankha Banerjee : Yes. So the Q4 contribution of price increase is only about 1%, 1.1% or something.

Dheeresh Pathak : 1%, okay. And provided if you don't take any price increase further, then this will be almost negligible in the coming quarters, right?

Dr. Om P. Manchanda: Impact of price, that's what we are saying that we want to beat the last year growth without the price increase this year.

Moderator: This will be the last question for today, which is from the line of Sumit Gupta from Centrum India .

Sumit Gupta: So just want to understand about like pricing scenario for overall test menu. So just can you highlight like how much is the specialized test menu? Like what kind of price increase are you taking in the specialized menu v/s obviously the routine test?

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Classification: Confidential Dr. Om P. Manchanda: No. as of now, we don't plan to take any price increase neither on specialized nor on routine.

Sumit Gupta: Okay. And how much is the specialized test menu as the overall test menu , specialized and overall test menu?

Shankha Banerjee : So specialized portfolio today for Q4 has contributed to about 20% of our overall revenue.

Dr. Om P. Manchanda : And by the way, they a

re not comparable because each company may have its own definition of specialized, so just keep that in mind as well. So it's not that somebody tells you that they're specialized menu is 30%, you start assuming that we're lower.

So ev erybody has his own way for defining these tests.

Moderator: Ladies and gentlemen, we'll take one more question, which is from the line of Shaleen Kumar from UBS Securities India .

Shaleen Kumar: So when you said that you will be delivering better performance compared to FY24 and there will not be any price increase there, right? So assuming that our sample per patient growth would be similar of what we have in FY24 , so we are looking at roughly high single digit of volume patient growth. Is that the right thing? At least, if not high, that means it is something like 7% odd kind of a growth quarterly?

Dr. Om P. Manchanda: No, no. How it is 10%?

Shaleen Kumar: No, no 6 %-7%, not 10%.

Dr. Om P. Manchanda: Okay. It's a bit early. Yes, I think that's the sort of target we are keeping in our mind that it must definitely move beyond 5 %-6%. Because I think for year basis , right now, this year is about 3%?

Ved Prakash Goel: 3.2%.

Dr. Om P. Manchanda: 3.2%. We want to see definitely this number moving up. And should be about 6% quarterly was the better number, yes, you're right.

Shaleen Kumar: So basically, even if 12% revenue growth we're looking at, 5% coming from higher tests .

Dr. Om P. Manchanda: So Shaleen, I think that depends on how ramp up on Swasthfit happens because that will give us a higher number of tests per patient. But you are right, we are looking at definitely, if you look at this quarter exit, which is 11.1%, has some advantage of base in pricing due to pricing. But we want a really better base and with a combination of volume as well as number of test per patient. But tests per patient, in any case, is baked in. So most of the growth will come for higher volume g rowth.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Ved Prakash Goel: Thank you, everyone, for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries satisfactorily. Please feel free to reach out to us in case you have any further quest ions or queries. Thank you once again.

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Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street

Mumbai – 400 001
Scrip Code: 539524

Sub: Transcript of Q1 FY25 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q1 FY25 Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral

Company Secretary & Compliance Officer

Encl.: As above

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Classification: Restricted Dr. Lal PathLabs (DLPL)

Q1 FY25 Earnings Conference Call Transcript

August 07, 2024

Call Duration •1 hour

Management Speakers •(Hony) Brig. Dr. Arvind Lal – Executive Chairman

•Dr. Om Prakash Manchanda - Managing Director

•Mr. Shankha Banerjee - Chief Executive Officer

•Mr. Ved Prakash Goel – Group Chief Financial Officer

Participants who asked

questions •Rahul Agarwal - IKIGAI Asset Management

•Karthik Chellappa – Indus Capital Advisors Hong Kong Limited

•Binay Singh – Morgan Stanley

•Prakash Kapadia – Spark PMS

•Sumit Gupta – Centrum India

•Adrit Chaturvedi - Nomura Financial Advisory and Securities India

•Kunal Randeria – Axis Capital

•Shaleen Kumar – UBS Securities India

•Deven Kulkarni – Marcellus Investment Managers

•Abdulkader Puranwala – ICICI Securities

•Pranav Chawla - Antique Stock Broking

•Ashutosh Parashar - Mirabilis Investment Trust

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Classification: Restricted Moderator : Ladies and gentlemen, good day and welcome to Dr. Lal PathLabs ' Q1 FY25 Earnings Conference Call. As a reminder, all participant lines will be in the listen - only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Siddharth Rangnekar of CDR India. Thank you and over to you.

Siddharth Rangnekar: Good evening, everyone, and welcome to Dr. Lal PathLabs ' Q1 FY25 Earnings Conference Call. Today, we are joined by senior members of the Management , including (Hony) Brig. Dr. Arvind Lal , Executive Chairman, Dr. Om Prakash Manchanda, Managing Director, Mr. Shankha Banerjee, Chief Executive Officer and Mr. Ved Prakash Goel, the Group Chief Financial officer.

I would like to share that some of the statements made on today's call could be forward -looking in nature and actual results could vary from these forward -looking statements. A detailed statement in this regard is also available in the results presentation that is available on the stock exchange website s and has been

separately circulated to all of you.

I would now like to invite Dr. Arvind Lal to share his perspectives. Thank you and over to you, Dr. Lal.

Dr. Arvind Lal: Thank you, Siddharth . Good afternoon and a warm welcome to all participants on this call. We are here to discuss the Q1 FY25 Earnings Performance of Dr. Lal PathLabs. I would like to commence by sharing my insights into the changing market dynamics and the advancements we have achieved.

This year also marks our 75th anniversary. Dr. Lal PathLabs continues to be a trusted healthcare partner, renowned for its commitment to quality, accuracy, accessibility and affordability. The diagnostics market in India is expected to grow as people live longer, more tests get prescribed, and the shift from unorganized to organized sectors builds momentum.

Our unyielding attention to excellence in diagnostics has earned us the trust of both patients and physicians over the years. Utilizing our digitally enabled network infrastructure, we remain agile to service growing demand for testing. Our custom - built logistics solutions further enhance our capability to serve our patients more efficiently.

The integration between Dr. Lal PathLabs and Suburban diagnostics is progressing as per our expectations. With the Suburban brand, we are making volume expansion a priority. We are making strategic investments in both marketing and process efficiencies to fortify the brand while enhancing the patient experience. We are creating opportunities to grow sample volumes with the help of hub labs in important clusters around the rest of the country. Our synergistic approach enables us to provide patients with a comprehensive array of tests across multiple categories with speed and precision.

Looking ahead, our growth trajectory will be driven by the ongoing transition from unorganized to organized sectors, heightened awareness of accurate diagnosis and emphasis on wellness among others.

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Classification: Restricted We remain dedicated to expanding our network and enhancing our service standards through operational excellence and innovative technological advancements. I am confident that the capabilities that we have created to achieve sustained growth will further consolidate our market share. Thank you and I would now like to hand over to Dr. Om Manchanda. Over to you.

Dr. Om P. Manchanda: Thank you, Dr. Lal. Welcome to our call today. I shall walk you through the evolving industry scenario and progress on our strategy.

I am pleased to report that we have achieved 11.3% growth rates on a YoY basis.

This is a result of the consistent

execution of strategy of expanding presence in identified towns and cities where our brand enjoys salience, particularly in West, North, Central, East and across the rest of the country.

The contribution from suburban diagnostics to the top line is scaling up in line with the initiatives that we are driving on franchising and brand communication. We continue to prioritize broadening and deepening the brands , geographical presence in order to derive sustainable volume and value growth. Our focus, therefore, is on scaling the next tier, expanding into Tier-3 and Tier-4 towns with lab infrastructure. We are establishing 20 new labs to broaden our reach with plans to add more labs as the year progresses.

DLPL is present in most prominent cities where we continue to densify our PSC network. Suburban is the fulcrum for the thrust into Mumbai, parts of western India including Goa and Pune. We are actively exploring opportunities to expand our footprint there, focusing on assets that align with our core values and strategic goals. Our Swasthfit brand, a steady revenue contributor has delivered around 25% of the total revenue and continues to expand. The medical centers of excellence which we have established are scaling effectively and enhancing patient engagement and outcomes. Driven by strategic emphasis on sustainable volume growth, these efforts aim to draw patients from unorganized labs, promoting the adoption of a quality and accuracy -backed approach to testing.

Our investment in technology and into infrastructure are delivering concrete outcomes, strengthening brand visibility and facilitating smoother interactions with patients. Patients today access our services across channels and our infrastructure naturally has kept pace with these requirements. As its matrix evolves, brands like ours are in a strong position to access growth. We remain attentive to market dynamics, adapting our approach to ensure sustainability of outcomes.

Moving forward, maintaining a solid financial foundation is crucial. To focus efforts, we aim to maintain our robust operating metrics and drive a strong cost structure that

will form the basis of the growth initiatives.

I now hand over to Shankha to delve deeper into business initiatives and operational highlights. Over to you Shankha.

Shankha Banerjee: Thank you. I welcome all participants joining this call today. I will proceed to share the business and operating highlights with you.

We are pleased to announce a strong start to the year, achieving robust performance in both revenue growth and profitability. In Q1 FY25, we achieved a revenue of Rs. 602 crore, representing a 11.3% growth over Q1 last year. This growth in revenue is driven by patient volumes, which is at 7.2 million and 21.1 million samples. Patient volume growth for Q1 FY25 is 5.5% and sample volume growth is 9.6% over Q1

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Classification: Restricted FY24. Notably, we saw a 10.4% increase in patient numbers in Q1 FY25 as compared to Q4 FY24. Our Q1 FY25 revenue per patient stands at Rs. 833, which is same as Q4 FY24.

We are strengthening our digital infrastructure and capabilities encompassing cutting edge data analytics in order to enhance testing and service outcomes for patients.

This is going to be a focus area for us. Dr. Lal PathLabs and Suburban are driving the omni -channel experience forward with the help of technology.

In the period to come, we expect a combination of volume gains and realization mix to drive the revenue growth. This shall include favorable tests and geographic mix, the traction in bundle testing and build up in volumes in specialized tests shall also contribute to this.

In conclusion, I would like to emphasize that we are progressing as per plan on our growth objectives. Our future performance will be guided by expanding our footprint in identified clusters, enhancing our digital capabilities and enhancing patient servi

ce

outcomes. Thank you for your ongoing support and trust in our journey.

This concludes my opening thoughts. I would now request Ved to take you all through the Financial Performance. Over to you, Ved.

Ved P Goel: Thank you, Shankha. Good afternoon, everyone and a warm welcome. I am sharing some of the key Financial Highlights for Q1 FY25 . Revenue for Q1 FY25 came in at Rs. 602 crore against Rs. 541 crore last year same quarter, a growth of 11.3%.

Revenue per patient for Q1 FY25 came in at Rs. 833 v/s Rs. 789 in Q1 FY24.

Sample per patient for Q1 FY25 is 2.92 with a growth of 3.9%. EBITDA for Q1 FY25 came in at Rs. 170 crore v/s Rs. 146 crore in Q1 FY24, registered a growth of 16.2% with the EBITDA margin of 28.2%. PBT for Q1 FY25 came in at Rs. 150 crore, registered a growth of 27.6% with PBT margin of 24.9%. PAT for Q1 FY25 came in at Rs. 108 crore v/s Rs. 84 crore last year same quarter, registered a growth of 29.1% with PAT margin of 17.9%. EPS in Q1 FY25 is Rs. 12.8 v/s Rs. 9.9 in Q1 FY24. Net cash and equivalents as on June 30th, 2024 is Rs. 1,044 crore.

We are pleased to announce that the board of directors has approved an interim dividend of Rs. 6 per share for FY25.

With this , I conclude my opening remarks , and I would now request the moderator to open the forum for Q&A.

Moderator: We will now begin the question -and-answer session. We will take our first question from the line of Rahul Agarwal from IKIGAI Asset Management. Please go ahead.

Rahul Agarwal: I have three questions. Firstly, Dr. Om, on the Tier-3, Tier-4 markets, as you are expanding infra, I just wanted to know any learning from patient behavior here like, how are these patients different from metros and Tier-1, what are they seeking from a brand like yours?

Shankha Banerjee: In terms of expansion into Tier-3, Tier-4 primarily it's a lot about looking at local unorganized players and we are going to compete with them and our offering there is quite similar to what we offer in Tier-2 and Tier-1 cities as well. We lead with quality and service and obviously we are finding quite a lot of patients and clinicians who are embracing us in these cities. The offer is really not significantly different from what we are doing in our other cities.

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Classification: Restricted Rahul Agarwal: Is the pricing very different or even that remains the same?

Dr. Om P. Manchanda: Rahul, I will probably slightly give you a higher -level perspective. What we have seen is that most of these smaller towns, their behavior is influenced by nature of the prescription given by doctors. What we are seeing is that the healthcare is now travelling to Tier-3, Tier-4. And I think it's a bit of a qualitative comment. I think overall quality of medical manpower is also going up as more qualified doctors settle down in smaller towns, they tend to prescribe tests which are also higher order. I think this is more a market evolution where we will see level of testing going up with time and directionally it's quite visible in our numbers as well. I think that's where I would want

to make comment.

Rahul Agarwal: Secondly, on this Medical Center of Excellence, just wanted to know, could you elaborate what is this exactly, are we putting up a different infrastructure here or is this the current lab is getting transformed into something?

Dr. Om P. Manchanda: I think it's a good question. I will explain to you. I think you guys cover lot of pharma businesses. If you look at the journey of this entire pathology, most of the differentiation in the past has been service, branding, brand awareness, top of the is stuff and all of that. But as we have progressed in the last 20 years, the market has grown, more competition has come, so everybody's offering the same thing. Now the only way you differentiate is that you start looking at different segments. Like in pharma, you have a Gastro division,

you have an Oncology division, you have that and this. Answer to this question is we are trying to segment the market and look at broad segments and especially on high end, the segmentation is done based on busy state and that's how we have identified certain segments like Oncology, Autoimmunity, Reproductive diagnostics and trying to get more focus from the corporate side, so that whether it's introduction of newer tests, whether it's introduction of new communication with the medical fraternity, organizing seminars or creating patient cohorts. A lot of those activities in a focused manner, we are trying to create these categories and that's the whole idea. Operationally, though we have some people on ground, but we are not trying to create different teams for these segments.

Rahul Agarwal: These are like existing people working at different labs which are identified as where you want to increase communication and you want to focus on specific specialties, is that correct?

Dr. Om P. Manchanda: I would say higher focus on marketing efforts, everything else remains the same, that's it.

Rahul Agarwal: Lastly, on this Swasthfit specialized portfolio, just wanted to know, I know that it's small right now, but what kind of scale or total mix into overall Swasthfit, do you visualize over the next 3 years?

Shankha Banerjee: The Swasthfit specialized portfolio is primarily aimed toward clinicians, and we are still kind of working on it. It's not something which has reached the level of scale for us to differentiate and start reporting those numbers separately.

Rahul Agarwal: Shankha, I understand. What I was asking was is more like a 3-year view, would you visualize like what kind of a specialized mix would you have within Swasthfit, would you have that?

Dr. Arvind Lal: The people around India especially in these Tier-3 and Tier-4 towns they have not realized the importance of these so-called non-communicable diseases and lifestyle diseases which we are trying to make an awareness and that is also a part of why the Swasthfit is doing well.

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Classification: Restricted Moderator: We have our next question from the line of Karthik Chellappa from Indus Capital Advisors, Hong Kong Limited. Please go ahead.

K Chellappa: Sir, I have three sets of questions. The first one is, if we were to look at our volume growth, let's say of about 9% or so, could you give us some sort of perspective on geography wise, how that is faring, I mean, which are like the geographies driving double digit and which are the ones which are still lagging?

Ved P. Goel: Karthik, I think we are not giving geography wise split quarterly, but once in a year we are giving it. Having said that, the geography is like rest of north and east which obviously we are performing well, and they are doing better than our company average. Where we are going much deeper, which is, let's suppose rest of north and especially UP where we are going Tier-3 and plus, that is obviously the higher growth as compared to company growth.

K Chellappa: My second question, sir, is what would be the like-for-like price increase that we have taken on a YoY basis across tests?

Dr. Om P. Manchanda: We took last price increase, that was in February 2023, and we completed one full year of cycle of that in Feb '24 and we have not taken any price increase and neither we have intent to take as of now.

K Chellappa: Which means if I were to look at, let's say the realization per test, which is probably up 2% YoY, that's probably driven more by mix, right, rather than anything else on a like-for-like?

Dr. Om P. Manchanda: Absolutely. This is the mix which is driving this revenue per patient up.

K Chellappa: Sir, would you be able to share the revenue and EBITDA margin for suburban this quarter?

Ved P. Goel: Suburban revenue growth for this quarter is about 8% and EBITDA is about 14%.

Moderator: We will

take our next question from the line of Binay Singh from Morgan Stanley.

Please go ahead.

Binay Singh : The 8% growth in suburban you commented so basically , we would have done around Rs. 40 crore or so of revenue. Is that YoY that you gave?

Dr. Om P. Manchanda: Yes, it's YoY, same quarter last year.

Binay Singh : Any reason for suburban growth to be lower than your overall growth, any comments on that?

Dr. Om P. Manchanda: I know this is the question which is in everyone's mind. I think the number doesn't show, but we are not unduly worried about this. I think the fundamental thing what has happened in Suburban is there is a very strong strategic shift that is taking place in that company. This company used to be more driven through Doctors network in the past, own infrastructure in the past, we are strictly going by franchising, infrastructure direct -to-consumer, there was a lot of the business of CRO al so which actually was not a business which was very B2B, not a sustainable thing. So, we are trying to build this company more direct -to-consumer, make sure that health checkups and things like that. I think overall directionally 8.3% is definitely an improvement from what we have experienced in the past. Directionally, I think we're getting more confidence about suburban. I am not really too much worried about this number. Ved is telling me core business growth is 13%. I don't want to get into this.

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Classification: Restricted To me, one number is where we stick to, we don't really slice this number into core and non -core. I stay very confident on suburban. That's the way I would say. See, it's actually not run QoQ. It's a need -based business. You just keep doing your work slowly, slowly, one gets the results. I am very, very sure about this. Look at margins of 14.1%. Even before our transition , pre-COVID had single digit margin. So, there is an improvement which is visible. There are green shoots here.

Binay Singh : And sir, secondly, on your own margins, because in the past we have always seen that Q2 is usually the strongest quarter for the company and given revenue leverage that falls in, so margins also tend to be the highest in Q 2. Would you sort of expect a similar trajectory into this year also?

Dr. Om P. Manchanda: Yes, Q2 is always higher. You are right. Most of the spike actually happens around September. But there is no reason why it should not happen because the season, rains, etc., all that happens every year. But as the revenue tends to be in absolute terms also higher in Q2, everything just flows into the margin. I presume that Q2 will always remain higher margin compared to a new quarter.

Binay Singh : In the sense, your margins could at least hold up or inch up on an annualized basis from these levels as you are getting leverage gain like it's visible in your Tier-3?

Dr. Om P. Manchanda: But it gets knocked down in Q3. I just don't look at quarter wise. My eyes always are on the annualized margin where we should really focus.

Binay Singh : And lastly, like after many calls, you've not made much comment on competition this time around in the call. Is that because you see it easing or there was not much to add, that's why it was not there in the opening remarks?

Dr. Om P. Manchanda: I think there are two directions one can talk about competition. One is the intensity of competition which to my mind will continue to remain, it was always there. I think competition became visible because a lot of private equity money and I think there were different format s of cash burn, etc., they were spending a lot of money on promotions which to me has eased off. That's why they are not that visible now. So, competitive intensity from visibility perspective has come down, but it' s not that they are out of the business, they are still there. Those companies that one used to talk about , it continue s to operate. But I think th

e spend to acquire customers, spend to

actually just be visible anywhere and everywhere, that too has come down. I think the other intensity in terms of predatory pricing, this kind of promotion, this Rs. 199, Rs. 99 tests and all, all that actually also has gone away. But in terms of number of players, I think that intensity still continues. And I think as the overall market is growing and you know a healthy sort of margin profile in the industry will always attract competition. And I've always maintained competition is good for the industry because it overall brings up the table in terms of quality level -playing field, and at the end of the day, organized market stil l has to grow because it has to grow from unorganized and which is a very large segment. So, there is a space for everyone to be there as long as there is healthy competition. I think to my mind that extreme, unhealthy competition has now become healthy co mpetition, but competition is still there.

Moderator : We will take our next question from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: Could you give us some sense of specialized test as a percentage of overall revenues maybe last year and do we see that as a differentiator going forward

because we seem to be investing in R&D, we seem to be doing a lot of work on newer tests, so where are we in our journey? Secondly is on Suburban, what will it take to get that inflection point because it's almost now 3 years and we have been

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Classification: Restricted investing in process, we have built the infra we have transitioned from being a doctor advocacy to a consumer brand and I think we are still #3 in Mumbai, what will it take for us to grow Suburban faster than the company average?

Shankha Banerjee: I think on the first question regarding specialized, the specialized portfolio is again the way we define it, because specialized portfolio definition can vary from organization to organization, the way we define it, it currently contributes about 23% of our revenue v/s let's say similar period last year, the contribution would have grown by about 1 -1.5% points. So, yes, that contribution is slowly increasing. It is a differentiator in terms of our ability to really get into larger institutional businesses and also get key opinion leaders to kind of endorse the brand. Is it a huge volume builder? As of now like I said, the contribution is more or less stable, maybe 1% up over a similar period last year. Coming to the question on Suburban, I think the way to look at Suburban is that there is a core geography focus that we talk about, and that geography is slowly and steadily responding for us. But, yes, as a number one when we look at what is happening to suburban, there is quite a lot of non-core business which is still a drag on the overall number, and one would imagine that it still would take maybe few more quarters for us to start seeing the overall suburban number growth rates to maybe move to double digit and early teen level. And after we reach there, we will see how we can get it going faster than DLPL.

Prakash Kapadia: That would be Shankha, from Mumbai only or this Pune or Goa initiative will get us there?

Shankha Banerjee: No. So, we have defined Mumbai, Pune and Goa as the three core. I think those are the three geographies that we are spending all our efforts on right now and those markets are responding quite positively as well.

Moderator: We have our next question from the line of Sumit Gupta from Centrum India. Please go ahead.

Sumit Gupta: I have some questions on Swasthfit. I just want to understand what is driving Swasthfit. So, if we see like overall contribution has been increasing on a sequential basis also? And can you explain how much is coming from rural and urban?

Shankha Banerjee: Let me try and take the first part of the question as to what is driving Swasth

fit. So, there are three screens that we see. First and foremost, there is this whole awareness amongst the public regarding preventive testing. The whole area in the metro cities as well as kind of approach that we put on online, etc., so there is a slice of that preventive wellness market that we see is helping Swasthfit growth. Secondly, there is also something which we see with channel expansion. We also notice that there is some upselling which happens in the channel. And lastly, we are also seeing in certain markets that clinicians are moving toward starting to prescribe packages. So, those are really drivers. And as we expand our reach further in metro as well as Tier-2, Tier-3 kind of towns, we see that Swasthfit growth is likely to continue. That's really maybe the first part of the question. In terms of split between urban and rural, I think we haven't got those numbers ready. So, for us it is primarily from Tier-1, Tier-2, Tier-3 towns, we will have to see what is the exact split and we can maybe share that number subsequently sometime.

Sumit Gupta: To take a broader view over the next 3-4 years, what is the peak contribution that you expect the Swasthfit to contribute to overall revenue?

Shankha Banerjee: That's a quite a pie in the sky kind of a question. At least one thing we can definitely foresee is that if you see next 3-4 years, it should be higher than where we are today. But how much higher and where is the limit? I think that's something which we are still trying to work out.

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Classification: Restricted Dr. Om P. Manchanda: We did a math and we found out that for the last 5 years, directionally every year we are increasing by 1.5%. Now, obviously that was on a much smaller base. So, let's hope that continues for some time for next 2-3 years.

Sumit Gupta: How do you see the margins inching up, faring well due to Swasthfit or overall business?

Ved P Goel: As Dr. Om mentioned, we don't see margins QoQ, but on annualized margins.

Sumit Gupta: Overall like over the next 3-4 years?

Ved P Goel: On a longer term, we said in the last call around 27% kind of margins because we also are doing lot of investments, we are going deeper, we are spending more and lot of new investments are required which will give obviously top line in a longer

period. But in the longer term, I think you don't think these kind of margins, much sustainable is around 27% kind of mark.

Sumit Gupta: What was the revenue contribution from North and West, the overall geography mix for this quarter?

Ved P Goel: I think again quarter wise it will not be representative, but I am saying we had like Delhi and NCR which is 31 %-32% kind of contribution and rest of India is balance.

Moderator: We will take our next question from the line of Adrit Chaturvedi from Nomura Financial Advisory and Securities India. Please go ahead.

Adrit Chaturvedi: I would just like to follow up on the awareness campaign bit that you had mentioned earlier. So, in these awareness campaigns, are there also any discounts that you are offering to drive up a lot of these bundle testing volumes?

Shankha Banerjee: So, I think these are two different things in itself. I think bundle testing by definition is discounted. There are awareness campaigns which are built on promoting the bundle test. Like I said, if you look at a package, the package is inherently itself discounted. That's one part of it. But I think some of the maybe visible campaigns that you observe, those are more to do with building awareness of the brand and the service, which is slightly independent from specific package promotion.

Adrit Chaturvedi: Would you have the number handy and if you could give it like at MRP if there are any kind of increases in these bundle testing, right now they're discounted, so there's no true sense of volume that we could gauge, if there's something like a GMV number on that?

Ved P Goel: Generally, I mean these packages are discounted at a 20% -30% kind of discount on MRP and depending on the packages.

Adrit Chaturvedi: Also, I think a lot of these bundle testing are also driving up your test for patients, right? So, right now it's at 2.9. As the share of this increases, do you have like a sense of how up it could go like it will go across 3 or like a 3.1 or is there a vision there that you would like to have more tests per patient or an average number that you track?

Shankha Banerjee: We don't have any operational target on saying that I want to drive test per patient. I think our operational targets are more around increasing our reach and also making these bundle tests available and promoting them. Naturally we see by doing all of these test for patient grows as well as revenue per patient grows. Now, we don't really work on a target saying that I got to grow so much basis. Because it's also a

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Classification: Restricted lot of other factors which drive including geography mix, specialized test portfolio and also the channel through which we sell. We really don't work on a specific target on that. But as the contribution increases of Swasthfit, the test per patient is likely to go up.

Adrit Chaturvedi: This specialized portfolio, so now that it's like 23%, what kind of gross margins are you seeing on that? And I presume that there will be on the lower end of the whole portfolio?

Ved P Goel: Gross margin is different on this specialized v/s routine, but net -net on EBITDA level, I don't think there is much difference because these are high end mostly, we are getting outsourced from hospitals or some 3rd parties where we just spend on logistics. But on B2C there is an infra cost, servicing cost which is much higher. Gross margin is different but on EBITDA level there is not much difference.

Adrit Chaturvedi: Could you qualitatively or like tell me how different could they be like an average specialized versus a routine like what kind of gross margins differentials are you observing?

Ved P Goel: Those are very difficult to measure, but we are measuring on bottom line which is net-net on EBITDA level.

Adrit Chaturvedi: And do you have like a mix of the institutional business through Swasthfit that you are doing versus your retail?

Shankha Banerjee: Swasthfit is predominantly a retail product. It isn't so much into institutional product.

Moderator: We will take our next question from the line of Kunal Randeria from Axis Capital. Please go ahead.

Kunal Randeria: You've been expanding aggressively in this year. From Tier-3 and Tier-4, on company level average for Swasthfit is around 25%. So, even from Tier-3 and Tier-4, is it similar or would it be substantially lower?

Shankha Banerjee: I think this question was asked earlier, urban -rural split, etc., I don't think we have the numbers handy. I think once we have those numbers, we will look at it and see whether it's different or not. However, that may not be the right metric because the competitive set and the growth strategies in these markets could be slightly different, Tier-3, Tier-4 v/s Metro or a Tier-2 town.

Kunal Randeria: Second question is actually for Ved. Ved, you mentioned a 27% is the kind of

EBITDA margin you are comfortable guiding with. You are already at 27% and I am assuming that the Suburban should improve, which means the core margin, the core ex-per cent margins should turn around in the coming year. Is that because you'll be making higher investments if you can just run it through your cost?

Ved P Goel: Yes, Kunal, it will compensate through higher investment in newer markets and spending on awareness, some bit of digital and automation, all these things.

Moderator: We will take our next question from the line of Shal een Kumar from UBS Securities India . Please go ahead.

Shal een Kumar: Two parts of the portio

n. First, we can see that our margins are improving , and they've been consistently improving , and I believe that we have been investing in growth as well. Can you talk a little bit about how we are investing for future growth in terms of maybe more accelerating our patient service centers or entering into new

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Classification: Restricted segment? And second part, when will we see the impact of that coming in? It could be any positive momentum on the growth maybe 3-6 months down the line you see because of investments happening?

Shankha Banerjee: Shaleen, let me try and answer that. In terms of where we are investing, it is in quite a few dimensions. I think Dr. Om mentioned about these 20 labs that we are in the process of setting up. First and foremost, it's in new lab infrastructure and accompanying that obviously you have other overhead costs that get built into the system. Over and above that, we are also improving our spending, whether it's on the digital channel as well as in offli ne to build awareness and also see how we can drive more patient acquisition activities. And as we expand more into non -core geographies in West and South, obviously, the spending needs are substantially higher than in brand strong markets. Primarily those would be where these spends are going to keep going. And lastly, some of it will also go into our frontline, manpower , expansion in sales and marketing kind of areas. Your second question was where we can see the impact. Now actually this is an ongoing activity. It isn't as if that we started this action a few quarters back. This is an action we continue to do over a period of time. So, what we see benefits today are result of acti ons which have been taken maybe a few quarters or a year back. So, it's a rolling action. In case you are expecting a certain spike because of these investments we are making, it may not be so, but yes, steady growth and also ability for us to do better in the newer geographies that we are entering is what we see as the likely outcome.

Shal een Kumar: Is it right to assume that, again, the intensity of our efforts has gone up because of our healthy profitability , strong position in terms of the character and all possibility.

The intensity vis -à-vis later 2 -3 quarters v/s where we are right now, it should be better, right because of where we are and hence , I am not expecting step jump, but a gradual improvement should be followed if the intensity is high , or intensity is increasing?

Shankha Banerjee: Intensity I would say is because of two reasons , one can look at. In our core geographies, this whole expansion into Tier-3, Tier-4 is happening which needs quite a bit of investment from our side, and also in some of our weak brand markets in west and south, there is investment required and the new channel that we have started spending on which is in digital. These is where the whole promotional spend is going. And yes, the outcome expected is a gradual improvement in our growth rates.

Shal een Kumar: Any new segment you are trying to tap in , kind of segments which you are kind of getting excited about?

Shankha Banerjee: I think in terms of segmenting in the market, Dr. Om mentioned that there are various categories that we are trying to create and work on. Obviously, the exciting area is around non -communicable diseases. That's the whole area where Swasthfit and some of our other approaches are really focusing on, variou s things around that, and also some of these specialized areas in and around genetics and testing in that area.

Moderator: We have our next question from the line of Deven Kulkarni from Marcellus Investment Managers. Please go ahead.

Deven Kulkarni : Hi, Ved. For this quarter effective tax rate is around 28%, which seems to be on the higher side. So, just wanted to understand why is that?

Ved P Goel: There is the deferred tax and Suburban depreciation, w

hich obviously has impacted.

There is some adjustment on account of deferred tax. But on an average, this tax rate is not changing, if you will see annualized basis, I don't think there is a change in tax rate. Don't see this quarter as a representative quarter for tax rate.

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Classification: Restricted Deven Kulkarni : And secondly, currently what's the net cash position as on 30th June?
Ved P Goel: Rs. 1,044 crore.

Moderator: We have next question from the line of Abdulkader Puranwala from ICICI Securities.
Please go ahead.

A Puranwala: In the past, we have highlighted some bit of competition coming directly from hospitals where they are trying to forward integrate. I mean any thoughts on how we can tackle this or any measures we have already taken in those areas, if you could highlight?

Dr. Om P. Manchanda: Our experience is the hospital competition is very limited to where the brand salience is there and that's mainly in some catchment area of 4 -5 kilometers. Building a large network for hospital business is very, very challenging, We are not unduly worried about that. As I keep mentioning that it's better that good quality name come into the market, so the overall table will go up. But there have been 4-5 such players who come in, but they have done well only in those markets where the hospital brand is strong and most of them are also listed players, I don't think there's any other listed player who's trying to do this. It's okay, there's a lot of space in the market for competition to be absorbed.

A Puranwala: My next question is with regards to the underlying growth in Q1. So, if I exclude the 8% growth what you mentioned about suburban and Swasthfit, the balance business also seems to have grown at 7 %-8%. Any thoughts over there, I mean is there some bit of lumpiness in a few tests due to which the growth was in single digit?

Shankha Banerjee: Looking at growth excluding Swasthfit may not be appropriate because like I said in terms of the way Swasthfit business is coming, there are places where upselling is happening, also now some doctors ' prescriptions are moving towards Swasthfit, so something which was a non -Swasthfit earlier may also be turning into Swasthfit. I think we need to look at the totality of the business . You can look at maybe suburban separate and DLPL if you want to look at in organic, maybe that will be a more appropriate way to look at it.

A Puranwala: In terms of the margins for Suburban, I mean I understand we have put considerable efforts to improve the margins here. But on a sequential basis also, we did I think close to 17% last quarter and now it's close to 14%. Just wanted to understand is there any lumpiness into this business and on an annualized basis what is the kind of a sustainable margin we should look forward for next 2-3 years perspective?

Dr. Om P. Manchanda: I really won't go by a few percentages here and there because base is too small, Rs. 40-50 lakhs here and there can swing that margin. Our focus clearly in Suburban is to see higher top line growth. I won't be worried about too much about this margin fluctuations. For the next 2 years, we just want to make sure that we get our growth trajectory right and then everything else will fall in place if you have good top line growth.

Ved P Goel: Plus, if you see QoQ , it's improving, last year same quarter it was much lower. So, from that sense because there is a quarterly impact, increment also. On an annualized basis we are very confident that we should be improving.

Moderator: Our next question is from the line of Pranav Chawla from Antique Stock Broking.
Please go ahead.

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Classification: Restricted Pranav Chawla: Sir, I have just one question majorly on your Delhi NCR market. Have we seen the growth rate of NCR slowing down over the past years?

Shankha Banerjee: I think the good news is that we aren't really see

ing a slowdown in the growth rate

rather we see it being sustained at a level which is slightly higher than what we had maybe pre -COVID.

Pranav Chawla: If we are seeing NCR, that is our biggest geography, if that continues to maintain its growth momentum, which would be the market that we are seeing a slowdown as of now, because NCR is among the biggest markets that we have?

Dr. Om P. Manchanda: My sense is that Delhi NCR market has gone through 2-3 phases where there was a phase, there was heightened competition, I presume some bit of a shift may have happened to them in the last couple of years, many of our old customers whom we lost may have come back to us. In between , we had a slightly slower growth in Delhi NCR, but as Shankha mentioned that we are inching towards slightly higher number. Can it go up further? I still doubt because in our business planning, we still don't want to project higher Delhi NCR growth because the base is too high and the market, size is still 3 -4-5 crore of this cluster, while all other clusters like UP, Bihar, Uttarakhand, they are firing for us, where the market size is nearly about 20 -25 crore population. So, that's the way I would put it. Basically, we are very strong in Delhi NCR. Some of our customers who may have gone to the competition may have come back in the last 8 -12 months. This is what we are seeing, and I think Swasthfit

contribution also has pushed up the revenue growth. But I will still stay conservative on Delhi NCR and push the other markets.

Pranav Chawla: Have we seen any change in patient behavior because our patient volumes have been pretty muted for the past couple of quarters, have we seen any improvement on this front?

Dr. Om P. Manchanda: There is an interplay that we are seeing between as the contribution for Swasthfit goes up, the number of patient visits are slightly muted. That may have impacted this growth. Because what we are reporting as a patient volume growth is technically, not unique patients, they are patient visits. But in Swasthfit per visit you end up doing a lot of tests in one -go, the number of visits may actually get reduced. So, I think we are seeing some kind of interplay as the contribution of Swasthfit is going up. I have a sense that patient visits may be inversely related to that. I think there may be some impact that we are seeing. But I think overall one should see the sample growth, which is nearly about 9.6% which is a combination of number of tests per patient into patient footfall.

Pranav Chawla: Can you give us some color on FY25 as well as FY26 growth that you are internally projecting?

Dr. Om P. Manchanda: I think FY25, I told you last time we will try and beat the FY24 number. This quarter is 11.3, its better than 10.4%. I have a feeling we will continue to do better and then definitely beat 10.4% of last year. As I mentioned that September is a crucial month for the entire industry, not only for us, I don't know how it turns out to be, but I think I can confidently say that we will beat the last year's number.

Shankha Banerjee: Adding to that, we beat last year growth number without having to take a price increase. That is what we are trying to drive the sales increase. And once we see 2-3 quarters of this year, maybe we will be in a position to then start looking at what the next year outlook.

Dr. Om P. Manchanda: I think October might be just a good month to talk about FY26.

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Classification: Restricted Pranav Chawla: What is the strategy of cash utilization given that we have cash building on the balance sheet?

Dr. Om P. Manchanda: Time and again we have said, our priority #1 is to look for strategic assets, inorganic which are in South and West Region, in West we have done Suburban. If you look at contribution wise, region wise obviously South is still very weak. We would want to fill that gap. There is no doubt about that. But

we also need to have a quality asset

at the right price, much high on governance, etc., so that's given. In absence of that happening because you don't know that doesn't happen every month or every quarter, it may happen, may not happen. But in the meantime, we have increased our dividend payout, we are trying to balance between the two and also investment in our new labs and technology. I think combination of all these 3s where we want to utilize cash. Cash and the balance sheet is also good because our ability to do certain transactions can actually go up as well. #1 priority is to see deploy this money for growth primarily in acquisition because it's a global experience that in new markets it's very challenging to establish organically. So, one has to look at that.

Moderator: We will take our next question from the line of Ashutosh Parashar from Mirabilis Investment Trust. Please go ahead.

A Parashar: So, just a couple of questions. On the 20 labs that we are planning to open this year, can you give us some color on the geographic spread of these labs. Are these likely to come up in the markets of Bihar and UP? And how many of these are planned for Suburban?

Shankha Banerjee: The primary focus is going to be, we have been doing in Tier-3, Tier-4 geographies of North and East. That's where the primary focus is going to be. Suburban, there isn't really a plan to add too many labs, there are 1 or 2 maybe gaps which we will fill, we are still evaluating that, but this is more in our core geographies of North and East.

A Parashar: On the Suburban trend, it's been some time since we have integrated the acquisition. Are there still some gaps on testing mix and all that we have to address or we have largely addressed that?

Shankha Banerjee: There are various layers of integration that are currently working. From a test perspective, now the whole test menu, which Dr. Lal PathLabs operates is today available through Suburban as well. But having said that, there are certain background integration, IT integration work, which is still happening to make it even more seamless, that work is going to take some more time. But as of now, clients in the geography, in which Suburban is operating have access to all test menu from Dr. Lal PathLabs.

A Parashar: What would be the average realization for our Swasthfit test?

Dr. Om P. Manchanda: You see, my reading is, you can do a reverse math in any case, right? 25% contribution, you know the portfolio revenue contribution earlier and now I think you can figure that out, but exact figure we have not really shared so far.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to management for closing comments. Over to you sir.

Ved P Goel: Thank you everyone for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries satisfactorily. Please feel free to reach out to us in case you have any further questions. Thank you once again.

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Call: Oct 2024

Classification: Internal
October 28, 2024

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q2 & H1 FY25 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q2 & H1 FY25 Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 15

Dr. Lal PathLabs
Q2 & H1 FY25 Earnings Conference Call Transcript
October 23, 2024

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer & CEO – International Business
Participants who asked questions • Amey Chakraborty – JM Financial
• Tausif Shaikh - BNP Paribas Securities
• Karthik Chellappa – Indus Capital Advisors Hong Kong Limited
• Prakash Kapadia – Spark PMS
• Anshul Agrawal - Emkay Global
• Nancy Yadav - Allegro Capital Advisors
• Saion Mukherjee - Nomura Securities
• Prashant Nair - Ambit Capital
• Bino Pathiparampil - Elara Capital
• Sumit Gupta – Centrum India
• Siddhesh Raje - ICICI Prudential AMC Ltd.

- Bhavya Sanghvi – Phillip Capital
- Shyam Srinivasan - Goldman Sachs
- Vaibhav Saboo - Nippon AIF

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Moderator: Ladies and gentlemen, good day and welcome to Dr Lal PathLabs Q2 FY25 earnings conference call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand over the conference to Mr. Siddharth Rangnekar from CDR India. Thank you and over to you, sir.

Siddharth Rangnekar: Thank you, sir. Good evening, everyone, and welcome to Dr Lal PathLabs Q2 & H1 FY25 Earnings Conference Call. Today, we are joined by senior members of the management, including Hony. Brig. Dr. Arvind Lal , Executive Chairman, Dr. Om Prakash Manchanda , Managing Director , Mr. Shankha Banerjee , Chief Executive Officer and Mr. Ved Prakash Goel , Group Chief Financial Officer and CEO of International Operations.

I would like to share that some of the statements made on today's call could be forward -looking in nature and actual results could vary from these forward -looking statements. A detailed description in this regard is available in the results presentation , which is available on the stock exchange websites and has been separately circulated to all of you.

I would now like to invite Hony. Brig. Dr. Arvind Lal, to share his perspective. Thank you. And over to you, Dr Lal.

Dr. Arvind Lal: Thank you, Siddharth and good evening, ladies and gentlemen and a very warm welcome to all the participants on today's call to discuss our Q2 FY25 performance. I will commence by discussing the evolving market dynamics and our key achievements.

Our ability to maintain our market position in the highly fragmented and competitive Indian diagnostics industry is a testament to our strong brand, exceptional service and extensive network. We are leveraging these strengths to penetrate deeper into under served Tier-3 and Tier-4 regions by offering affordable high-quality diagnostics to the patients.

Dr Lal PathLabs have scaled up nationally into a well -run operation with an extensive franchise base, consistent delivery of high quality and affordable diagnostics reports to our patients has earned us this position. We believe this to be a core factor why patients have faith in our brand.

In today's evolving healthcare landscape, digital integration has become imperative, and we are actively leveraging technology to enhance patient experience and operational efficiency. Currently, we are also expanding our home diagnostic services to meet the growing demand from patients who value convenience. We are committed to achieving these objectives without taking any price hike and focusing on driving patient volume, growth and efficiency gains.

India has a vast unserved and underserved population in terms of healthcare and diagnostics. The headroom of growth therefore is very much there. Together, with policy impetus, tech innovation and growing propensity to avail of quality and accurate diagnostics, the share of national brands like ours is rising. We continue to benefit from our scale of operations and brand recognition, both of which are crucial factors to seed consolidation at the industry level. In the interim, our focus remains on building out organically while evaluating inorganic options that fit our objectives and values.

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As we celebrate 75 years of excellence, it is an opportunity to reflect on the past, celebrate achievements and look forward to the future. We remain committed to serving the patient community through consistent service and quality of diagnosis. And at this point of time, I would like to apprise you of a development. As you are all aware that Dr. Om Manchanda ends his term on March 31st, 2025, he will be stepping down from his position as managing

director into an advisory role. We

personally thank him for all the contribution he has made in building Dr Lal PathLabs into the leading and best diagnostic company in India. Nearly, 20 years of his operational and management excellence has helped the company in achieving this goal. We wish him the very best in his new role and look forward to his continued guidance and direction.

Thank you and I would now like to hand over to Dr . Om. Over to you, Om.

Dr. Om P Manchanda: Thank you, Dr. Lal and very warm welcome to all the participants on this call. First things first , as Dr. Lal mentioned that my term ends on 31st March 2025, post that I shall transition to my new role as an advisor to the company. Incidentally, I also turn 60 next year and this milestone coincides with end of my term. I thought this is the best time for me to transition into a new phase of my career from an operating role to an advisory role. A large part of my executive role is already transitioned to our CEO , Mr. Shankha Banerjee. I shall further transition to him during the remaining period of my term. Now, let me shift to the business and industry insight .

I think the very important insight that I have picked up and which we have been talking about the last few quarters, that is the concept of bundled test packages, has become a very, very firm trend in industry . In my view, it is having the following impact :

Number one, it is leading to higher realization per patient resulting due to higher number of tests per patient and this is further leading to slight improvement in the industry margins is the trend that I notice.

The second important impact of trend is that the business processes are getting very simplified in terms of whether it's ordering by the patient or collection of blood samples and even in the lab operations.

Our Bundle Test Program, which is "Swasthfit " continues to do well and is now almost 24% of the company's overall revenue. As we reach out to a wider patient base, the growing contribution from such bundled offering both in routine, semi -specialized and also specialized testing will further add to our performance going forward.

The second point that I want to make is that our efforts in building West region platform has started yielding results. West business is now nearly 15% of the total revenue, that is nearly Rs. 100 crore in a quarter. It is a very unique business in nature as 60% of the turnover is inorganic. This quarter the West region has delivered better growth than overall company growth. It will be a great learning platform for us as we build out our hybrid strategy in non -core markets, that is West and South.

We are continuously investing in our capabilities not only in technologies that drive growth and efficiencies, but also on the cyber security aspect, further solidifying defense around patient data. Going forward, we are also investing towards organizing a nd analyzing data better so that it helps in raising the quality of our offering.

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With that, now I'd like to hand it over to Shankha for his thoughts. Thank you.

Shankha Banerjee: Thank you, Dr. Om. A warm welcome to all participants on this call today. Let me share the business and operating highlights with you .

The positive momentum in performance has continued with revenue and profit after tax growing by 9.8% and 18.1% respectively in the 2nd Quarter of FY25 . This performance is a testament to our strategic focus on both expanding our service network and meeting evolving patient needs.

The growth in revenue is driven by sample volumes, which stand at 23 million. Our patient volumes came at 7.8 million. Sample volume growth for Q2 FY25 is 8.6% and patient volume growth is 3.9% over Q2 FY 24. Our Q2 FY25 revenue per patient stands at Rs. 844, an increase of 5.7%. The realization improvement does not have any price increase impact, rather it is due to product a

nd geography mix.

We are happy to share that in Q2 FY25 Suburban has delivered revenue growth in double digits in line with our plans. It has started contributing positively to our overall and West region growth.

Our ongoing expansion initiatives are taking concrete steps. As indicated earlier, we will be opening 15 to 20 additional labs in this financial year and are accelerating the addition of collection centers to support these labs and meet incremental demand.

These developments are crucial in widening our footprint across Tier-3 and Tier-4 markets and deepening our presence in core regions.

To align with our long -term vision, we are expanding our services to include bundled testing for non -communicable diseases beyond the Tier -2 market. This addresses the increasing demand for healthcare services, particularly in managing chronic illnesses.

Our commitment to reaching underserved markets remain steadfast and we are also strategically strengthening our footprint in creek clusters, particularly in West and South India We are undertaking targeted brand campaigns in selected cities to increase visibility and stay connected with local communities. In addition, we are driving several initiatives to foster loyalty to encourage patient growth within our core

audience.

We remain confident in realizing growth organically backed by network outreach and investments in communication and technology. As we have the benefit of scale, we are in an advantageous position to benefit from positive trends shaping the diagnostic landscape.

I would now like to hand over the call to Ved who will walk you through the financial performance. Over to you, Ved.

Ved P Goel: Thank you, Shankha. Good evening, everyone, and a warm welcome.

I will be sharing the key financial highlights for Q2 and First Half of FY25

- Revenue for Q2 FY25 came in at Rs. 660 crore compared to Rs. 601 crore in the same quarter last year, reflecting a growth of 9.8%. The first half of FY25 revenue stands at Rs. 1,262 crore, a growth of 10.5%.
- Revenue per patient for Q2 FY25 is Rs. 844, 5.7% higher compared to Rs. 798 in Q2 FY 24.

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- Test per patient for Q2 FY25 is 2.94 v/s 2.81 in Q2 last year, registering a growth of 4.5%.
- EBITDA for Q2 FY25 came in at Rs. 202 crore compared to Rs. 178 crore in Q2 FY 24, registering a growth of 13.9% with an EBITDA margin of 30.7%.
- EBITDA for first half of FY25 stand at Rs. 372 crore v/s Rs. 324 crore in FY24 , registering a growth of 15% with EBITDA margins at 29.5%.
- PBT for Q2 FY25 came in at Rs. 183 crore, registering a growth of 20.3% with the PBT margin of 27.7% PBT for first half of FY25 stands at Rs. 333 crore with the margin at 26.4%.
- PAT for Q2 FY25 came in at Rs. 131 crore compared to Rs. 111 crore in the same quarter last year, registering a growth of 18.1% with the PAT margin of 19.8%.
- PAT for the first half of FY25 stands at Rs. 239 crore versus Rs. 194 crore in FY24 , registering a growth of 22.8% with the margin of 18.9%. Improved profitability is on account of operating leverage and strategic initiatives to optimize cost by using technology.
- Earnings per share for Q2 FY25 is Rs. 15.5 compared to Rs. 13.2 in Q2 FY24 . First half FY25 EPS stands at Rs. 28.3 as compared to Rs. 23.1 in FY24 .
- Net cash as on September 30th , 2024, is Rs. 1,095 crore.
- The DSO as on Q2 FY25 is 25 days and we are still enjoying negative working capital of 22 days.
- Further, I am pleased to share that the board of directors of the company have approved a second interim dividend of Rs. 6 per share for FY25 .

With this I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A. Thank you.

Moderator: We will now begin the question -and-answer session. First question is from the line of Amey Chalke from JM Financial. Please go ahead.

Amey Chalke: The first

st question I have is on the Suburban for the improvement in the operational performance for QoQ . Is it possible for the management to clarify? Slight margins have improved to 20%, last quarter it was around 14%. What were the drivers during this quarter , is it only the operating leverage which has played out or something else which we should watch?

Ved P Goel: Yes, Amey, you are right that operating leverage is playing and there are certain strategies and initiative which we are taking, with use of technology, with the back - end efficiency, productivity enhancement and so on and so forth. I think that is the reason for Suburban margins is inching up.

Amey Chalke: Going ahead, how should we think the trend for the following quarters for 3Q and 4Q because 3Q could be stronger, but again, there could be a weaker quarter, so like how should we think about for a whole year?

Dr. Om P Manchanda: I still will continue to maintain that we should not focus too much on margin in Suburban. Our intent is to drive growth rate even further and in any case, our overall company margins are pretty healthy. I would rather invest in driving growth at the moment than worry about improving the margins. I will sacrifice some margins to drive growth in Suburban , that's the way I look at it. If we get leverage benefit, no problem, but I think our eyes are focused more on top line right now.

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Amey Chalke: Is it possible for the management to give a growth region wise for the quarter or at least how much the growth was in north and ex of north?

Dr. Om P Manchanda: I do not think we actually share region wise growth on quarterly basis, but one thing

I can share with you is that interestingly it's uniformly spread across the regions. Maybe couple of percentage higher and lower here and there. As I mentioned in my comments, We st is slightly better. I also get a sense that the West region had higher incidence of fever this time than maybe rest of the region. But I think overall our growth rates across all regions is uniformly spread.

Moderator: Next question is from the line of Tausif Shaikh from BNP Paribas Securities . Please go ahead.

Tausif Shaikh : I have two questions mainly on your expansion plan in Tier-2 and Tier-3 cities. Can you throw some light how the realization is in these cities and how is the volume ramp up , are they slower compared to the metro cities when you open a new lab?

Shankha Banerjee: Our Tier-3 and Tier-4 city expansion is predominantly in our core geographies of North and East and in these geographies , we are going deeper. So, pricing in these geographies is quite similar. In the contiguous market, the pricing is not lower. So, realizations are similar in terms of the contiguous geographies that we are going into. And in terms of ramp up, it takes the normal usual time that one would take to ramp up. But the throughput of these labs is managed in a way that it gathers slightly bigger radius to ensure that the volume throughput or lab is taken care of.

Tausif Shaikh : And also, what kind of challenges do you see in these kind of cities ? Is it fair to assume the unorganized player offer s a path and radio mix business, so that's some kind of challenge which these kind of players like Dr . Lal face a challenge instead of a pure path player?

Shankha Banerjee: In all of these markets, the predominant task is converting the unorganized to organized and I think challenges are more or less similar whatever we have seen in our previous years as we have done it in other geographies. There is always a mix of players of fering path and non -path as well as pure path. Those are not really challenges which are different than what we have done in the past. I think differential challenge to that extent is not something which is very different. It is more or less similar. It's just that it's a smaller ground that we're reaching out today.

Dr. Om

P Manchanda: Yes, I think I may just add what Shankha just mentioned. I think it's true not only for diagnostics, but for all healthcare sector, even hospitals also I am sure must be facing a challenge. As we go in smaller towns, there are very strong relationships between patient and doctor and between the doctors also. For any brand which is slightly global or acting in nature to penetrate to in that relation becomes very, very challenging. It's a very slow process , gradual process , you have to really communicate your proposition very well so that you can overcome the power of those relationships with the kind of service that you offer. But once you reach that tipping point, then I think the whole shift happens from unorganized to org anized. And it takes a little bit of time, but as we build scale my sense is the time it takes now is much lesser than what it was taking before.

Moderator: Next question is from the line of Karthik Chellappa from Indus Capital Advisors Hong Kong Limited. Please go ahead.

K Chellappa: My first question is, if we were to look at our volume growth at least on samples, for the last 4 quarters, it has been somewhere in the range of about 8% -9% and it is backed by very healthy margin expansion. I am just curious to see whether there is scope for us to reinvest some of these margins to drive a higher volume growth? And

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in your opinion, what needs to happen for volume to kind of move up to a more sustainable double -digit level?

Shankha Banerjee: Firstly, let me try and answer the question that the sample growth is driven by also improved contribution which is coming through Swasthfit and we are seeing even in the non -Swasthfit portfolio the test per patient is slightly up compared to what we were seeing maybe a year before , that is what is driving sample per patient and therefore our overall sample growth. Now, I think investing back into growth is something that we are continuously doing. Like I mentioned in my opening remarks, this year we are planning to open a higher number of labs between 15 to 20 labs, we will be adding this financial year. Also, we are doing investment in terms of communication in key cities very strategically wherever we feel that adds value to our brand and our position.

We are also strengthening our market connect and our sales teams and those kind of investments are continuously being made. It is not as if whatever is there investments to be done that are being held back, because if you see our A&P spends are actually inching up every quarter when you compare with the same period last quarter , those are definitely there. And whether it will be a double -digit patient volume growth or not, I think that's something which we will have to come back to, I think that's an aspiration, but not really in the near future because there is a mix of two

things. One is the existing patient volume getting impacted because of more Swasthfit and also on the balancing factor, the outreach we are doing to newer geographies and newer Tier-3, Tier-4 towns and the activities we are doing there which is adding new patients, it's still under churn. When does it reach double digit is something which we will come back to that a bit later.

K Chellappa: My second question is, as far as competition is concerned, let's say, compare to your commentary a quarter ago or even two quarters ago, has there been any change either for the positive or negative, either in the form of pricing, discounting or number of players or any other attribute that you measure?

Dr. Om P Manchanda: I think our commentary is exactly very steady. First, I think you all know that our numbers should be viewed with the backdrop that we have not taken any price increase. Many of our competitors have taken price increase and we also have adverse impact of higher base as well. From that perspective, we always said that, we try and meet or

slightly beat our last year's performance, which I think at first half level, our growth is 10.4% compared to what we did last year full year. Last year 10.4% had an impact of nearly 3% of price increase. And this year, we have not had that and despite that not only the revenue, but we have also improved our margins a bit as well.

We continue to maintain that steady sort of commentary as to what we have been saying. I think if you really want to know about competitive intensity, I think the general perception in the market is that competition is less. I do not think that is true. Competitive intensity is still there. I think what is not there is irrational pricing, promotion, discounts, those are happening. But competitive intensity to my mind still continues to be there. I think many of these young new age players are looking for a path to profitability. My sense is they have stopped deep discounting their promotions, etc. But definitely one competition from hospital is on the horizon because I continue to see hospitals trying to move into retail pathology business, to my mind intensity is still there in the market. It's just that irrational competition on pricing is reduced.

K Chellappa: As far as the management succession is concerned, so once Dr. Om steps down in March '25, how is the succession going to be planned, will it be someone internally
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or are we going to look for somebody externally or could you give us some color or thought on that?

Dr. Om P Manchanda: If you look at we in any case have a full-time CEO who is Shankha Banerjee. We split this position a few years back and a large part of my role has already been transitioned to Shankha and he will spearhead as far as the internal management is concerned. And along with that, Ved is there as a group CFO as well as the international CEO. That's the way it is. As of now, there is no other plan as far as succession is concerned.

Moderator: Next question is from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: A couple of questions from my end. On Swasthfit, if you could give us some sense how much of this is a doctor recommended, how much is directly and is there a scope to increase this contribution in Tier-3, Tier-4 cities also? Given the work we have done over the last two years now in Suburban, so is it fair to assume now Suburban will continue to grow higher than the company average from here on? And lastly from a higher revenue growth perspective if I were to just guess the drivers in the medium term, will it be market share gain from smaller players, will it be higher number of patients or will it be more revenue from patients or you see Tier-3, Tier-4 contribution increasing, what could be the pecking order in terms of just trying to assess the potential revenue over the medium-term?

Shankha Banerjee: Prakash, let me try and answer them to the best possible degree. I think firstly on the Swasthfit contribution from direct patient v/s doctor prescription, we do not have accurate data on that, but one would assume that quite a bulk of it is also upgrading prescriptions, they may not directly be a Swasthfit prescription, but some prescriptions get upgraded, so quite a bulk of our Swasthfit kind of comes through that. We will not have exact numbers but that is one bit. Secondly, whether we are taking it to Tier-3 and Tier-4, I think that's something which I mentioned in my opening comment as well that we are actively taking Swasthfit packages into our Tier-3, Tier-4 markets and we are also seeing some traction build up in those markets on Swasthfit which is giving us the confidence that the growth trajectory we are seeing on Swasthfit still has the legs to run on.

I think on Suburban, yes, lot of work has been done in the last two years and I think we have now started seeing results on that. Independently, we definitely feel that, or we believe

ieve that Suburban growth rates are going to be sustained or better as we keep moving forward. Will it be always better than the company? Actually, my expectation would be even the other part of the company should grow faster. So, to me those two remain independent. But yes, Suburban growth, we should be able to now sustain double -digit and grow maybe even faster than that.

Your last question , in terms of how will future growth come and what are the driver? Yes, we continue to look at newer geographies, which is the Tier-3, Tier-4 expansion. I talked about new labs. We are opening more sales headquarters reaching more directly to more clients across more of geography, that gives wider coverage. And in South and West, we continue to focus on our core cities and core clusters and building our brand and writing on whatever we have built in the past, getting higher throughputs in our existing infrastructure. Those will be the key drivers and also the bundled test portfolio growth continuing.

Dr. Om P Manchanda: I just want to add one more thing about Suburban point that while we track Suburban performance separately, but to my mind we do not look at that number in isolation.

In fact as I mentioned that 60% of our business is coming from an inorganic in the West region. It is not only Suburban. It is also coming from that small subsidiary company that we have Pathlabs Unifiers. We actually look at this as a driving network

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effect of labs, collection center of all the brands put together in the region. We track first is West region, the entire region's growth first and then look at each component separately. As long as we are able to drive the region's growth, which is significantly better than company, I think we probably would have achieved our objective.

Prakash Kapadia: Lastly, Shankha, you mentioned about the growth drivers for a higher kind of revenue growth. Historically we had seen post -COVID lot of these new labs had sprung up. Any sense on these unorganized or these players getting marginalized because I guess over the mid -to-long term that also would aid growth and now should translate or show into higher growth, is that happening on the ground, not happening if you could give u s some sense that would be helpful.

Shankha Banerjee: It's a bit of mixed feedback there. The COVID period actually created a bit of a cushion for a lot of the single operating labs because lot of them need quite a bit of money. Therefore, some people have tried to expand, or they are sustaining their businesses and still continuing to run their operations , that is at one end. At another end, we also see people who may want to be exiting or wanting to close their businesses. But yes, it is a bit different than what we were seeing pre -COVID. There is still some bit of aggression that we see in the local lab ne tworks, but we have our eyes and ears open, I think the situation should improve in terms of consolidation as we move forward.

Moderator: Next question is from the line of Anshul Agrawal from Emkay Global. Please go ahead.

Anshul Agrawal: My first question is on network expansion. Is the target of adding 15 to 20 labs in the current year more back ended in nature? How many labs have we added in H1.

Shankha Banerjee: Although we do not maybe share the number, but one can say that we are on track, 50% of these labs have already been added in H1.

Anshul Agrawal: The reason why this is we have only incurred a CAPEX of about Rs. 10 crore in H1 if I am not mistaken. So, would that mean that our CAPEX numbers would be materially lower than what we had incurred last year?

Ved P Goel: I mean most of these CAPEX is happening in second half of the year. Though we have started the labs but maybe most of the real operation will start in the second half. That's why the CAPEX is low. And historically also if you see the CAPEX in first half is always low.

Anshul Agrawal: Our CAPEX guidance of roughly around Rs. 50-60-odd crore would stay as is. Secondly, the moment we sort of commission these labs, would our margins also get impacted in the second half of the year?

Ved P Goel: I think it's a good question. I mean that's why I always say that do not look at margins for QoQ basis, but on an annual basis, I think I mentioned on the last call as well that we are hoping that we can do slightly better than last year, but our margins will be sustained as last year. This Q2 always has a higher margin because of operating leverage and efficiency and due to most of the investments that come in the second half, that's why I think the overall margin for Q2 will be lower than H2.

Anshul Agrawal: Second question is on Swasthfit portfolio. If you may throw some light on this, what percentage of the Swasthfit portfolio would be illness testing and what percentage would be wellness?

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Shankha Banerjee: I think somebody in the past also asked this question how much is home v/s patient prescription upgraded. We do not have any accurate data to say that, but one can

look at it, either illness part is also the sizeable component.

Anshul Agrawal: What are our plans for the cash that we have on books? I mean, I am sure we do not have any CAPEX sort of requirement. Do we plan on giving out increasing dividend payouts or what is your thought process on the cash that we have on books ?

Dr. Om P Manchanda: See, obviously, our first priority is to use this cash for growth with purpose and growth consists of both organic and inorganic. Those efforts will continue. While organic we have better control on how we expand. Inorganic is very difficult to time it, but consistently we have been also paying out dividend, that will also continue. I think it's a combination of all 3 organic, inorganic and dividend and we will continue to progress as we have been doing in the past. Our effort on inorganic continues. If we cannot use this cash, then obviously a dividend will be paid out.

Moderator: Next question is from the line of Nancy Yadav from Allegro Capital Advisors. Please go ahead.

Nancy Yadav: I wanted to ask if we can give out any numbers on the IndAS adjustment that we have made for this quarter?

Ved P Goel: Nancy, IndAS adjustment is not new, I mean, it is implemented 3-4 years back and there is no exceptional in this quarter, I would say it's very much comparable as last year.

Nancy Yadav: And also, sir, if it's possible to give any sort of exact numbers for the Suburban labs like if we could get a percentage of exact growth from the previous quarters or any numbers that you will be able to provide?

Ved P Goel: Suburban growth this quarter is about 11.6% and EBITDA margin is about 20%.

Moderator: Next question is from the line of Saion Mukherjee from Nomura Securities . Please go ahead.

Saion Mukherjee: This strategy of not taking price increase unlike your competitors, I think, historically,

Dr. Lal used to take low single digit price increases. I am just wondering is this a strategy for a year or you think it should sustain this on a slightly longer -term perspective? And generally, we see inflation across everywhere. There's like 2 -3% price increase is something where the volume sensitivity is very high. How do you assess the strategy of keeping or not taking any price increase?

Dr. Om P Manchanda: We are not averse to taking price increase. I do not think it's our stated strategy that we will not take price increase as our effort is to go down to Tier-3, Tier-4 towns. As all of us are aware that those are the markets that require affordability and to achieve that if we continue to take price increases, then we do not achieve that objective. As long as we are able to achieve this without diluting our margins, that's the reason why we resist taking price increases, but it's not that if infla

tion is higher than what

we can absorb through this leverage, then we will obviously be forcing a price increase. But our intention is to make sure that our proposition appeals to wider footprint, otherwise, we can get narrowed down to large cities.

Saion Mukherjee: The second question I have on this employee expense in the quarter. It seems to have gone up almost 18% YoY, much higher than the revenue growth. If you can throw some light on the dynamics there?

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Ved P Goel: One is obviously, every year we give some increments and as Shankha mentioned, we are in the process of opening these 15 -20 labs. New additions, new manpower and so on and so forth , there is nothing which is exceptional here, but these are all where some increment every year we give to the employees and plus additional manpower which we are hiring.

Saion Mukherjee: And finally, now you have guided for more than 10.5% growth this year. Now, as you look forward with the lab expansion, Suburban picking up growth and possibly some price increase, are you expecting an acceleration in growth as we go forward, or you think low double digit is a level that we should sustain from a slightly medium -term perspective?

Shankha Banerjee: We have been saying equal to or better than last year growth, which is 10.4% and first half now we have delivered 10.5%. Therefore , our guidance still remains the same from that point of view. I think the point about Suburban growth coming through and price increase, like this financial year, definitely we may not be taking price increases like even Dr . Om mentioned. We continue to evaluate , if we feel that we need to take a price increase, we will take a price increase, but definitely not to drive revenue growth, that is not going to be a reason to take price increases because we like to be at broad base as possible. As of now we will stay with the revenue guidance that we have given in the past few quarters.

Saion Mukherjee: I was just looking at let's say for FY 26. Are you expecting growth to accelerate from let's say 10.5% this year or it would more or less remain at the current level?

Shankha Banerjee: See, our next year's financial plan we will start thinking once we are through with Q3

and maybe we will be in a better position to start talking about that a quarter or 4 -5 months from now.

Dr. Om P Manchanda: And also, please keep in mind that our base is much higher than many of our competitors.

Moderator: Next question is from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: Related to how do you plan your network as you go into the future. 5-6 years back, you had a certain hub -and-spoke model with a certain ratio of collection centers to labs which has served you well so far. Now as you go into Tier-3 markets increasingly or into newer markets, would that change in any manner? So, I am talking more about number of collection centers in relation to number of labs. Would you be able to do with more collection centers relative to labs or would you need to set up more labs, how do you think about the network as you go into your next phase of development?

Shankha Banerjee: Over the last few years, we have been steadily increasing the number of collection centers per retail lab that we have, and we reached a reasonably good number of almost 28 -29 labs, in the last financial year we reported I think close to about 29 collection centers for retail labs. I think that kind of a ratio we should be able to sustain even with our expansion into Tier-3 and Tier-4 models. The broad hub -and-spoke model remains in place. It is just that we are now going into as closer to the consumer as possible even in the hinterland, but the overall model is not really changing.

Moderator: Next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

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B Pathiparampil: Looking at thi

s 15 -20 labs this year number, is that going to be a strategy going forward as well, so can we look at a similar number of lab additions for the next few years per year?

Shankha Banerjee: If you see our historic trend, typically we have added between maybe 10 -15 labs if you go back to pre -COVID that kind of run rate we usually had. In the last two or three years, there has been a bit of a slowdown in terms of the number of labs that we have added. This year is also a bit of a catch up in terms of doing 15 -20 labs. At best, this rate may be sustained for one more year, but chances are we will start going back to what our normal pre -COVID level of lab addition would be in a given year.

B Pathiparampil: Just one question to Ved. Depreciation expense has actually come down a little bit YoY. Is it going to be like this at the given level of CAPEX or will this change and start moving up again?

Ved P Goel: No. First half depreciation is about Rs. 70-odd crore, which is similar as last year. I mentioned that most of the CAPEX is usually in the second half. I think depreciation will be higher in the second half compared to the first half.

B Pathiparampil: Yes, but YoY comparison, would it be similar, or it will keep going up?

Ved P Goel: Yes, it is similar unless and until we are doing large CAPEX because there is a depreciation, there is addition, net-net you are finding that this is the impact.

B Pathiparampil: For the current level of CAPEX that you are doing, should depreciation be a kind of stable expenditure going forward?

Ved P Goel: Yes.

Moderator: Next question is from the line of Sumit Gupta from Centrum Broking. Please go ahead.

Sumit Gupta: Just want to understand on the margin guidance that you highlighted 27%. However, let's say for the second half then it comes out to be around 24.5 %-25%. I just want to understand where those investments will be largely since 50% of the lab addition has been done, just want to understand on the investment part? And what kind of returns that you can generate?

Ved P Goel: Sumit, as I mentioned, I mean, though we have started and opened few labs, but the real impact will start coming in second half, that is number one. Second, historically also if you see our Q3 particularly our margins are quite different than Q2. Overall, I am saying margin, we can do slightly better than last year, but it is not something which is like you have seen in Q2, which is not a representation of the full year.

Sumit Gupta: Just want to understand the majority of the breakup of the CAPEX that you would be investing in apart from the lab side?

Ved P Goel: There are investment in the technology, digital, there are higher in spending on the A&P, we are going deeper, we are going newer markets, there are investments, awareness and so and so forth. There are logistic expenses which will go up, when you added new infra you have to add newer routes and so on and so forth. There are expenses which are coming along with this new infra. It is not only the CAPEX, but the running expenses will also go up.

Sumit Gupta: What kind of margins can we target for the Suburban, let's say, over the next one year or so?

Ved P Goel: As Dr. Om mentioned, right now the focus is to drive top line rather than to see margins. Having said that and naturally whatever efficiency we can generate whether it is maintaining the efficiency or improving the efficiency. As Q2 is 20%, it is not representation of the full year as Q2 is always higher, but overall basis maybe you can take 16 %-17% kind of steady margins for the Suburban.

Shankha Banerjee: Also, I think it will be important or one thing would be to look at company total margin and that guidance. I think that will be a much more stable indicator because our key thing for Suburban will continue to be how to drive the top line. The margin profile of Suburban can fluctuate. But I think the important thing would be to keep looking at the company total margin guidance.

Dr. Om P Manchanda: I think margins can fluctuate because we are also investing heavily into IT systems there. Some quarters those costs may come, A&P as well. I think my reading is that just let's focus on top line, right, because overall company margins are taking care of Suburban.

Sumit Gupta: On the geographical side, Delhi NCR still contributes around 30 %-32%?

Shankha Banerjee: Yes, Delhi NCR contribution is still at 31% for Q2.

Moderator: Next question is from the line of Siddhesh Raje from ICICI Prudential AMC Limited . Please go ahead.

Siddhesh Raje: I have two questions. One is we have seen this number of patient volume growth of 3.9% YoY and Swasthfit revenue is growing by more than 20%. Is it fair to assume that this new patient volume growth would have come largely because people are taking this Swasthfit packages and in that context where can the Swasthfit contribution settle down at company level? Second, in the last few calls you mentioned that the company will look to reinvest back margins into business so that even incremental competition gets very attractive, right. So, in that context when we say that we may do better than last year in terms of margin, can it incrementally attract more competition?

Shankha Banerjee: I think firstly to answer Swasthfit , where is it likely to settle down? I think in one of the previous calls also I had mentioned that we are seeing traction in terms of the newer geographies even in Tier-3 and Tier-4 as well as the source of revenue coming or these packages even coming from patients who come with prescription. So, therefore the scope of Swasthfit is much, much wider than only wellness. We still believe that there is a reasonable headroom for it to continue to grow at this level and improve contribution further. What that number would be is something which I do not think we are looking at putting a figure on right now. We will see how that evolves.

Also, in terms of overall market requirements, the incidence of non -communicable diseases keeps going up and that also is a driver for some of these health packages being utilized by patients , that's one. I think on the part about margins attracting competition, I think competition is there from multiple sides. I think Dr . Om mentioned in his comments, now we are seeing competition emerging from the hospital players wanting to get into retail pathology and they are driven by a very, very different thought process than just pure what is the retail or the margin profile that we see. As an industry I think again Dr . Om had made a remark in his opening comments that in the overall industry we are seeing improvement in the margin profile , those things

are happening. Whether it will be attracting more investment? I am not too sure whether that's something I will be able to comment on right now.

Dr. Om P Manchanda: Obviously. I think it's a broad factor. If the margins improve, people would like to become aggressive in growth. Competitive intensity does grow if the industry has healthy margins. I think I will wait for this quarter results for other players. But my reading is that this year industry might just show a slightly better margin than last year mainly on account of higher contribution of bundle packages , that also augurs well with our strategy of not trying to unnecessarily take a price increase and be a loser on our proposition.

Moderator: Next question is from the line of Bhavya Sanghvi from PhillipCapital. Please go ahead.

Bhavya Sanghvi: I just have a single question. in our presentation and annual reports, we talk about offering close to 4,500 tests. I just wanted to get a sense as to how many of these tests are outsourced and how many of these are conducted in -house? And if you could just give a sense of the percentage c

contribution and revenues from these outsourced tests?

Shankha Banerjee: I do not think that's the data that is something we share, or we would like to share, but suffice to say that predominant tests are all in-house.

Bhavya Sanghvi: And over the last 5 years, the trend would be the same, right, the contribution from the outsourced or it has changed?

Ved P Goel : No. I think over the years we are in fact converted some of the tests in-house rather than sending out. And it's a very small number of tests which probably we are sending out of India and that's all.

Moderator: Next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

S Srinivasan: Just the first one is on the Suburban acquisition we announced three years back and now I think in this quarter you started giving us more granularity. Just some of the learnings is what I was looking not just generally from a Suburban perspective, but also just from an M&A perspective. What are some of the key learnings maybe even some color around the Mumbai market?

Dr. Om P Manchanda: It's a good question and we have actually done about I think 28 M&As of different sizes and scale. Obviously, Suburban is of a different scale. I have actually got many learnings out of this and fundamentally I think most important learning that I have is to really understand the business post-acquisition as to how in our lab it will look like. I think if you are able to assess restated financials and restated infrastructure, which is very important, because sometimes those challenges we tend to underestimate like say for example in Suburban itself, I would say that large part of Suburban infrastructure before acquisition was own infrastructure and that was one of the reasons why the margins also were lower because like LPL actually has grown out of entrepreneurial behavior of partners and franchising does help.

Now, the Suburban was not like that, this whole transition as we made, we ended up actually sharing revenue share with our partners that caused a little bit of a stress.

So, to me, I think I would sum it up our ability to visualize a scenario that post-acquisition what is the state of the business is one learning that I have out of this.

And that reason helps in all the negotiations, evaluations and operational challenges one would face post-acquisition.

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S Srinivasan: And the second question is just a data-keeping on the 11.6% growth. How is that split by the volume and price for Suburban again?

Dr. Om P Manchanda: That probably, Shyam, we have not really shared. That granular I would say as of now I think this data probably as the time comes, we will start sharing, but as of now I would say that we are not fully there yet because of the system integration.

Ved P Goel: There is no price increase in Suburban.

Dr. Om P Manchanda: This has not come out of price increases.

Moderator: Next question is from the line of Vaibhav Saboo from Nippon AIF. Please go ahead.

Vaibhav Saboo: If I look at the material cost and if I evaluate the gross margin for Q2 last year, the gross margin was around 79.7%, which was 80.2% in the previous quarter and 80.7% in the current quarter. Now considering we have not also taken any price increase, can you give any insight as to how we are able to increase the gross margin just on the material front, and whether this is something which is sustainable going forward or was it something like a one-off?

Dr. Om P Manchanda: Yes, I think I have an answer for that. I think again and again we are talking about, this is what bundling is leading to and if you study most of the bundle packages of various companies in the industry they consist of a lot of routine tests and relatively routine test cost is lower as a percentage of top line compared to higher end tests.

And Swasthfit is growing nearly about twice the rate of our company growth.

That

tends to reduce the consumption cost. Now, it will obviously stabilize at some point.

It cannot really keep going down again and again. I would say your question is, is it sustainable at this number? I get a sense, yes, right.

Ved P Goel: And in shorter term, yes.

Dr. Om P Manchanda: Yes, and obviously there is an impact of higher top line. In those quarters where top line is lower you might just see but on a yearly basis I would say they are sustainable.

Vaibhav Saboo: In terms of Swasthfit contribution, while we have been able to increase on a YoY basis but if I look at it, Q1 stood at 25% whereas for the current quarter stands at 24%. Like any particular insight from that?

Shankha Banerjee: Historically, the trend is that the Q2 contribution is lower than Q1 and it is because of the overall base number going up due to a lot of seasonal fever, etc., Even if you see QoQ, last year Q2 contribution was lower than last year Q1 contribution for Swasthfit.

Moderator: Ladies and gentlemen, that was the last question for the day. I would now like to

hand the conference over to the management for the closing comments.

Ved P Goel: Thank you, everyone for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries.

Please feel free to reach out to us in case you have any further questions or queries.

And once again thank you and wish you a very, very Happy Diwali and the rest of the festival. Thank you.

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February 03, 2025

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q3 FY25 Earnings Conference Call

Dear Sir/ Madam,
Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q3 FY25, Earnings Conference Call. We request you to please take the same on record.

Thanking You, Yours Faithfully,
For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer
Encl.: As above
Page 1 of 16
Classification: Confidential

Dr. Lal PathLabs
Q3 FY25 Earnings Conference Call Transcript
January 30 , 2025

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group CFO & CEO – International Business
Participants who asked questions • Saion Mukherjee - Nomura Securities
• Karthik Chellappa – Indus Capital Advisors Hong Kong Limited
• Rishi Modi - Marcellus Investment Managers
• Prakash Kapadia – Spark PMS
• Karan Vora - Goldman Sachs
• Saurabh Kapadia - Sundaram Mutual Fund
• Anshul Agrawal - Emkay Global
• Punit Pujara – Helios Capital
• Sumit Gupta – Centrum India
• Abdulkader Puranwala – ICICI Securities
• Yogesh Soni – InCred Capital
• Pradnya Ganar – UTI AMC
• Prashant Nair - Ambit Capital
• Rahul Jeewani – IIFL Capital

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Moderator: Ladies and gentlemen, good day and welcome to Dr. Lal PathLabs Q3 FY25 earnings conference call for investors and analysts. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Nishid Solanki of CDR India. Thank you. And over to you.

Nishid Solanki: Thank you. Good afternoon, everyone, and welcome to Dr. Lal PathLabs Q3 FY25 earnings conference call. Today, we are joined by senior members of the management team, including Hony. Brig. Dr. Arvind Lal, Executive Chairman, Dr. Om Prakash Manchanda, Managing Director, Mr. Shankha Banerjee, CEO, and Mr. Ved Prakash Goel, Group CFO and CEO, International Business.

I would like to share a standard disclaimer. Some of the statements made on today's conference call could be forward-looking in nature and the actual results could vary from these forward-looking statements. A detailed statement in this regard is available in the results presentation, which has been circulated to you and also available on the stock exchange website.

I would now like to invite Dr. Arvind Lal to share his Perspectives. Thank you. And over to you, sir.

(Hony) Brig. Dr. Arvind Lal: Thank you very much. Good afternoon, everyone, and thank you for joining us on today's call to discuss our Q3 FY25 Results.

I would like to begin by sharing my thoughts on the dynamics of the diagnostic sector in India. And give you an overview of the key initiatives we are rolling out.

The Government of India has committed to increasing the public health spend to 2.5% of our GDP from the current approximately 1.8%. The need to strengthen primary and secondary healthcare infrastructure especially in rural areas and in cities classified as Tier-3 and beyond has never been more pressing. At the same time, we are also witnessing a rise in non-communicable diseases, also called NCDs or lifestyle diseases and these include diabetes, hypertension, cardiac diseases, strokes, cancers, kidney, liver and lung diseases and obesity. NCDs account for nearly 65% of deaths in the country and a financial loss of about 2% of GDP every year. Therefore, it is imperative that both the government and the private healthcare work together to control the rise of these NCD, starting with timely screening and preventive management, creating accessible testing capabilities, which often serve as the first steps in delivering health care has become increasingly crucial, as was also seen during the COVID pandemic.

In this context, Dr. Lal PathLabs network plays a key role in enhancing healthcare accessibility in an affordable and reliable manner, particularly in underserved regions. The awareness around preventive diagnostics is also growing. We are in a position to facilitate timely screening and early detection, enabling proactive healthcare management and improving overall health outcomes for all.

Our ongoing investments in digital infrastructure have given us the flexibility to efficiently deliver services to a wider customer base. We were pioneers in leveraging AI-driven diagnostic modules with introductions of our AI-enabled test for breast

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Classification: Confidential cancer and prostate cancer many years ago. Genomics, where we have made significant investments and established our Center for Excellence for 'Genevolve', serves as another platform that expands our diagnostic capabilities to have a laser focus on genetics and personalized medicine. It is these innovative and cutting edge diagnostic solutions that have made us the numero uno in diagnostics in India and the most

preferred testing partner for both patients and doctors.

Our operations continue to scale up according to our predefined strategy. That's by over 75-years of experience. As a leader in healthcare, it is our goal to provide timely, efficient quality care to everyone in the country.

With that, I would like to hand to Dr. Om Manchanda. Over to you, Om.

Dr. Om P Manchanda: Thank you, Dr. Lal and warm welcome to all the participants on this call today. The last time I spoke about bundle packaging and our strategic efforts in Western region.

This time I want to share about as to how we navigated our business without the price increase and still delivered better than last year revenue and EBITDA margin in the first nine months of the current financial year.

The first initiative that we took was a very sharp focus on geographical and test mix that delivered higher realization per patient as well as EBITDA margin. For example, Delhi NCR, a region with a better margin profile has delivered double digit growth in

line with the national average. This has happened after many years of single-digit growth in this region.

The second has been Swasthfit contribution continues to rise and has delivered higher realizations of patient. This is as a part of our stated strategy.

Second initiative has been that we are expanding lab and collection centers infrastructure in Tier-2 and Tier-3 towns.

Third initiative has been relentless focus on the productivity initiatives.

And lastly, a very sharp focus on franchisee collection centers that helps us to increase width as well as depth of the reach. The contribution from franchisee collection centers now has gone up to nearly 45%.

With that, I would now hand it over to Shankha for his thoughts. Thank you.

Shankha Banerjee: Thank you, Dr. Om. I extend a warm welcome to all participants joining us on call today. I will present the key business and operational achievements.

We maintained a strong performance trajectory in the third quarter of the fiscal year 2025, delivering 10.7% growth in revenues. The growth is delivered through sample volume growth and mix improvement and does not have any price increase benefit. I am pleased to share with you that Delhi NCR recorded double-digit revenue growth for the second successive quarter.

Let me share some of the key operating statistics. Sample volume stood at 20.6 million in Q3, a growth of 10.3% YoY, while patient volumes came in at 6.9 million, an increase of 3.8% over the same period last year. Revenue per patient for Q3 FY25 stands at Rs.861, a growth of 6.7% compared to Q3 FY24. This growth in

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Classification: Confidential revenue per patient is on account of mixed management initiatives on geography, channel and products.

We are accelerating our reach in the underserved markets with the planned opening of 15 to 20 new labs in this financial year. We continue to expand our reach into Tier-3 and Tier-4 markets and have started strengthening our presence in the core regions as well. We are now making investments in new infrastructure in metro and Tier-one cities.

Our bundle test program, Swasthfit, continues to contribute favorably to the overall performance with a robust growth of 27.2% in Q3 FY25. Building on positive results, we are developing a wider range of bundle packages and expanding our market penetration beyond the traditional Tier-one and two markets to address the growing demand. While it's still early days, the response thus far has been encouraging.

Our future growth will be driven by a combination of strengthening our operations in core metro and Tier-one cities, penetration in Tier-3 and Tier-4 in North and East and building clusters in South and West. We are committed to maintaining the highest standards of diagnostic quality and accuracy while expanding our service offerings to include high end and super specialty test. Our robust digit

al infrastructure will

enhance the patient's experience by providing seamless access and maximizing solutions.

With that, I would like now to invite Ved, who will take you through the financial performance. Over to you, Ved.

Ved P Goel: Thank you, Shankha. Good evening, everyone and a warm welcome.

I will be sharing the key financial highlights for Q3 FY25 and year till date.

- Revenue for Q3 FY25 came in at Rs.597 crore compared to Rs.539 crore in the same quarter last year, reflecting a growth of 10.7%.
- YTD FY25 revenue stands at Rs.1,859 crore against Rs.1,681 crore in the same period last year, a growth of 10.6%.
- Revenue per patient for Q3 FY25 is Rs.861 against Rs.807 in Q3 FY24 mainly due to test and geography mix.
- Test per patient for Q3 FY25 is 2.97 v/s 2.79 in Q3 last year, a growth of 6.3%.
- EBITDA for Q3 FY25 came in at Rs.154 crore compared to Rs.141 crore in Q3 FY24, registering a growth of 9.6% with the EBITDA margin of 25.8%.
- YTD FY25 EBITDA stands at Rs.527 crore v/s Rs.465 crore in last year same period with a growth of 13.3% and EBITDA margin of 28%.
- PBT for Q3 FY25 came in at Rs.138 crore v/s Rs.116 crore in last year same period, registering a growth of 19.4%.
- PAT for Q3 FY25 came in at Rs.98 crore compared to Rs.82 crore in the same period last quarter last year, registering a growth of 19.3% with a PAT margin of 16.4%. For YTD FY25, PAT stands at Rs.337 crore against Rs.277 crore in same period last year with a margin of 18.1%.
- EPS for Q3 FY25 is Rs.11.6 as against Rs.9.8 in the same period last year

and YTD FY25 EPS stand at Rs.39.9 against Rs.32.9 last year same period, registering a growth of 18.7% and 21.4% respectively. Net cash as on 31st December 2024 is Rs.1,123 crore.

- At last, I am pleased to share that the Board of Directors of the company have approved a Third Interim Dividend of Rs.6 per share, taking the total dividend for the current year till date to Rs.18 per share.

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Classification: Confidential With this I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A.

Moderator: Thank you very much. We will now begin the question -and-answer session.

We will take our first question from the line of Saion Mukherjee from Nomura Securities. Please go ahead.

Saion Mukherjee: Yes, hi. Good evening and thanks for taking my question. Just some P&L numbers I just wanted to get some color. We have seen consistent improvement in gross margin despite no price increase. If you can take us through the dynamics in terms of volumes, Swasthfit mix and you have raw material prices, currency, etc., how should we think about gross margins going forward? And the second one was on employee expense. What I notice, it has been growing in mid-teens higher than the revenue growth for the past few quarters. What's driving this and how should this grow going forward in your view? Thank you.

Ved P Goel: I think just to correct, the gross margin in this quarter compared to last year same quarter has not improved. Basically, this gross margin is slightly lower than last year same quarter. Second, in terms of employee cost, yes, definitely, there is an increase because we have started investing in new infra and we hired about roughly 100 people in our sales and marketing team as well. So, that is the impact where we are investing in the business for future growth.

Dr. Om P Manchanda: I want to add what Ved just mentioned. I think in the last sort of 1-2 years post COVID, there has been a certain jump in gross margins and mainly because of the COVID testing decline, our margin improved and we also invested a lot behind technology initiatives, which helped us to improve productivity. But I would say that these margins one should assume more or less stable at this number rather than any improvement further going forward. As far as employee cost

is concerned, as Ved

mentioned, we are now relooking at our organic infra and not only just in terms of lab but also adding manpower in our business, which is combination of both these two.

Saion Mukherjee: If I can ask one more just on depreciation and tax rate, we see depreciation, amortization numbers kind of quite flattish or even declining. Any reason there? And you are still on the old tax rate and any plans to move into the new tax rate and what should we expect in terms of an effective tax rate in the years ahead?

Ved P Goel: Our applicable tax rate is 25%. If you see the standalone, the tax rate is similar to that effective rate, but in consol we are charging some depreciation on account of impairment of intangibles, where we are not getting any tax benefit right now, that's where the effective tax rate is about 29% in consol. But in future, I don't think there is any change we have on this applicable tax rate.

Saion Mukherjee: And any comment on depreciation, amortization number?

Ved P Goel: These are similar because there is not much CAPEX which is done in terms of newer investment. These will be coming in the future. Right now, depreciation is more or less in the same range.

Dr. Om P Manchanda: Getting offset.

Ved P Goel: Yes.

Saion Mukherjee: But you were adding labs, right, I understand your organic infrastructure you are adding.

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Classification: Confidential Ved P Goel: This is offsetting against the depreciation we are charging against the new addition.

Moderator: Next question is from the line of Karthik Chellappa from Indus Capital Advisors Hong Kong Limited. Please go ahead.

K Chellappa: Sir, two questions from my side. First, if I look at Swasthfit, that contribution to overall revenue has remained at about 23% -25% in the last four quarters. What is our medium-term aspiration in terms of, let's say, revenue composition and what needs to happen for us to take it to a higher level? That's my first question.

Shankha Banerjee: If you see every quarter v/s the same quarter last year, you will see that it's a 2.5% - 3% contribution over and that's been a trend that we have seen through the last two, three years period that the contribution remains kind of flat between Q4 to Q3 and then again in Q4 it jumps. All the actions that we are taking, you will likely see some benefit of that accruing to us in Q4 and the baseline will shift again for the next three quarters. The growth rate on Swasthfit, and if you also compare the contribution v/s

same quarter last year, you will see the delta.

K Chellappa: Got it. Which means that going into next year, this ratio, at least in a YoY basis, will continue to improve. Is that how I should interpret it?

Shankha Banerjee: Yes, that's really the plan.

K Chellappa: My second question sir, if I look at Suburban, this quarter at about 9.2%, that revenue was soft relative to both our overall revenue growth as well as Suburban's nine-month revenue growth as well. What would explain this relative weakness in Suburban this quarter and what is the plan or let's say aspiration for FY26?

Shankha Banerjee: I think we've been saying all along that in our business QoQ may not be the right metric to look at growth rates. I think what we are saying is that the growth trajectory on Suburban definitely is moving in the right direction. There is obviously some quarter there can be a slight up, some quarter there could be a slight down, but overall that trajectory will improve and obviously going forward our plans are to grow even faster than what you're seeing now.

Dr. Om P Manchanda: I also want to add more point. Actually, while the questions get asked on Suburban, but we are looking at West as a region for our total business. We have 3 verticals there. One is Suburban, the other is Dr. Lal PathLabs parent business and third also we have done a smaller acquisition under the vehicle calle

d PathLabs Unifiers. Our

front-end team now is common for all the three and we really don't evaluate them whether it has to be Suburban or parent as long as overall business grows. Because there's a huge sort of a network advantage we get one over the other and obviously there is a latent sort of brand equity for Dr. Lal PathLabs and I was looking at the number actually West region growth rate is even higher than our national average for not only for the quarter even on YTD basis. So, we've also seen some bit of uptick on the parent business as well in this geography. That network of Suburban is also helping us in a positive way to get our Dr. Lal PathLabs business growing as well.

Moderator: We will take our next question from the line of Rishi Modi from Marcellus Investment Managers. Please go ahead.

Rishi Modi: Om, I heard your comment on the gross margin where you said that large part of the gross margin expansion is done some with respect to COVID going away, the normal business having higher gross margins and the others the initiative that you all up taken at your end. One thing I wanted to understand right, today, on the other industry in the domestic reagents business, which is coming up, which is at a lower

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Classification: Confidential pricing than the international players. Like, are we not planning to increase that mix or like we going to pass on the benefits that we get from those and hence lower pricing or test pricing, like I am just trying to understand why wouldn't gross margin expansion take place if we incrementally keep using domestic players reagents?

Dr. Om P Manchanda: Ved can comment on this domestic part of it, but I also want to highlight because most of these regions are imported and dollar becoming expensive, while we are still holding on to this, I don't know what the response of our vendors going forward, but that pressure also is going to come not only for us, but for the entire industry, but as of now, we're just holding on to these prices. So, there are headwinds on the reagent cost. On the domestic v/s international, maybe Ved, you want to comment.

Ved P Goel: Yes, Rishi, I think as Om said, these are mostly reagents which we are directly not imported but a n imported material. However, having said that, we have started using some of the consumables, which is local Indian manufactured and there we are definitely getting some leverage and benefit. If you see last 2-3 years consumption cost, it has reduced substantially because of mix which we are changing from 22.5% to now roughly 20.9% this quarter. That is where wherever we have opportunity, we are doing that, but at the same time there is a inflationary pressure which is coming and that's where I don't think in future we should build any further improvement in the gross margin.

Rishi Modi: Dollar appreciation primarily in the near term will lead to pressure, sir. Second, I wanted to understand on the Swasthit , right. When I look at the contribution, from Q1, the contribution has come down to 23%, while YoY it's still up. I am just trying to understand, is there something which is missing here like this contribution has not historically dropped as a percentage of revenue ever since in the last 3 years at least that we have the data for, it's kept on decently well over a year upon growing on a QoQ basis as well. Is 25% the peak or like what's happening here if you can give some color?

Dr. Om P Manchanda: I think the data that I have in front of me, what I've seen is that Q3 generally is a soft quarter, 23% one see it as to what was the contribution last year in the same quarter, that number is 20%. And our sort of assessment is that Q4 actually peaks because of a lot of people actually go for health packages because there's reimbursement, I

think there's a tax advantage they get. It peaks in Q4 and then somewhere stabilizes after that in Q1 as well. But then it

softens in Q2 and Q3 and a gain it peaks in Q4. I

think 23% is not a representative figure for the year, but it's only a figure for the quarter.

Shankha Banerjee : I think the trend is similar last year. So, it's not as if every quarter last year was growing over the previous quarter. If you see the trend last year was exactly the same on contribution. Even last year, the QoQ contribution was highest in Q1, then it dropped in Q2, dropped in Q3, again peaked in Q4. I think that is the trend we have seen for the last 2 years and the same trend continuing this year.

Dr. Om P Manchanda: Or other way to look at is that peak contribution aside, Swasthfit this quarter has grown by 27% over last year same quarter.

Rishi Modi: And I think last time, Om, you mentioned like this can go much beyond the 25 %-30% mark. Is that still the case or you are still maintaining that, right?

Dr. Om P Manchanda: Directionally, I think industry is showing those trends and I haven't collated other companies data, but I think one should probably look at the trends in other companies as well, where the data is available publicly, but my own gut says that directionally it's moving towards bundling and we keep growing. Now I don't know what that number is, but definitely if 50 %-60% of disease incidence is coming out of

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Classification: Confidential NCDs, I think NCDs are, that segment is prone to bundling because they're all metabolic disorders, they are all club together and they are all falling in the domain of preventive testing, etc., It's only the infectious or communicable diseases you can't bundle it. You don't need to go for a malaria testing if you don't have fever, but you can still go for a lipid profile and kidney function test, etc., I think if you were to ask me as to where it can go, it is a reflective of total NCD burden in our society .

Rishi Modi: Finally, for Ved, just a couple of numbers I wanted your help with. Suburban EBITDA margin for the quarter v/s the same quarter last year if you could help me with? And on the tax piece I am seeing like a 28 %-29% tax rate for several quarters now. Just wanted to understand like what's the reason for this elevated tax rate? I maybe you have listed a few mentioned in the past.

Ved P Goel: Rishi, Tax as I mentioned earlier also that if you see standalone, there is a absolutely around 24 %-25% which is the applicable rate, but on consol there are some adjustment, maybe some depreciation, intangibles where we are not getting any tax and then dividend adjustment which is intercompany removal of those. For that, it is reflecting a higher tax rate, but normal tax rate is 25%.

Rishi Modi: Like this tax amount that we're showing in the P&L is actual amount that we are paying, not just the P&L amount?

Ved P Goel: Yes.

Rishi Modi: And if you could help me with the EBITDA margin for Suburban for the quarter v/s same quarter last year?

Ved P Goel : It is lower but again on an yearly basis, I think margins for Suburban will be in the range of 15 %-16% because in Q3 as revenue dropped as so as the margins.

Rishi Modi: If I take H1, we would have done about 17 %-18% of EBITDA margins, you're saying nine months is at 15 %-16%. This quarter, the EBITDA margin would be around 10 %-12%, is that right?

Ved P Goel: Yes, 12 %-13%.

Dr. Om P Manchanda: Yes, because higher fixed cost component is so high, it's very sensitive to revenue fluctuations.

Rishi Modi: You are saying it's declined v/s last quarter . So, last year we were in the 15% range. Last year, the margin profile would be the same, same quarter?

Ved P Goel: From last year, YoY, it has improved.

Dr. Om P Manchanda: Q3 FY24 EBITDA margin for Suburban was 11.5%, now it's 12.3%.

Moderator: We will take the next question from the line of Prakash Kapadia from Spark PMS.

Please go ahead.

Prakash Kapadia: Two questions from my end. Any comments on how ar

e smaller players in the

industry doing, any specific trends you witnessed in terms of consolidation or regional players performance that would help? And secondly, any price increases on the anvil because some of the larger players are contemplating price increases?

Those are my questions.

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Classification: Confidential Dr. Om P Manchanda: Let me answer the second question first. Yes, I know that many players have been

thinking of taking price increase, some of them have already taken as well. I think,

internal, our view is that we want to get our volume growth going up and the good news is that this year, we have seen significant improvement v/s last year and we believe our strategy of holding on to the prices and still managing revenue growth in line with our internal budget as well as over last year, we think our strategy is working and without dilution of margins, we want to hold on to this for some time and see if we can get our volume growth up. That's our current view. We will probably review this going forward maybe 6-9 months later, but as of now, we don't plan to take any price increase.

Second is coming to the smaller players as to how it is, I think there is some sort of a stability and calmness in this whole industry right now because a couple of years back because of this deep discounting and aggressive pricing by some of the players which are really uneasy times for the industry. Overall, it is stabilizing. I don't see too much of a sort of stress in the smaller labs, but I also don't see too many new labs coming up. I think it's a stable time I would say. And if some people are taking price increase, it does help smaller labs as well, one must know that, because it gives them sort of leeway to take price increases even locally also. I think it is going to be a stable time for smaller labs going forward for some time.

Prakash Kapadia: And lastly, Dr., in this context of what you just mentioned, is it possible directionally we should grow faster than some of the other players, given that we are spreading geographically, we've been one of the earlier players in Tier 3-4 cities, so when do you know some of these initiatives fructify and lead to still higher sales growth for us?

Dr. Om P Manchanda: Yes, I think you're right, but the challenges also is our base is equally large because at such base, let's say give or take Rs. 2,500 crore base, if you have to grow over 10%, you technically have to add Rs. 250 crore of extra business every year. That's a kind of broad math one has. And growing it organically in the same market sometime becomes challenging and the newer markets are slow to respond. I probably would say what you are saying is right and we also want to make sure that it's a sustainable growth and my personal experience is, 1% or 2% slower growth is still fine, but as long as you are structurally building it up for long-term, slowly, slowly build your infrastructure. Because the reason for people to come back again and again is quality and trust. At no point in time, our expansion should compromise on that because at times in a hurry to get extra that 1%, one then compromise on those aspects. I think it's a more process-driven business, make sure that you build it brick-by-brick is what probably helps. And we are seeing that actually in the market. And one of the factors, in fact Delhi NCR, we've not talked about this. In the last two quarters, our growth rates are pretty good. And I actually would say some of the labs users which went to some of the new competition have started coming back to us so which is a sort of a proof to at least very positive. I don't know whether it's working that way, but it's just a hypothesis I have. I want to see for next 2-3 quarters, but I probably would say that it's a process-driven business. Go steady, slow, slow, build it up and it stays with you.

Moderator: We will take our next question

from the line of Karan Vora from Goldman Sachs.

Please go ahead.

Karan Vora: My first question is with respect to the patient service centers and lab ratio. So, when we mentioned that we are trying to improve productivity and efficiencies, what are the metrics we kind of look at, do we look at revenue per lab or revenue per patient service center or patient service center per lab, which are the right metrics, and what is the ideal metric you would like to have like over a medium term?

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Classification: Confidential Dr. Om P Manchanda: I think clearly this particular metric of ratio between lab and collection center is very

important. Because a throughput through a collection centre is definitely more efficient than throughput through a lab. Not only from a cost perspective, but also giving higher market access because you are spreading your infrastructure closer to the customer.

The second area has been area of overall fixed block that we have, how does it compare in terms of cost per test, basically that ratio. And we have seen simple things like printing a stationary, if you reduce the physical paper use, that itself gives you a lot of benefits. The second area could be if you franchise more collection center, your rental cost as a percentage of revenue actually is coming down. Because your own infra is high street infra, you end up paying more rent and now I think GST impact is also there, right on that?

Ved P Goel: Yes.

Dr. Om P Manchanda: And if it moves closer to collection center, it's really that kind of cost structure is not

there in their hands. So, some of these small, small initiatives, they all add it up and that's how we get the benefit.

Karan Vora: Right, now I think our patient service center to lab ratio is around 20 -21x. What is the ideal number you would look at and is it different between a Tier-1 and say a Tier-2 or Tier-3 city?

Dr. Om P Manchanda: Yes, Tier-1 definitely this number would be higher because you are operating in a small geography. If we go deeper, then obviously it will be much lower. Your average number, I don't know where it can stop. There was a time when we were 1:7 also and I've seen the journey from 7 to now 20 -21. I think this is technically competing with turnaround time as well because more collection centers you have, they are far away from lab, your turn around time actually gets sacrificed. I think it's a very dynamic number. I don't know whether we have a target in our mind, as long as overall sort of a cost overhead, we just keep an eye on that. I don't think we just decided target and go after that number because at some point in time like this year, we are trying to expand more collection centers. This ratio may go off as well. But it's a very dynamic number. I don't think we have a number in our mind to achieve. The higher the number better it is for us.

Karan Vora: My second question is with respect to the competition in Delhi NCR region from hospitals. So, are you seeing the competition kind of increase like on a YoY basis you think it is more or less in line with what you are expecting?

Dr. Om P Manchanda: More or less in line with what we expect, but I think last time also we spoke about, there are hospitals, especially the ones which have ambition to become bigger and get listed. They are the ones who are trying to look at retail pathology. But at least in our sales meetings, we don't really get to hear from them that hospital is a very big threat, but definitely it's a competition which is here to stay, there is no doubt about that and there's a large enough market. In fact, I personally always have believed that organized competition helps all of us because technically you are taking the market away from unorganized players and which is what we're seeing, the market has started responding now better and bundling is effect of aggressive competition activity around bundle packa

ges. Overall I have a feeling that these people will help market to grow on an organized play, shift from unorganized to organized will move further.

Karan Vora: Are they pricing like in line with what we are pricing or are they like more price competitive, do they price higher, how is their pricing?

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Classification: Confidential Dr. Om P Manchanda: They cannot afford to compete with us in pricing because they have to match their internal hospital pricing as well. I think hospital guys will never be aggressive on pricing. They can be aggressive on building infra, but my sense is they will not go to deep cut the pricing.

Karan Vora: My last question is with respect to the kind of region wise growth. I think we mentioned that West and Delhi NCR have actually grown faster, which kind of maybe implies that East or South have kind of grown below average. What has happened there? And on a normalized say medium term growth perspective, which regions do you think can be above corporate average and which regions would be below?

Dr. Om P Manchanda: I think see, the difference is not very stark. When I probably mentioned about West region growing faster, doesn't mean the East has grown very slow. I would actually say if I had to pick one region which probably requires focus is South for us. I think all other regions, in front of me I have the numbers, more or less in line with national average, few 1 or 2% higher or lower. But I think one region which probably requires focus for us is the South. This is going to be a focus area for us.

Karan Vora: Even from a medium -term standpoint, more or less growth should be split equally in across regions?

Dr. Om P Manchanda: Yes. It's a very uniform growth. It does help us to see that we are not dependent on one region for higher for the growth, it is uniform across India.

Karan Vora: And when we look at the margins like do you think or the margins across regions are similar or like if there's a rank order maybe Delhi NCR would be the highest, but like what others would be higher than corporate average and what others would be lower. Not asking for this particular quarter, but in general on a normalized basis?

Dr. Om P Manchanda: Ranking is I think it's no -brainer, Delhi NCR is the highest, then come rest of North and then East and West. Any concentrated geography will always have for us, not for the industry.

Karan Vora: And where would corporate average stand? So, corporate average would be better than rest of north ?

Dr. Om P Manchanda: Maybe we can chat offline.

Karan Vora: Okay.

Moderator: We will take our next question from the line of Saurabh Kapadia from Sundaram Mutual Fund. Please go ahead.

S Kapadia: You mentioned about making investment in infra in metro and Tier-1 cities. So, is it related to the lab investment or it would be more of a marketing infra?

Shankha Banerjee: So, we are looking at lab and marketing both.

S Kapadia: What could be a quantum of investment?

Shankha Banerjee: No. I think that's more in terms of number of labs that we look at. We are starting to look at new infrastructure in metro and Tier-1 in terms of labs.

S Kapadia: I am seeing now with the voluntary liquidation of Suburban, now we mentioned the production in the overhead cost. What kind of cost saving we could see post that?

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Classification: Confidential Ved P Goel: This is administrative and legal and compliance costs primarily, which can be saved because right now there are multiple entities, now it will be one. There are certain costs which we can save on account of this.

S Kapadia: Any number to it?

Ved P Goel: It's not very substantial, but yes, definitely there will be something.

Moderator: We will take our next question from the line of Anshul Agrawal from Emkay. Please go ahead.

Anshul Agrawal: If you could just help me with the number of labs you have already added in

the nine

months ended for this current year?

Shankha Banerjee: We have set 15 -20 labs. We are well on track to achieve that by the end of the year.

Anshul Agrawal: I have a follow -up question on that. Has the Delhi region grown on the back of network expansion or would this be a sort of an SSG kind of growth that we have witnessed over the last two quarters?

Shankha Banerjee: Primarily it is SSG kind of growth as of now. Yes. Whatever we are seeing in the last two quarters is still primarily at is.

Moderator: We will take our next question from the line of Punit Pujara from Helios Capital.

Please go ahead.

Punit Pujara: My first question is for Dr. Om. You mentioned that the new labs have stopped coming in, but has that resulted in let's say lower valuation in the existing labs? I am asking this because you have Rs. 1,100 crore of cash and I think it makes sense to follow the roadblock strategy . Yes, that would be my first question.

Dr. Om P Manchanda: The stable environment, I essentially meant that, one tends to see a lot of new labs being launched by smaller players. Especially in the unorganized space, I don't see a lot of new players coming and putting up new labs. That's what I meant by that.

But there is aggression , there is a competition from the organized because those players are putting up new labs, but I what I meant was unorganized sector.

Punit Pujara: I see. And the second is clearly you are focusing a lot on growth. Are there any adjacent areas that you are exploring to further accelerate the current rate of growth?

Shankha Banerjee: We are currently looking at a few options, but I think there are too immature right now to talk about. But yes, overall, a slightly longer term horizon there could be some adjacencies.

Dr. Om P Manchanda: For us essentially we are looking at higher end tests in the area of genomics and Dr.

Lal mentioned in his comments about 'Genevolve ' and it's in the area of autoimmune disorders. There is a lot of effort going behind promoting higher end tests. So, that actually grows on back of the network that we have expanded. I think that's the way we look at adjacencies. And the second area is in terms of basic radiology, especially ultrasound and X -ray and we have large number of labs where we have the services. That's another area of adjacency. So, that's the way we look at it.

Moderator: We take our next question from the line of Sumit Gupta from Centrum India . Please go ahead.

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Classification: Confidential Sumit Gupta: As we plan to expand geographically across the nation, so like, what is the patient volume growth that you expect going forward over the next 3-4 years?

Shankha Banerjee: Last year I think we ended the full year at about 2.7% and I think right now YTD we are about 4.4%. Give or take let's say we are in the range of 4.5% -5% is what I think we are expecting patient volume, and our endeavor is to improve that as we go forward. I think we are not taking any specific target on it, but the idea is with the new infrastructure, collection, network expansion, etc., we can reach and access larger patient pools and therefore grow better in terms of patient volumes going ahead.

Sumit Gupta: Okay. And then those geographies also you plan to expand through volume growth through Swasthfit ?

Shankha Banerjee: Swasthfit is just a one product which plays more prominently in maybe metro, Tier-1 and some Tier-2 cities. Infra growth and Swasthfit are two different access for growth.

They need not be overlapping. A certain level could be complementary to each other.

Dr. Om P Manchanda: I want to pick up this question on the volume. We first of all look at volume both these metrics together, it's not only just the patient growth but also sample growth. Because what we are seeing as a contribution of Swasthfit is moving up. There is a slight decline that we see on the patient growth

because actually technically these are patient visits, they are not unique patients. We are seeing the frequencies drop and that's why you see slightly slower or lower patient volume growth. We want to see both together and we want to see both growing as we move forward.

Sumit Gupta: Understood, sir. You also allocated revenue contribution from franchisee collection center, was 45%. Overall, how is the trend being and how do you expect it to go forward?

Dr. Om P Manchanda: I get a sense it's now stabilizing here because in addition to this, we have pickup points which is nearly about another 20 %-25%. The rest of it will remain as a walk - in and home collection, which we normally call retail business which is our own infra. I would say it is probably stabilizing at this number.

Sumit Gupta: And sir, lastly, how much CAPEX was there in nine months FY25 and what is your guidance for next year?

Ved P Goel: In nine months roughly we have made a CAPEX of around Rs. 30 crore. As Shankha mentioned, we are planning to open 15 -16 labs in this year. Maybe this is coming in the later part of this year, so Q3 and Q4. And going forward, on an ongoing basis, maintenance CAPEX is about Rs. 60 - Rs. 70 crore per year excluding any major investment in any other venture.

Moderator: We will take our next question from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

A Puranwala: Just a follow up on what the previous participant was talking about. If I see your Swasthfit revenues, they are growing much faster and I think that is a broader trend for the last couple of quarters and we have been talking about network expansion into Tier-2, Tier-3 markets, that has worked quite well, but what in terms of the overall growth, that's end up to single. Just also wanted to understand on the volume front, how was the volume growth been in the Swasthfit v/s the residual part of your portfolio, is there any material difference as compared to how your revenues are getting shaped?

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Classification: Confidential Dr. Om P Manchanda: We don't have that data in front of us. We will come back to you offline. But YTD

overall volume growth for the business is about 4.4%.

A Puranwala: Second is on a similar line. While historically we have seen that bundling test is something which has always attracted a large competition say it from online or you know from other channels. If you could share your thought process about carefully selecting this as one of your growth engines for the near future despite the threat of competition any day coming into it?

Shankha Banerjee: I think Bundle Testing is a phenomena which is now across the industry, and I think to a certain extent the promotions done by the online competitors have even spurred this demand overall. It may not necessarily be one eating into the other, rather it helps building the category much faster.

Moderator: Ladies and gentlemen, to ask a question, please press "*" and "1" on your phone now. We will take a next question from the line of Yogesh Soni from InCred Capital . Please go ahead.

Yogesh Soni: Just wanted to understand , what has been the West region contribution in last year? And also wanted to understand your plans on expansion into West and Southern regions? Thank you.

Dr. Om P Manchanda: West region contribution is now nearly 15% all put together, which is similar to what we do in East. East and West are now easily sort of balanced now, gone up from 14 to 15 is what I see.

Yogesh Soni: Sir, what endeavors are we making to expand into these two regions, West and South?

Dr. Om P Manchanda: South, obviously, the region for us to focus . As we have always stated that these new regions, we look at both organic and inorganic and we will continue to put those efforts in these markets. And while on these calls we can talk about South, but South itself is very big cluster of Bang

alore, you have a cluster of Chennai, Hyderabad. We look at that cluster and then grow our business. We will definitely focus much more in South this year than what we've done in the past.

Moderator: We take a next question from the line of Pradnya Ganar from UTI AMC. Please go

ahead.

Pradnya Ganar: Sir, my question is on the M&A strategy. While you indicated that M&A you look at it to expand into new geography, but can you also look at M&A to add new capabilities, are there any kind of testing capabilities that you would want to add and for which you might want to evaluate M&A?

Dr. Om P Manchanda: Yes, that's a good question. Actually, when we look at some of these businesses and I would say there's a routine testing which I think every one of us has. Then there are a lot of these high-end tests that are done in India, and nobody has these test instruments, these instruments are actually available off-the-shelf. So, you don't need to go and acquire, if we Illumina is one machine for genetic testing which cost me about Rs. 8 - Rs. 10 crore. So, you just go and buy this machine, and you add these tests. You don't need to go and acquire any company. The effort really needs to be put on the sales and marketing side and then acquiring some of these because these technologies come from outside, available on the shelf and you just go and buy these equipment and you are adding those capabilities. I think the real effort is on the front end side to grow the business.

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Classification: Confidential Pradnya Ganar: Currently what would be our mix of specialized and semi-specialized set?

Shankha Banerjee: There is no industry standard definition of specialized, but the way we look at our business, the way we define specialized, that portfolio for us is about 22% right now.

Moderator: We will take our next question from the line of Prashant Nair from Ambit Capital.

Please go ahead.

Prashant Nair: Good evening, everyone. I just wanted to reconfirm the volume growth number for nine months. Did you mention 4.5% in that range?

Shankha Banerjee: Yes, YTD patient volume is 4.4%.

Prashant Nair: And is there a test volume number?

Dr. Om P Manchanda: 9.5%. This number that I am mentioning to you 4.4% volume and 9.5% sample growth is for nine months.

Prashant Nair: Dr. Om, when you mentioned you want to take volume growth higher. That's the internal thought process. Where you think you would want to settle, is it like 100 -200 bps higher or can you reach a higher number than that?

Dr. Om P Manchanda: Yes. Okay. Don't hold me for that number, but I would say that if you go back in time before COVID our average volume growth used to be in the range of about 8% -odd CAGR and sample growth was not much during that period. Then we went into a phase where the sample growth picked up and the volume growth declined and we were actually struggling to figure out what's really going on. And one of the variables we found was Swasthfit. Because bundling contribution was going up, sample growth was going up, but the volume was coming down. But then I look at volume growth as surrogate measure for our expansion efforts, I would want to see that more and more patients coming from newer markets, gives us that we're expanding. Sample growth at times tends to convey that you are actually upselling to the existing customer base. I think probably my message to the team would be at least go back to this number of 7 %-8% that we were doing in the past.

Prashant Nair: Right. And just one follow up question. Dr. Om, I think while the volume growth in terms of patients has come down, I think your test per patient has probably gone up over this period. Do you think you'll be able to retain that number where it is now, or maybe improve on it while you also step up on patient growth?

Dr. Om P Manchanda: I think directionally test per patient, there are lots

of tailwinds on that. One tailwind,

of course, is the bundling. I think you guys cover pharma as well. Maybe you may want to study some of these prescription behavior. I am noticing that now more and more tests get prescribed on the same prescription than before. That is also happening. I think there is a favorable sort of a situation for test per patient in future.

Moderator: We will take our next question from the line of Rahul Jeewani from IIFL Capital.

Please go ahead.

Rahul Jeewani: Sir, now you're talking about lab expansion in metro and Tier-one markets as well.

And as you rightly pointed out that we see a lot of these bundle tests on Swasthfit primarily in larger metro markets. With the expansion which we are targeting in metro markets, is there a potential that the sample volume growth, which has been around 9.5%-10% for us, that can accelerate to a low double digit kind of a number going forward?

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Classification: Confidential Dr. Om P Manchanda: But it'll get offset by newer markets going up for Swasthfit because it's not Delhi NCR

alone, you have entire UP, rest of north, these labs are also responding very well to Swasthfit kind of packages. I think if there is a headwind in bigger towns, then it'll

definitely get offset by smaller towns. That's my take away on this.

Shankha Banerjee: I think it is all just a slight shift to say that there is still quite a bit of headroom in the metro and Tier-one town, which we want to capture. And not only will it be captured through Swasthfit or bundle package, but also, the prescriptions which come out and where there is a lot of tag dependent delivery expected.

Rahul Jeewani: Sure, sir. I wasn't talking about any headwind. Actually, I was talking about this tailwind that over let's say past 2-3 years we have been expanding in Tier-2, Tier-3 markets, in North and East. Now, we are talking about expansion in metro markets as well. With that expansion in metro markets, is there a potential that we actually start seeing an acceleration in volume growth both on the patient volume as well as the sample volume growth?

Shankha Banerjee: These are in my view, it's a part of the overall mix. If you see the contribution of metro and Tier-1 today and we've been talking about our Tier-3 and Tier-4 kind of contribution, therefore, it's not as if metro and Tier-1 is a huge contributor, it's also a part contributor. It's just about ensuring that there are lot of tag dependent testing opportunities or patient opportunities that we sense in these markets as well which we would like to capture. Yes, the idea is to help us get newer patients or patients who may not be coming to us, and in a certain sense, it will help patient volume growth as well as sample growth.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to management for closing comments. Over to you, sir.

Ved P Goel: Thank you, everyone for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries and questions. Please feel free to reach out to us in case you have any further queries. Thank you once again. Now we can close the call.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: May 2025

Classification: Internal
May 01 , 202 5

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q4 & FY25 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q4 FY25, Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 14
Classification: Restricted

Dr. Lal PathLabs
Q4 & FY25 Earnings Conference Call Transcript
April 25 , 2025

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group C FO and CEO, International Business
Participants who asked questions • Prakash Kapadia – Spark PMS
• Karthik Chellappa – Indus Capital Advisors Hong Kong
• Anshul Agrawal - Emkay Global
• Shyam Srinivasan – Goldman Sachs
• Bharat Celly – Equirus Securities
• Raman Sivi – Sequent Investments
• Divyansh Gupta – Latent Advisors
• Amey Chalke – JM Financial
• Deven Kulkarni – Marcellus Investment Managers
• Sumit Gupta – Centrum Broking
• Harshal Patil – Mirae Asset

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Classification: Restricted Moderator : Ladies and gentlemen , good day and welcome to Dr . Lal PathLabs ' Q4 and FY25 Earnings Conference Call. As a reminder, all participant lines will be in the listen - only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand over the conference to Mr. Siddharth Rangnekar from CDR India. Thank you, and over to you, sir.

Siddharth Rangnekar: Thank you. Good afternoon , everyone, and welcome to Dr. Lal PathLabs ' Q4 & FY25 Earnings Conference Call. Today, we are joined by senior members of the management team , including (Hony) Brig. Dr. Arvind Lal , Executive Chairman, Mr. Shankha Banerjee , CEO and Mr. Ved Prakash Goel , Group CFO and CEO , International Business .

I would like to share our standard disclaimer : some of the statements made on today's call could be forward -looking and actual results could vary from these statements. A detailed note is available in the results presentation on this , and the presentation has been circulated to you and is also available on the stock exchange website.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspective. Thank you, and over to you, sir.

Dr. Arvind Lal: Thank you, Siddharth . Good evening, everyone, and thank you for joining us today on our annual earnings call.

I will share perspectives on diagnostics as an industry and the opportunities that it presents for Dr. Lal PathLabs . As we reflect on the strides made in India's healthcare sector during FY25 , it is evident that both Government and private entities have intensified their commitment to enhancing health infrastructure and services. The Union Budget for 2025 - 26 has increased healthcare allocation by nearly 11% to nearly Rs. 1 lakh crore. Flagship schemes like Ayushman Bharat , PMJAY received increased funding. Concurrently, private equity investments in healthcare have reached \$5 -6 billion in 2024, nearly doubling pre -endemic levels and expected to maintain at these levels concurrently. Both trends point towards breadth and depth of opportunity in this space.

Demographically, India stands at a pivotal juncture. The working age population , that is 15 - 59 years of age , is projected to constitute 65.2% of the total population in 2031, offering a substantial demographic dividend. The population of seniors in the same period is expected to stand at 13.1%. Some estimates project that India shall have 34 crore seniors by 2050, accounting for approximately 17% of world's elderly population.

This dual demographic trend underscores the pressing need for a healthcare system adept at addressing both preventive and geriatric care. In this evolving landscape, the diagnostics industry plays a crucial role. The diagnostic sector, growing in double digits annually, is witnessing steady expansion. However, it is noteworthy that the organized segment accounts for only a fraction of this, highlighting significant opportunities for consolidation and standardization.

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Classification: Restricted I would like to share a few important trends with you at this moment , given how the growth in testing is shaping -up, Tier-2 and Tier-3 towns and beyond are expected to account for a larger share of the diagnostic demand in the coming years.

Secondly, the burden of non -communicable diseases or NCDs or lifestyle diseases has significantly increased with cardiovascular conditions, diabetes and cancer estimated to impact about 23 crore people by 2030.

At Dr. Lal PathLabs, we are committed to serving this requirement for higher testing by leveraging technology, expanding our reach and ensuring that quality diagnostics are accessible a

cross the country. Our brand has introduced a new test and overseeing the establishment of new laboratories with a view to enhancing accessibility.

Investments have continued in digital technologies in order to streamline operations and improve patient experiences. This includes implementation of advanced laboratory information systems and usage of AI to enhance diagnostic accuracy. As we navigate these transformative times, our focus remains steadfast on innovation and excellence. Together, we can shape a healthcare ecosystem that not only meets the current demands, but it is resilient and responsive to future challenges.

Thank you for your continued trust and partnership. I now hand over to Shankha .
Over to you, Shankha .

Shankha Banerjee: Thank you, Dr. Lal. A warm welcome to everybody on our Q4 & FY25 earnings call.

On the operational front, we sustained a strong growth trajectory in Q4 driven by volume growth and mix improvement without taking any price hikes. Sample volumes grew by 9.5% to 20.9 million, while patient volumes increased by 3.8% to 6.8 million. Our bundle d test program, SwasthFit , also delivered robust growth of 22%, contributing meaningfully to the overall performance.

During the year, we added 18 new labs, expanding our presence in Tier-3 and Tier-4 markets, while strengthening our network in Metro and Tier-1 cities.

On the system side, we have successfully implemented Microsoft D365, Starlims and the full IT stack in Suburban , creating a unified digital infrastructure across both brands.

I also want to take this opportunity to share my perspective on the overall direction for Dr. Lal PathLabs . India remains vastly unorganized in terms of diagnostic testing. Thus, it is our primary objective to develop the market, expand our reach and enhance our impact across healthcare landscape of India.

We are strengthening our network in Metro and Tier-1 cities, ensuring we remain the trusted partner for diagnostics in places where we are already well regarded. We also continue to develop our presence across Tier-3 and Tier-4 cities, primarily in North and East. We have built a strong brand preference in the North and East, but we continue to work on building brand reach and strength in the West and South. Our vision also includes a relentless focus on expanding our test portfolio. We are strategically growing our capabilities in specialized verticals such as genomics, reproductive diagnostics, autoimmune disorders and other advanced tests. In line with this, we recently launched an advanced Amyloid Typing Test. We are proud to be the first diagnostic chain in South Asia to offer this highly specialized test under the guidance of the National Amyloidosis Centre, London , UK.

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Classification: Restricted Our bundle d test program, SwasthFit , is a potent instrument to gain scale while providing value to patients. We are extending the bundle d test philosophy into the illness segment, ensuring we provide comprehensive and cost -effective diagnostic solutions across the healthcare spectrum. As we progress strongly into the hinterland, here too we are strengthening SwasthFit .

Integral to our future success is digital transformation. We are leveraging technology across our value chain from enhancing patient experience through user -friendly digital platforms to streamlining our operational efficiencies through sophisticated logistics and data management systems. This digital backbone is a strategic tool for scalability, security and for delivering innovative services.

We believe that this clear strategic roadmap coupled with focused execution will lay a strong foundation for sustainable and scalable growth in the years to come. We are cognizant about the opportunities and challenges ahead and remain dedicated to our mission of providing high quality accessible diagnostics to all.

With that, I now hand over the ca

ll to V ed.

Ved Prakash Goel: Thank you , Shankha . Good evening , everyone , and a warm welcome.

I will now proceed to share the financial performance for the quarter and the full year 2025.

- Revenue for Q4 FY25 came in at Rs . 603 crore compared to Rs . 545 crore in the same quarter last year , reflecting a growth of 10.5%.
- Revenue for the full year stands at Rs . 2,461 crore against Rs . 2,227 crore in FY24, a growth of 10.5%.
- Revenue per patient for Q4 FY25 is Rs. 887 , up by 6.4% , compared to Rs. 833 in Q4 FY24. This is mainly due to improvement in test and geographic mix.
- Test per patient for Q4 FY25 stood at 3.07 compared to 2.91 in Q4 last year , and 2.97 for the full year FY25 against 2.83 in FY24 .
- EBITDA for Q4 FY25 came in at Rs. 169 crore compared to Rs. 145 crore in Q4 FY24 , registering a growth of 16.9 % with the EBITDA margin of 28.1%. For the full year, EBITDA stands at Rs . 696 crore compared to Rs . 609 crore in FY24, registering a growth of 14.2% with a margin of 28.3%.
- PBT for Q4 FY25 came in at Rs. 154 crore compared to Rs. 120 crore in the same period last year, registering a growth of 28.1% with a margin of 25.5%. Full year PBT stand at Rs. 625 crore against Rs . 505 crore in FY24, registering a growth of 23.6% with a margin of 25.4%.
- PAT for Q4 FY25 came in at Rs. 156 crore compared to Rs. 86 crore in Q4

FY24 , registering a growth of 81.4% with a PAT margin of 25.8%. Please note that in Q4 FY25 , we have a one -time deferred tax benefit of Rs. 40.8 crore on account of voluntary liquidation of Suburban Diagnostics . For the full year, PAT stands at Rs. 492 crore against Rs. 362 crore in FY24 , registering a growth of 35.9% with a margin of 20%.

- EPS for Q4 FY25 is Rs . 18.6 compared to Rs . 10.1 in Q4 FY24 . For the full year, EPS stands at Rs. 58.5 compared to Rs. 43 last year , registering a growth of 35.9%. This EPS has a one -time deferred tax benefit as explained earlier. Excluding this one -time benefit, normalized EPS for Q4 FY25 is Rs . 13.6 and for the full year, normalized EPS stand at Rs . 53.5.

- Net cash and equivalents as on 31st March 2025 is Rs. 1,229 crore.

Further, I am pleased to share that the Board of Directors of the Company have approved a final dividend of 60%, that is Rs . 6 per share, taking the total dividend for the year to 240%, that is Rs . 24 per share.

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Classification: Restricted I would also like to update that pursuant to the approved scheme of liquidation of Suburban, the entire business undertaking of Suburban has been transferred to Dr. Lal PathLabs Limited on a going concern basis in Q4 FY25 .

With this, I conclude my opening remarks , and I would now request the moderator to open the forum for question and answer. Thank you.

Moderator: Thank you very much. We will now begin the question -and-answer session . The first question is from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: Yes. Thanks . A couple of questions from my end. So, at a time when most estimates for the diagnostic industry are showing an 8 % - 9% growth, our growth has been 10%. So just wanted to understand, you know, what it will take at our end to grow at faster than the current rate. So, you know, because we have been expanding and derisking from North , we have expanded to East, to West is now almost 14 % - 15% of contribution. I guess with scale, you know, the low cost and the quality assurance and branding should ensure slightly offer better growth than this. This is obviously better than industry, but what will it take for us to grow still higher than the current growth rate? So is it going to be a specialized test? Is it going to be some specific region contribution where we are very low or weaker ? So, some ca rds on that will help.

Shankha Banerjee: Yes, so, the industry growth rate is obviously an estimate. And a s of now, you are

right. It could be anything between 8 % - 10%. I think, you know, this year, the year that has gone by, we had started the year saying that we would like to do slightly better than the previous year without having to take a price increase. And I think we have been able to do that. Effectively, it's a 3% better performance than last year because there is no price increase in FY25 v/s there was a price increase in FY24 . Directionally, I think we are moving in the right direction and some of the initiatives, it is unlikely to be any specific geography because as you have said, there are at least now three geographies which are reasonable sized, which is Delhi NCR, North, East as well as West. So, it is unlikely to be any specific geography driven.

But I think our investments in terms of new infrastructure, the whole expansion of our collection network that we have done a bit of aggressi ve on that in the last 12 - 18 months should start seeing benefits of access that we get to newer markets. And also we are looking at increasing our focus on rather the SwasthFit portfolio, like we said, is something which we are moving into the Tier-3 and Tier-4 towns as well. There is still juice left in that strategy . And that's something which we will continue to push for. And even in our North and East markets, the Tier-3, Tier-4, growth is still continuing. I think a mix of all of these is what we are aiming towards to see how we can maybe grow faster than what we did in FY25 .

Prakash Kapadia: Okay . And anything you want to call out on the contribution of specialized test ? Is that a focus area given our legacy and experience in the industry ? Can that be a big contribution going forward?

Shankha Banerjee: So, you know, again, I think we had mentioned in one of our previous calls, the specialized portfolio is something which each company would be defining differently. But the way we define our specialized portfolio; it is about one -fifth of our portfolio comes from the specialized test. But again, like I said, you know, specialized test is something, yes, we will focus on.

But what we realize is that any given geography when we go, we have to look at the whole range, right from routine , specialized, super specialized, bundle d test, you know, all of that put together is how the ecosystem moves forward and the growth

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Classification: Restricted trajectory builds up. So, it may not be right on my part to call out just one element,

but yes, there is focus on the specialized part of the portfolio as well.

Prakash Kapadia: Okay. Understood. And one question for Ved. Ved, given that Suburban is now part of Dr. Lal on an ongoing basis, so how does it impact the amortization and the balance sheet? Because we were writing part of the acquisition cost through amortizing it through P&L. So, what is left? How does it change the P&L and balance sheet impact?

Ved Prakash Goel: Prakash, there is, you know, on consolidated basis. I don't think there is any change because earlier also in consolidated financials we were impairing intangibles and the same will continue. It is just that it is coming in standalone now instead of consolidated.

Prakash Kapadia: Okay. And would you have the figure of amortization for this year and what would be pending?

Ved Prakash Goel: As I mentioned earlier also about Rs. 50 crore kind of, an amortization which is coming every year on account of intangibles.

Moderator: The next question is from the line of Karthik Chellappa from Indus Capital Advisors.

Please go ahead.

Karthik Chellappa: Thank you very much for the opportunity, sir. Congrats on the quarter. Two questions from my side. First, if we were to look at the sample volume growth, could you give us some color on how did NCR North and, let's say, the West and East did individually?

Ved Prakash Goel: Karthik, hi, this is

Ved. I think we are not splitting sample patient volume geographically. Right now, we have given the split in terms of revenue. So, maybe volume geographically we have not mentioned here.

Karthik Chellappa: Okay, or let me ask this question qualitatively. I think your endeavor was to improve your volume growth in NCR, which is like your biggest market, and that was probably growing lower than your overall volume growth. Are you starting to see traction there and in FY26 can we expect to see NCR volume growth at least better? At least in qualitative terms would you be able to comment on that?

Shankha Banerjee: Yes, I think, you know, what we were saying in maybe one quarter back that we were mostly talking about NCR revenue growth and since we are not taking any price increase, obviously it is driven by sample volume and mix. And we would like to, you know, we have done about a double-digit revenue growth this year for Delhi NCR. And going forward, we would like to maintain that trajectory.

Karthik Chellappa: Okay, great. My second question, is if I were to look at our patient service centers, this year, we have probably added more than what we added in the last 2 years, around ~845 or so, which is also a healthy 14 %-15% growth Y-o-Y. So, is this like a prelude that we can see to a better volume print, given that you have actually ramped up your infrastructure? And I know there is a lead and lag for sure. But just given the investments that you have made in the PSC count, is this a sign that we should start to see better volume growth?

Shankha Banerjee: You know, the whole reason for ramping up the infrastructure numbers is to ensure that we get access to newer micro markets and even newer geographies. And in some of those geographies and depending upon the geography that the access we

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Classification: Restricted are getting, the kind of time horizon on which the infrastructure, the collection network really matures will be different.

It could be anything between 18 -36 months that would take for the collection network or a new collection center and a lab network to really start maturing and contributing significantly to the volume and the revenue numbers. But directionally, yes, you know, ramping up of infrastructure should show better patient volume s and revenue in the future.

Moderator: We will move to the next question, which is from the line of Anshul Agrawal from Emkay. Please go ahead.

Anshul Agrawal: Hi, thank you for the opportunity. My question is on network expansion. Are we looking to aggressively add labs in the next 1 or 2 years or do we think that the current lab network should suffice our growth plans in this period?

Shankha Banerjee: We have added 18 new testing labs last financial year. And for this financial year and maybe one more, we definitely would like to keep up the tempo at this pace. We have been adding actually close to about 14 -15 labs pre -COVID and post -COVID there was a period when the lab addition was slightly slower. We need to maybe catch up in these few years and after that we will come back to our normal lab addition trajectory.

There is still a lot of, you know, geography space which is available for testing infrastructure. Therefore, you know, that lab addition will continue. And not only that, even existing infrastructure of labs, you know, there are improvements and

upgradation of instrumentation, etc., that we keep doing. And that is also a part of the normal annual operating cycle.

Anshul Agrawal: Just a follow up on this, sir. Would we see an increase in our PSCs as well commensurate to what we have seen in the current year? The reason why I ask is this, our lab utilizations surely seem to be going up. And would that continue in your view?

Shankha Banerjee: Yes, collection centers, so lab and collection centers or PSCs as we call them, go hand in hand. It is the

whole hub and spoke and the whole network approach that we have in every geography. It is not either or. It will happen simultaneously. And as I was mentioning to a previous question that it is that ecosystem of lab and collection center network, which once added, takes a bit of time to really mature and build up. Yes, you will see addition of collection center, PSC network as well.

Anshul Agrawal: Sir, my second question is on the West region. Assessing by the split that you have given, geography-wise, our West region portfolio seems to have just grown about 5% in FY25. Could you give us an update on how Suburban has panned out in the last quarter?

Shankha Banerjee: We did the whole IT stack changeover in Suburban in Q4 and also the voluntary liquidation process was carried out in Q4. So, because of that, there has been a bit of an impact in terms of the revenue for Suburban in Q4. And we have already started recovering and we believe that we should be able to recover quickly in the next 1-2 quarters.

Anshul Agrawal: Okay, so it's been mid-single digits. Would that be a right understanding?

Shankha Banerjee: Yes, it will be a mid-single digit.

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Classification: Restricted Anshul Agrawal: Great. I will fall back in queue. I have more questions. Thank you.

Moderator: Thank you. The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Thank you. The first question on FY26. What are we sharing in terms of our revenue guidance? Maybe I can start there.

Shankha Banerjee: Yes, Shyam, so this year we have ended at 10.5% in terms of revenue growth. So, next year I think we should be better than that, maybe between 11% - 12%.

Shyam Srinivasan: Okay. And just another point here, as you mentioned, not taken price increase couple of times in the FY25. Is there a contemplation of that in the near term or in the medium term?

Shankha Banerjee: Price increase is something which for us will be a very strategic decision. I think in some previous call also we had discussed that if there is, you know, if we see the need, whether it is in terms of costs or whether it is in terms of the whole pricing table in the industry moving in a certain direction where we are left behind, if any of those things happen, then we may consider, but it is not something which we will do for any tactical reason right now, because maintaining the price table also has its advantages in terms of overall growth that we are trying to achieve.

Shyam Srinivasan: Got it. Just the 10.5% going to 11%, let's assume. Are we expecting improvement in volume growth? Or is it still the SwasthFit contribution that keeps going up 26-28-30%? I am just making it up. But what is driving higher growth?

Shankha Banerjee: I think it will be both. You see, the RPP growth that we did this year was about 6%, right, and patient volume and then sample volume at 9.5%. The uptick, we definitely are trying to do the incremental growth over last year driven primarily through volume and try to maintain the RPP growth.

Shyam Srinivasan: Got it. Last question, just again on guidance on margins. We have reached 28.3%, so very healthy numbers. But I just want to understand how we should look at margin for FY26. Thank you.

Ved Prakash Goel: Shyam, margins, I think, we have close at 28.3%. Going forward, I think, since we have to make a lot of investment in newer geography, newer talent and some digital initiatives and so on and so forth, so around 27% kind of margins we are looking or we are confident to maintain.

Shyam Srinivasan: Ved, 100 bps lower, you are saying.

Ved Prakash Goel: Yes.

Shyam Srinivasan: Okay, and the Rs. 1,229 crore of cash, any thought process on how that will be likely allocated, Ved? Thank you.

Ved Prakash Goel: Again as a stated strategy, we are keeping this cash with us for any op

portunity

which will emerge and as earlier mentioned, I mean, obviously we are growing organically and we have started our own efforts in South, particularly South and West, but if there is any opportunity there, definitely we will be keen to do that

opportunity. And that's why we are keeping this cash. And in addition, obviously, we are paying dividends as well.

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Classification: Restricted Moderator: Thank you. The next question is from the line of Bharat Celly from Equirus Securities. Please go ahead.

Bharat Celly: Thanks for the opportunity. We have been actually seeing volume growth of around mid-single digit since last almost like 7-8 quarters. And despite the fact that we are moving to the hinterlands of India, can we see these numbers inching up to the high single digit?

Shankha Banerjee: Like I was answering in one of the previous participants, the infrastructure expansion push that we have taken in the last 12 - 18 months over the new labs and collection networks and incremental areas around that, those will obviously start maturing. It takes anything between 18 - 36 months for these networks to mature. And we believe that they will help us improve our patient volume number. For overall, like we have been saying earlier, for volume, look at sample volume and patient volume both as indicators of volume, not only the patient.

Bharat Celly: When we will start seeing this Hinterland started picking up probably Tier-2, Tier-3 towns, will it be a case where our patient volume growth will look higher, but the realizations will start taking a hit considering the prices there will be relatively lower?

Shankha Banerjee: So, our pricing approach in Tier-3, Tier-4 is not really different compared to whatever are the contiguous Tier-2 or Tier-1 markets. Our pricing is similar, it's not essentially lower pricing on an MRP basis. But the revenue per patient and those angles, we obviously will look at that, how it develops and emerges. As of now, we are not seeing any significant difference between the Tier-3, Tier-4 realizations v/s. what we are seeing in maybe a Tier-2 or a Tier-1 town.

Bharat Celly: And according to you how the pricing for the competitors will be in these markets?

Shankha Banerjee: The competition in these markets are primarily unorganized local labs. I mean, that is the main competition in the Tier-3 and Tier-4 markets. And pricing there is not very different. I mean, it could be slightly lower, but it is not as if it is very different from the kind of pricing we are able to work on in these markets.

Moderator: Thank you. The next question is from the line of Raman Sivi from Sequent Investments. Please go ahead.

Raman Sivi: Hello. I just have two questions. One is, I want to understand the competition in the industry, because from what I am hearing and what I have seen, that there is a competition from pharma players also, like pharma players entering the diagnostic industry. For example, recently Lupin announced its foray into diagnostic sector. Is the competition going to intensify in coming quarters or years?

Shankha Banerjee: You know, on the competition front, I think this industry has always been quite competitive. And there have been phases when newer kind of players have entered the industry. And I think as long as more and more organized players come into the industry, it is good. Because the overall industry benefits from more organized players. It becomes more level playing field. So, from that point of view, this competition with more organized players is not something which is very new and is likely to maybe benefit the industry at an overall level as well.

Ved Prakash Goel: See, the market is so big, this market has space for everyone. I mean, it's not that we are the only player remaining here.

Raman Sivi: Okay. And my second question is with respect to the CAPEX. How much CAPEX

are we planning for FY26?

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Classification: Restricted Ved Prakash Goel: Normally our maintenance CAPEX is about Rs. 60 – Rs. 70 crore a year. And I believe in next year, FY26, similar kind of CAPEX will be there.

Raman Sivi: You are saying the CAPEX will be around Rs. 70 crore and as of now there is no plan of any additional acquisition?

Ved Prakash Goel: No, that is something different. That's why I am saying maintenance CAPEX, which includes normal expansions where we are opening 15 -20 labs every year and some maintaining CAPEX on account of let's suppose investment in IT or software and all that stuff. But if there is any opportunity, obviously that will be different.

Raman Sivi: With respect to this follow-up question on CAPEX itself, how much usually is the CAPEX? If I have to set up a lab or pathology lab or a radiology lab, how much usually is the initial investment to set up a new lab or pathology center?

Ved Prakash Goel: It depends. I mean, we have different kinds of set up, reference lab. Then we have hub labs and then we have set labs. So, smallest is set labs where generally Rs. 80 lakhs to Rs. 1 crore kind of CAPEX is required. And if you go smaller lab, which is like Tier-3, Tier-4, even lesser amount is required. So, that's how the CAPEX is required in these labs. Obviously, we are not planning any reference lab further in

this FY26 , that is not required.

Raman Sivi: Okay. And one last question. So, in FY26 , what is the revenue from pathology and what is the revenue from radiology?

Shankha Banerjee: Our revenue from radiology is actually miniscule. It is very low single digits. I mean, it is all pathology revenue only.

Ved Prakash Goel: Less than 5%.

Raman Sivi: 4% - 5%.

Shankha Banerjee: Less than 5% .

Ved Prakash Goel: Yes, less than 5%.

Moderator: Thank you. The next question is from the line of Divyansh Gupta from Latent Advisors . Please go ahead.

Divyansh Gupta: Hi, a couple of questions. First is on , let's say, weaponization of chemicals by China. They stopped selling of Gadolinium , which is MRI agent, and today they stopped shipping, exporting of rare earth magnets , right. And I understand that a lot of chemicals come from China for the reagent purposes. Two questions in this , how much of it is coming from China as of today? Is there Indian capacity which can service in case China takes some similar steps in this domain? How are we thinking about that?

Dr. Arvind Lal: Yes, this is Dr. Lal here. The answer to your question is that we hardly get anything from China , miniscule it is, you know, would be even less than 1%. So, this actually does not involve us and since we are not in radiology, we would be able to answer that question, what you were asking is that there is some rare earth chemical goes into a dye which is injected into the patient. We wouldn't know that.

Divyansh Gupta: Sorry, I wasn't asking about radiology . I was saying just for the pathology diagnostics , our reagents .

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Classification: Restricted Dr. Arvind Lal: In pathology diagnostics, no reagent in our case comes from China. None.

Divyansh Gupta: Got it. That's good to hear. The second one was an earlier presentation you used to share this Tier-3 revenue footfall numbers. But in the current presentation I am not able to find. Would it be possible to share these numbers?

Shankha Banerjee: Yes, We have had a good growth in our Tier-3 revenues even in this year. We had a healthy, mid -teens kind of growth . So, it is definitely in the similar trajectory as what we would have seen earlier. That trend continues.

Divyansh Gupta: Got it. And just the last question . On a per patient basis in Tier-3 v/s. on Tier-1, what is the difference in realization purely because of mix of pricing difference? Or at an overall level.

Shankha Banerjee: As I was answering the

previous question, we don't have a differential pricing in a Tier-3, Tier-4 town , we have a geography approach in terms of pricing. Markets in that vicinity all have similar pricing. So, there is no differential in terms of pricing. If at all , it would be maybe a bit of number of tests on per prescription or on that basis or a mixed differential , that would be the only difference. It is not anything to do with pricing.

Divyansh Gupta: And so what would be that end result because of the mix change? Like 5% on a per footfall basis, we end up making 5% lower, 10% lower , and purely because of mix change.

Ved Prakash Goel: I think realization is lower than let's suppose Tier-1 in Tier-3, Tier-4, but that split obviously is varied because of many reasons as Shankha mentioned. I don't think we have exact split of that right now.

Moderator: Thank you. The next question is from the line of Amey Chalke from JM Financial. Please go ahead .

Amey Chalke: Yes, thank you for taking my question. Although most of my questions have been answered, I have one question on the expansion. So , this year we have seen something like 18 labs being added in a year and close to 1 ,000 PSC. Is it possible to tell how much would it be in the core areas of like North and how much would it be in all core areas?

Shankha Banerjee: So, we can look at that number and come back to you, but in terms of you asking only labs ?

Ved Prakash Goel: Mostly in our co re geography.

Amey Chalke: I am saying the diagnostic paradigm, the way it is done largely, it is In Vitro Diagnostic at present, the dependence of it. So , going ahead, you see the importance of In vitro diagnostic to go down in place of, let's say, AI -powered image techniques, etc., if a lot of these ventures are using them. So , you expect the In Vitro Diagnostic testing as an importance to go down going ahead.

Dr. Arvind Lal: No, we don't think so. I think In Vitro Diagnostics will continue the way they are continuing.

Shankha Banerjee: You know, our view is, there is, if you look at imaging and pathology, firstly, there is a lot of AI deployment that is happening on the imaging side. In pathology, there is still AI work happening. And either ways, a lot of it is going to assist the current evaluation and reporting and maybe help enhance that rather than maybe replace it .

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Classification: Restricted Moderator: Thank you. The next question is from the line of Deven Kulkarni from Marcellus Investment Managers. Please go ahead.

Deven Kulkarni : Hi, Shankha . Hi, V ed. So , this quarter, I see that our fees to collection centers has grown by just 3 % odd YoY, which is like far better than our revenue growth. So , can you help me understand what's happened here? Have we cut channel margins or is it some mix change?

Ved Prakash Goel: There is no change in the margins. Deven, it's purely on the mix.

Deven Kulkarni : Okay, so then how to interpret that? Sales from collection centers has grown by just 3% odd Y-o-Y in Q4. And the rest of it has come from our own labs or direct channel, right?

Shankha Banerjee: No, that would not be the right way to read it. The sales from collection center has also grown in double digits. It is also, like he said, there is a test and a geography mix. You know, that is what will be impacting. But there is no change in terms of the revenue share that we are giving to the channel partners.

Deven Kulkarni : Okay, understood. And secondly, Ved, what was the EBITDA margin in Suburban in this quarter?

Ved Prakash Goel: So, as I mentioned this has been merged in the parent company in this quarter. Now we are looking overall margins, maybe it is the company margin. But as earlier mentioned, I mean couple of quarters back, this was having 15 %-16% kind of EBITDA margins. But really now after m erging with this, it's only one which is coming and not separately.

Mo

derator: Thank you. The next question is from the line of Sumit Gupta from Centrum B roking. Please go ahead.

Sumit Gupta: Hi, thank you for the opportunity. It's a follow -up to the previous participant's question. So, just to understand on the competitive scenario in the Mumbai market for Suburban and what are the other competitors , like how do you see this competition? For overall market, how is it shaping up?

Shankha Banerjee: If you see Mumbai market, you know, we are not the leaders. So, there is existing players who are the market leader there and number two strong player. For us obviously, since we are a challenger brand in that market, for us that competition has always been there and it remains similar.

Sumit Gupta: Okay, so how do you see Suburban pairing over the next 3 - 4 years? What is the plan for Suburban ?

Shankha Banerjee: The plan for Suburban is focused on these three geographic places, Mumbai, Pune and Goa. And we continue to work on those markets. These are large, you know, Mumbai as well as Pune have become large diagnostic hubs in their own right. And we believe both these markets have a huge headroom available for us to play both the brands and therefore we will continue with our two brand , dual brand strategy in these markets . And that's how we want to really move to becoming one of the top 2 brands in both of the geographies .

Moderator: Thank you. The next question is from the line of Harshal Patil from Mirae Asset Capital . Please go ahead.

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Classification: Restricted Harshal Patil: Good evening, and thank you for the opportunity. I just have two questions, probably from an understanding perspective. We did see that for FY26 , we are kind of expecting some 100 bps lower in terms of operating margins. And so I just wanted to also understand from the perspective that we have some, you know, we have kind of expanded our network and lab network and collection networks over the past 2-3 years. The benefits of leverage would definitely also accrue going ahead as these centers ramp up. And simultaneously, we are also expecting 100 bps lower margins. Is it that the initial start -up cost for the recently commissioned labs is higher or are we kind of talking of higher marketing expense?

Shankha Banerjee: You know, that is a very good question. I think the initial aggression in terms of opening of labs collection network is about 12 - 18 months old. And they are still in the maturing phase and even in this financial year, we are saying we want to open 15 - 20 new labs. And I also mentioned, you know, there will be upgradation in existing labs. It is not only about the newer labs, but upgradation in existing lab infrastructure that we will do. And also there is more frontline sales teams to work on in the newer geographies that we are going and intensifying our approach in the markets that we are already there and also advertising costs. Therefore, marketing

spend, it is a mix of all of that. Basically, you know, as that previous thing matures over a 18 - 36-month cycle, we should start maybe seeing those benefits, but then we may have newer areas to invest at that time.

Moderator: Thank you. The next question is from the line of Kunal Lakhani from CLSA. Please go ahead.

Kunal Lakhani: Hi, good evening. Just a follow-up on an earlier remark. You said that the radiology revenue mix is less than 5%. Is that a conscious strategy to keep the concentration lower or it could be a revenue driver for future? What's the thought process there?

Shankha Banerjee: You see currently, we have what we call as basic radiology in some of our core markets which is X-ray, ultrasound, maybe TMT and things like that. High-end radiology which is CT, MRI is something which we haven't really gone into on a big way. You know, as of now there is a lot of market opportunity in the pathology area itself and therefore

, being our core strength, we want to focus more energies in that direction where we have an advantage. But having said that, radiology as an option is always on the table and we continue to evaluate it. And as and when we find that to be something viable, fitting into our growth strategy, we will get into it.

Kunal Lakhani: Sure. I mean, the reason why I was asking that was because we do have plans to expand in the South and some of our Southern peers have a very strong radiology mix, which kind of in a way helps them in terms of branding and sort of brand recall.

You think that would not be an issue for us when it comes to expanding South India?

Shankha Banerjee: You know, in South India currently, our footprint is relatively quite weak and we are looking at both organic as well as inorganic opportunity in South. There could be a possibility that if there are inorganic acquisitions which are coming with the radiology business attached to it, then that becomes a part and parcel of our business also going forward. But organically trying to do radiology in the South is really not on the agenda right now.

Kunal Lakhani: Understood. Thank you so much. My second question was on the margin outlook again. This year you have guided for like 100 bps lower margin. But would it be more transient or you think this 27% margin is something that would go on for like, say, mid-to-long term?

Ved Prakash Goel: I think, Kunal, this is where we are investing and that is where that doesn't mean that operating leverage or efficiency is not coming. Whatever improvement or whatever

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Classification: Restricted increase in the margin, we are investing back into the business. And this is definitely for the FY26 we have narrated. Maybe for a longer term, how we will go, maybe it is difficult to tell at this point of time. But definitely for the next year, we have plans to invest in the business.

Kunal Lakhani: Sure. And lastly, on the pricing bit, right, I mean, we have seen a very prolonged period of new price hikes. And it's for any industry, right, price growth is like a litmus test or metrics to measure the health of the industry per se, right? Where do you think we are or how far away you think we are in terms of like the pricing power coming back to the industry, not just talking about Dr. Lal or any other operator, but for the industry per se, pricing power coming back. When do you think that can happen?

Shankha Banerjee: You know, I don't think this question of taking a price hike or not is something to do with the pricing power. It is more about a conscious choice and a strategic choice of not taking a price increase, given some of the strategies that we have deployed for growth, moving into Tier-3, Tier-4 towns and some of the other ways that we want to look at sample volume growth also being one of the major drivers, looking at how can we improve maybe even patient volume in the future.

It is more of a conscious reason. And since we are able to kind of drive efficiency through operating leverage, a lot of technology that we deploy, we are able to eat a lot of the inflation that is otherwise going to be in the system. In that scenario, it is a conscious choice of not taking a price increase.

But like I mentioned to a question in the past, you know, we obviously will keep evaluating if there is a cost pressure or if there is any other reason or if we see the pricing table in the industry is moving in a certain direction and we are kind of, you know, there is always a linkage in our industry of price and quality. We don't want to be caught on the wrong end of that. If those opportunities present to us, then we may consider a price increase.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments.

Ved Prakash Goel: T

Thank you , everyone , for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your questions. Please feel free to reach out to us in case you have any further queries. Thank you once again.

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August 05, 2025

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001
Scrip Code: 539524

Sub: Transcript of Q1 & FY26 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Earnings Conference Call on Financial Results for Q 1 & FY26.

We request you to please take the same on record.

Thanking You,
Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl. : As above

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Dr Lal PathLabs
Q1 FY26 Earnings Conference Call Transcript
July 31, 2025

Call Duration • 1 hour

Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman

- Mr. Shankha Banerjee - Chief Executive Officer
- Mr. Ved Prakash Goel – Group CFO and CEO, International

Business

Participants who asked

questions • Karthik Chellappa – Indus Capital Advisors Hong Kong Limited

- Anshul Agrawal - Emkay Global
- Prakash Kapadia – Spark PMS
- Surya Patra – Phillip Capital
- Karan Vora – Goldman Sachs
- Pranaya Jain – Banyan Tree Advisors
- Yogesh Soni – InCred Capital
- Aashita Jain – Nuvama Institutional Equities
- Harshal Patil – Mirae Asset Capital Markets (India)

Moderator: Ladies and gentlemen, good day, and welcome to Dr Lal PathLabs ' Q1 FY26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing “*,” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand over the conference over to Mr. Nishid Solanki from CDR India.

Thank you. And over to you , sir.

Nishid Solanki: Thank you. Good afternoon, everyone, and welcome to Dr Lal PathLabs ' Q1 FY26 Earnings Conference Call.

Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal – Executive Chairman , Mr. Shankha Banerjee , CEO and Mr. Ved Prakash Goel , Group CFO and CEO, International Business.

I would like to share out standard disclaimer . Some of the statements made on today's conference call could be forward -looking in nature and the actual results could vary from these forward -looking statements. A detailed statement in this regard is available in the results presentation , which has been circulated to you and also available on exchange website s.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to Share his Perspectives.

Thank you , and over to you, sir.

Dr. Arvind Lal : Thank you very much. Good afternoon, ladies and gentlemen, and welcome to our 1st Quarter Earnings Call . I would first like to highlight the broader opportunity in healthcare and the role that we can play as a leading diagnostic brand.

Healthcare in India continues to grow at a healthy pace, characterized by digital transformation and shift towards tech -driven and holistic healthcare delivery. However, I would like to add that there is more than ample room for growth. When it comes to hospitals alone, it is said that India has a shortfall of 2.4 million beds to meet global standards. This is getting addressed with major hospital chains adding to capacity judiciously by upwards of 30% till FY27. Central Government flagship initiatives like Ayushman Bharat and Heal in India are propping up public and private investment in the healthcare system. One can be assured that this kind of growth will translate into a higher requirement for quality diagnostic services going forward.

On the tech side, we are witnessing increased integration of AI in healthcare services, including diagnostics. It is estimated that by the end of 2025, the Indian artificial intelligent healthcare market would have a size of US\$1.6 billion. The impact of telemedicine and digital health records is being felt given enhanced access in rural and semi -urban regions. The role of digital and tech within our industry is growing, where on the one hand, it appears to be operational efficiency and on the other, it enhances patient and clinical outcomes.

Let us look at the other side. The prevalence of chronic and lifestyle diseases in India continues to expand. We are the topmost country with incidence of diabetes, with 100 million patients afflicted. Rates of hypertension are also

climbing, with some surveys indicating that maybe 30% of adults having elevated blood pressure. Obesity is experiencing a surge in young adults and children, given linkages to higher consumption of processed foods and general inactivity. And these trends are coming up in the rural settings as well as lifestyles undergo change over there. The only drawback for rural populations has been access to regular screening and specialist care. Delayed diagnosis is also an important exacerbating factor here, especially in underserved populations.

Given this background, the role of diagnostics and management of these healthcare outcomes cannot be overstated. As a national brand, we remain committed to providing

quality diagnostic services to the country. Our operations are steadily spreading across hinterland markets, where demand for such services is rising. This is especially so in Tier-2 and beyond. We are building stronger presence in the West and South, to complement presence in the North and East. FY25 saw addition of 18 new labs to our network, and we expect to sustain a similar number in the present year too. Our patient service centers also saw a strong increase to match this, and again we expect this franchising trend to continue.

On the tech side, we are making the right investments to strengthen operations and create scale with flexibility.

Our journey will be marked by growth milestones as we seek to broaden the scope of services and geographic coverage. The model is scalable, and we have the intent to grow it to meet unserved healthcare requirements. With that, I would like to hand over to Shankha to continue. Over to you, Shankha. Shankha Banerjee: Thank you, Dr. Lal, and a very warm welcome to everyone. Let me share some insights into Dr Lal PathLabs performance in the 1st Quarter of Fiscal Year 2026.

As evident, our results are a testament to the enduring strength of our business model and the disciplined execution of our dedicated teams across the nation. The new financial year began on a strong footing, with 11.3% growth in revenues and 24.3% improvement in profit after tax. This was achieved by steady volume momentum and a favorable test mix. Our sample volumes grew by 10.7% to 23.4 million, while patient volumes increased by 5.3% to 7.6 million.

At an industry level, the competitive landscape continues to evolve. We have recently observed the entry of a new e-commerce player into the diagnostics arena. This will add to the existing online competitors that have been in the industry for nearly a decade. The foray from hospitals and pharma companies into the diagnostics sector continues to help in speeding up the unorganized to organized shift for the industry.

Strategically, we are expanding our capabilities in high-complexity testing. We have launched 58 new tests in the quarter, strengthening the genomics portfolio and introduced component-resolved diagnostics for allergy testing. We are also supporting antimicrobial stewardship with the launch of an in-house smart culture reporting algorithm for the right antibiotic recommendations for all types of culture reporting. We see a very strong future potential in high-end and specialized testing.

We are also seeing positive trends in bundle testing under "Swast hFit," which continues its strong growth trajectory. The bundle testing offer is being strengthened on the illness segment front as well. Further, we have added new

offerings in genomics, reproductive health, and autoimmune disorders to bring sharper focus to these areas.

On the operational front, our network continues to expand in line with our cluster-based strategy. We are strengthening our leadership in core urban markets in North and East, including Delhi NCR. We are also deepening our presence further in Tier-3 and 4 towns.

We are maintaining our calibrated pricing strategy and continue to hold our prices. The gains in realization are driven by premiumization of our offerings and driving a favorable test and geography mix.

On the digital front, we are investing more in automation and digital systems to improve patient experience, enhance cyber security, and drive operational efficiency.

Our outlook is defined by three strategic pillars: driving volume-led growth through aggressive market expansion, achieving operational excellence via comprehensive digital transformation, and steadfastly upholding our brand leadership through unwavering quality and widespread accessibility.

With that, I will now hand over the call to Ved.

Ved Prakash Goel: Thank you, Shankha. Good afternoon, everyone, and a very warm welcome once again.

Let me now walk you through the financial performance for the 1st Quarter of FY26:

- Revenue for Q1 FY26 stood at Rs. 670 crore compared to Rs. 602 crore in the same quarter last year, reflecting a strong growth of 11.3%.
- Revenue per patient rose to Rs. 880, up 5.7% from Rs. 833 in Q1 last year, driven by a favorable change in the test mix. Tests per patient increased to 3.07 compared to 2.92 in the same period last year, highlighting continued traction in bundled and preventive test adoption.
- Our Swast hFit portfolio contributed 27% of revenue this quarter, up from 25% in the same period last year, underscoring its growing relevance in preventive healthcare.
- EBITDA for the quarter came in at Rs. 192 crore v/s Rs. 170 crore in Q1 FY25, a growth of 13.1% with a stable and healthy EBITDA

margins of 28.7%.

- Profit before tax rose to Rs. 181 crore from Rs. 150 crore, up 20.8% with a PBT margin of 27%.
- Profit after tax stood at Rs. 134 crore compared to Rs. 108 crore in Q1 FY25, delivering a robust 24.3% growth and maintaining a PAT margin of 20%.
- Earnings per share for Q1 FY26 came in at Rs. 15.9, up 24.4% from Rs. 12.8 in the corresponding quarter last year.
- Our balance sheet continues to be strong and resilient, with net cash and equivalents of Rs. 1,389 crore as of June 30, 2025.

In recognition of this performance and to reward our shareholders, I am happy to share that the Board of Directors has approved an interim dividend of 60% i.e., Rs. 6 per share.

These results reaffirm the strength of our business model, the effectiveness of our execution and our disciplined financial stewardship. We are making strong progress across all strategic fronts, expanding in both core and emerging
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markets, scaling up our high-end and specialized test portfolio and accelerating our digital transformation journey. We remain confident in our ability to deliver sustainable, profitable growth while staying true to our purpose of delivering trusted diagnostics with care and precision.

With this, I conclude my opening remarks, and I would now request the moderator to open the forum for question-and-answer. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session.

The first question is from the line of Karthik Chellappa from Indus Capital Advisors Limited. Please go ahead.

Karthik Chellappa: Yes, thank you very much for the opportunity, and congrats on the quarter. I have three questions. The first one is on the volume growth for the quarter, which is very healthy. The commentary that we heard from a lot of consumer companies is there were a lot of unseasonal rains and erratic weather this quarter. Did our volume growth benefit from any of these unseasonal trends, or is this like a very clean kind of unseasonal volume growth that we saw this quarter? That is my first question.

Shankha Banerjee: Yes, hi, Karthik. Thanks for your question. As of now, in the 1st Quarter numbers that are there, there is not too much of a favorable impact because of the unseasonal rain. It is more or less like-for-like compared to Q1 last year.

Karthik Chellappa: Excellent. The second question is, if I were to look at our realizations this quarter, they have not grown much, probably even less than one percentage or so. But despite that, we have seen a very strong gross margin expansion both YoY and QoQ. Is there anything specific on the cost front that we have done this quarter? And is this kind of improvement something that we can sustain, or are we hitting a normalized level of gross margin where we like to sustain it at an 80%-81% level?

Ved Prakash Goel: Hi, Karthik. I think realization has gone up from Rs. 833 to Rs. 880, which is not 1%. I mean, that is one. But there is no price increase as such we have taken in this. This is all due to test mix, or maybe hi

gh end test contribution

which has contributed to this realization. Margin specifically has increased, and we got the benefit because of this SwasthF it which has contributed 27% this time, which is highest ever contribution in this quarter. So, that is the reason we are getting some benefit out of this.

Karthik Chellappa: Excellent. Okay. Because the 1% I was referring to was the realization per test. But it is okay. I think I got the message or so. My last question, in last quarter, we indicated in our outlook for FY26 that we may see up to, let's say, 100 basis points margin compression because we are reinvesting into our brand as well as lab infrastructure, etc. This quarter, margins have actually been quite strong. So, should we see the impact of this margin compression on a lag basis, let's say, in the remaining quarters or was this quarter it just surprised us more than what we had anticipated?

Ved Prakash Goel: You are right. If you see first half, we generally have higher margins, because most of the investments we are getting into second half. I do not think these 28.7% margins are representative of the full year. Yes, we have improved, but let us see how the remaining quarters goes. But most of the investment will be in later quarter of the year.

Shankha Banerjee: But Karthik, just to add to that, I think there is internal view, although we are recalibrating that the overall annual margin also could be slightly better than

what we had projected at the beginning of the year.

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Moderator: The next question is from the line of Anshul Agrawal from Emkay . Please go ahead.

Anshul Agrawal: Hi, thank you for the opportunity. My first question is on volume growth. Could you provide some color, is this growth secular geographically? Is it some particular region which has led to this strong patient volume growth or sample growth?

Shankha Banerjee: If you look at the overall number, revenue at 11.3 % and let us say sample growth at 10.7 %, which is really very closely hugging the overall revenue. I think we are seeing similar trend of the sample growth being very close to our overall revenue growth across all our major geographies.

Anshul Agrawal: Okay. Core Delhi region is still growing at double -digits and same for West ?

Shankha Banerjee: Yes, Delhi has grown at double -digits for this quarter as well. West, like we said that post our changeover in Suburban of the whole IT stack, there has been a bit of a discontinuous impact in the market, which we had mentioned would take maybe 2 quarters to recover. We are still in that recovery phase. It is better than previous quarter, but I would not say we have still reached the level that we were 2 quarters back.

Anshul Agrawal: Got it. If at all that comes in after two quarters, we still have that lever to improve our volume trajectory.

Shankha Banerjee: Yes, that is what it would seem that we still have some headroom there to go back to our previous numbers there.

Anshul Agrawal: Great. Second question, is on the current quarter, which is so basically unseasonal monsoon is the question on that. Q2, we should see the beneficial impact of that . Would again that be correct?

Shankha Banerjee: You see Q2 has traditionally been the high quarter specifically driven by fever and seasonality. So, like if we are saying this year, the similar thing was there last year. I do not think there is an expectation of this year high being differential from others, because there is still quite a lot of the quarter still remaining to play out. It has been a seasonal high always, and we expect a seasonal high this year as well.

Moderator: The next question is from the line of Prakash Kapadia from Spark PMS . Please go ahead.

Prakash Kapadia: Yes. Thanks for the opportunity. A couple of questions from my end. In the last, I think, 3 odd years, we have launched the Bangalore Reference La

b as well

as the Mumbai Reference Lab. Historically, we have seen the sales growth is much higher in these markets when the reference lab is launched, be it east, be it north. Towards the latter half of the year, would we expect west and specifically south Bangalore side to have revenue growth faster than the company average growth, because that is what I think the trend shows and all the efforts which we typically put once the reference lab is there, they generally fructify and in addition to that, from a revenue side, also, I think last call you had alluded, Shankha, the patient service centers we have invested. So, if I see that last 2-3 years, they are up by 40 %, pickup points are up by, I think, 17%, 18%. When does the step up in revenue happen for us? Obviously, we are growing better than industry, but still the step up in revenue say 15 % revenue growth, when do some of these things fructify as we move forward?

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Shankha Banerjee: Right. Thanks, Prakash. Very interesting questions. I think firstly on the reference lab question that you asked. The reference lab setup is like, basically, we have a test menu, which is significantly larger than, let us say, a hub lab or a cluster lab. I mean, that is how the reference lab setup is and the higher test menu is typically a lot of not so frequently tested, but it is something we outsource from quite a lot of clients. Now, if you look at how this whole thing works is that, firstly, clients in that same geography who we were maybe sending the samples to Delhi or somewhere else, now start getting reports better and faster from the local lab or the local reference lab that we have, and then the conversion of the other clients for those high end tests will happen. It is a slightly long-term thing compared to a satellite lab. So, this is a slightly longer term horizon thing and it is a bit of a slow burn. So, to kind of directly link it only to opening a reference lab may not be appropriate because some of the other capacity of routine tests which get built in a brand strong market, the ability to leverage that is more quicker than in markets where brand strength is still under development. I would say that the reference lab impact

will be a slightly more longer-term one.

Coming back to your second question, which is on infrastructure and the front end, whether it is the collection network or the pickups that we are opening, now, as you would have also noted in our previous commentary, quite a bit of the infrastructure gets opened up in Tier-III, Tier-IV towns and some of the throughput per infrastructure may not be as high as the throughput per infrastructure in the larger urban towns. Just the number increase getting converted to a direct, same kind of a number increase on revenue side, may not always be appropriate. But, yes, there is also a buildup in terms of revenue that happens over a period of time. But it is a constant process. Although we have stepped up some of these additions, there has always been addition happening every year at certain level and that is a rolling benefit. It is unlikely to show you a serious step jump going forward. But, yes, it will all contribute because finally the whole network, the hub-and-spoke model of the collection network and the lab and the test menu in that lab is what will help us grow the revenues in the future.

Prakash Kapadia: And this, Shankha, you said would happen over a period of time. So, that would take more time in the Tier-III, Tier-IV markets where we are focusing since the last few years because there I think penetration, habits, awareness would take time. What would be that inflection point, which we have to monitor to see we have got there in terms of the faster revenue growth, which seems to be missing as of now? Obviously, it is better than what others or industry players are doing. But how does that step up happen

? Is it habit change? Is it awareness? Is it per capita?

Shankha Banerjee: Infrastructure is definitely one big contributory factor in terms of step up of revenue growth, but that is not the only factor which will determine. There are other things that need to be executed. Like I said, in terms of our test menu expansion, in terms of speed to market, in terms of the brand salience getting developed. There are a lot of other factors to play but yes, infrastructure is a large contributory factor. Again, directly linking to say that, in terms of numbers, 40% infrastructure means a 40% increase in revenue may not be the right correlation to build.

Prakash Kapadia: Sure. Understood. One question is for Ved. The amortization for Suburban would continue at Rs. 60 crore run rate and this should be over in the next 4 years, right, is that understanding, correct?

Ved Prakash Goel: Total is Rs. 50 crore, Prakash, for the year, Rs.12.5 crore per quarter, which will continue for some time.

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Moderator: The next question is from the line of Surya Patra from Philip Capital. Please go ahead.

Surya Patra: Yes, thank you for this opportunity, and congratulations for the great set of numbers. My first question is on the number of tests per patient. If I see there is a kind of a very steady secular rise that we are witnessing and possibly that is helping you deliver growth without taking even any price hike, although the industry in following. Although there is kind of a consistent rise that we are seeing in the number of sample per patient or the test per patient, is there any benchmark that one can think, see, the number could be, let us say or based on your any matured sub-segment or sub-market, the number of a test per patient could go as high as 4 or 5 or what number that one can kind of target and achieve and over what time period, if you can give some sense on that?

Shankha Banerjee: I think that is a very interesting question. And, at the outset, let me say we do not have a target that we are following on a test per patient basis. But one thing that we definitely see is that there are a lot of factors which are contributing to this test per patient increase, and some of those factors definitely in the near-term would continue.

First and foremost is this whole SwasthFit acceptability and growth that we see, and I think I mentioned that sometime before as well, we see traction in terms of more people wanting to use it not only in urban areas, but we are also seeing some of our smaller towns, the acceptability is rising, we are also seeing some traction coming from the prescription channel. So, that is one driver for test per patient. And obviously, there is another driver, which is in terms of prescribing habits of clinicians where there could be, more tests being prescribed on a single prescription now than previous. Yes, there are certain factors which are contributing, we are not running after a target on this one. But you are right. This is definitely contributing to our growth and obviously

helping us to maintain our price stance the way we have taken.

Surya P atra: Sure . My second point was, let us say, what is the growth over last year that we have seen in terms of the number of test panels and whether it is fair to believe that with the rising trend of the test panels that we offer, that will lead to kind of a faster growth in either the Swast hFit growth or the overall growth for us ?

Shankha Banerjee: I assume by panels you are implying Swast hFit. So, Swast hFit portfolio Q1 this year has grown about 22 % over the same quarter last year and obviously that is ahead of our overall revenue growth and therefore contributes positively to our overall revenue growth as well as to our profitability mix.

Surya P atra: But it is not the number of panels that also contribute to the growth of Swast hFit portfolio

o ?

Shankha Banerjee: I am not too sure what you mean by number of panels. We have got some set predefined Swast hFit bundles. There are 1-2 new that we add bundles, but these are all predefined bundles.

Surya P atra: Okay. Currently what 385 test panels that we are having , that number would not be very frequently changing , is that understanding right ?

Shankha Banerjee: Yes, that understanding is right.

Surya P atra: Okay. Then my last point was, do you believe likely a significant boom in the usage of the GLP -1 medication , is likely to have a kind of a big potential boost

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to the overall test volume numbers for us because of our pan India presence that we are having?

Shankha Banerjee: No, I think that is a very interesting question. We have not really maybe figured out the connection between GLP -1 usage and diagnostic testing or direct connection to that. But let us as a team maybe we will look into this slightly more deeply and try and see if there is a direct connection that could be there between this and what impact it could possibly be.

Moderator: The next question is from the line of Karan Vora from Goldman Sachs. Please go ahead.

Karan Vora: Yes, thank you for taking my question. My first question is with respect to Suburban. While we have been one of the best -run diagnostic companies, for a long time now. Like for Suburban, we have not been able to work it out like probably the way we would have wanted when we had acquired the asset. Any color if you could provide on what are the things where we have struggled or what went wrong and what were the learnings from it , would help ? And also a sub-part to that is this IT restack, which we are doing on Suburban , any particular reason we are doing it right now v/s. say do it when we acquired or immediately after we acquired it , that is my first question.

Shankha Banerjee: I think the Suburban question has been asked quite a few times in the past and I think on the investor call, we have tried to answer that quite a few times. But if you reflect back and see what are the challenges, I think we said that there was one big change that we had wanted to make with the collection network, which was there , when the COVID test went down, a lot of our existing collection network within Suburban actually started becoming unviable and closing the franchise part and then, we also wanted to change the whole structure from an owned collection network to a franchise collection network model. So, that changeover, I think, took longer, specifically because of the slowdown of the existing franchise network, which happened post the COVID reduction. I think that was obviously one core reason.

I think the other thing is that , starting itself we said that there are quite a few parts of the business that we felt were non -strategic and non -core, which we started de -emphasizing and that obviously went on a slow decline. And those kind of non -core geographies as well as non -core business lines , on the top line side, one would not see the benefit , although internally, there would be segments that we wanted to grow, were starting to see those results. Coming back to the point on the IT stack changeover , we had done quite a lot of work , we have to prepare the teams. The operations part had to be aligned to a certain level before the IT stack changeover could be done. I think the acceptability within the system, the team, as well as the preparation that would have gone into it and we felt that now is the right time for us to really start looking at more backend synergies. And that is why the timing was chosen to do the IT stack changeover to align fully to the Dr. Lal PathLabs IT stack, which we did in February this year.

Karan Vora: Got it. And is this the last step or, post which all the measures like conversion

of own centers to franchisee centers and all the other things

are done and we

should start seeing recovery , would that be a fair understanding , like maybe after a quarter or two?

Shankha Banerjee: I would not say all is done because there are still certain geographies there, we have actually now even transitioned the business to Dr. Lal PathLabs system. There will still be some transition which will continue because, we do not want to do everything in one shot , it really does not work in an ongoing business , we have to obviously handle it carefully.

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Karan Vora: Got it. And my second question is with respect to the inorganic opportunities and the cash allocation, which we have in the balance sheet. So, like while we have always mentioned that we want to explore for assets in the South and in untapped geographies for inorganic acquisition. But just to get a sense on, what are the challenges when you go and approach the labs? First of all, are there enough labs for you to target like potential targets? And then if they are, then what are the issues you are facing other than the valuation part, which might always probably be expensive in India? Any color on that would be helpful.

Ved Prakash Goel: Yes, Karan . I think, we already stated that inorganic is the idea and especially in South. Right now, there is nothing which is we can share right now. But the exercise is on in terms of what should be the value , what is the strategy and all. I think the foremost thing we want a platform where we can ride and, make that platform big. We do not want to just acquire for the sake of adding turnover. But wherever we feel there is a strategic fit into our whole long -term strategy, I think that is where we require. But yes, there are opportunities . At the right time maybe it will come.

Moderator: Thank you. The next question is from the line of Pranaya Jain from Banyan Tree Advisors. Please go ahead.

Pranaya Jain: Hi, thank you for taking my question. Actually, I wanted to understand our gross margins outlook. When we look at gross margins over the last few years, they have been constantly expanding. As we scale up further, should not gross margins go up even beyond, say, 82 %-83% over a 3-4 year horizon?

Ved Prakash Goel: I think your observation is right and the gross margin is increasing. Obviously , a couple of things contributing. One is, I alluded on my opening remarks because of contribution of Swa sthFit where the test mix is very different. Second is scale, obviously and third, geography mix also. I can not comment on where it can go but our efforts is always to create efficiency where we can still get some benefits, whether it is volume, whether it is test mix and all. But right now it is hovering around 79 %-80%. And I would say near -term, I do not think it will go substantially, from here. So, that is where you should consider that it should be around at the current levels.

Pranaya Jain: Got it. And my second question is in our non -core geographies, what are some of the strategic initiatives that we have taken to fasten organic growth?

Shankha Banerjee: Non-core geographies, obviously , there is a slightly slower turnaround in terms of building the organic business. And it will follow a quite simple mechanism, obviously building the testing infrastructure , collection network and then put feet on the ground, who can then meet up with the prescriber community and be able to promote our services and how we can give good quality services compared to maybe a lot of other local labs and maybe some of the smaller regional labs that may be operating in that geography.

Moderator: Thank you. The next question is from the line of Yogesh Soni from In Cred Capital . Please go ahead.

Yogesh Soni : Yes. Thanks for the opportunity. My first question is on the Delhi NCR region. We have seen consistently double -digit growth for the last couple of quarters. I want

to understand, I mean, what has changed in the said region - whether the market competitiveness has changed or whether we have made certain efforts that is resulting in double -digit growth?

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Shankha Banerjee: Yes, I think firstly, there is a slightly benign market competitiveness in terms of the online players and pricing , that is a contributing factor. But over and above that, there are quite a lot of service enhancement initiatives that we have taken in the market, whether it is on logistics, test menu, people on ground. A lot of those and even looking at our channel partners and kind of motivating the

channel partners performance, etc. So, a lot of those initiatives have gone into play, which is resulting in what we are seeing as the consistent double-digit growth in NCR.

Yogesh Soni : Okay. That helps me to understand. Another question is on the genomics. I just wanted to discuss about the genomics market, what kind of market size is it now and the kind of growth rate that we are seeing in the market ? Also, I mean, if you could let us know what is the overall contribution of genomics in our revenues and how we can see genomics becoming affordable often in the coming time ?

Shankha Banerjee: Like our overall industry, I think, having a sub-sector market sizing is something which is not available at an industry level. I may not be able to comment on the market size currently but one thing which is sure is that genomics as a segment is definitely, getting more acceptability amongst the clinicians and is also seen as a way to kind of pursue or go ahead on the whole personalized care and personalized medicine, which is evolving, globally and also in India. Also, I think the whole cancer or the onco area that is getting more focus in our country, also uses genomic testing. We believe that in the future, the requirement for genomic testing is going to grow from wherever it is today, and we definitely would like to get a sizable share of that growth going forward and we are therefore investing in that basis. As of now, I do not think I am in a position to share, sectoral breakup of what percentage of our business is genomic.

Yogesh Soni : Okay, that helps. Just to get a further clarity, if you will be able to provide on the affordability part, I mean, how will that be achieved?

Shankha Banerjee: I think affordability of genomic testing is improving day-by-day. It is a combination of volume and as well as the type of testing equipment that one is going to use, and technology improvement that we are seeing from time-to-time. So, affordability and the testing costs are getting competitive. So, I think if that trend continues the way it is, the user acceptability will grow even further.

Moderator: The next question is from the line of Aashita Jain from Nuvama Institutional Equities. Please go ahead.

Aashita Jain: Good day, everyone. My first question is on your West India strategy. I understand you are in the last leg of integration, but with the dual branch that we have now with Suburban and Dr. Lal, how should we see your West India region performing going forward? Could you give us more color around your organic expansion, maybe focus cities, or how are you dividing the local areas, and also the white spaces that you are seeing in the West India region? That is my first question.

Shankha Banerjee: Right. West India for us is, Maharashtra, MPCG, Gujarat and Goa, that is what West India is. And amongst this whole wide geographic array, the dual branch strategy is operational in Mumbai, Pune, and Goa primarily. The rest of the parts of West India is still fully Dr. Lal PathLabs organic and a few inorganic acquisitions that we have done in the past. In terms of our strategy, the core medical hubs are Mumbai and Pune and that is the area that we really want to build a strong base in. I mean, the other parts of

West India in MPCG, MP and

Chhattisgarh, there is quite a lot of organic Dr. Lal PathLabs presence and quite a lot of those markets we are the number one brand and we continue to

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expand our footprint as well as our business there, whereas in markets like Maharashtra and Gujarat, we are not the leaders and could be quite far behind some of the incumbent brands there. Those are the geographies that we are trying to build for the long term and working on that to see how we can now grow these two brands organically in Mumbai and Pune.

Aashita Jain: Is it fair to assume that the majority of the expansion would be franchisee collection centers going forward rather than the lab addition in these markets or how should we see it from say, 3-5 years down the line?

Shankha Banerjee: In a 3-5 year scenario, definitely we will have lab additions as well. So, it is not that lab additions are not going to happen. But, since we are first focusing on a very compact geography of Mumbai, Pune, I think the lab infrastructure for Mumbai, Pune is already established. In these markets, it could translate into more of a collection network expansion. But when you look at West as a whole, which includes other parts of Maharashtra, Gujarat, there is also a possibility of adding lab infrastructure which we continue to do on a steady basis.

Aashita Jain: Understood. That is helpful. My second question is on the radiology side. I think in the last earnings call, you made a remark that radiology is also on the table.

How should we think radiology as a growth lever for you? And also, one of your competitors have also started ECG at home and they are doing a couple of other basic radiology as well as advanced. How should we see this for Dr Lal PathLabs going forward?

Shankha Banerjee: Radiology has a gamut of tests, there is something called basic radiology which would be X-ray, ECG, Ultrasound, TMT tests, etc., and then there is high-end radiology which would be CT, MRI, maybe even let us say a PET / CT scan, scanners, and things like that. Basic radiology, we do in some of our urban markets we do basic radiology, although it is still a very low contributor. But we were running some pilots which we continue to do on high-end radiology and we are still working on a plan as to how we will expand into high-end radiology, which is CT, MRI. That business plan is still not fully frozen. But yes, that is an opportunity that we are aware. And at the appropriate time, we will look at that business model as well as a part of our growth strategy.

Aashita Jain: Sure, that is helpful. And last couple of quick bookkeeping questions. What was the growth in the Suburban this quarter as well as the margin for this quarter?

Ved Prakash Goel: Ashita, as we have merged now, separately it is not Suburban but west as a whole which we have already answered, Shankha has given that. Due to this IT stack change now maybe going forward it will come back what we used to do 2 quarters earlier. But right now, this quarter also is impacted because of this changeover. And obviously margins also is now whole as a Company, not separately. But this quarter and the last quarter was impacted due to this IT stack change.

Aashita Jain: Okay. And this quarter your tax rate is 26%. Is there been any change or 26% is something we should build in for the full year and the coming years as well?

Ved Prakash Goel: Yes. 25%-26% is the normal tax rate. If you are comparing with the last year if you see there was some deferred tax reversal because of Suburban and that was the one-time impact, we have explained last time. But going forward, I think this is the standard rate, 25%-26%.

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Aashita Jain: Understood. And I think lastly on the guidance you said you are expecting better than the earlier guidance of 27% EBITDA margin. But are you also revising up your growth

guidance revenue of 11%-12%?

Shankha Banerjee: No. We will stay with our 11%-12% revenue growth projection.

Moderator: Thank you. The next question is from the line of Harshal Patil from Mirae Asset Capital Markets (India). Please go ahead.

Harshal Patil: Thank you for the opportunity. Most of the questions are answered. Just need one understanding from you. You alluded to quite a lot of reasons for a very good volume growth that we have achieved. I just also wanted to kind of have your qualitative views on, we are seeing a good volume growth, also the competitive intensity as you kind of rightly said is benign. In this whole thing, how do you see the mix from unorganized to organized moving around across markets? Maybe has that picked up or probably how should we see that going ahead just in the light of the volume growth that we have got?

Shankha Banerjee: The caveat always is that we do not have a very clear industry level number. All that we are saying is obviously some extrapolation from what we observe from the other listed players and what kind of results they are declaring and some of the maybe quasi-listed players and what kind of results they are declaring. I think at an overall level, one can see that the quasi-listed space definitely seeing a slightly better revenue volume growth, and to that extent one can imagine that maybe the shift towards organized is getting slightly more accelerated, but these are all directional statements, I think the actual percentages or rate of change, contributions, etc., are very difficult to pinpoint.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand over the conference to management for closing comments.

Ved Prakash Goel: Thank you, everyone for joining us today. We truly appreciate your continued trust and support. We hope we have been able to address all your questions.

Please do not hesitate to reach out to us if you have any further queries.

Thank you once again and my best wishes to all of you.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Nov 2025

Classification: Internal
November 06, 2025

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q2 & H1 FY26 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Earnings Conference Call on Financial Results for Q 2 & H1 FY26.
We request you to please take the same on record.
Thanking You,
Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 16
Classification: Internal

Dr. Lal PathLabs
Q2 H1 FY26 Earnings Conference Call Transcript
October 31, 2025

Call Duration • 1 hour 9 minutes
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group CFO and CEO, International Business
Participants who asked questions • Binay Singh – Morgan Stanley
• Karthik Chellappa – Indus Capital Advisors Hong Kong Limited
• Prakash Kapadia – Kapadia Financial Services
• Anshul Agrawal - Emkay Global
• Bino Pathiparampil - Elara Capital
• Chirag Maroo – Keynote Capital
• Rishi Modi – Individual Investor
• Aashita Jain – Nuvama
• Yogesh Soni – InCred Capital
• Krishna Raj K. – Ekvity Wealth Management Private Limited
• Vivek Choksey – Emkay Global
• Surya Patra – Phillip Capital
• Abdulkader Puranwala – ICICI Securities
• Sumit Gupta – Centrum Broking

Classification: Internal Moderator: Ladies and gentlemen, good day and welcome to Dr Lal PathLabs Q2 FY26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen- only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand over the conference to Mr. Nishid Solanki from CDR India. Thank you and over to you, sir.

Nishid Solanki : Thank you. Good afternoon, everyone, and welcome to Dr. Lal PathLabs' Q2 FY26 Earnings Conference Call.

Today, we are joined by senior members of the Management Team, including (Hony) Brig. Dr. Arvind Lal , Executive Chairman, Mr. Shankha Banerjee, CEO , and Mr. Ved Prakash Goel , Group CFO and CEO , International Business .

I would like to share our standard disclaimer. Some of the statements made on today's conference call could be forward- looking in nature, and the actual results could vary from these forward- looking statements. A detailed disclaimer in this regard is available in the results presentation, which has been circulated to you and also available on stock exchange websites.

I would now like to invite Dr. Arvind Lal to share his perspectives. Thank you, and over to you, sir.

Dr. Arvind Lal: Thank you, Nishid. Good evening, ladies and gentlemen. Thank you for taking out the time and joining us on our second quarter and half yearly earnings call for Fiscal 2026.

Healthcare in India stands at a crucial inflection point. The sector continues to expand at a healthy pace, shaped by both demographic realities and transformative policy shifts. Initiatives like Ayushman Bharat are steadily broadening access with millions of new admissions under coverage, and hospitals being impanel led at a fast pace. This is opening the doors of organized healthcare to Tier -2 and Tier -3 cities in India, creating a structural tailwind for quality diagnostic services also. As India accelerates healthcare spends , it is imperative that early detection and diagnostics must be at the heart of this national healthcare agenda.

Another trend that is gaining momentum is rapid digitization of the healthcare delivery. The Ayushman Bharat digital mission and other incentive schemes are pushing adoption of standardized digital workflows across hospitals and labs. This shift is not very administrative. It is also redefining the patient's experience, ensuring better traceability, enabling us to integrate our services more efficiently. For diagnostic players with established digital infrastructure like ours, this is a significant advantage. Turning to disease patterns , non-communicable diseases remain the most pressing healthcare challenge for the country. Cardiometabolic demand stay elevated as India continues to grapple with high burdens of diabetes, hypertension and obesity conditions that increasingly require continuous monitoring and preventive testing. At the same time, environmental factors like air quality and lifestyle shifts are compounding risks, expanding population that requires regular testing and early

Classification: Internal interventions. These developments translate directly into higher demand for chronic care diagnostics, preventive panels and high- end testing across the country.

The diagnostic sector is undergoing gradual consolidation. Organized chains still account for less than one-fifth of the overall market. However, the bar for quality, digital readiness and compliance is rising fast , due to consumer and institutional demands. This shift is continuously helping the migration from unorganized to organized providers. This transformation also presents a generational opportunity for India's in-vitro

diagnostics (IVD) industry. As the

nation expands its screening coverage, demand for robust, affordable and scalable diagnostic solutions is increasing at a fast pace. The next decade offers a defining opportunity to align India's economic strength with its healthcare ambitions. The time to act is now , to invest, innovate and integrate diagnostics as the gateway to India's universal healthcare dream.

We remain deeply committed to our mission of delivering trusted diagnostics with care and precision. At Dr. Lal PathLabs with our reference labs backbone, franchisee- led growth model and investments in high- end diagnostic testing, we are well positioned to address emerging healthcare challenges. Together, these elements will enable us to grow responsibly , while strengthening our leadership in the country.

With that, let me invite our CEO , Mr. Shankha Banerjee, to share performance highlights of the quarter. Thank you. Over to you, Shankha.

Shankha Banerjee : Thank you, Dr. Lal. Good evening to everyone. It's a pleasure to connect with all of you again as we

review the financial performance for the second Quarter of fiscal year 2026.

In Q2 FY26, we have maintained a healthy growth trajectory and strong profitability, reflected in the operational discipline across our network. Revenue growth in Q2 stood at 10.7% YoY with H1 revenue growth of 11%. The Q2 revenue performance was supported by sample volume growth of 10.3% to 25.4 million and patient volume increase of 5% to 8.2 million. In Q2, we recorded a PAT growth of 16.4%.

While our core growth was strong, the overall volumes appear moderated on YoY basis, mainly due to lower incidence of seasonal illnesses such as Chikungunya, Malaria, Dengue and general fever, compared to what was observed in the same period last year.

Our focus on deepening our technical expertise in high complexity testing areas is continuing. Our Genevolve division incorporates integrated cutting-edge sequencing technology, significantly enhancing our capacity and accuracy for specialized services like personalized medicine and molecular diagnostics across our network.

Furthermore, underscoring our leadership in new age diagnostics, Dr. Lal PathLabs has become the first lab in India to integrate a deep learning-based AI module for detecting lymph node metastasis, including elusive micrometastasis, in cancer cases. This innovation dramatically enhances the precision and speed of oncology reporting, helping clinicians make quicker, life-saving treatment decisions.

Our flagship "Swasthfit" program contributed 26% to Q2 revenues. As an affordable bundled preventive health checkup package, it is the key to securing B2C market access. Given the immense potential of India's nascent preventive testing market, we are confident in driving a continuous increase in Swasthfit's revenue. We are

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Classification: Internal actively developing new offerings to embed within the bundled program to sustain this trend.

Our network expansion is disciplined, driven by a cluster-based approach. We are reinforcing the depth and breadth of our leadership in the core markets of North and East, including Delhi NCR and are seeing higher revenue contributions from these strengthened clusters. Parallely, we are intensifying efforts to deepen our reach into

Tier-3 and Tier-4 towns to secure future scalability.

Our investment in digital assets and technology platforms is helping us improve patient and client experience. We are also increasing applications for AI in our operations, including in-house developed algorithms.

Moving forward, growth will be underpinned by geographic expansion, portfolio innovation and digital enablement.

Operationally, our focus is to maximize utilization of the expanded network, drive efficiency and convert our infrastructure investments into significant operating leverage. This disciplined approach ensures our continued leadership in the diagnostic sector, d

elivering sustainable and profitable growth for our shareholders over the long term.

With that, I will now hand over the call to our Group CFO and CEO, International business, Ved. Over to you.

Ved P Goel: Thank you, Shankha. Good evening, everyone, and a warm welcome.

I will be sharing the key financial highlights for Q2 and first half of FY26.

- Revenue for Q2 FY26 came in at Rs. 731 crore compared to Rs. 660 crore in the same quarter last year, reflecting a growth of 10.7%. The first half FY26 revenue stood at Rs. 1,400 crore, making a growth of 11%.
- Revenue per patient for Q2 FY26 came in at Rs. 889, 5.4% higher compared to Rs. 844 in Q2 FY25.
- Test per patient for Q2 FY26 stood at 3.09 v/s 2.94 in Q2 last year, registering a growth of 5%.
- EBITDA for Q2 FY26 came in at Rs. 224 crore, compared to Rs. 202 crore in Q2 FY25, registering a growth of 10.8% with an EBITDA margin of 30.7%.
- EBITDA for first half of FY26 stood at Rs. 416 crore v/s Rs. 372 crore in H1 FY25, registering a growth of 11.8% with EBITDA margin of 29.7%.
- PBT for Q2 FY26 came in at Rs. 204 crore, registering a growth of 11.2% with a PBT margin of 27.9%. PBT for first half of FY26 stood at Rs. 385 crore with PBT margin at 27.5%.
- PAT for Q2 FY26 came in at Rs. 152 crore compared to Rs. 131 crore in the same quarter last year, registering a growth of 16.4% with the PAT margin of 20.8%.
- PAT for the first half of FY26 stood at Rs. 286 crore v/s Rs. 239 crore in H1 FY25, registering a growth of 20% with the margin of 20.4%.
- Earnings per share for Q2 FY26 was at Rs. 18.1 compared to Rs. 15.5 in Q2 FY25. First half, FY26 EPS stood at Rs. 33.9 as compared to Rs. 28.3 in H1 FY25.

- Net cash as on September 30, 2025, stood at Rs. 1,367 crore.
- In recognition of this performance and reward to our shareholders, I am happy to share that the Board of Directors has approved an interim dividend of 70%, that is Rs. 7 per share and a bonus issue of shares in the ratio of 1:1, i.e. 1 bonus equity share for every fully paid equity share, held on the record date.

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Classification: Internal With this, I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A.

Moderator: Thank you very much, sir. We will now begin the question- and-answer session.

Ladies and gentlemen, we will wait for a moment while the question queue assembles. First question is from the line of Binay from Morgan Stanley. Please go ahead.

Binay Singh : Hi, team. Thanks for the opportunity. My first question is on the revenue growth. When I look at FY25 , our network expanded by almost 9% , in that I am adding the labs, pickup point and the service center. But if you look at the first half revenue growth is around 10% or so. So, linked to that, how do you rate this revenue performance? Because we are also seeing asset turnover, which you shared on Slide 10 coming down. How is this revenue trajectory v/s your expectation? And what do you think is needed to go into at least a mid- teens or so revenue growth at the company level?

Shankha Banerjee : Binay, we had at the beginning of the year, guided a revenue growth range between 11% to 12% for this financial year, and we are staying with that number even as of now. But I think there were 2 other related points that you make.

I think one must understand that new network that one creates isn't going to be at a productivity level of existing network starting from day 1 or even in year 1. It takes some time for networks to mature. And it's an ongoing process. So, trying to correlate the number of new network points directly to a percentage revenue growth may not be an appropriate conversion ratio to look at.

And also, most of the network that we are talking about is a franchise- led network growth. So, that doesn't really need a lot of our own asset creation. So, I think the number on the a

asset turnover, et cetera, is slightly different, which maybe Ved can allude to or try and answer for you. But I think the point on percentage number of new network points to revenue growth, there is not a direct one- to-one correlation on that.

Ved, you want to talk about the asset turnover?

Ved P Goel: So, Binay, this asset turnover is compared to maybe previous one, because we have invested in some property for our expansion, that has also impacted in this asset turnover at this time.

Binay Singh : Okay. So, that has come in the H1 only?

Ved P Goel: Yes.

Binay Singh: And secondly, earlier, we talked about margin compression this year of around 100 basis point. And then in the last conference call, you said that the actual impact could be lower than what you earlier anticipated. How do you see that in the remaining quarters? Because this, again, is a pretty good quarter on the margin side. So, we have not seen any pressure.

Shankha Banerjee: So, typically, the margin impact starts reflecting mostly in the second half of the year, because the investments that are happening, the cost of that start building up in the second half. So, that is yet to pan out for us. One thing is that we are definitely maybe looking at a lower impact than maybe what we envisaged at the beginning of the year. And I think we think that our margins for the year are likely to be in the range of 27% to 28%.

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Classification: Internal Binay Singh: Okay. So, the investments will continue, so you are managing it well through cost - cutting initiatives or so, which is why the impact is lower than anticipated.

Ved P Goel: So, maybe, Binay, not the cost cutting, but yes, some of these efficiency programs, some of these mix . Swasthfit also is growing faster, and some of these things are helping us to maintain our margin.

Moderator: Thank you. Next question is from the line of Karthik Chellappa from Indus Capital Advisors Hong Kong Limited. Please proceed.

Karthik Chellappa: Yes. Thank you very much for the opportunity , sir. Just 2 questions from my side. If we were to correlate the revenue growth along with the volume growth, and if we were to look at our realization per test, in the last 6 quarters or so, that has remained more or less in the region of about 1% or below.

I am just curious to understand why is mix not having a bigger impact on our

realization per test. And what do you think needs to happen for us to see some sort of acceleration in this metric? Because clearly, we have resisted taking price hikes. So, whatever growth here has to probably come from mix. And I am just curious to understand what will it take for us to see a better outcome on this metric? That's my first question, sir.

Shankha Banerjee: So, Karthik, realization per test is not a metric we are actively tracking or also even trying to bump up. For us, the more relevant metric that we look at more closely is the realization per patient. And the reason is that the cost to serve, et cetera, is more driven by the number of patients that we will be handling in our own network as well as the franchisee network, their cost structure also is about handling more patients. You see realization per test metric that you are looking at is not something that we are even actively trying to manage or trying to push up. So, the mix improvement that you see is a resultant, which is at the patient level, the realization per patient is a resultant of higher test per patient, geography mix and some test mix improvements. All those are bundled in the realization per patient.

Karthik Chellappa: Got it. My second question, sir, if I were to look at our employee cost growth this quarter, that's only been about 6% to 7%, whereas in the last few quarters, it used to be in the double-digit range. So, just curious as to how should we read this? Is it a case where you have already upfronted a lot of your hiring and investments

. So,

from here on, there is some operating leverage possible? Or is it just something which is transient like the timing of festive bonus or so?

Shankha Banerjee: So, I think that number, we will have to do a slightly more deep dive to maybe come back to you at that level. But one, I can tell you that from an employee cost structure or the increments and things like that, there is nothing out of the ordinary in this quarter v/s maybe what we have done. There could be some accounting or the things around it, which we will have to just check and maybe come back on why is it looking very different than maybe the previous quarter. Maybe Ved can add.

Ved P Goel: No. Karthik, there is no exception here. Personnel cost is normal, 7%, 8% growth, which is more driven by the increments and all. But there is no exceptional addition which is happening in this quarter, and that's why you might not be looking. But having said that, the personnel cost is normal, which is some bit of increment, which we always give, maybe 7%, 8% and then some addition which generally we do in second half of the year.

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Classification: Internal Karthik Chellappa: Got it. One data point, sir. Can I get the contribution of West for the quarter into revenue?

Ved P Goel: So, we give this once in a year. Generally, we give the geographical contribution once in a year.

Moderator: Thank you. Next question is from the line of Prakash Kapadia from Kapadia Financial Services. Please proceed.

Prakash Kapadia: Yes. Thanks for the opportunity. A couple of questions from my end. In your opening remarks, you mentioned incidence of dengue and malaria was slightly lower despite monsoons being good. So, was that felt in the Northern region or pan-India for us, the lower incidence of diseases?

Shankha Banerjee: So, that lower incidence was seen across the country, all the markets that we are operating in, we are seeing that impact.

Prakash Kapadia: Okay. And Shankha, how critical is new product development or specialized products to drive higher revenue growth? Over the last few years, we have been talking about Tier-2, Tier-3 expansion. We have been talking about using our network. We are investing in technology, customer experiences, app differentiation. So, how critical is new product development or specialized products?

So, is there a piece of revenue which we target from here on every year as a percentage, this should contribute to higher growth, because you seem to be doing all the right things, but still we are in that channel of growth. Obviously, you have guided for this growth only, but as investors, the heart desires more. So, how critical is this piece to achieve higher growth? There have been obviously some news on technology, cancer-led this thing. So, how is this portfolio going to differentiate us from others? If you could highlight that, that will really be helpful.

Shankha Banerjee: So, taking one element and trying to correlate it directly to a growth number for the overall company might be a very difficult connect. However, as an overall portfolio and as our business model, I think new tests are quite important and critical to our success.

See, Lal PathLabs as a brand is known for being on the leading edge of the diagnostic technology. And that is something that we are proud of, and we would like to maintain, because when there are high complex cases which come up or difficult cases that come up with clinicians, we believe that their choice of diagnostic partner

will become Lal PathLabs.

And when that change happens, then automatically lot of other portfolio of tests can come to us. So, it's a portfolio approach. But from a company perspective and the business model and the profile that we have, launching new tests or having the newest cutting-edge technology on diagnostics, we feel is quite important.

Coming back to your question, is there a specific target we set? Ther

e are certain

divisions that we earmark. And yes, there are certain specific targets for divisions, which the sales team looks at. But they may not be one-to-one again correlated with the tests that we are talking about right now, because the new test when they launch, it also takes time to get disseminated, accepted and then start using in the clinician network for the benefits that we offer.

Prakash Kapadia: Okay. R&D investments could increase because I think last year, the spend on R&D was around Rs. 4 crore- odd. So, is this an area which will increase, and we will build

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Classification: Internal a differentiated product and leverage on all the things which we have done to also have a complex or a differentiated portfolio?

Shankha Banerjee : So, it will come from different sources. So, there will be, like you said, R&D, which could be what we do in-house. There could be partnerships. And there can also be tie-ups with other companies or players who are helping doing some research and then bringing it to life through companies like Lal PathLabs.

Prakash Kapadia: Okay. Understood. And lastly, if you could comment on the Suburban IT integration, is it over? And is growth coming back? Has growth been in line with what the company average is? Are things stable now?

Shankha Banerjee: So, the Suburban IT integration is completely done. And the good news is that now both the brands, Suburban and Lal PathLabs in that geography are seamlessly moving between the systems. So, I think that has started.

In terms of the overall growth, I think it's taking slightly longer than what we had envisaged. And right now, I think the way I am looking at it is maybe by or around

Q4 is when we should be able to come back to our original double-digit growth that we were seeing in Suburban before we went for this whole IT changeover.

Moderator: Thank you. Next question is from the line of Anshul Agrawal from Emkay Global.

Please go ahead.

Anshul Agrawal : Hi. Thank you for the opportunity. First question is on trajectory revenue growth despite a muted season and if I may call it, an unfavorable base, we have continued

to grow in double-digits. Would you want to throw some light around how we have been able to achieve it? Is it like a particular region that has grown outpaced or has the growth been broad-based or any strategy that we followed, which could help us sort of sustain this momentum despite an unfavorable base?

Shankha Banerjee: So, the growth is quite broad-based for us. And like maybe I had spoken about it a few quarters back, as we start ramping up our overall testing infrastructure, and also the new lab that we are adding, it will start reflecting over a period of time in terms of the organic growth that we should be able to deliver. I think some of those things have started to maybe flow through and more should come. I think that's one factor.

And second is we have put a lot of renewed focus on Delhi NCR as a region. And that has responded, and we have been successfully recording double-digit growth in Delhi NCR as well.

Anshul Agrawal : Any plans to take price hikes this year? I am assuming it will be almost 2 years by the end of this calendar year that you have taken any price hike. Any plans to take any price hikes in the current year?

Shankha Banerjee: So, a bit of a correction, by February of 2026, it will be 3 years since our last price

increase. So, we will look at, evaluate price increase opportunities maybe around that time. But as of now, there isn't something which is immediately on the cards, getting the organic growth and all the required elements right is the topmost priority.

I think once we are confident that the organic growth, the revenue and the sample volume and the patient volume that we want to see are moving in the right trajectory.

I think adding a layer of price increase on top of that will be something which we

will

be able to execute.

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Classification: Internal Ved P Goel: Just to add, Anshul, I don't think in this year, this price increase is on the cards, because another development on account of GST, where people like us are getting some benefit from the reduction of GST rate on our reagents and chemicals. And obviously, that has to be passed on to the customers.

So, this is other way around, at least, which I can think of. So, we have to pass on this benefit to the customer, and we are in the process of evaluating how much and

at what stage we have to do this. So, definitely, price increase in this year is unlikely .

Anshul Agrawal: Got it. Just one more question, one last question from my end. We seem to be heavily investing in the genomics, the NGS or next-gen sequencing specialty space. Any color on the market size or go- to-market strategy that we plan on doing in this segment?

Shankha Banerjee: See genomics and sequencing technology , the reason we are investing is not really driven by the market size of today. But the way we envisage, I think going forward, this market is going to develop quite fast in the future. And we need to be rightly positioned when this market becomes much bigger than what it is today.

So, as of now, it's not so much about the revenue numbers, but getting the right technology, the platforms, the test menu, the TAT around it, the cost and the pricing right, and our ability to really disseminate all of this to the clinician network, which positions us as when the need starts growing significantly in this area.

Moderator: Thank you. Next question is from the line of Bino Pathiparampil from Elara Capital.

Please go ahead.

Bino Pathiparampil: Hi, good afternoon, all of you. Congratulations on a great set of quarters. A couple of questions from my side. We have seen the new rates being published by CGHS , and possibly other government agencies will follow. Is there any way in which the stand- alone diagnostic industry benefits from that , and our company in particular?

Shankha Banerjee: So, the new CGHS rates, which have been announced, I think, effective around 20, towards the end of September. So, it definitely has a beneficial impact on Lal PathLabs . We have CGHS business as a part of our portfolio. So, we are still evaluating the level of that impact or the quantum of that impact , because some of it would flow maybe quickly, but some of it may come after a lag. So, we will definitely see some positive impact, but the quantum of that is something which we are still trying to work out.

Bino Pathiparampil: May I know what percentage of your top line would be benefiting from this, assuming other government agencies also follow.

Shankha Banerjee: So, like I said, we have business which is CGHS, then there is another linked business, which is ECHS, which has the defense services come through it. Then there are public sector units which follow CGHS price. So, there are many such things inside. But the overall contribution of this portfolio will be maybe 5% or maybe lower than 5% to our overall revenue. Ved, anything you want...

Ved P Goel: Yes. Maybe less than 5%, yes.

Bino Pathiparampil: Got it. Got it. Understood. And you just alluded to the benefit from GST in the answer to a previous question. Would you be able to quantify a little bit in terms of what sort of margin benefits you expect and how much of that you would retain?

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Classification: Internal Shankha Banerjee: No. So, we are not willing to retain anything. Whatever benefit will come, we are willing to pass on to the customer. As far as quantum is concerned, we are in the process to evaluate, because there is a process going on. And whatever we will get from our suppliers, the benefit in terms of that will go to customers. So, there is unlikely that on account of GST, our margins will go up.

Ved P Goel: Yes. So, I think just to add to

that, we have no intention of using any benefits through GST to flow through margins. It will get passed on to customers, clients, et cetera. I think the quantum and the modality of how that will happen is something which is still under review , because it's quite a complex calculation, given the number of types of reagents and consumables and the mix of that in our whole test menu. So, it's a very complex calculation. So, it's taking some time for us to really figure out and who's passing how much, et cetera. But we don't have any intention of using any GST benefits pass on for improving margins, but we will rather pass it on to the customers.

Bino Pathiparampil: Understood. Just a bookkeeping question. The depreciation amount for the quarter is a little higher than previous few quarters. Anything new that has come into that?

Ved P Goel: I don't think anything new. The only thing now after Suburban liquidation and getting into this parent company, there is additional depreciation on intangibles, which is coming, but nothing else.

Moderator: Thank you. Next question is from the line of Chirag Maroo from Keynote Capital.

Please go ahead.

Chirag Maroo: Yes. Thank you for the opportunity. Most of my questions are answered. I just have 2 questions. One is, will it be possible for you to give the H1 mix of specialized portfolio in terms of revenue?

Shankha Banerjee: So, this is not really a number we are sharing at a quarter or a year or a half year level. I think one thing also one must keep in mind is that there is no standard definition of specialized business across the industry. I think each company has its own way of defining what is specialized and what is not. We have a specialized portfolio. I think that growth and contribution, et cetera, we will share at the end of

the year.

Chirag Maroo: Okay. No issues. My second question is, will it be possible for you to share Tier-3 plus revenue mix, it was not provided for FY25 also. So, if that is possible for FY25 and H1 FY25.

Ved P Goel: So, again, Chirag, some of these data points, we are generally giving end of the year, because there are certain way of calculating, certain way of where there are some complexity. And as Shankha mentioned, like specialized, each company has own definition and so on and so forth. So, maybe end of this year, we will provide both these data points.

Moderator: Thank you. Next question is from the line of Rishi Modi, who is an individual investor. Please go ahead.

Rishi Modi: Shankha, so what my research tells me is you all are running an advanced radiology pilot program in Delhi in a few centers. So, how is that shaping up? And how are you looking at it? Like are you all there where you all can scale this up at least in Delhi? Or how is that panning out?

Shankha Banerjee: So, currently, we are running high- end radiology, which is CT, MRI related test that we have started is in one center in Delhi NCR. And what we are seeing is that the

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Classification: Internal scale -up is more or less happening as per our plan, and it is looking robust, fitting in with some of the business practices the way we run it. So, we have decided to expand that pilot into a few more centers.

And as and when we would also want to test that the same trajectories and the assumptions hold not only for one center, but at least a few more before we really go for a full scale- up scenario. So, those investments are also likely to happen in this financial year, the other few pilots.

Rishi Modi: Okay. So, let's say, we understand from the other listed integrated players, it takes about Rs. 25 crore, Rs. 30 crore to put up one of these larger centers, which are MRI, CT, everything else. So, basis your study, like how big is the opportunity, say, even if you just start with Delhi within the next couple of years?

Shankha Banerjee: We haven't yet done a full opportunity business plan

scoping. I think right now, we

are still at the pilot stage. And for us, since this is an addition to existing infrastructure, the investment to start high -end radiology isn't in the range of Rs. 20 crore, Rs. 25

crore, like you mentioned. It is lower than that. So, I think once we are done with all our assessments of our pilot, and when we have created our whole scale- up business plan, I think we will be able to share that granular detail at that point of time.

Rishi Modi: Okay. Got it. Second, I wanted to understand, we have been mentioning in our presentations, annual reports that we are actively looking at South India for an inorganic opportunity. Has there been any progress? Is there any opportunity that

one can expect to come through or anything which you can disclose without violating any nonpublic information.

Ved P Goel: So, Rishi, right now, there is nothing which we can share with you. But having said that, this activity is ongoing. And as stated earlier, we are always scouting and keen to expand our South India presence, especially through M&A. But right now, there is nothing which I can share with you.

Rishi Modi: Got it. And finally, Shankha, you mentioned you all are using an in- house AI for improving your own process systems. Just wanted to understand if you could delve a bit deeper on what sort of initiatives apart from that one cancer detection that you

mentioned on the operations side, if you could tell me what sort of AI algorithms or AI tech, you are using to improve processes? And then what sort of talent have you hired for this?

Shankha Banerjee: Okay. So, I may not be able to tell you very specifically on what in- house AI we have rolled out. But let me first give you an overview. So, for us, there is obviously the medical tech side, which I spoke about in terms of new tests. So, there is AI, which is there in terms of the patient and the testing side of it.

There is also lot of operations that we run where AI algorithms are and can be deployed, and that is where some in- house AI algorithms have been created.

Because of this operational thing, the talent is primarily in- house, and we have developed the algorithm and maybe some more in the pipeline. Most of that is in- house work.

Rishi Modi: So, if I have to understand the objective is to reduce the cost of current operations or it's to enable scale for further operations?

Ved P Goel: So, it would depend upon the particular use case, and both kinds of use cases are there. So, there is about enabling scale. There is about improving efficiency. There

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Classification: Internal is also about enhancing quality with certain algorithms, it helps improving in any of these 3 areas.

Moderator: Thank you. Next question is from the line of Aashita Jain from Nuvama. Please go

ahead.

Aashita Jain: Hi, good day, everyone. So, I just have 2 questions. Firstly, on the Radiology side, given that you have highlighted you are planning for couple of pilot centers, what's the initial outlay that we have thought for our pilot radiology program, if you can highlight that? And also, the CAPEX for this year?

Ved P Goel: Aashita, it's in the range of, let's suppose Rs. 12 crore to Rs. 15 crore outlay for any center, including MRI and CT. And CAPEX in this year, normal maintenance CAPEX is about Rs. 50 crore, Rs. 60 crore, but we have bought one property in this year, which is another. So, overall about Rs. 130 crore, Rs. 140 crore CAPEX in this year is what we are estimating.

Aashita Jain: Okay. Thank you. And secondly, just related to this, the property that we have bought in Shalimar Bagh, is this also related to our radiology foray or separately? How should we think about it?

Shankha Banerjee: So, it's about enhancing our super specialty testing capabilities. So, there would be newer departments, higher -end equipment, new technologies that we will be bringing. And it

is about enhancing our super specialty capability centrally. I think that's the primary objective. Depending upon size, location, right now, it may or may not result in MRI, CT kind of a thing. That's not really the objective for this location.

Aashita Jain: Okay. Understood. And any guidelines for the lab or the collection center addition for this year and the next year?

Shankha Banerjee: Right now, we can talk about this year. I think beginning of the year itself, I had said 15 to 20 new labs is what we are looking at, and we are still guiding the same number. And in terms of collection center network, we should be maybe 600 to 800 number of new collection centers this financial year.

Moderator: Thank you. Next question is from the line of Yogesh Soni from InCred. Please proceed.

Yogesh Soni: Yes. Thanks for the opportunity. First question is on the Tier -2, 3 cities. If you could let me know how has been the growth in the first half? And secondly, if you could discuss about how the competitive intensity has been in these markets? And how are we strategizing our further growth?

Shankha Banerjee: So, I think Ved also mentioned, you see tier -wise growth is a number which we are not giving at a quarterly level. Maybe we will look at it and provide that information at an annual level. But coming back to the competitive intensity, competitive intensity, there is a visible competition and there is an invisible competition.

We must still keep in mind that the overall market structure is such that the unorganized pathology sector is still the most dominant player. And the competitive intensity in that part of the business remains quite high. So, one can't say that intensity has come down at an overall level. But yes, visible competition definitely has been more rational in the recent past than some time back.

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Classification: Internal Yogesh Soni: If one wants to understand whether the presence of online players or other hospital diagnostic players has improved in these markets, can you help us understand on this?

Shankha Banerjee: So, I think the hospital players are also expanding, I think the retail network. I think that's something which has been visible for the last few quarters. And I think that continues. So, as online players, I think they are obviously trying to do their bit, in terms of their revenue as well as profitability, which they are doing.

So, I think none of those have really changed in terms of any strategy that they would be following. Obviously, I can only comment on what we see from outside. But like I said, the growth is not a zero- sum game, because still the huge market is unorganized. So, there is space for everybody, all types of business models to grow in the diagnostics space.

Yogesh Soni: Understood. One last question, which I would want to know is on the growth front. For last couple of years, we have been doing heavy lab additions, and adding the collection centers as well. Our growth has now reached from 10% to around 11%, 12% or so. When can we expect growth to reach early teens or maybe mid- teens over the next 2, 3 years or 3 to 5 years of time?

Shankha Banerjee: Yes. So, I think the basic idea is exactly the way you have outlined it. You see once we are secured of a steady underlying organic growth of, let's say, 11%, 12%. I think building a few things on top of that, whether it is a once in 3 years, once in 4 years price increase CAGR, which is lower than inflation as well as some one- off kicker which can come through M&As, the long- term CAGR moving it to a 13%, 14% level is definitely will be in a realistic framework for us.

Moderator: Thank you. Next question is from the line of Krishna Raj K. from Ekvity Wealth Management Private Limited. Please go ahead.

Krishna Raj K: Hi. Thank you for the opportunity. So, it was mentioned in the presentation as new

therapies for diabetes are increasing, the role

of diagnostics will intensify. With

weight loss drugs going off patent next year, do you intend to plan test bundles which will cater to GLP -1? And how do you see this opportunity?

Shankha Banerjee: So, I think the GLP -1 drugs going off patent is definitely an opportunity, very right for the pharma industry. I think on the diagnostic front, I would still say that there are many bundles that were already available from our side rather in the market, which can help monitor the health of the different organs and multiple sets of organs. Will there be a need for any specific testing bundle just for people who are on GLP -1 or can be on GLP -1 drugs is something which we haven't really still come to a final conclusion on.

Krishna Raj K: Okay. And secondly, in the industry, Q3 is generally a softer quarter. But with monsoon being extended this time, do you see more volumes the next quarter?

Shankha Banerjee: So, I assume by more volumes, you are thinking v/s last year in terms of a trend line.

Krishna Raj K: Yes. So, in terms of more patients reaching out to the diagnostics, because the monsoons are getting extended, so.

Shankha Banerjee: So, you see it is not only about monsoons getting extended, there is also a temperature angle related. So, it's not just that monsoons, I think there is a thing about monsoons, and then there is a thing around temperature. I think there are multiple things which come together for the fever or the seasonal fever, et cetera, to

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Classification: Internal come into play. Our belief is that this year, those conditions weren't too favorable. And unlikely that Q3 will see a catch-up because of that.

Krishna Raj K: Okay. And my last question was, when I look at revenue per patient, we are slightly lower compared with peers. So, how would this number ideally go up? Is it volumes that are driving or it is more complex tests that would drive this number?

Shankha Banerjee: So, when you say revenue per patient down compared to what?

Krishna Raj K: Compared to peers.

Shankha Banerjee: Peers?

Krishna Raj K: Yes.

Shankha Banerjee: So, I think you see the revenue per patient is a derivative of multiple things. So, you know it has a geography, it has a channel mix, and a type of test mix. So, I think inherently, each organization or each company can have a slightly different way of looking at these multiple factors. I think the way we look at it, we are quite happy with the progress we are making on revenue per patient. It is currently growing at about 5% over same period last year. I think that's quite healthy. And we don't envisage this to organically step up to much higher levels any time in the near future.

Moderator: Thank you. Next question is from the line of Vivek Choksey from Emkay Global.

Please go ahead.

Vivek Choksey: Hi, sir. Good evening. So, just a point that was mentioned in your presentation, right?

So, just wanted to understand that in the Indian market, there has been a narrowing penetration in non-Tier-1 cities over the last 5 to 6 years, as you can see in the ppt, right? So, does that mean that demand for pathology in such markets could be a headwind in terms of if you look at our strategy of expanding into non-Tier-1 market?

Shankha Banerjee: Sorry, maybe I didn't get your question. What are you saying has reduced in non-Tier-1?

Vivek Choksey: The penetration of diagnostics across Tier -2, 3, 4 cities, if you see Page 34 of your presentation.

Shankha Banerjee: It has reduced.

Vivek Choksey: Sorry, my bad, I actually saw it in the reverse. Okay. Sorry. My bad.

Moderator: Thank you. Next question is from the line of Surya Patra from Phillip Capital. Please proceed.

Surya Patra: Thanks for the opportunity, sir. My first question was on the lab utilization front, sir. So, on an overall basis, what lab utilization that you would be currently having? That is one. And a clarification a

bout the CAPEX that generally maintenance CAPEX of around Rs. 60 crore-odd that you have mentioned for the year, but the total CAPEX could be around Rs. 130 crore, Rs. 140 crore, what you have mentioned, whether that is indicative about lab addition only or something else?

Ved P Goel: So, Surya, this Rs. 60 crore plus we bought this property, Rs. 60 crore includes the lab addition, which we are saying maintenance, because generally, we are adding 15 to 20 labs. Last year also, we added 18, this year also, we are planning 15 to 20

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Classification: Internal labs. This is including Rs. 60 crore and Rs. 75 crore-odd we have spent on this property addition.

Surya Patra: Sure.

Shankha Banerjee: And on your lab utilization point, I think the capacity is not a fixed number in labs. So, we always try and keep some capacity free for building up volume, because adding new equipment or better or larger throughput machines, increasing the timing of a lab. There are multiple ways in which capacity gets enhanced. So, there is no formula on saying that this is the capacity utilization one is looking at or working at, rather we would always try and keep some capacity available for ensuring that the growth doesn't get impacted.

Surya Patra: Sure, sir. Second question was on the margin front. See, we had in the 4th quarter mentioned about some margin pressure for the full of FY26. But the run rate what we are seeing despite a relatively weaker season in the 2nd Quarter, better than what we have seen in the previous period. So, are you still likely to maintain the margin pressure indication for the full year, so which could be seen in the second half? Or how should one think or would you revise upward or how to think about it, sir?

Ved P Goel: So, yes, as earlier mentioned, this year, we are estimating that between 27% to 28% margins, we are confident to maintain. Yes, we had mentioned earlier, there is some dilution. But in last quarter, we mentioned that it might not be that much. So, right now, we are estimating between 27% to 28% EBITDA margins.

Surya Patra: Okay. And just last one question from my side, sir, about the same point, the previous question, revenue per patient. So, I am seeing that, okay, you are very smartly without raising any price, maintaining more than 5% growth annually on a revenue per patient levels. So, how sustainable that can be? And what levers that you have to sustain this over a period of time, if we are not taking any price rise?

Shankha Banerjee: So, I think sustaining 11% to 12% revenue growth without price hikes is what we are talking about. Now that comes through a mix of patient sample, RPP. It's kind of a mix playing there, because there can also be a scenario where we can have a slightly higher patient growth, but a lower RPP growth, right?

So, I think the ability for us to maintain 11% to 12% revenue growth, sustain that consistently is what are the initiatives and the plans that we have been deploying in the last 4 to 5 quarters. I think we seem to be on that track and on that trajectory. And the levers are what I have mentioned earlier that there is a testing network expansion, there is this whole Swasthfit bundling that is there, there is a geography and a channel mix improvement that we are doing, and the portfolio test mix improvement. So, I think those are the points which we mentioned earlier as well are helping us achieve this number.

Moderator: Thank you. Next question is from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Mr. Puranwala, your line is unmuted, please go ahead with your question.

A. Puranwala: Hi. Thank you for the opportunity. Sir, just one question on about this pivot towards Radiology business. So, sir, how should we look at this as an opportunity? I understand it's on a pilot basis. But in terms of acceptability and the opportunity, this would be in y

our home market? And this is something which can be rolled across

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Classification: Internal entire Delhi NCR market? Or you believe that this would be within certain pockets in the Western side or on the Northern side of the home market?

Shankha Banerjee: So, I think I would not term it as a pivot right now. I think it is more about testing and building our assumptions and looking at the metrics and the levers to use to grow the business sustainably. I think once we are clear, obviously, the first benefit case will be in Delhi NCR.

And I think if our pilots are all successful, then Delhi NCR will be our business case. I think going beyond that would need fine-tuning of elements, which will take its own time for us to really develop and build. So, it is unlikely to be looked at as a pivot. But definitely, it's a way to handle the emerging patient expectations of being able to get the integrated diagnostics service under one roof.

Moderator: Thank you. Next question is from the line of Sumit Gupta from Centrum Broking. Please proceed.

Sumit Gupta: Hi, good evening. Thanks for the opportunity. Sir, just one question. Like how do you plan to increase the B2C contribution?

Shankha Banerjee: Right now, our B2C contribution is almost 3/4 of our overall revenue. And we believe that this is a reasonably good contribution number. So, in the range of, let's say, if we maintain our range of anything between 74%, 75% of B2C contribution, I think that's good. We are not trying to say that increasing this contribution is going to be a key objective for us going forward.

Moderator: Thank you. Ladies and gentlemen, that was the last question for the day. I would now like to hand the conference over to the Management for the closing comments.

Ved P Goel: Thank you all for joining this call today. We appreciate your continuous trust and support. We hope we have been able to answer all your questions. Please reach out to us in case you have any further queries. Thanks once again. Thank you very much.

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Call: Feb 2026

Classification: Internal
February 03, 2026

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q3 & 9M FY26 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Earnings Conference Call on Financial Results for Q3 & 9M FY26.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 19

Dr. Lal PathLabs Limited
Q3 FY26 Earnings Conference Call Transcript
January 30, 2026

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group CFO and CEO, International Business
Participants who asked questions • Prakash Kapadia – Kapadia Financial Services
• Vivek Agrawal - Citigroup
• Anshul Agrawal - Emkay Global
• Raman KV - Sequent Investments
• Bino Pathiparampil - Elara Capital
• Saion Mukherjee - Nomura
• Shyam Srinivasan - Goldman Sachs
• Surya Patra – Phillip Capital
• Vamsi Hota - ASK Investment Managers
• Reet Jain - Niveshaay
• Lokesh Manik - Vallum Capital

Moderator: Ladies and gentlemen, good day, and welcome to the Dr. Lal PathLabs Q3 FY26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Nishid Solanki from CDR India. Thank you, and over to you, sir.

Nishid Solanki : Thank you. Good afternoon, everyone, and welcome to Dr. Lal PathLabs Q3 FY26 Earnings Conference Call.

Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal – Executive Chairman ; Mr. Shankha Banerjee, CEO , and Mr. Ved Prakash Goel , Group CFO and CEO International Business.

I would like to share our standard disclaimer. Some of the statements made on today's conference call could be forward -looking in nature, and the actual results could vary from these forward -looking statements. A detailed statement in this regard is available in the results presentation, which has been circulated to you and also available on stock exchange website.

I would now like to invite Dr. Lal to share his perspectives. Thank you, and over to you, sir.

Dr. Arvind Lal : Thank you, Nishid, and good afternoon, everyone, and thank you for joining us for the Q3 and 9M earnings call for fiscal 2026.

Healthcare in India is undergoing a profound transformation. We are witnessing a steady rise in lifestyle -related and chronic diseases, an aging population with evolving care needs and a growing awareness among consumers about the importance of early detection and preventive health. These shifts are redefining diagnostics from being episodic and illness driven to becoming an essential continuous part of long -term health management.

At the same time, access to high -quality diagnostics remains uneven across the country, reinforcing the relevance of organized technology -enabled players who can deliver reliable and standardized services at scale. This is where our purpose at Dr.

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Lal PathLabs remains deeply aligned. We are committed to building an accessible, dependable and clinically rigorous diagnostics ecosystem for every Indian.

This quarter, we made meaningful progress on several strategic priorities :

- First one was launch of Sovaaka, our personalized preventive healthcare platform : A major milestone in this direction was the launch of Sovaaka operating under the philosophy of science behind wellness. Sovaaka represents a strategic pivot from traditional disease detection towards science -led disease prevention. By integrating high -end diagnostics and AI -supported imaging with curated programs, we are directly addressing the rising demand for personalized preventive care in India.

- Next is the expansion of specialized and advanced testing capabilities : We continue strengthening our offering in high -end diagnostics, including oncology, genomics, advanced infectious disease testing, and investments in our reference labs and collaboration with global technology partners remain key enablers of this strategy .

- Next is deepening our presence in underpenetrated markets : Our scalable franchisee -led model continues to help us expand into Tier 3 and Tier 4 markets. Improving diagnostic access in semi -urban and rural India remains a core long -term priority.

Together these initiatives reflect our integrated approach that is combining a robust physical network, deep clinical capability and digital innovation to serve India's evolving healthcare needs while driving disciplined and sustainable growth.

With that, I would now like to invite our Chief Executive Officer, Mr. Shankha Banerjee, to share the key performance highlights

for the quarter. Thank you, and over to you, Shankha.

Shankha Banerjee : Thank you, Dr. Lal, and good afternoon to you all. I am glad to connect with everyone today to walk through our Q3 FY26 performance.

Our Q3 FY26 results reflect sustained growth and operational excellence. We delivered strong financial performance this quarter with revenue rising 10.6% YoY, resulting in 10.8% growth for the 9 -month period. Patient volume growth was lower

than our current trend at 2.7% in Q3 due to unexpected decline in our "seasonal fever portfolio". The YTD patient volume growth is 4.4%. Our YTD sample growth remains robust at 9.6%.

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In Q3, we continued the journey of significant clinical advancement with introduction of more than 15 new tests, including more than 5 which are "First in India". These additions were across multiple portfolios, further strengthening our high-end complex testing capabilities. Our technical team had 5 international journal publications with 1 award-winning paper in the area of "using ML for rapid acute leukemia diagnosis". We have upgraded our enterprise IT infrastructure to next-generation technology, which has an AI-enabled environment at compute and network layer. We have rolled out a new agentic bot for patients at multiple touch points and a clinician-facing digital tool in pilot mode. We have enhanced our patient experience at walk-in labs and home collection with faster reporting standards in Delhi NCR. All these initiatives and more in the pipeline are aimed to enhance the patient and clinician experience with the brand. Our recent launch of "Sovaaka" preventive wellness program model also reimagines the patient experience with diagnostics. Together these developments solidify our position, not just as a laboratory service provider, but as a comprehensive high-tech partner in our patients' long-term health journey.

During the quarter, Swasthfit contributed 26% to total revenue and continues to play a primary role in our B2C market growth. We are scaling this affordable preventive health model to Tier 2 and below towns to capture an untapped preventive testing market.

The transition from unorganized to organized diagnostics in India is sustained as patient priorities shift towards clinical reliability. Our competitive edge lies in stringent quality protocols and superior service architecture. Beyond the test itself, our differentiation is defined by the end-to-end patient experience from seamless sample collection to digital report delivery. Ultimately, these superior service levels are the primary drivers of patient acquisition and brand loyalty.

We are scaling through expansion in newer markets, innovative offerings and digital enablement. This approach cements our diagnostic leadership and enables long-term profitable growth.

With that, I will now hand over the call to our Group CFO and CEO International Business, Ved. Over to you.

Ved Goel : Thank you, Shankha. Good evening, everyone, and a warm welcome. Thank you for joining us today.

I will take you through the key operational and financial highlights for Q3 and 9 M of FY26, followed by our year-to-date performance.

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Let me begin with some key operating metrics.

- Revenue per patient for Q3 FY26 stood at Rs. 927, representing a 7.7% YoY increase over Rs. 861 in Q3 FY25. This improvement was driven primarily by favourable test mix and geographic mix rather than pricing.
- Test per patient increased to Rs. 3.11 in Q3 FY26 compared to Rs. 2.97 in the corresponding quarter last year, reflecting our continued focus on menu expansion and preventive healthcare offerings.
- As part of our commitment to strong governance and customer-centric practices, the company has gladly passed on the benefits arising from reduction in GST on reagents and chemicals from 12% to 5% through price reduction for patients. While this had a modest

impact on realization, we

believe it strengthens our long-term trust and demand sustainability.

Overall, our growth continues to be volume led with realization improvements coming largely from mix enrichment, which we believe are structurally sustainable drivers.

- Revenue for Q3 FY26 came in at Rs. 660 crore compared to Rs. 597 crore in the same quarter last year, reflecting a YoY growth of 10.6%.
- For the 9 months ended December 2025, revenue stood at Rs. 2,060 crore versus Rs. 1,859 crore in YTD FY25, registering a growth of 10.8%, driven by steady volumes and operating leverage.
- During Q3 FY26, we incurred a onetime cost of Rs. 30 crore related to implementation of new labour code, which has been reported as an exceptional item. This is a non-recurring adjustment and does not impact the underlying operating performance or steady-state cost structure of the business.
- EBITDA before exceptional item for Q3 FY26 stood at Rs. 179 crore compared to Rs. 154 crore in Q3 FY25, reflecting a growth of 16.3% and

EBITDA margin of 27.2%.

- For the 9 months ended December 2025, EBITDA before exceptional items came in at Rs. 596 crore compared to Rs. 527 crore in the same period last year, registering a growth of 13.1% with EBITDA margin of 28.9%.

- Profit before tax for Q3 FY26 after taking the hit of labour code was Rs. 124 crore compared to Rs. 138 crore in Q3 FY25 with a PBT margin of 18.8%.

For the 9 months ended December 2025, PBT stood at Rs. 509 crore v/s Rs. 471 crore in YTD FY25 with a margin of 24.7%.

- Profit after tax for Q3 FY26 after taking the hit of labour code stood at Rs. 91 crore compared to Rs. 98 crore last year, resulting a PAT margin of 13.9%.

For the 9 months period, PAT was Rs. 378 crore compared to Rs. 337 crore in YTD FY25 with a margin of 18.3%.

- Earnings per share for Q3 FY26 was Rs. 5.4 compared to Rs. 5.8 in Q3 FY25. For the 9 months ended December 2025, EPS stood at Rs. 22.4 compared to Rs. 20 in the previous year.

- Our balance sheet remains robust with net cash and cash equivalent of Rs. 1,411 crore as on December 31, 2025, providing us significant flexibility to support organic growth, calibrated inorganic opportunities and shareholder returns.

- I am pleased to share that the Board of Directors has approved an interim dividend of 35% amounting to Rs. 3.5 per share on enhanced equity share capital post 1:1 bonus issue.

Our capital allocation priorities remain unchanged, investing in growth, pursuing M&A opportunities and returning surplus capital to shareholders while maintaining financial prudence.

Overall, we remain focused on delivering sustainable growth and healthy profitability while continuing to invest in quality, compliance and customer experience.

With this, I conclude my opening remarks, and I now request the moderator to open the forum for question -and-answer. Thank you.

Moderator: Thank you very much. The first question is from Prakash Kapadia from Kapadia Financial Services. Please go ahead.

Prakash Kapadia : Thanks for the opportunity. A couple of questions from my end. Shankha, at a time when industry revenues have been growing in the 7% -8% range, our growth, if I look at last 11 -12 quarters, has been 10.5% on an average. So what am I missing? We have brand, we have technology, we have franchisee network, we have reach, we have doctor empanelments, sales force, low cost of test, on-time delivery. What is the missing factor which is leading to this kind of a growth? Because if I look at some of the other listed players, they have been reporting higher growth. So that is the first question on revenue.

And secondly, on Swasthfit, it has been growing almost 2x of our company revenue.

How is the journey been? Is it now a Tier 2, Tier 3 phenomenon also? Is that also contributing there? What is the journey in Tier 2, Tier 3 cities? Is it movement from

ent from

basic diabetes and thyroid to some of these packages? And is there any possibility that Swasthfit is cannibalizing some repeat usage or revenues at the company level?

Those are my 2 questions.

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Shankha Banerjee : Thanks, Prakash. I think on the first one in terms of revenue, I think we have been consistent in trying to say that right now we are focusing only on organic revenue growth. If you remember, about 2 years back, we had slipped to about a 7% organic revenue. And so we were quite clear that we want to build the organic momentum back across the organization. And that is what the team has been working on.

Simultaneously, the building blocks of how to take the next jump forward are being put in place. Revenue growth in the future, we are obviously working towards making this better than whatever we have now. But the stability that you see is kind of designed to at least bring the organic growth trajectory at a level that we are comfortable.

I think we have been saying 11% to 12% organic growth for this year. And even as of now, we are confident that we should be able to deliver between 11% to 12% for the year. And once the whole base kind of comes to that level, I think other initiatives which are already being laid out should help us step up going forward.

On the second question on Swasthfit, yes, I think even in some of the previous calls I had mentioned that Swasthfit now with our expanding reach we ourselves are able to now put Swasthfit into more geographies, including the smaller towns, and we are definitely seeing offtake or acceptability of this in the smaller towns.

Also, even in the urban centers, our penetration or availability of this is , we are seeing not only direct consumer or patient pickup, but like I mentioned, in some cases, we are even seeing doctor recommendations coming for the portfolio. Those levers are still in play, and that is what is helping us maintain the growth momentum on Swasthfit.

Prakash Kapadia : Okay. And in some of these initiatives which you have been calling out over the last few quarters, be it distribution, be it new product development, be it enhancing the menu, I think increasing the sales force. Where will that trajectory come in? Now is FY27 going to get us there? Because I think last call, you had hinted suburban has stabilized, the IT issues are done, Mumbai reference lab has stabilized.

Will West and South contribute more meaningfully to that, then we will see that delta or incremental revenue coming through us? What is the tipping point or what as analysts and fund managers we should focus on, this is what it is and we are there?

Shankha Banerjee : I think for a size of organization and the scale that we are running, I do not think one single city, geography will really help in kind of taking the overall number higher.

What has to work for us is slightly different strokes in different geographies, but all
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geographies will contribute towards a higher growth momentum. I would say, starting Delhi NCR, North, East, West, even South, international as well, all of these geographies put together will contribute to the higher growth trajectory that we are aiming for. It is unlikely that one geography will help the overall number of the organization to go to the level that we want it to move towards.

Prakash Kapadia : Okay. Understood. And from next year, we should see some better growth? Is that the path we should expect?

Shankha Banerjee : Yes, that is the trajectory that all of us expect, including me.

Moderator: Thank you. Next question is from Vivek Agrawal from Citigroup. Please go ahead.

Vivek Agrawal : Thanks for the opportunity. In the opening remarks , you talked about that the quarter has been also impacted or the volumes have been impacted by adverse season. So is it possible for you to break it down how the seasonal volumes or the fever -related volumes

have grown in this quarter or how the specialty or semi -specialty test -related volumes have grown in the quarter? Thank you.

Shankha Banerjee : Yes. I do not think we will get into that micro detail, Q3 is usually not a thing where we discuss season or fever or things like that. But it has been an unexpected quarter in the sense that we have seen an impact due to our seasonal fever portfolio has got negatively impacted. We have declined on that portfolio. And yes, that has resulted in lower patient volume numbers and even to a certain extent the lower sample volume numbers.

Vivek Agrawal : Understood. Basically, it has declined, right? And approximately, what is the share of this portfolio in the overall patient volume, approximately? Can you give any ballpark number?

Shankha Banerjee : You see that share is different by quarter, so I am not too sure, and that is not something which we focus upon. I am not too sure that is the data we would want even any of you to be focusing upon.

Vivek Agrawal : Yes. No problem at all. Another question is if you look at the patient volume growth, so although this quarter has been impacted, but even if you look at the 9 months, it is around 4 %-5% kind of growth. And this is despite that you are pushing significant growth as far as in the Tier 2, Tier 3 town, expanding the network and all. Is it the right way to look at that? Your core markets, like, for example, Delhi NCR or in the Tier 1 markets, is there any kind of a structural slowdown as far as the growth is concerned? Or how to put it.

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Shankha Banerjee : I think the patient volume numbers are seeing a steady growth. I think quarter -to-quarter fluctuations are there. But if you at least see in the last 2 -3 year movement, from an annual low of almost 2.7% that we saw 2 years back, we are going upwards. I think expecting a jump at the scale of patient volumes that we do, expecting this number to suddenly go up by 3 %-4% in one calendar year or one financial year may not be the right expectation.

But yes, there is a slow and steady improvement that we are trying to build on the patient volume number. And that you will see in the YTD number this year also. We have obviously this impact right now in this quarter, but we are seeing some increase in the patient volume numbers this financial year as well.

Moderator: Thank you. The next question is from Anshul Agrawal from Emkay Global . Please go ahead.

Anshul Agrawal : Thank you for the opportunity. Sir, my first question is on margins. I think generally, we see a dip in margins in the current quarter, and we had previously indicated that

H2 margins would also be lower than H1 owing to network investments and other expansion projects. Are the current margins, I am talking about 9-month ended margins, sustainable at around 29% going forward?

Ved Goel: I think if you see H1 margin was higher this quarter. If we take out the impact of labour code, it is 27.2%. And later part of the year we said it will be lower than first half of the year. Overall, earlier, we said a similar number, around 28% kind of margin. And we are confident that we can achieve in this financial year between 27%-28% even after this impact of labour code.

Anshul Agrawal: Got it. Going forward, the margin trajectory would remain firmly around this 28% is what I am trying to understand despite us passing on those GST-related benefits, et cetera, and all that stuff.

Ved Goel: Yes. I think we are hopeful that we can maintain the trajectory between 27%-28%.

Anshul Agrawal: Got it. Second question I had was, can you provide us with an update with the advanced radiology pilot project that we had undertaken in Delhi? Is there any update that you can share with us with regards to traction or any plans to sort of expand on that?

Shankha Banerjee: The traction on the advanced radiology is that we were running a one center pilot in

Delhi NCR, and we are looking at it and it is kind of going as per the planned trajectory that we have. We are now starting or just started another center in Delhi

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NCR on advanced radiology because we want to now see how replicable is our first pilot in another location which is geographically apart. So that is the status as of now.

Anshul Agrawal: Got it. This would be an advanced center only, sir, right? It will include MRI and CT scan machines.

Shankha Banerjee: Yes, this is another center with the MRI and CT scan.

Anshul Agrawal: Just one last bookkeeping question from my end. Can you help us with the CAPEX number for the current quarter? Any full year guidance incorporating these high investments in radiology and even Sovaaka preventive wellness program?

Ved Goel: Anshul, this year, we have made 2 investments, one, we bought one property which is for our precision diagnostic center, and, of course, the Sovaaka, which we have just started. Roughly, we will be spending about Rs. 150 crore to Rs. 160 crore kind of CAPEX in this year. Normally, we spend about Rs. 50 crore - Rs. 60 crore - Rs. 70 crore on an operating kind of capex.

But these radiology center, let us suppose if we take one center, it takes away about Rs. 15 crore - Rs. 16 crore kind of investment. And if we take, let us suppose, going forward 1 or 2 more, then accordingly someone can factor in 2 more investments.

Anshul Agrawal: Right. So sustainably, capex could settle at around, say, around Rs. 60 crore - Rs. 70 crore of operational capex plus 2 center capex on this figure. About Rs. 100 odd crore could be the right number?

Ved Goel: Plus some new initiative or new investments. So near to time, maybe you can see between Rs. 100 crore - Rs. 150 crore kind of CAPEX someone should assume.

Moderator: Thank you. The next question is from Raman KV from Sequent Investments. Please go ahead.

Raman KV: Sir, I have 2 questions. One is, sir, can I get the total test volume for the quarter?

Shankha Banerjee: Total test volume?

Raman KV: Yes, number of tests conducted, not the sample collected?

Shankha Banerjee: Number of tests.

Ved Goel: We have done 22.2 million samples in this quarter.

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Raman KV: Yes, that is the sample collected, right? I just want to understand the number of tests performed on those samples?

Ved Goel: Raman, we generally give sample numbers. What you are talking about the parameter where, let us suppose, if we do one test, which is TFT, LFT, let us suppose, and within that, there are so many parameters, which we do not report directly. So these are not reported. So we generally give sample numbers.

Raman KV: Okay. Understood, sir. And sir, my second question is with respect to Swasthfit. I just want to understand the margin difference between Swasthfit and the rest of the portfolio. Where I am coming from, I just want to understand if once your contribution of Swasthfit improves further, will it aid margins or will it be a pressure on margins or the margins will remain flat?

Ved Goel: So realization is, of course, higher in Swasthfit because of bundle and packages, but same time, we provide discount because these packages are at discounted rate.

Obviously, we get operating leverage. And overall, net-net, it is not like very different, but yes, realization is higher and operating leverage is always coming, be it consumption, be it other operating overheads. But somebody should not factor in

that higher contribution from Swasthfit will give very high margins. That one should not factor in.

Shankha Banerjee : It is not margin dilutive.

Raman KV: Okay. Understood, sir. Sir, my third question is with respect to the newly launched Sovaaka . Can you just elaborate what are the tests the company will be performing under this?

Shankha Banerjee : Sovaaka is a very new conceptual setup. It is focused on p

reventive wellness testing

only, and if you want more details, you can go on to Sovaaka.com, which is the website dedicated to Sovaaka. You can type S -O-V-A-A-K-A, Sovaaka.com, and you will get details. But it is all preventive health checkups .

These are packages , there is a pre -consultation with the doctor, and depending upon individual conditions, family history, etcetera, packages are suggested and finalized, and then the testing happens and report consultation and diet consultation post that.

So it is a wellness and preventive focused testing only.

Raman KV: Understood, sir. And sir, B2C share during the quarter, is it still 75% or has it increased?

Shankha Banerjee : B2C share is still around 75%.

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Moderator: Thank you. Next question is from Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: A couple of questions from my side. On one side, we have seen a sharp depreciation in Rupee s. and at the same time, we have seen this European Union trade deal which cuts tariffs. Does either of this favourably or unfavourably affect our cost base and can it impact margins?

Ved Goel: I think dollar is moving up, we all know. And most of our reagents and chemicals are imported. Indirectly, we are also impacted. But we are able to absorb that inflationary cost because of our volume. Right now, we do not have any which is linked to - directly linked to dollar because we buy from Indian companies. On the other side, this EU trade deal is very early to comment on. Definitely, there may be some opportunity in future which can come. But right now, it is too ea rly to comment on that.

Bino Pathiparampil: Okay. And in case these reagent prices actually go up, do you think you can pass them on without impacting volume growth?

Ved Goel: We have long -term contracts with most of our large suppliers. Right now, we are not having that. But yes, of course, if dollar trajectory is upwards like this, then definitely we have to review our pricing again.

Bino Pathiparampil: Understood. And one question on the radiology plan. So you mentioned that it is going as per plan. In the normal course if it goes as per your plans, when can we see a significant ramp -up on that strategy? Would it be in a couple of years or would it be in 5 years?

Shankha Banerjee : Radiology contribution to our business is less than 5%, and even a significant ramp - up may or may not immediately impact the overall fortunes of the company. But having said that, we are definitely understanding the business model around high - end radiology in brand strong urban markets, and then we will also maybe try and understand in some other markets and then figure out our long-term strategy. Having said that, in terms of time horizon, like you said, it is unlikely to be a significant contributor to top line growth at least not in the next 2 -3 year scenario.

Moderator: Thank you. Next question is from Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee : Good evening. Thanks for taking my question. Sir, you mentioned about the patient volume growth at 4% -5%, which you think can increase going forward. It may not be a step-up, but it can go up. If you can share your thoughts as to what we should look

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at, what would drive that? And also acquisition, small, big task, is that a possibility you see in the current environment? And can that emerge as a driver for you?

Shankha Banerjee : Right. For the organic patient volume growth there are 1 or 2 obviously quite clear levers that I think we have spoken about earlier, and the most important one is the spread and the access that we are increasing for patients. You see the number of new testing labs and the collection network expansion that we have stepped up over the last 2 years is aimed at gettin g more wider geographic spread, and therefore, ability to access a larger population and give more access

to population to come and

use our brand for their diagnostic needs. I think that is number one.

And then primarily over and above that, even through our existing network, there are patient acquisition activities that we keep doing from time to time. And our Tier 3,

Tier 4 and rural push that we have been undergoing for over, I would say, maybe 4 - 5 year journey that we have had on that is also going to help us get the organic volume.

On the inorganic side, definitely, there is appetite for us to be looking at inorganic opportunities and we are on the lookout for suitable inorganic opportunities which can fit into our portfolio and our strategic growth areas.

Saion Mukherjee : And sir, I mean, are you looking at, how should I say? Like is it like midsized chains, big chains or individual labs? What kind of acquisitions are most likely?

Shankha Banerjee : It is not that we look at all kinds of options because it is geography specific. I think the South market is something where we would definitely like to have something which is reasonably large, at least maybe a top 3, top 5 player in a given city with a slightly larger platform play, which we can then adopt and grow.

Whereas in some of our other Northeast, maybe even some parts of West, we are open to doing standalone lab acquisitions, which gives us maybe access to some micro market or some other benefit which we are maybe not able to get through our normal organic business.

Saion Mukherjee : Understood. Sir, on the pricing front, I understand like it has been like 3 years there has not been any price increase. In fact, you mentioned there has been the GST cut on inputs were passed on. So what is your view on pricing? When do you plan to take some price increase? Can we expect this year or next year? Any thoughts around that?

Shankha Banerjee : Like Ved mentioned, we have just taken a decision to do a price cut to pass on the GST benefit. So that cycle has to run for some time before we consider a price

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increase. So therefore, I definitely do not see a price increase on the horizon, at least not for the next 2 -3 quarters.

Moderator: Thank you. The next question is from Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan : Just one on the realization part. Again, I am just calculating revenue per patient from your revenue numbers and the patient numbers. So that comes up by 7.5%. You mentioned I think the GST cuts have not come in Q3. So I am assuming it is in Q4. What would be some of the enablers of this? Would it be higher Swasthit? Can you just explain that? Or has the environment from a competitive standpoint been a lot more easier, do you think?

Shankha Banerjee : The GST price cut was taken in Q3, but it was maybe towards the end of Q3. So obviously, the full impact is not reflecting in the RPP of Q3. To that extent, you are right. It is a combination of test and geography mix that we are seeing in terms of RPP. As we said, Delhi NCR continues to grow double digits and we all understand that Delhi NCR is a higher RPP market for us compared to the rest of the country, and we also have some test mix benefits that are there. So those are the major contributing factors for the geography and the test mix for the RPP increase that we see.

Shyam Srinivasan : That is helpful, sir. Just the second question is you alluded to higher growth over time. Are we talking about not the 11% to 12%, but are we talking about higher than that? We think, and at some point of time, if it is 3-4-5 quarters later, we are looking at price increases. Could we then go beyond the 12% on a medium-term basis? I am not holding you to a specific year, but just want to understand are we in the trajectory looking up at any point of time?

Shankha Banerjee : Yes. I think the thought process that you are saying is reasonable. We are looking at 11% to 12% is what we have been saying for this

financial year. So going forward, if that becomes our baseline, we obviously would like to be slightly better than that. And maybe 3 -4 quarters down the line, the price increase window may open up again.

Shyam Srinivasan : Yes. And what would be a trigger for a price increase? I am just curious. We have not done it for many, anything that you think will then say this prompts us to take prices? Is there something that we monitor closely? Is it inflation? I am just curious. What would trigger the price increase.

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Shankha Banerjee : I think there are two triggers we look at. One is, obviously, internal cost is one thing which we continue to look at, how much we can absorb or what is it that cannot get absorbed, et cetera. And second would also be the external pricing table, because we do not want to get into a situation where we are not or we seem to be falling off the pricing premium table because there are quality connotations associated with price in our country still.

Moderator: Thank you. Next question is from Surya Narayan Patra from Phillip Capital. Please

go ahead.

Surya Patra : Thanks for the opportunity. My first question is on the Sovaaka initiative . So I just wanted to understand, sir, how is this different from the Swasthfit and whether this will be complementing Swasthfit growth. And another point here is that, can the Sovaaka initiative be scalable and scalable beyond the NCR?

Shankha Banerjee : Sovaaka and Swasthfit are very different in their idea and execution. Swasthfit is a pathology only and widely distributed and accessible testing portfolio, which we said not only do we do in urban centers, it also goes into Tier 2, Tier 3, Tier 4 towns across the country. Sovaaka is a personalized, customized wellness , preventive wellness setup which integrates pathology and imaging, including high -end imaging and is a specific center -driven touch and feel experience for people who are looking at really in-depth understanding of their individual health status. So this is kind of center driven.As of now, it is the first center that we have launched. Again, this will be on a mode where we will see how we are scaling this, the patient footfall, the walk -ins, etcetera, and the buildup of this center. I think only on the basis of that will we decide how do we scale it. But there could be possibly multiple modes or models of scaling. But yes, the scalability is way beyond Delhi NCR once we get the model right.

Surya Patra : Okay. Is it fair to believe Swasthfit is a kind of annual testing mechanism, while Sovaaka would be a once in lifetime or once in 10-year kind of thing?

Shankha Banerjee : Both are annual. It is just that the type of testing is different and the offering is very different. Both are annual in nature from a preventive side.

Ved Goel : Surya, I mean, just on the Swasthfit, it is not only preventive wellness, but in our case, most of the time it is upselling. Maybe patients who are coming for individual tests, they are trying to take this package because it makes sense and avoid multiple visits where they can get a lot more tests in a single visit. It is not always preventive wellness. While Sovaaka is purely preventive, preventive and wellness kind of thing. But Swasthfit is always where we have patients who comes in need also.

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Surya Patra : This is helpful. Just one more. Whether we do get any benefit out of this Ayushman Bharat Digital Mission success? What we have seen?

Shankha Banerjee : As of now, we are enabled to incorporate ABHA IDs into our system. So whosoever is giving us their ABHA IDs, we put it into their records the ABHA IDs. But primarily, ABDM is still focused on the hospital care. Diagnostics as of now we have not yet seen any reasonable thing to believe that this can become a key driver of growth for us right now.

Moderator: Thank you. The next question is from Vamsi Hota from ASK In

vestment Managers.

Please go ahead.

Vamsi Hota : My first question is on the extent of utilization of both the Mumbai and Bangalore reference labs. Given that we continue to open about 600 to 800 collection centers annually with greater focus on the high -growth North markets, except of Delhi, how do you plan to ramp up the utilization of these 2 specific reference labs?

Shankha Banerjee : The reference lab capacity by design is very high because it depends on what type of machine modularity that is currently deployed, and there is obviously ability for us to keep deploying more. A lab is designed with a life which says , we are looking at maybe a 10 -15-20 year outlook on a reference lab kind of a setup. So I do not think that kind of correlates with the structure of network collection that is going in other parts of the country.

The reference lab actually helps in building the tier -wise hub -and-spoke model that we work on. We have got reference labs and hub labs and cluster labs and satellite labs, and then front end, we have the collection network.

It is about putting a certain test menu in the market and then our ability to really build that test menu with the local prescribing community in the catchment area and our ability to deliver that timely reports and the quality report from the reference lab that we have for that geography. So that is an independent task and does not necessarily correlate with saying that more collection network will certainly mean more capacity utilization at a reference lab.

Vamsi Hota : That is helpful, sir. I will just ask one more question. Given that we follow a revenue sharing arrangement with our franchise network and that in markets, except of Delhi, we end up sharing more than 25% of our revenue with the network, do you foresee incremental margin dilution given franchisee -led growth? If not, how much more bandwidth do we have on expanding our EBITDA margins via process efficiencies

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and leverage, given that all of our growth has been volume and price led at least over the last 2 years?

Shankha Banerjee : I think Ved will add to whatever I say. I think one must understand that when a

franchisee is given a margin, there are certain tasks of the whole process which also get outsourced. And there is a certain benefit that we also get because of accumulation of samples, and on top of that, we get an independent entrepreneur who is also kind of really running after generating business for us.

So, there are multiple reasons why the franchisee footprint works for us, and we have been able to really develop a very strong mechanism of franchisee management. If you see, besides Delhi NCR, the model of franchisee revenue share is different in certain other geographies, depending upon how the competition, layout, etc. is. We have been able to manage it quite well till today in terms of our overall portfolio margin and we do not see any reason why that cannot be managed going forward as well. So Ved, do you want to add anything to that?

Ved Goel: No. So I do not think margin, where it is dilutive because, first of all, this model of having a franchisee is because of scalability and not putting a fixed overhead on our head because there is an entrepreneur who always drives, and it is a variable model where you share out of your revenue share instead of incurring on your own infra cost.

In spite, you see our contribution has increased so much. We are not diluting our margins. In fact, we are getting operating leverage because we already have testing outlets. We have already capacity which is available, and we can leverage on that.

So I do not think this will dilute our margins.

Vamsi Hota : Sure, sir. Incrementally, how much more scope or rather another way to ask this is, what is the dream state EBITDA margin that you kind of look at?

Shankha Banerjee : I think we are very near the dream state, if you ask me, be

cause we would rather

see if we feel that we are in a position to generate more margin, I would rather see how can it be deployed back to generate growth, sustainable growth, over the long term. I think the margin profile that we run, 27% -28% EBITDA, we are maybe in the quote what you say, "dream state".

Moderator: Thank you. Next question is from Reet Jain from Niveshaay. Please go ahead.

Reet Jain: Sir, basically my question was the molecular diagnostic players like 3B BlackBio or Mylab, Molbio, they are coming into the industry, like Molbio filed their IPO, DRHP.

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Basically, are they a competition or are they a supplier to us for kits? And how is that molecular diagnostic industry doing? Can you give a color on that?

Shankha Banerjee : Molbio is a vendor, is a supplier. They are not really into front -end retail pathology as far as I am aware. And so that is one question. And molecular testing, obviously, has seen increase over time and PCR testing, et cetera. Yes, I think that part of the portfolio should increase. I think maybe more traction will come on that portfolio.

Also because of the oncology testing, et cetera, happening, I think that portfolio will see, and I think the second thing which Dr. Lal is also mentioning is that we see Genetics, again, another area where quite a bit of traction we foresee will come in the market.

Reet Jain: Basically, can you give the reasons why that side of portfolio is doing well and such extensive tests are taking market share from this simple testing?

Shankha Banerjee : I do not think it replaces simple testing. Please understand what is happening is when you get into Genetic testing or molecular testing, these are more personalized, customized and more deeper testing. But that does not take the need away of routine or regular testing. Actually, on the contrary, as people get diagnosed with more difficult or complex conditions, for therapy and the patient management the routine testing is required and that need may actually go up.

I do not see it to be a replacement. Rather, it is more about building and helping build your clinical superiority as well as the trust with the clinicians of our ability to really solve the complex diagnostic problems. I think that is where these portfolios also play a role in our business.

Dr. Arvind Lal : We are also acutely aware of the increase in testing for all these noncommunicable diseases, NCDs, which are basically basic diseases in our jargon. But at the same time, the numbers are humongous, led by diabetes, high blood pressure and lipid profiles. And of course, cancer is catching up, liver, kidney. All these are a very large set of diseases which we club under NCDs. So there is a remarkable improvement for performing because 65% of the Indians are dying from these diseases every day, noncommunicable diseases, lifestyle diseases. Of course, infectious diseases have not left us. They are about 35%. But right now, NCDs are catching up.

Moderator: Thank you. Next question is from Lokesh Manik from Vallum Capital. Please go ahead.

Lokesh Manik : Sir, just one clarification I needed. The new initiative of Sovaaka, would you say this is more like a DTC foray, direct -to-customer foray, v/s. our established model where

the doctor and the lab setup may not exactly be in the same premises v/s a Sovaaka, which would be in the similar premises with obviously more advanced testing and advanced services product portfolio? Would that be a correct assessment?

Shankha Banerjee : Sovaaka , like I was mentioning earlier, is a preventive testing and a wellness testing - driven platform. So to that extent, it can see more self -generated test, but it does not mean that there is not going to be clinician -referred testing here as well. I think even annual health checks which people do, they may end up showing it to a clinician after that. Therefore, it is not

either/or. But yes, compared to the standard diagnostic lab testing where primarily much more prescription -driven testing is happening, in this situation, we might see slightly more self -driven testing as well.

Lokesh Manik : Right. So this would be company owned, right? This would not be franchisee or channel partner.

Shankha Banerjee : Yes, this is company owned, fully company owned.

Lokesh Manik : And even when you plan to scale, it will be a company -driven model. It will not be outsourced to franchisees.

Shankha Banerjee : The scaling model we will enumerate later, because once we have the experience of running the center, obviously, many more details will be with us for us to then decide. But the flagship store is company owned.

Moderator: Thank you very much. We will take that as the last question. I would now like to hand the conference back to the management team for closing comments.

Ved Goel : Thank you all for joining the call today and for your continued trust and support. We hope we have been able to address your questions satisfactorily. Should you have any further queries, please feel free to reach out to us. Thank you once again, and have a good evening.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: May 2026

Classification: Internal
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National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q4 & FY26 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Earnings Conference Call on Financial Results for Q4 & FY26.

We request you to please take the same on record.

Thanking You,
Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 18
Classification: Internal
Dr. Lal PathLabs Limited
Q4 & FY26 Earnings Conference Call Transcript
April 30, 2026

Call Duration • 1 hour
Management Speakers • Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group CFO and CEO, International Business
Participants who asked questions • Tausif Shaikh – BNP Paribas
• Amey Chalke – JM Financial
• Anshul Agrawal – Emkay Global
• Abdulkader Puranwala – ICICI Securities
• Bino Pathiparampil – Elara Capital
• Rajat Baldewa – Kizuna Wealth
• Hrishikesh Patole – Tokai Investors
• Prakash Kapadia – Kapadia Financial Services
• Aniket Shinde – SMIFS
• Rishi Mody – RDM Advisory LLP
• Rahul Salvi – Franklin Templeton
• Gaurav Tinani – Ambit Capital
• Hafeez Patel – ASK Investment Managers

Classification: Internal Moderator: Ladies and gentlemen, good day, and welcome to the Dr. Lal PathLabs' Q4 & FY26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star, and then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Nishid Solanki from CDR India. Thank you, and over to you, sir.

Nishid Solanki: Thank you. Good evening, everyone, and welcome to Dr. Lal PathLabs' Q4 & FY26 earnings conference call. Today we are joined by senior members of the management team, including Mr. Shankha Banerjee, CEO, and Mr. Ved Prakash Goel, Group CFO, and CEO, International Business.

I would like to share our standard disclaimer. Some of the statements made on today's conference call could be forward -looking in nature, and the actual results could vary from these forward -looking statements. A detailed statement in this regard is available in the results presentation, which has been circulated to you and also available on stock exchange websites.

I would now like to invite Mr. Shankha Banerjee to share his perspectives. Thank you, and over to you, sir.

Shankha Banerjee: Thank you, Nishid, and good evening, everyone. I am pleased to join you today to discuss our performance for the fourth quarter and full fiscal 2026.

We concluded the year on a strong note, maintaining our steady growth trajectory. Our FY26 performance underscores the resilience of our business model and the continued preference of patients towards clinically reliable organized diagnostic providers.

We achieved revenue of Rs. 703 crore in Q4 FY26 with a growth of 16.6% and Rs. 2,763 crore in FY26 with a growth of 12.2%. This performance has been primarily driven by growth in sample volumes.

A significant highlight this quarter was the successful hosting of Medillumina 2026, a first of its kind international medical conference from Dr. Lal PathLabs. This landmark event brought together over 500 clinicians and global thought leaders to deliberate on the 'New Era of Diagnostics', specifically focusing on high complexity domains

Classification: Internal such as Oncogenomics, Transplant Immunology, Infertility, Auto-Immunity, and Rare Genetic Disorders. By creating a platform over 2 days of scientific dialogue and collaboration, we are not only reinforcing our scientific leadership and brand equity among the clinician community but also accelerating the adoption of high -end specialized testing. This initiative underscores our position as a vital clinical partner, deeply integrated into the precision medicine ecosystem.

To further build on our Scientific Leadership, we have started a wide -ranging R&D program. It encompasses tie -ups with leading academic institutes with specific research projects, international companies, and start -up ecosystem collaborations as well as in -house research projects culminating with publications in reputed medical journals.

On the operational side, we successfully executed our expansion plan for the year, adding 14 new labs and more than 1,100 collection centers. These additions, coupled with the integration of cutting -edge AI diagnostic tools and specialized testing platforms ensure that we are well -positioned to meet the rising demand for high-quality accessible healthcare across the country.

Our preventive healthcare brand, Swasthfit, contributed 27% to our total revenue in FY26, proving to be a critical lever for B2C growth. Sustained growth in Swasthfit continues as we deepen market penetration and offer a wider range of packages. An important milestone of our strategic journey this year has been the launch of Sovaaka

in the premium wellness space. Sovaaka is not just an addition to our portfolio, it represents a foray into 'AI-powered Precision Health Screening'.

Unlike traditional diagnostic models, Sovaaka offers a holistic concierge -led experience that bridges the gap between high -end diagnostics and personalized health management.

In parallel, the larger health care ecosystem is also expanding with many hospitals adding new infrastructure and capabilities. Thus, it is not surprising that diagnostics are also growing alongside this expansion. This further opens opportunities for better integration, stronger linkages, and expansion of super specialized testing over time.

We are entering FY27 with strong operational momentum, a strengthened digital infrastructure, and a clear pathway towards sustaining early to mid -teens revenue

growth.

I will now hand over the call to Ved to discuss the financial metrics in more detail.

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Classification: Internal Ved Goel: Thank you, Shankha. Good evening, everyone, and a warm welcome. Thank you for joining us today.

I will take you through the key financial highlights for the quarter and the full year 2026.

- Revenue for Q4 FY26 came in at Rs. 703 crore compared to Rs. 603 crore in the same quarter last year, reflecting a growth of 16.6%.
- Revenue for the full year stands at Rs. 2,763 crore against Rs. 2,461 crore in FY25, a growth of 12.2%, driven by sample volume growth of 12.9% in Q4 and 10.4% in FY26.
- Revenue per patient for Q4 FY26 is Rs. 956, up by 7.8% compared to Rs. 887 in Q4 FY25. This is mainly due to improvement in tests and geographic mix.
- Test per patient for Q4 FY26 stood at 3.21 compared to 3.07 in Q4 last year.
- EBITDA for Q4 FY26 came in at Rs. 187 crore compared to Rs. 169 crore in Q4 FY25, registering a growth of 10.5% with an EBITDA margin of 26.6%.
- The full year EBITDA stood at Rs. 752 crore compared to Rs. 696 crore in FY25, registering a growth of 8.2% with a margin of 27.2%.
- PBT for Q4 FY26 came in at Rs. 160 crore compared to Rs. 154 crore in the same period last year with a margin of 22.8%. Full year PBT stood at Rs. 669 crore against Rs. 625 crore in FY25 with a margin of 24.2%.
- PAT for Q4 FY26 came in at Rs. 132 crore compared to last year Rs. 156 crore in Q4 with a PAT margin of 18.8%.
- Full year PAT stood at Rs. 510 crore against Rs. 492 crore in FY25 with a margin of 18.4%.
- EPS for the full year is Rs. 30.2 compared to Rs. 29.2 last year.
- Please note that the results for this quarter and the full year have some exceptional items :
 1. One-time cost of Rs. 30 crore related to the new Labour Code , which was accounted for in Q3 FY26.
 2. There was an additional benefit of Rs. 41 crore in Q4 FY25 on account of voluntary liquidation of Suburban Diagnostics.
- Excluding these one -time and exceptional items, EBITDA margin for FY26 is 28.3% with a growth of 12.5% and PAT margin is 19.3% with a growth of 17.9%.
- We continue to maintain a strong balance sheet with our net cash and cash equivalents standing at Rs. 1,526 crore, providing ample liquidity for future

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Classification: Internal growth and M&A. Our commitment to operational excellence is reflected in our lean working capital, which is negative by 26 days.

- Further, I am pleased to share that the Board of Directors have approved the final dividend of 40%, that is Rs. 4 per share, taking the total dividend for the year to Rs. 20.5 per share, that is 280% after adjustment of bonus issue of 1:1 in Q3 FY26.

With this, I conclude my opening remarks and now request the moderator to open the forum for Q&A. Thank you.

Moderator: Thank you very much. The first question comes from the line of Tausif Shaikh from BNP Paribas. Please go ahead.

Tausif Shaikh : Good evening and thanks for the opportunity, A couple of questions on the recent asset acquisition of Shahbaz kers in Mumbai.

Just wanted to know, I mean, what is

the business mix over there? What is the share of Radio and Path? Are 100% of revenue considering walk -in patient? And what has been the rationale of Dr. Lal acquiring this asset?

Shankha Banerjee: Right. I will take the last part first, the reason we have acquired this asset is this is quite an old operating lab. It has got a legacy of over 45 years in that geography. And it is in a micro market in Mumbai, where we actually with either Lal PathLabs or Suburban do not really have a significant presence. So, this is going to add to our portfolio in that market, given that we are looking at really building our presence strongly in Mumbai and the West region. So that is the reason why we have acquired this entity. Yes, it is a business which has radiology as well as pathology, but I think the exact mix is not something which we are really kind of disclosing.

Ved Goel: Largely, it is a pathology, but having a basic radiology, which is sonography and X - ray, but not high -end radiology. Largely it is a pathology business.

Tausif Shaikh : And what would be the EBITDA margin profile for this asset?

Ved Goel: We have not disclosed the EBITDA margin as of now. We have disclosed it is about a Rs. 6 crore kind of turnover , top line. It is a small asset, it is not large.

Tausif Shaikh : And does this asset have scope to further grow in that micro market that Dr. Lal can scale up over there?

Shankha Banerjee: Yes. Idea is it obviously gives us access to that micro market. And over a period of time, as we deploy some of our products and marketing techniques, we would expect this geography to grow.

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Classification: Internal Tausif Shaikh : That is helpful Any color on this quarter volume growth? It looks better compared to previous quarters?

Shankha Banerjee: Yes. So, this quarter, volume growth has come at 8.2%. But even in the last quarter, I had mentioned that quarter -to-quarter, even on volume is not really a very, I would say, a robust way to look at it. But if you look at the annual progression of the patient volume growth, we have steadily increased over the last 3 years, and we feel that going forward also, we should be able to inch up further in terms of patient volume growth with our new access points, labs, and collection network that we are increasing, we should see some increase in that number as well.

Tausif Shaikh : That is helpful. I will get back in the queue.

Moderator: Thank you. The next question comes from the line of Amey Chalke from JM Financial.

Amey Chalke: Thank you for taking my question and congrats with a good number. So, one question I have on the margin side. This quarter, we have seen a sharp jump in the other expenses. I understand that second half is generally marketing heavy. But given this jump looks much sharper even on 3Q, how sustainable it is? And if you can also give color on margin for next year in line with that? Thank you.

Ved Goel: Amey, this quarter, we have spent a little bit extra , as we said, we are investing in the business. We have spent extra amount on our infra, uplifting of infra, including Delhi NCR as well. Second is, we are spending more amount on A&P, which is, while we are going deeper or spreading across other geographies . Spending on the A&P is also step up. So, these are a few of the expenses where we have done in this quarter. While for the next year, as we are closing this year 27.2%, even after taking the one -time charge of Rs. 30 crore on account of new Laboratory Code . So, we are hopeful for next year also, we are looking something similar margin like between 27% to 28%.

Amey Chalke: You do not expect margin to expand next year, what you are telling?

Ved Goel: Yes. So that is why I am saying, we are expecting this margin in the range of 27 %-28% and the reason because we are investing in the business, be it infra, be it opening

new labs. If you see in last 2 years, we have added 32 labs. This year also, we have added 14 labs and we will continue to add a few more labs in next year also. Those investments are going in the business, and that is where we are looking to maintain the similar margins.

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Classification: Internal Amey Chalke: And also, I wanted to understand, I believe last few quarters, you have also given an indication on the price hike, which we have not taken over the last 3 years. Any thoughts on that? Also, if it is going to come, should not that also will help you to improve margins in the coming year?

Shankha Banerjee: The price hike, we have said that we have completed 3 years since we took our last price increase. But we also said that we will kind of wait and watch, especially because we have taken a decision to pass on the GST -related benefit. A few quarters, we will wait and watch and see how the market is reacting and what position we are in, how is our business growing? How is the competitive situation looking like?

I think basis that maybe we will decide whether we need to take a price increase or not. But definitely, if it is there, it is a few quarters away. It is not something which is immediately on the cards. And on the margin front, you know the overall margin that we see 27 %-28%, we feel is quite a healthy margin. anything extra that we feel can be generated or if it is available for us, we will invest back into the business for growth.

Amey Chalke: Thank you so much, sir. I will join back.

Moderator: Thank you. Your next question comes from the line of Anshul Agrawal from Emkay Global .

Anshul Agrawal: I just wanted to confirm the FY27 revenue guidance that you mentioned, sir, is it early to mid -teens?

Shankha Banerjee: Yes, early to mid -teens.

Anshul Agrawal: Great. So that suggests that the volume growth that we have delivered in the current quarter seems to be sustainable for the entire year. Is there any particular geography or channel which is sort of disproportionately contributing to this growth, your thoughts and some more colour on this. I am guessing you are growing faster than industry here?

Shankha Banerjee: I think I would not kind of correlate it to a quarter alone because, again, if I am talking about, let us say, if you look at the annual trajectory, and that is a much better way to judge because between quarters, there could be some movements and seasonality and other impacts, which can happen. So best way to look at is the annual number. If you look at the annual number, the patient volume growth is at 5.3%, which is better than 4.2% and we have given, let us say, 12.2% overall annual revenue growth.

Now if we move up, obviously, there is some part of it will be through the patient
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Classification: Internal volume, but we are also seeing samples per patient increasing. The sample volume growth is another factor and because of geography and test mix, we are seeing a revenue per patient also going up. All of these 3 will finally contribute to the overall revenue delivery. So, the 8.2% patient volume growth that we have seen in this quarter is something which I would not hastily built into the plan for next year.

Anshul Agrawal: Got it clear. What I wanted to probably understand just a follow-up on this was whether realization per patients which have improved as well, I understand Ved Goel mentioned that it is because of geographical and test mix. This is despite us increasing contribution from Tier 3 geographies. I am trying to understand again here whether Tier 3 geographies are dilutive in nature or accretive in nature to our baseline realizations? Yes, your thoughts.

Shankha Banerjee: I think this is a discussion we have been pondering in the last quite a few of these calls and you see close to 39% of our revenues is now coming from Tier 3 plus geographies, and we have a realization,

which is in front of you. So obviously, it cannot be dilutive and I think I have tried to explain it in the past as well. The way we run our pricing is actually in clusters. So it is not as if I move from a city like Lucknow to, let us say, a city or a town, which is smaller nearby, the pricing is going to change. The pricing in that cluster is actually the same. It is not as if I am going to a Tier 3 market naturally means that pricing is going to be different. The cluster pricing remains the same and parallelly, even when I am going into the Tier 3, Tier 4 towns, with more access, we will be able to sell our health packages, preventive check-ups and all of those, which even on a revenue side per patient side is slightly higher revenue. So I think there are those factors there. So as of now, it has not been dilutive, and we do not believe it is going to be dilutive going forward as well.

Anshul Agrawal: Great. Clear. Could you help me with the capex guidance for FY27 and the B2C contribution, B2C revenue share in the current year?

Ved Goel: Anshul, for capex, I think we are planning to be in the range of Rs. 100 crore - Rs. 120 crore kind of capex for the next year.

Shankha Banerjee: The B2C contribution this year is about 75%.

Anshul Agrawal: Sorry, just if I can squeeze in one more. Ved -ji, this capex guidance, I would suspect there are incremental capex in addition to sort of the lab infrastructure. Could you call out whether, are there any radiology projects planned, which are built in this capex number?

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Classification: Internal Ved Goel: Yes. We are planning to have 1 or 2 radiology centres. So that includes in there. This Rs. 100 crore - Rs. 120 crore is one is maintenance capex. Obviously, another is we are opening like we opened 14 labs in this year. Next year also, we are looking 12 to 15 labs, another labs. And one you know, another investment we are making in our precision lab. So those are the additional investment in addition to maintenance capex.

Anshul Agrawal: Okay, many thanks sir all the very best.

Moderator: Thank you. Your next question comes from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala: Hi Sir, thank you for the opportunity. My first question is with regards to, you know, the FY27 revenue growth guidance. If you look post-COVID, our run rate has been around 10 %-12% kind of a growth, and you are guiding for early to mid-teens. So just wanted to understand what are the kind of structural tailwinds you are seeing into the business?

And secondly, if I look at your FY26 performance, it has been quite broad-based across regions. If you could also highlight when we talk about Tier 3, Tier 4, which are these geographies exactly contributing to the growth?

Shankha Banerjee: The confidence behind the early to mid-teens is driven by the work which has been

happening in the last 2 years. I think the continuous expansion of lab infrastructure and the collection network. If you see over 2 years, we have added close to about 32 labs and almost close to 2,000 collection centres.

Now we all know that these infrastructure matures with time. Typically, that is what is going to be building up for us, number 1. Number 2, we have quite a bit of a focus back on Delhi NCR, which is our stronghold, and we have been able to sustain the double -digit growth in Delhi NCR. And we believe that even going forward, we will be able to sustain a double -digit growth in Delhi NCR.

Our West region, our Suburban business in the last quarter as we had spoken earlier has started picking up. We are seeing better growth trajectory coming back to Suburban. So those are the, those are really helping us you know, project a number that we should be able to do early to mid -teens. So that is the place. You had a second part to the question. What was that exactly?

Abdulkader Puranwala: Ye

s, sir. Second part to the question was when I look at your FY26 growth of 12% and on the PBT, when I refer to the revenue split you know across the region, it is quite identical to what it was in fiscal 2025. When we talk about much of the growth
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Classification: Internal coming from Tier 2 and Tier 3 cities, how does that pan across the regions in which you operate in?

Shankha Banerjee: Most of our Tier 3 + towns are in our stronger brand markets in North and East. So that is where you know, most of them are. But there are Tier 3 towns that we operate in West and South as well. All of them are showing growth. And like I said, you know some of the metro areas like Suburban business is also showing some upside and Delhi NCR also is doing well for us, it is quite broad -based.

Abdulkader Puranwala: Understood. And just one last one, if I may. Yes. So sir, I mean, if you could also highlight on the Sovaaka centres, how many centres we have? And you know when we talked about next year capex guidance, what are we factoring? And you know what is the revenue run rate across the centres now?

Shankha Banerjee: Sovaaka, we have launched one center, which was launched in January. I think it is a new concept and even I think in the last call, I think we had highlighted this that we would first like to stabilize the centre before we work out the expansion plan. So immediately in the next financial year, we are not really looking at more centres, which are like S ovaaka, but there are other integrated high -end radiology centres that we have opened in Delhi NCR.

We may open in Delhi NCR. We may also try and see if the same model can be operated in maybe a Tier 2 town in North. Those are some of the things we will try. But Sovaaka is one center, and there is no plan to add centres in the next financial year. Next financial year is more about building that centre up and being very sure about the expansion plan after that.

Abdulkader Puranwala: Very clear sir, thank you.

Moderator: Thank you. Your next question comes from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: Hi, good afternoon. Congrats on a good set of numbers. First, question on the Middle East war and the raw material price inflation. Do these things have any impact on our operations in terms of availability or cost of reagents, etcetera?

Ved Goel: Bino, as of now, no, because we are, obviously, we have ample sufficient inventory for the next 3 -4 months, and we have long -term contracts as well. Having said that, I cannot comment, I mean, what happen after 3 -4 months. If this war continues, obviously, there will be some impact may come on our supply chain. I mean, because we import most of our reagents and consumables also, there are linkages with oil
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Classification: Internal and all that stuff. But as of now, we are able to maintain. But yes, in future, I do not have visibility right now.

Bino Pathiparampil: Got it. Now just a couple of bookkeeping questions. One, this entity we have acquired, does that have just one lab or is it a few more labs?

Ved Goel: No. See this Shahbazkers is one lab. It is a single lab.

Bino Pathiparampil: Okay. And the tax rate consolidated tax rate for the year is a bit lower than previous years. So, this 21 %-22% you are looking at or will it swing back to 25 % tax rate?

Ved Goel: No. Tax rate is similar because as I explained in my opening remarks, last year, we got some additional benefit due to Suburban liquidation. And that is why Rs. 41 crore was the exceptional benefit, which was there last year, but tax rates are same.

Bino Pathiparampil: The current year's rate will stay for next year as well because the current year is a little below 25 %, 23% range.

Ved Goel: Yes. It is in the same range, which is around 25%.

Bino Pathiparampil: Thank You.

Moderator: Thank You. Your next question comes from the line of Rajat Baldewa from Kizuna Wealth. Please go ahead.

Rajat Baldewa: Hi sir, thank you for giving me the opportunity, my first question is on the acquisition side which you have acquired Shahbazkers Diagnostic Centre for upto Rs. 20 crore, about 3.3x FY26 revenue of Rs. 6.11 crore.

Given that Mumbai's crowded lab market, like Metropolis, ID Labs, and there are many phenomenon labs, what was the competitive intensity there? And is this a mainly 2 holed acquisition or the first of multiple bolt-ons in Maharashtra?

Shankha Banerjee: I think like I was explaining to one of the questions earlier, so within Mumbai, there is a micro market where we do not have a presence either through the Suburban brand or through Lal Path Labs. This acquisition kind of fills that gap for us and every large market has a lot of opportunity. And not only is the opportunity because of there will be large labs present, but there are a lot of unorganized labs also in those markets, plus the overall demand in these markets are also growing.

The opportunity for growth is available in these markets, and we definitely want to participate and grow our business in Mumbai city as well. And that is really the rationale behind the acquisition.

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Classification: Internal Rajat Baldewa: Great sir, and sir, is there any plan on radiology side given that 3-4 year growth plan particularly on radiology?

Shankha Banerjee: Our plan is a very slow and calibrated as of now on radiology because we are still working on that how we will be able to replicate one center success to more, and we need to work that out on a very organic basis. It will be very slow and calibrated. We have not set any ambitious targets for us on radiology growth in the next 4-5 years, the way you are suggesting.

Rajat Baldewa: Okay, great. Thank You.

Moderator: Thank you. Our next question comes from the line of Hrishikesh Patole from Tokai Investors. Please go ahead.

Hrishikesh Patole: Hi. Good evening. Could you please share how you are prioritizing your investments in new labs v/s the old collection centers? And also, what kind of ROI thresholds and the payback periods that you typically look at when you are trying to expand in these?

Shankha Banerjee: I did not get your question. Investment in lab v/s collection center, what was the question?

Hrishikesh Patole: Okay. Let me step back. Let us talk about capex, right? You talked about how your there is maintenance capex and growth capex. Could you please elaborate on your growth capex, how you are going to spend it?

Shankha Banerjee: I think Ved talked about it. So, there are new satellite labs that we are going to open up, right? Then there is maybe a few high-end radiology setups that we will do. Plus, we have acquired an asset to set up a precision diagnostic lab, which has kind of high-end complex testing and those kind of machines, equipment, etcetera, will be there. All of these are part of our capex plan for next year.

Hrishikesh Patole: Okay. Thank you.

Moderator: Thank you Once again. Our next question comes from the line of Prakash Kapadia from Kapadia Financial Services. Please go ahead.

Prakash Kapadia: Thanks for the opportunity. Congrats to the team, after a long time, we have seen growth being broad-based across most of our geographies on an annual basis. The good sign is Delhi NCR has really done well this year. So that is good. If you could give some insights, is it focusing on existing customers, some quicker turnaround, high-end test? What is leading to Delhi NCR growth? And if I look at the quarter growth has finally come above 15%. Now channel check suggests it is lesser

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Classification: Internal competitive intensity across the board. There are selective price hikes in some of the packages, which is also leading to this growth?

You mentioned in your remarks, Shankha, you

are pretty confident of growth being

mid-teens. We should expect higher growth in Suburban and some of the other geographies, which has just started to continue, which will give us steady state 14%-15% growth in the coming quarters. Is that the aspiration we are working?

Shankha Banerjee: Thank you, Prakash, for the question. I think, firstly, on Delhi NCR, I think it is a lot to do with maybe all the things that you said because we have got a very strong brand equity and presence. We have just tried to activate all our channels, including our own infrastructure, our partners as well as improved our service levels.

I think I had mentioned in one of the previous calls, we have also added a few testing locations in Delhi NCR to improve the turnaround time. And a lot of work is happening

on the specialized portfolio as well. It is an all-around effort, which is carrying on. And then we are seeing results and that is how Delhi NCR growth at double digits is getting sustained.

Going to the other question about guidance for next year. When we say early to mid-teens, I am talking of a range it could be between 13 %-15%, and basically, annually, we have already seen that we have been able to deliver 12 -12.2 and one quarter, which is quite good, the last quarter. But like I said, one quarter is not the way we kind of judge the business. But there are a few things which are working for us. Our lab and network expansion that we have been able to deliver, that is going to cumulate each year as we move forward, that cumulative benefit will flow through. Our Delhi NCR growth is sustained and we also are seeing Suburban business now picking up in terms of growth rates. All of these are going to be contributing and helping us add a few percentage points to our growth rate.

Prakash Kapadia: Great. And Suburban, any sense if you could give, is it going to be package driven? Is it going to be individual price driven? And when we talk of the overall 28% revenue coming from packages, is Suburban also included in this? Or it is just the Swasthfit of Dr. Lal, which comes under this in terms of the contributions?

Shankha Banerjee: No. Even Suburban packages are included in that.

Prakash Kapadia: Okay. Suburban also has some of these packages, and that is a decent portion of Suburban revenues. Is that right understanding?

Shankha Banerjee: Yes, it is a decent portion of Suburban revenues as well.

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Classification: Internal Prakash Kapadia: Okay. Fine. Thank you. All the best. I will rejoin back if I have more questions. Thank you.

Moderator: Thank you. The next question comes from the line of Aniket Shinde from SMIFS .

Please go ahead.

Aniket Shinde : Yes, thank you for the opportunity. I guess, in starting, you mentioned the acquisition cost of Shahbazzers, Can you please repeat that?

Ved Goel: Total deal size is about Rs. 20 crore for this asset.

Aniket Shinde : Okay. And what would be the overall capex for FY27 and FY28?

Ved Goel: So as I said, Rs. 100 crore - Rs. 120 crore for next year.

Aniket Shinde : Okay. Yes, that is it.

Moderator: Thank you. Our next question comes from the line of Rishi Mody from RDM Advisory LLP. Please go ahead.

Rishi Mody: Shankha, just wanted to get your understanding on Suburban. You mentioned that a large part of the growth contributor has been Suburban. I understand one would be that operations normalized after that software update that you all were talking about. There would be a portion of lost revenue, which is still normal. But beyond that also, is there growth which has come from either market share gains or like what has led to that, if there is significant growth from that piece as well?

Shankha Banerjee: Yes. I do not think I mentioned it is a significant growth from Suburban. I think what I was saying is that the growth has been very broad-based and which includes Suburban business had not been real

ly doing very well for 1 or 2 - about 3 -odd

quarters. I think that is something which we are now seeing in the last quarter coming back. And therefore, that momentum we will be able to carry forward into our next year business as well.

Rishi Mody: All right. So could you just help me with the numbers for Suburban revenue like this quarter v/s Q3 and say, last year Q4? So I just get an idea of what runway to expect for FY27 on Suburban?

Ved Goel: Rishi, we are not now reporting separately these numbers. These are all part of our West number, which has been given in the split, geographical split because now Suburban is no more a separate entity. It is merged with main parent company.

Rishi Mody: Okay. All right. I will pick it up from the Western region numbers. All right. Thank you.

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Classification: Internal Moderator: Thank you. Your next question comes from the line of Rahul Salvi from Franklin Templeton. Please go ahead.

Rahul Salvi: Yes. Thanks for the opportunity. I had a question on improvement in volumes, if any, are we seeing because of the GLP -1 launch in the last 40 days. Is there any patient volume accretion happening on that front? And which are the tests basically which these patients are choosing? Any insights on that will be helpful.

Shankha Banerjee: The patient volume growth to a certain extent is also a factor of the improved collection network and the lab network that has been put into place. It is definitely not driven through GLP and like I said that this is just one quarter performance because there are sometimes the base numbers can be slightly different in different quarters.

The best way to look at the patient volume growth is at an annualized level, which is better than last year. And we believe that going forward, we should be able to do slightly better on the patient volume number as well in the next financial year.

Rahul Salvi: But as I understand, you will not attribute the FY27 growth even to a slightest extent to patients who are opting for GLP and the doctors prescribing them those tests, right?

Shankha Banerjee: I would not ascribe any differential impact due to GLP.

Rahul Salvi: Thank you. That is helpful.

Moderator: Your next question comes from the line of Gaurav Tinani from Ambit Capital .

Gaurav Tinani: Hi, good evening , and thank you. Question is on the incorporation of the subsidiary in Dubai, UAE. Can you share what are your plans from a business build -out in these geographies? And what percentage of capital or capex of Rs. 120 crore, if any, is allocated to this geography as well in FY27?

Ved Goel: Yes. Thanks, Gaurav, for asking this. As I mentioned on the last call as well that we are making inroad to our international expansion. It is not something immediate, but over a period of, let us suppose, next 3 -5 years, we are looking to expand a few of the geographies.

Right now, we have on -ground presence in Nepal and Bangladesh. But we are looking some of the new geography on ground operations, including Middle East. This incorporation is in line with that expansion plan.

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Classification: Internal Gaurav Tinani: You have significant cash on balance sheet. Would inorganic opportunities be also explored in the Middle East or Dubai, UAE markets over the next 2 -3 years, you would be open to that?

Shankha Banerjee: Right now, I think the idea is to incorporate a holding company kind of or a company in Dubai, which can also maybe operate as a holding company for the region. Now in terms of our expansion plan, M&A opportunity can also be evaluated. That is always on the cards. But yes, I think both organic and inorganic can be looked at.

Gaurav Tinani: Got it. Some accounting questions. I think you have kind of reallocated some costs from these collection centres or employee to other expenses this quarter. What was the primary reason for that?

Ved Goel: This cost

was in the nature of courier and transportation costs, which was grouped under employee benefit, which has been regrouped as per the nature of the expenses. This is the cost which has been regrouped from employee benefit to other expenses.

Gaurav Tinani: If you look at the reclassified employee expense for Q 4, we are seeing a jump of almost 19%. Is that some part allocated to the division in the labour code and restructuring of the compensation structures?

Ved Goel: Yes. Regrouping is nothing to do with this new labour code. But having said that, as I mentioned, Rs. 30 crore is the additional cost, which is reflected in this year on account of new labour code.

Gaurav Tinani: That was in Q3. If I just look at employee expenses in Q4 this year, which was close to Rs. 129 crore v/s Rs. 108 crore in Q4 last year. So that is almost a growth of 19%?

Ved Goel: This is because, as we mentioned that we have added infra, which is 14 labs and this is more towards the end of Q3 and Q4 mostly. And plus, we have started operations in Sovaaka. So those are the expenses which is also factored in here in Q4.

Gaurav Tinani: Sorry, last question, just the previous colleague or peer also asked this. This quarter, tax expense or tax rate is closer to 17%. Any benefit that we realized this quarter on the effective tax rate?

Ved Goel: There is a reversal of deferred tax in this quarter on account of some income tax assessment has been done, we got the refund and accordingly, we have reversed. But as I mentioned, tax rate is same, which is around 25%, nothing changing.

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Classification: Internal Gaurav Tinani: Thank you. All the best. I will join back the queue.

Moderator: Thank you. Your next question comes from the line of Hafeez Patel from ASK Investment Managers. Please go ahead.

Hafeez Patel: Congratulations on the good set of numbers. My first question is around the ongoing transition of Suburban collection sales from in -house to a franchisee -led model. To what extent has that been completed? And secondly, in terms of the EBITDA margin profile of Suburban, I think the last quarter figure was somewhere around the high teens range. Is that kind of improving towards the upward trajectory and kind of reaching the company level margins? Or is there further scope of improvement there?

Shankha Banerjee: On the collection network, now I think the transitions that were to be made have

mostly been done. Now the Suburban expansion is also happening mostly through a franchised setup. However, there will be certain geographies where there will be company -owned collection network also that we will consider.

But primarily, that whole transition towards having more centers to franchisees is already kind of underway and mostly done, so that is one. I think on the margin, maybe Ved can answer.

Ved Goel: On margins, as I mentioned, we are not tracking separately because this is no more separate entity. But margins, obviously, for different geography, different margin structure, even let us suppose, West as a whole, if we compare Delhi NCR v/s West, obviously, margins are different. But in spite of that, we are looking the margins on an overall basis as a company is in between of whatever 27% -28% margin.

Hafeez Patel: Understood sir. So just a small clarification there. I mean, while you may not disclose the specific number there, but the margins that you kind of have on the West geography, has Suburban reached up to that scale? Or is there further scope for improvement? If you could just guide on that?

Ved Goel: No. As I said, I mean, margins for West, including Suburban, there are still rooms to improve because, obviously, it is not a onetime activity. It is an ongoing where we continuously have some levers where we can optimize our cost. And that is why improvement in margins for a few geographies are possible.

Hafeez Patel: Understood sir. Thank you and

all the best.

Moderator: Thank you. As there are no further questions, I now hand the conference over to the management for closing comments.

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Classification: Internal Ved Goel : Thank you all for your participation today and for your continued trust in our vision.

We trust we have addressed all your questions comprehensively. If you require further clarification or have additional queries, please do not hesitate to reach out to us. We look forward to engaging with you again next quarter. Thank you once again and have a good evening. Thank you.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an

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Call: Jul 2026

Classification: Internal
July 30, 2026

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q1 & FY27 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Earnings Conference Call on Financial Results for Q1 & FY27.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 20

Dr Lal PathLabs Limited
Q1 FY27 Earnings Conference Call Transcript
July 24, 2026

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal - Executive Chairman
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel - Group CFO and CEO, International
Business

Participants who asked
questions • Amey Chalke – JM Financial
• Tausif Shaikh – BNP Paribas
• Yogesh Soni – Haitong Securities
• Anshul Agrawal – Emkay Global
• Prakash Kapadia – Kapadia Financial Services
• Saion Mukherjee – Nomura
• Shyam Srinivasan – Goldman Sachs
• Rahul Jeewani – IIFL Securities
• Sudarshan
• Parth Sodha – Trinetra Asset Management
• Abdulkader Puranwala – ICICI Securities

- Sumit Gupta – Antique
- Akash Shah – Investec
- Surya Narayan – Phillip Capital
- Mohammed Patel – Edelweiss Public Alternatives

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Moderator: Ladies and gentlemen, good day and welcome to Dr Lal PathLabs ' Q1 FY27 Earnings Conference call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the call, please signal an operator by pressing star then zero on your touchtone phone.

I now hand the conference over to Mr. Siddharth Rangnekar from CDR India.

Thank you and over to you, sir.

Siddharth Rangnekar: Thank you. Good evening, everyone, and welcome to Dr . Lal PathLabs ' Q1 FY27 earnings conference call. Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal, Executive Chairman , Mr. Shankha Banerjee, CEO and Mr. Ved Prakash Goel, Group CFO and CEO International Business.

I would like to share our standard disclaimer. Some of the statements made on today's call could be forward -looking in nature and actual results could vary from these forward -looking statements. A detailed description in this regard is available in the results presentation that has been circulated to you and is also available on the stock exchange websites.

I would now like to invite Dr. Arvind Lal to share his perspectives. Thank you, and over to you, sir.

Dr. Arvind Lal : Thank you very much. Good evening, ladies and gentlemen , and welcome to our first earnings call for the financial year 2027. FY 2027 is, for us, a year of execution: scaling what we have already built, deepening our reach into new markets, including rural.

The Indian diagnostics industry remains firmly in a structural growth phase. Demand is no longer anchored only to episodic illness; it is increasingly shaped by the rising burden of lifestyle and non -communicable diseases, growing health awareness, and rising disposable incomes that are making regular, preventive testing a part of everyday life.

We also remain deeply committed to providing access to quality diagnostics in the underserved regions of the country. Towards that cause, we have launched a rural outreach program, which is now active in 7 states , and we have tes

ted more than

1,10,000 patients in these areas in the first quarter of this year.

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We have continued to widen our footprint with purpose. The incorporation of our wholly owned subsidiary in Dubai marks a considered first step in extending our diagnostic expertise outside India, building on our existing presence in Nepal and Bangladesh.

In parallel, the broader healthcare ecosystem is expanding, with hospitals across the country adding new capacity and capability. This tailwind is opening up deeper integration, stronger clinical partnerships, and greater scope for super -specialised testing over time.

Dr. Lal PathLabs is well placed to lead through this shift, and our continued investment in scientific leadership, R&D, digital infrastructure and service standards, is aimed squarely at capturing it responsibly for the long term.

I will now hand over to Shankha Banerjee to take you through our operational and financial performance for the quarter. Over to you, Shankha.

Shankha Banerjee : Thank you, Dr. Lal, and good evening, everyone. I'm glad to be here today to review our operational performance and strategic milestones as we conclude the first quarter of financial year 2027.

We have entered the new fiscal year with strong operational momentum, sustaining the resilient growth trajectory established in FY26.

We achieved a revenue of Rs. 798 crore in Q1 FY27, registering a year-on-year growth of 19.1%. It is important to note that this revenue growth rate is the highest quarterly growth rate achieved in last 4 years. The revenue growth is mainly driven by Patient volumes which stood at 8.2 million with a growth of 8.2% and sample volumes which stood at 25.9 million with a growth of 10.7%.

It is important to note that our performance continues to be driven by underlying organic business strength. Our execution strategy rests on three key pillars:

1. Scientific Leadership: We have launched 116 new tests with 4 first in India

tests in this quarter . Expanded advanced diagnostics through NGS Chimerism, Flow Cytometry MDS Assay, Perforin Assay, AI -enabled Histopathology, and 81 specialized Genomics assays.

2. Enhanced Patient Experience: We have launched a GenAI Patient Bot on WhatsApp - SwasthAI enabling conversational transactions. Delivered with journeys for Test Enquiry, Prescription Upload, Location Enquiry, and Report

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Access. It reduces booking friction, personalises interactions, and positions DLPL at the forefront of GenAI patient engagement.

3. Operational Excellence: The sustained focus on operational excellence has resulted in Industry leading TAT on Whole Exome Sequencing to 15 days and an Express Whole Exome Sequencing in 10 days. Our routine testing portfolio of more than ~400 tests is now delivered in 3 hours for almost 90% of walk -in patients.

We continue to leverage our Digital Infrastructure & AI tools to improve customer experience, optimize operations, and advance clinic al capabilities in areas such as histopathology and cancer diagnostics.

Looking ahead, we are well -positioned to maintain our targeted sustainable revenue growth over the medium term, backed by structural volume tailwinds, expanding diagnostic footprint, and enhanced focus on high -end specialized tests.

We also retain a significant pricing headroom as a strategic lever going forward.

I will now hand over the call to Ved to discuss the financial metrics for the quarter in more detail.

Ved Prakash Goel : Thank you, Shankha. Good evening, everyone, and a warm welcome. Thank you for joining us today.

I will take you through the key financial highlights for the quarter ended 30th June 2026.

- Revenue for Q1 FY27 stood at Rs. 798 crore, compared to Rs. 670 crore in the same quarter last year, reflecting a strong growth of 19.1%.
- Revenue per patient rose to Rs. 968, up 10% from Rs. 880 in Q1 last year, driven by a favourable

change in the test and geography mix and increase in CGHS, ECHS prices .

- Test per patient increased to 3.14, compared to 3.07 in the same period last year, highlighting continued traction in our Swasthfit portfolio.
- EBITDA for the quarter came in at Rs. 247 crore, v/s Rs. 192 crore in Q1 FY26, with a growth of 28.7%, with EBITDA margin of 31.0%.
- Profit before tax rose to Rs. 229 crore from Rs. 181 crore, up 26.3%, with a PBT margin of 28.7%.

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- Profit after tax stood at Rs. 170 crore, compared to Rs. 134 crore in Q1 FY26, delivering 27.2% growth with a PAT margin of 21.4%.
- Earnings per share for Q1 FY27 came in at Rs. 10.1, up 27.8% from Rs. 7.9 in the corresponding quarter last year.

• Our balance sheet continues to be strong and resilient, with net cash and equivalents of Rs. 1,693 crore as of June 30, 2026.

• Now I would like to give you an update on 2 transactions which the board (Subsidiaries) has approved:

o Acquisition of 80% equity stake in Sunshine Healthcare Limited, Ghana engaged in diagnostics business for a consideration not exceeding GHS 45.6 million.

o Acquisition of 30% equity stake in Neuome Technologies Private Limited, engaged in innovative solutions in sample preservation and biobanking, for a consideration not exceeding Rs 3.5 Cr. This is in line with our focus on strengthening healthcare innovation.

• Further, I am happy to share that the Board of Directors have declared an interim dividend of 50% (i.e. Rs 5 per share).

We remain confident in our outlook and our ability to deliver sustainable and profitable growth, while continuing to uphold our commitment to providing reliable and high -quality diagnostics.

With this, I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A.

Moderator: Thank you very much. The first question is from Amey Chalke from JM Financial. Please go ahead.

Amey Chalke : Yes, thank you for taking my question and congrats to the management on the good set of numbers. Our first question on the revenue per sample and the

revenue per patient growth, it is quite high this quarter. You mentioned in your remark that some of it is led by the hike in the CGHS prices. Is it possible to quantify how much portion of it would it be and should we assume that the growth will pick up for the entire year because of this?

Shankha Banerjee: Yes, thanks for the question, Amey. The CGHS, ECHS price increase is now flowing through into the system. Our assessment is that it kind of is to the tune of
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between 2 %-3% at an overall company level. And I think this benefit will continue for at least another 2-3 quarters.

Amey Chalke : Sure. And in terms of volume growth, this is second quarter we have maintained 8% volume growth, once a slightly softer quarter. So , will this volume growth continue, or will it improve in the remaining of the year? What is our sense?

Shankha Banerjee: At the beginning of the year, I think we had said that we are looking at a volume growth, which is patient volume, between 6% -7%. So Q1 has definitely come slightly ahead in terms of the patient volume growth number. I think it is too early to comment because, we have also mentioned in the past that one quarter is not maybe the way to really judge the performance of the business because there could be some shifts between quarters which may happen. I think once we are through Q2 as well, when we have done half the year, we might be in a better position to talk about what the outlook for the year could be more comfortably.

Amey Chalke : Sure. And just last question if I can squeeze in. So , we were talking about general price hike for second half of this year. Because of this now the CGHS contribution is helping us, so should we assume that this price hike will be delayed now?

Shankha Banerjee: Amey, I think price increase is something for us it is a strategic

lever. So, it depends on multiple factors, including how the pricing table of the competition is moving as well as are there any cost pressures etc ., that need to be looked at. So right now, we had anyway indicated that, we would be thinking about something related to price only towards the end of the year. But I think somewhere there is when we will reassess whether a pricing change is required or not. As of now, we were anyway not thinking about a price increase in the first half of the year. So , I think we will look at it once the first half of the year is over.

Moderator: Thank you. The next question is from Tausif Shaikh from BNP Paribas. Please go ahead.

Tausif Shaikh: Good evening and thanks for the opportunity and congrats on a good set of numbers. In your opening remarks, Dr. Lal has said that a rural program has started in 7 states. Can you just throw some light on the program and also the patient volume growth of 8%, is it fair to assume the large part of volume growth would be coming from Tier 2 cities of North India?

Shankha Banerjee: The rural outreach program is a drive that we have taken, it is kind of a n initiative where we are looking at NCD or the non -communicable disease burden is not only there in the urban part of the country, but we assess is maybe there in the rural
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part of the country. So , this drive is more of a health initiative for the rural people , and we have actually started this few quarters back and right now I think we are talking about it because it has reached some level of scale. Now in terms of the overall patient volume growth, the patient volume growth we are seeing is across the geography. It is not restricted to one geography.

Tausif Shaikh: That is helpful. Just last question, do you maintain your guidance of mid -teens revenue growth and EBITDA margin of 27% to 28% for this fiscal after strong start?

Shankha Banerjee: Like I was mentioning to the previous question that the first quarter results have come slightly ahead of our expectations and we will have more positive way of giving a forecast or a change in forecast after the first half of the year is over. But having said that, looking at what how we have moved in first quarter, chances are we might be more towards mid -teens rather than early teens.

Moderator: Thank you. The next question is from Yogesh Soni from Haitong Securities. Please go ahead .

Yogesh Soni: Yes. Thank you, for the opportunity. First thing is I wanted to understand on the Swasthfit. So , what I understand is that Swasthfit has been growing at a healthy rate, around high teens to near 20%. So , do we expect SwasthFit to continue to sustain this high teens kind of a growth going forward ahead as well?

Shankha Banerjee: Even this quarter our Swasthfit growth has been kind of in the 20% range as well. I think we are definitely looking at sustaining this growth at this level. That is the idea. And what we are seeing is that we are seeing traction of Swasthfit even in our Tier 2 and Tier 3 geography. It is getting much more wider acceptance and, therefore we believe that this growth rate we should be able to sustain for some

more time.

Yogesh Soni: And just to add on to what you said, I mean, your rural outreach program, is that also expected to elevate the Swasthfit growth?

Shankha Banerjee: The rural outreach program is not driven by Swasthfit. It is a different program. It is a much more affordable rural kind of a package as compared to SwasthFit. So there the objective is, like I said, it is more about, trying to see how we can serve the rural population, identify what kind of non-communicable diseases are prevalent there and then later on see how that part of the population can also get the benefit of good quality testing to identify their conditions. It is not Swasthfit there.

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Yogesh Soni: Okay. Understood, And second question, wa

nted to understand how are we tracking on the genomics and radiology front? I mean, whether the contribution from these segments have reached near high single digit or are we at a lower revenue contribution?

Shankha Banerjee: We do not really talk about genomics separately, in terms of the contributions, but suffice to say that we are focused on genomics given that it has quite a lot of application in the newer emerging cancer diagnostics and even some of the other diagnostic areas. Therefore, we are kind of focused on that portfolio. But in terms of contribution it is not moving significantly. It is still less than 5% of our portfolio.

Moderator: Thank you. The next question is from Anshul Agarwal from Emkay Global. Please go ahead.

Anshul Agarwal: Hi, thank you for the opportunity. First question is on the increase in realizations. Just wanted to understand this better. What kind of contribution would be coming from this favorable geographic or favorable test mix? I understand CGHS rate hikes has resulted into 2% to 3% realization hike. But the other components, what could be the contribution if you could just throw some light here?

Shankha Banerjee: I think Ved mentioned in the opening remarks, it is a mix of the test and geography. Now, different portfolios are there, the contribution mix from different geographies like for example we say Delhi NCR for us is a higher, realization geography. So, the performance in Delhi NCR contributes to better realization.

Likewise, when you look at the overall geography mix, now each individual item will be very difficult for us to say because our pricing is different in different geographies, geographic clusters. If you take out the CGHS, ECHS pricing benefit of 2% to 3%, the rest is because of the test and geography mix.

Anshul Agarwal: Okay. So, I can sort of deduce that, you know, Delhi or the core NCR region would have grown in line or slightly better than what the overall portfolio has done?

Shankha Banerjee: Our idea has always been to grow Delhi NCR in line double digit and as near to the overall revenue as possible and we are still able to achieve that.

Anshul Agarwal: Great. Second question is on the acquisitions that we have made in the current quarter. First on Neuome Technologies, is this, if I understand this correctly, is this sort of venturing into allied healthcare services or will this investment be used for internal consumption or a backward integration kind of purposes?

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Shankha Banerjee: Neuome Technologies is a startup which is working on quite some interesting innovative possibilities within the diagnostic supply chain. And we see if some of these fructify, they have huge operational benefit that can accrue to the business, and I think we will jointly be working with the teams there to make some of these things successful. It is not about foraying into something new, but lot of it will be direct benefit in our operational business.

Anshul Agarwal: Got it. And one question on the other acquisition. What percentage of the overall revenue is being contributed by international business currently, and any plans or any target for this business to sort of contribute say high single digit revenue to the overall revenues, say three to four years out, five years out?

Shankha Banerjee: International is a slightly longer wavelength project for us. And these are steps that we are taking today does not necessarily mean that the contribution from these markets or international is going to change significantly in the near term. But the idea is yes, over a slightly longer-term horizon, we should be able to increase the contribution from international. As of now, we have not set ourselves any target like that because we still need to learn a lot about running the international operations, fine-tuning it, settling it down.

I think once we get those things done and we understand the

dynamics of running,

more international geographies which are not really in the Indian subcontinent, we

may be thinking about a plan of really putting forward numbers on how we would like the contribution to look like 4-5 years down the line.

Anshul Agarwal: Got it. Just one last question if I can squeeze in. Any guidance on the number of lab additions that we plan to do for the current year? Will it continue just like last year?

Shankha Banerjee: Yes, it is going to be more like last year. We are looking at between 12 -15 labs to be added this year as well.

Anshul Agarwal: Great. And anything on the radiology front? Would we look to add more centers?

Shankha Banerjee: There are a few centers that we are looking at to add in Delhi NCR, and we may also try and pilot one or two centers outside of Delhi NCR into Tier 2 towns to see how that business model shapes up. It could be maybe three to four centers this year.

Moderator: Thank you. The next question is from Prakash Kapadia from Kapadia Financial Services. Please go ahead.

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Prakash Kapadia: Yes. Hello. Thank you so much for the opportunity. I really appreciate the growth trajectory which has come all the efforts which we have been taking have fructified and hope this becomes a structurally high growth company as we have always hoped for. So, congrats to the team finally all the efforts are showing. A couple of questions from my end. In West India, how has been the performance if you could give some insights because we had a low base earlier, there were integration issues. So how is West India shaping up?

Shankha Banerjee: Right. Prakash, I think the good news is post the integration, the LIMS integration that we did, whole digital thing, I think for this quarter also we are seeing our traction improving. It is the Suburban business specifically, which is the main driver of our West portfolio right now, is really turning around and on trajectory, it is very close double digits actually right now in terms of growth.

Prakash Kapadia: And that should continue, right? Worst seems to be behind us. So, on the growth front, should not that be slightly higher in growth than average?

Shankha Banerjee: No, you are right. I mean, that is exactly the plan, that this quarter also we have started, rather last quarter itself we had started now investing into newer stores, newer radiology centres, and collection network through the Suburban brand. And we are quite hopeful that in the coming quarters this trajectory should start moving up further.

Moderator: The next question is from Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Yes, thanks and good evening. You mentioned one pillar of growth, which is these new tests which you are introducing, some of them are first in India or even in Asia.

I just wanted to understand the, you know, kind of market opportunity these new tests provide and are you developing these in-house or you partner through business development efforts with other companies abroad? If you can just talk about the dynamics there and given India's demographics, what is the kind of addressable market size all these new tests can have?

Shankha Banerjee: Saion, the pillar that I talked about was not really new tests. The pillar that I talked about was scientific excellence. Now, one of the factors around scientific excellence and specifically in the clinician community is the ability to solve complicated cases and ability to really be able to do highly complex testing. So, the company which helps solve complicated cases also is a natural choice for all the other testing that might be emanating from that institution or for that clinician specialist.

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The way that we track the results are not for the new tests that are getting launched because they may be far and few in between, but it is about our ability to really convert

more clinicians or get more clinicians to look at Dr Lal PathLabs as their diagnostic provider, and therefore their share of business accrues to the company.

Saion Mukherjee: Understood. And so, these tests are developed by you in-house or they are, you know, kind of in partnership with other companies? How does that dynamic work?

Shankha Banerjee: I think there are both kinds, but right now a lot of this is in partnership, including international partnerships.

Saion Mukherjee: Okay, great. Thank you. And just one question, I think you talk about it for the international foray, currently the contribution is not significant, right? And it may not increase meaningfully at least in the next 3-4 years. Will that be a right assessment?

Shankha Banerjee: Yes, so the contribution is less than 5%, and I would say at least in the next five years, it significantly going beyond that number in terms of contribution.

Moderator: The next question is from Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Good evening. Thank you for taking my question. Just the first one on the increase in the revenue per patient. Just trying to dissect again. I thought in your remarks you said geographic mix, test mix, and CGHS you obviously called out at between 2% and 3%. I just noticed that the Swasthfit contribution Y-o-Y has not changed. So, when you mean test mix, what would that mean?

Shankha Banerjee: We have portfolio of specialized tests. We have other tests also which are high realization. So, the SwasthFit contribution remains the same because it has grown similar, so it has also grown at around 20% and overall revenue also grown around 19%. So, therefore contribution has not changed.

Swasthfit growth is still quite strong. But the contribution has not changed. So that is the reason why you see Swasthfit contribution not changing, but the Swasthfit growth is still quite good. And the other part of the portfolio, high realization portfolios are also growing.

Shyam Srinivasan: Got it. Shankha, historically Swasthfit was growing, and this is one quarter where that is not so the case. So, I was just trying to understand which are these

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additional tests outside of Swasthfit look like? Is it B2B tests you are saying that we have seen higher realizations or mix has moved to some of those?

Shankha Banerjee: I do not think we are commenting on a test level performance here, but what I can say is that we have a specialized test portfolio which has higher realizations and even that portfolio has done well.

Shyam Srinivasan: Got it. Yes. Very helpful. Second question, this is on this reclassification of I think costs between the different line items, right? I think footnote 15. So, some Rs. 8 crore has changed. So just want to understand between fee, collection fees I think, other expenses, and even in employee expenses there has been some change. Is it historical?

Ved Prakash Goel: It is reclassification. There are costs related to logistics & courier which has been reclassified from manpower personal cost to other logistics cost, which is part of other costs. There is no reclassification in fees to collection center and all.

Shyam Srinivasan: Understood. When we now look at employee as a percentage of revenue, the number is 16.6%. Historically this number used to be 18%-19%. So, this is a structural change, right?

Ved Prakash Goel: No. Like -to-like last quarter also we have reclassified. Because we have higher revenue, so this percentage is looking low. But as such there is no change otherwise. It is a like-to-like where from previous quarter as well as this quarter we have reclassified some expenses.

Moderator: The next question is from Rahul Jeewani from IIFL Securities. Please go ahead.

Rahul Jeewani: Yes. Thanks for taking my question and congratulations on a very strong set of numbers. Now, if we

see for the entire diagnostics industry, we have seen a growth acceleration for past two quarters, so including your peers and yourself, there has been some sort of a growth acceleration in fourth quarter and Q1 of this year.

Now, part of the reason which you pointed was CGHS price increases to the tune of let us say 3%, but apart from CGHS increases, has there been any structural change in the industry which is let us say now aiding the growth profile for all the diagnostic companies at least for the past two quarters?

Shankha Banerjee: No, that is an interesting call out and you are right even when we look at our numbers, the growth is structural, it is coming from all the geographies that we have. One thing is obviously like we said about CGHS, I think any other underlying thing, I think we got to wait for a few more quarters to get more clarity in terms of

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the numbers at a test level, client level, etc. before we can decipher, because there is no other differential trigger that we have been able to identify as of now. But the growth is from all around.

Rahul Jeewani: Sure. And let us say our patient volume growth, which two quarters back was hovering around 5.5% to 6% kind of a number and right now we are at 8%. Let us say of this incremental growth on the patient volume side, can you call out the levers which has led to this acceleration? You obviously talked about Suburban adding a larger number of radiology centers. So, is that contributing to your patient volume growth acceleration?

Shankha Banerjee: You see, right now I would not jump to that conclusion. The reason is in Q2 or Q3 of last year when we were doing the calls and the conversation, one of the reasons why the patient growth numbers were low was also to do with the fever season and I think we had spoken about that. So, I think we need the cycle to run through

quarters for us to say that it has really caught on because we have not yet entered fever season this time round. So once Q2, Q3 is done, is when I think there will be more clarity on the patient volume growth number, how is it kind of panning out for the year.

Moderator: Thank you. The next question is from the line of Sudharshan.

Sudharshan: Yes. First of all, congrats on a good set of results. So just coming back to the CGHS price hike, I understand that the top line kind of benefited from this, but in terms of margins, should we also kind of factor in because your cost would broadly be remaining similar on the fixed part? So, was there some benefit on the margin side as well coming in from the higher price realization at CGHS and will that also kind of continue for the next 2-3 quarters?

Shankha Banerjee: Yes. So, there will be a pass-through because of the higher realization there will be a pass-through to the margin level and one should see that for maybe one or two quarters more.

Sudharshan: And then the subsequent question is, will you kind of then upgrade your guidance in terms of 27% to 28% margins or will that be reinvested into the business at the end of the year?

Shankha Banerjee: So the idea always is to reinvest for future growth. That is the dominant idea. And I think the decisions will be clearer as we go through Quarter 2 because that will give us more visibility on where the annual number could potentially land and therefore if there are any additional investments etc. to be made in the business at that point

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of time. As of now, we will be looking at Q2 more closely before any of those decisions can be taken. But primarily we would be leaning towards investing more back into the business for growth.

Sudharshan: Got it. And lastly, in terms of, you know, sustainability of your improvement of test mix or geography mix, I understand we are kind of doing more complex work on scientific excellence etc., but this can sustain over the medium term in terms of

giving better realizations?

Shankha Banerjee: So, there is a test and a geography mix both. Even besides that if routine tests we sell more in markets, where our realizations are higher, even that contributes to the geography benefit. So therefore, the realization per patient we assume that still has some legs because right now Swasthfit contribution has flattened v/s last year because overall growth and Swasthfit growth is similar.

But there are other levers, specialized tests is a lever, high realization geographies are a lever. So, the realization improvement story is likely to stay at least in the next few quarters for sure and then we will see what else or how can it pan out for future that we will be able to look at maybe towards the end of the year.

Moderator: The next question is from Parth Sodha from Trinetra Asset Management. Please go ahead.

Parth Sodha: My question is on the utilizations and the new network addition in Q1?

Shankha Banerjee: What do you mean by utilization?

Parth Sodha: Like how many of our instruments in the labs are utilized right now?

Shankha Banerjee: So, I am not too sure I got your question. Are you talking about capacity utilization?

Parth Sodha: Yes.

Shankha Banerjee: Right. So, I do not think these are numbers that we kind of talk about specifically in pathology because capacity is really not a constraint ever. So, these are not really numbers we talk about or discuss. And on the second question, which is about network expansion, what was the second?

Parth Sodha: Network additions.

Shankha Banerjee: Yes, the network addition numbers is something we share once in a year. But like we answered one of the previous questions that this year also there is a plan to add between 12 to 15 labs.

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Moderator: The next question is from Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdul Puranwala: Yes, thank you for the opportunity. So firstly, just again on understanding your revenue growth and specifically your sample growth. So, just understanding how the season would have, you know, been one of the factors as compared to last year and, you know, how would that be influencing your test mix for the quarter as compared to what you were seeing last year?

Shankha Banerjee: So, quarter 1 definitely has seen a very different kind of a weather pattern than maybe what we are normally used to in quarter 1. We have seen a more heat, maybe some off shower sometime. It has been a very different weather pattern this quarter. However, what we do not know is whether and what is the impact of that

on diagnostic testing. So, like I was mentioning earlier that once we are through quarter 2 that is when we can maybe ascertain that was there any shift between the quarters or not. As of now, I do not think we have a very strong view on that. But one thing is there that the weather pattern we saw this quarter has been very different from last year.

Abdul Puranwala: Understood. And secondly, on the cost part. If I look at your material cost, so despite the realization improvement, you know, we are seeing your gross margin still at close to 81%. How should we read this and anything to read on, you know, the cost escalation which would have happened because of geopolitical issues here?

Ved Prakash Goel: So, I think as of now we are able to maintain our cost, last call I mentioned that we have stepped up our purchases or inventory. So, there is not much impact on that account. But yes, having said that, if this continues then obviously movement in currency and overall inflation may impact our material cost because we use most of the imported material for testing. But as of now there is not much impact on that.

Abdul Puranwala: Okay. And anything to read on the realization improvement v/s not much of an impact on your gross margins?

Shankha Banerjee: So, the realization improvement like we

mentioned that one part of that is what we

are seeing as the pass-through of CGHS and ECHS pricing. But the rest, which is to do with let us say test mix, now higher realization tests also have a higher COGS. Therefore, I would not kind of read too much to say that if realization is improving, therefore naturally the material cost as a percentage will start going down. I think we have one of the better gross margins and I do not think we would be looking at saying that this will significantly improve going forward.

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Ved Prakash Goel: And you see, while you are looking as a percentage, if you see test per patient is going up. Now this time we have a test per patient 3.14 which is last year same quarter it was 3.07. So, test per patient is also improving and maybe compensated through that also some bit.

Abdul Puranwala: Understood. Anything on our margin guidance? Earlier we talked about 27 %-28%. You know, how are we looking at the margins for the full year now after the stellar performance in Q1?

Ved Prakash Goel: So again, as Shankha also mentioned earlier that of course this flow through is because of this price increase will come, but idea is to invest back into the business, and we will be in a better position after maybe Q2 whether it is panning out or we will change our guidance on that.

Moderator: The next question is from Sumit Gupta from Antique. Please go ahead.

Sumit Gupta: Yes. Good evening. Thanks for the opportunity and congrats on a good set of results. First, how is the B2C segment performed this quarter? What was the realization growth?

Shankha Banerjee: B2C contribution for us has been in the range of 75% and even this quarter it is in a similar ratio.

Sumit Gupta: Okay. So, like we can expect this trend to continue, or can we expect this to improve?

Shankha Banerjee: The mix of B2C and B2B at 75 : 25 is a mix which we believe is a good mix. There is benefit of both these channels for us and we would be quite happy to keep growing both these channels to ensure that we are able to retain this mix also going forward.

Sumit Gupta: Sure. And second is on the Suburban. Like you mentioned there was a double-digit growth in Suburban. So, with respect to profitability, has it improved over the last 1-1.5 years?

Shankha Banerjee: So, we are not calculating Suburban profitability separately because the back end has been merged post the liquidation, voluntary liquidation of Suburban, the integration of all our back end structures has happened. So, there is an interoperability between both the brands in that geography. We are not separately calculating profitability for Suburban now.

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Sumit Gupta: Okay. And the lastly on the structural point of view from the industry. So like with respect to hospital-based labs, are you facing any competition or has it abated? Or are you gaining market share from the ally the standalone industry?

Shankha Banerjee: So, there is no industry-level data for us to pick out. Is it a shift of market share from one type of competitor or another type of competitor. I think one thing that we can say for sure is that we are seeing a broad-based improvement in terms of the growth in this quarter.

And competition in the industry has always been intense and is likely to stay

intense. If one part of the sector we think maybe reducing intensity, there will be another part of the sector which will increase intensity. I do not think any of us are banking on competitive intensity reducing in this industry.

Sumit Gupta: Understood . So basically , I was like trying to understand that it is the diagnostic chains and the hospital -based labs which will grow in tandem and take market share from the standalone labs or how should we look forward like over the next 5 years, 10 years, how the industry is shaping up?

Shankha Banerjee: Yes, I think as a long -term trend,

definitely there will be shift from unorganized to organized, but that is a slightly slow shift given that there are still low entry barriers and therefore, at the bottom end there could still be smaller labs entering the fray. And since the overall size of the pie is also increasing, they will have some way to kind of gain profitability even at a lower scale. But yes, structurally it is shifting and will continue to shift from the unorganized to organized.

Moderator: The next question is from Akash Shah from Investec. Please go ahead.

Akash Shah: Good evening, and congratulations on the good set of numbers. Just two questions. When you say that your Swasthfit has improved by 20% and also your overall revenues have improved by 19%, should we read that as the percentage of specialized tests improving and whether it is structural or not? And second, how do you see the overall competitive scenario on the price front? Thank you.

Shankha Banerjee: If we look at Swasthfit, which is 27% of our revenues, and look at the balance 73%, all the 73% is not specialized portfolio. Even that portfolio has routine tests because Swasthfit is a bundle. There are still tests which get sold outside of the bundle. It is not as if all of the other growth is only in the specialized area. Even the routine business in geographies has grown for us.

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In terms of your question on the competitive intensity or pricing, I think we are not seeing structurally any price change maybe in between the last few quarters. So, that remains neither has it become too favorable, if you look at the last few quarters.

Moderator: The next question is from Surya Narayan Patra from Phillip Capital. Please go ahead.

Surya Narayan Patra: Yes, thank you for the opportunity . My first question is on the patient volume growth what we are seeing whether anything to read from the rural -urban mix that you would be having?

Shankha Banerjee: Yes, so the patient volume this quarter there is no rural -urban divide like I mentioned in one of the previous questions. We are seeing a kind of an overall across geography kind of a good growth.

Surya Narayan Patra: Okay. In terms of your network, is it possible to kind of have a sense what is the mix between the rural and urban?

Shankha Banerjee: You mean labs?

Surya Narayan Patra: Yes.

Shankha Banerjee: We have disclosed in our annual results in the last quarter that the Tier 3, Tier 4, I mean, I do not know what you really mean by rural, but we are talking let us say Tier 3 and below. So , Tier 3 and below has is about 39% of our revenue in the last financial year.

Surya Narayan Patra: Okay. My second question is about the CGHS price rise what we are witnessing. So, it is across the test portfolio, or it is for some selective or specific area and hence it would be different for different players in terms of the benefit so far as CGHS price rise is concerned? How should we understand that?

Shankha Banerjee: CGHS has released a price increase, which is a common price list for the country. It is not different by different area, except I think they have a certain price cut by a different type of geography that they have done. But it is applied similarly across the country.

Surya Narayan Patra: Okay. I just wanted to understand your experience about the Sovaaka initiatives, and how is that helping us in any manner?

Surya Narayan Patra: So Sovaaka is a venture which is a high -end premium diagnostics -driven wellness portfolio. So , it is a completely new concept. And I think I had even mentioned it

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earlier that our endeavor right now is to look at the first center that we have launched, stabilize it, fine -tune it, understand the business growth model, how we are going to drive it.

I think once that model becomes fully established is when we will think about a scale -up and it becoming any significant part of the business

business. As of now, it is more

kind of implementation phase. So, we are still on the learning path on that.

Surya Narayan Patra: Okay. Just last one piece from my side, about your inorganic growth plans. In the previous quarter that you had mentioned about inorganic growth possibility in the Dubai region by creating a 100% subsidiary and all that. So that was one. And this quarter we are seeing your entry into the African market. So, are we becoming bit aggressive about creating our international presence generally, or it is for some specific reason or any thought process about the international growth, inorganic growth plan?

Shankha Banerjee: The international business growth for us is a slightly longer-term plan which we are seeing should play out in the 3-5 year horizon. So, these are steps that are being taken to ensure that our understanding in these markets that we are focusing on, which is Africa, some parts of Middle East and CIS and Southeast Asia, that how can we run local businesses, those things are where we are going to focus on in the near term so that maybe we can then build it more in the future.

Surya Narayan Patra: Okay. So, for the time being is it fair to believe that, let us say even if any focus or any interest that would be there, that is in the area of GCC and Africa?

Shankha Banerjee: So right now, the focus areas for us is Africa, Middle East, CIS, and Southeast Asia.

Moderator: Next question is from Mohammed Patel from Edelweiss Public Alternatives. Please go ahead.

Mohammed Patel: We are sitting on approximately Rs. 1,700 crore of cash. So, I wanted to understand how are we going to utilize this cash and what are your overall thoughts on capital allocation?

Shankha Banerjee: So, one of the major utilization of cash as we see going forward would be for M&As. We have said that if we look geographically also within India, there are parts of the country where we have low presence in markets of west & south. So, we are looking at possibilities of getting a play which could be slightly larger asset in a given city.

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So, primarily that is where the cash utilization will happen mostly on M&As. And secondly would be around some of the high-end radiology centers that we are slowly rolling out, whether it is in Delhi NCR and other Tier 2 markets in North, that is also a bit of capex that we put in besides our maintenance annual capex that we do.

Mohammed Patel: What should be the capex number for this year and next year?

Ved Prakash Goel: We touch roughly about Rs. 140 crore to Rs. 150 crore.

Moderator: Thank you very much. We will take that as the last question. I would now like to hand the conference over to the management team for closing comments.

Ved Prakash Goel: Thank you all for joining us this call today and for your continued trust and support.

We hope we have been able to address your questions satisfactorily. Should you have any further queries, please feel free to reach out to us. Thank you once again and have a good evening. Thank you.

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